II

COMPANION TO THE INTEGRATED NOTEBOOK

The Question Bank Dental Sleep Medicine

An IFDSM examination revision companion · Author edition

1,000 single-best-answer questions across ten IFDSM competencies

Integrated patient scenarios and clinical decision-making

Five options, one best answer, with an explanatory rationale

Syllabus-section references to the main revision notebook

A structured question bank supporting Notes In Dental Sleep Medicine: focused recall, clinical application, and examination practice mapped to the integrated revision notebook.

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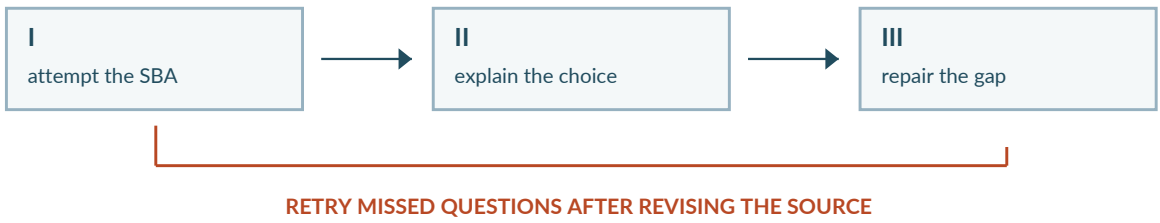
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The question bank

This is Volume II of a two-part study set. Volume I explains the ten IFDSM competencies; this question bank tests application and recall through 1,000 five-option single-best-answer questions.

I	**Attempt the question** Clinical judgement	Read the stem and all five options before choosing ONE best answer. Clinical cases may combine more than one syllabus competency.
II	**Review the justification** Learn from the explanation	Each answer includes a rationale immediately after the options. Cover it while attempting questions; reveal it only after answering.
III	**Repair gaps in knowledge** Return to the notebook	Follow the syllabus section label and notebook anchor to revise the underlying topic, then retry the question without prompts.

Which way to move



Three passes: (1) attempt 20–40 questions without looking at the answers, allowing approximately 75–90 seconds per item; (2) read the rationale and revisit the referenced notebook subsection; (3) repeat clinical cases and articulate why the best option is preferable.

ITEM TYPE	HOW TO INTERPRET IT
Integrated clinical vignette	**Patient-centred, often multi-domain reasoning**
Clinical-context / targeted SBA	**Applied rule or precise syllabus distinction**
All / none of the above	**Still exactly ONE defensible best option**

Study aid only: diagnosis, efficacy verification, and local scope or billing rules require appropriate medical and jurisdictional guidance.

Coverage map · learning outcomes

Question tags preserve the notebook's syllabus sub-numbering. Some headings are treated together in the condensed notes; the map below separately tracks all learning-outcome codes.

Topic 1 — Normal sleep, sleep disorders and pathophysiology of SDB

1.1 Benefits and functions of normal sleep (12 questions)
1.2 Sleep physiology and architecture (13 questions)
1.3 Anatomical/non-anatomical SDB mechanisms (14 questions)
1.4 Prevalence and comorbidities (8 questions)
1.5 Age, sex and other risk factors (9 questions)
1.6 Signs and symptoms (2 questions)
1.7 Impact on the individual (3 questions)
1.8 Impact on society (2 questions)
1.9 Insomnia (15 questions)
1.10 Other respiratory sleep disorders (24 questions)
1.11 Other non-respiratory sleep disorders (7 questions)
1.12 Medications and sleep (21 questions)

Topic 2 — Diagnostic sleep testing and interpretation

2.1 Normal PSG architecture and parameters (22 questions)
2.2 Abnormal respiratory and sleep indices (23 questions)
2.3 PSG interpretation and treatment choices (9 questions)
2.4 Assessment of therapeutic response (9 questions)
2.5 Attended vs unattended studies (18 questions)
2.6 Vigilance testing: MSLT and MWT (11 questions)
2.7 Screening and dental findings (23 questions)

Topic 3 — Oral appliance therapy and other treatments

3.1 History of OAT (19 questions)
3.2 Mechanism of OAT (2 questions)
3.3 Indications and contraindications (9 questions)
3.4 PAP therapies (18 questions)
3.5 Surgery (7 questions)
3.6 Positional and behavioural therapies (10 questions)
3.7 Emerging therapies (12 questions)
3.8 Combination therapies (13 questions)
3.9 Comparative efficacy (9 questions)
3.10 Mean disease alleviation (9 questions)
3.11 Guidelines and evidence-based care (17 questions)

Topic 4 — History, dental examination, imaging and patient selection

4.1 History, objectives and examination (21 questions)
4.2 Medical and lifestyle modifiers (11 questions)
4.3 Other sleep disorders (17 questions)
4.4 Medical comorbidities (5 questions)
4.5 Ethics and patient consent (5 questions)
4.6 Multidisciplinary communication (9 questions)
4.7 Sleep bruxism (17 questions)
4.8 Pain, TMD and sleep (15 questions)

Topic 5 — Appliance design, registration, manufacture and fitting

5.1 Clinical guideline and diagnosis (15 questions)
5.2 Upper-airway mechanisms and predictors (21 questions)
5.3 Indications and contraindications (5 questions)
5.4 Device designs and materials (21 questions)
5.5 Individual device selection (16 questions)
5.6 Construction bite registration (21 questions)
5.7 Impression accuracy (3 questions)
5.8 Laboratory prescription (4 questions)
5.9 Appliance delivery (5 questions)
5.10 Home care and maintenance (19 questions)

Topic 6 — Titration, treatment effectiveness and adherence

6.1 Chief complaint and adjustment (17 questions)
6.2 Symptoms and side-effects monitoring (14 questions)
6.3 Referral for objective assessment (8 questions)
6.4 Clinical and instrumented titration (15 questions)
6.5 Efficacy and effectiveness (13 questions)
6.6 Adherence measurement (13 questions)

Topic 7 — Long-term oral appliance follow-up

7.1 Age, weight, alcohol, drugs and sleep (15 questions)
7.2 Longitudinal symptom documentation (8 questions)
7.3 Treatment modification with progression (8 questions)
7.4 Annual device and clinical review (29 questions)

Topic 8 — Paediatric sleep-disordered breathing

8.1 Paediatric prevalence (10 questions)
8.2 Aetiology and pathophysiology (3 questions)
8.3 Signs and symptoms (4 questions)
8.4 Insufficient sleep and development (5 questions)
8.5 Adolescent sleep and performance (14 questions)
8.6 Paediatric screening (13 questions)
8.7 Treatment approaches (15 questions)
8.8 Differences from adult SDB (11 questions)
8.9 Underlying syndromes and conditions (12 questions)
8.10 Referral and diagnostic pathways (13 questions)

Topic 9 — Best-practice, documentation and ethical responsibilities

9.1 Physician diagnosis and portable testing (16 questions)
9.2 Clinical records and communication (15 questions)
9.3 Billing and payer codes (13 questions)
9.4 Ethics and professional competence (16 questions)

Topic 10 — Informed consent and management of adverse effects

10.1 Valid informed consent (8 questions)
10.2 Short/long-term side effects (19 questions)
10.3 Craniofacial and TMJ effects (7 questions)
10.4 Morning guide and exercises (19 questions)
10.5 Managing side effects (25 questions)

10.6 Suspending or abandoning treatment (22 questions)

TOPIC 01
Normal sleep, sleep disorders and pathophysiology of SDB

130 questions | Full notebook pages 9–35 | Each question has a justification and linked learning code.

Q0001 §1.6 · Integrated clinical vignette

An adult describes loud snoring, witnessed pauses, gasping awakenings and morning headache. Which interpretation is most appropriate?

A. A definitive indication for immediate surgery

B. Only an insomnia presentation

C. A pattern of nocturnal and daytime features warranting OSA assessment

D. A confirmed OSA diagnosis from history alone

E. Sufficient proof that no sleep study is needed

ANSWER: C · A pattern of nocturnal and daytime features warranting OSA assessment

Why this is best: The notebook lists these as common OSA features. They increase suspicion, but physician-led testing and clinical review are still required.

Notebook anchor: *Integrated notebook, Topic 1, §1.6*

Q0002 §1.1 · All-of-the-above integrated judgement

A person with suspected OSA, severe driving sleepiness and evening alcohol intake asks about evaluation. Which option gives the MOST COMPLETE safe plan?

A. Assess the driving-safety risk

B. Refer to a physician for diagnosis

C. Review alcohol and sedative factors

D. Evaluate daytime and night-time symptoms

E. All of the above

ANSWER: E · All of the above

Why this is best: The appropriate assessment must address immediate safety, sleep-medicine diagnosis, relevant risk modifiers and symptoms; each component is needed.

Notebook anchor: *Integrated notebook, Topic 1, §1.1*

Q0003 §1.1 · Clinical-context application

Clinical context. During a sleep-medicine consultation, an adult with unrefreshing sleep asks about the biological value of sleep. During this case discussion, how should the candidate interpret **Physical restoration and repair**?

A. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

B. REM-dependent processing of affect

C. protein synthesis, cell and tissue repair, anabolic dominance

D. declarative in NREM (sleep spindles), procedural and emotional in REM

E. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

ANSWER: C · protein synthesis, cell and tissue repair, anabolic dominance

Why this is best: In this context, the relevant notebook finding is **Physical restoration and repair**: protein synthesis, cell and tissue repair, anabolic dominance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0004 §1.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on benefits and functions of normal sleep, which description is specifically associated with **Physical restoration and repair**?

A. declarative in NREM (sleep spindles), procedural and emotional in REM

B. REM-dependent processing of affect

C. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

D. protein synthesis, cell and tissue repair, anabolic dominance

E. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

ANSWER: D · protein synthesis, cell and tissue repair, anabolic dominance

Why this is best: Physical restoration and repair: protein synthesis, cell and tissue repair, anabolic dominance The distractors correspond to other named concepts in this learning area (Memory consolidation, Emotional regulation, Biochemical refreshment, Hormone regulation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0005 §1.1 · None-of-the-above discrimination

Which single item alone is sufficient to establish an OSA diagnosis?

A. A history of snoring

B. A positive STOP-BANG

C. An elevated ESS

D. The presence of sleep bruxism

E. None of the above

ANSWER: E · None of the above

Why this is best: None of these screening or associated findings alone establishes OSA; diagnosis requires appropriate physician-led assessment and objective evidence.

Notebook anchor: *Integrated notebook, Topic 1, §1.1*

Q0006 §1.6 · Integrated clinical vignette

A patient denies sleepiness yet a partner witnesses recurrent apnoeas. What is the best conclusion?

A. Witnessed pauses prove the AHI is below five

B. A normal ESS excludes moderate OSA

C. OSA requires daytime sleepiness in every case

D. The absence of sleepiness does not exclude clinically important OSA

E. The dentist may independently diagnose and treat without referral

ANSWER: D · The absence of sleepiness does not exclude clinically important OSA

Why this is best: OSA may be minimally symptomatic or asymptomatic; screening and physician assessment should incorporate witnessed nocturnal events.

Notebook anchor: *Integrated notebook, Topic 1, §1.6*

Q0007 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **REM atonia applies to the pharyngeal dilators too**?

A. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

B. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

C. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

D. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

E. GABA and galanin from the ventrolateral preoptic area; overall reduction in neuronal firing

ANSWER: A · This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

Why this is best: In this context, the relevant notebook finding is **REM atonia applies to the pharyngeal dilators too**: This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 20, §1.2*

Q0008 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **REM atonia applies to the pharyngeal dilators too**?

A. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

B. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

C. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

D. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

E. GABA and galanin from the ventrolateral preoptic area; overall reduction in neuronal firing

ANSWER: A · This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

Why this is best: REM atonia applies to the pharyngeal dilators too: This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7) The distractors correspond to other named concepts in this learning area (Process C (circadian), NREM-promoting, Wake-promoting, Process S), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 20, §1.2*

Q0009 §1.8 · Integrated clinical vignette

A bus driver with untreated sleepiness asks why the condition matters outside personal health. Which societal consequence is most immediate?

A. Greater cognitive speed during night shift

B. Increased risk of transport and workplace accidents

C. Reduced healthcare use in every patient

D. Improved alertness at work

E. Elimination of public safety risk by snoring control alone

ANSWER: B · Increased risk of transport and workplace accidents

Why this is best: Excessive sleepiness impairs vigilance and decision-making, creating community and occupational harm.

Notebook anchor: *Integrated notebook, Topic 1, §1.8*

Q0010 §1.3 · Clinical-context application

Clinical context. An adult with confirmed OSA has incomplete benefit from mandibular advancement despite good use. During this case discussion, how should the candidate interpret **Awake upper airway collapsibility index**?

A. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

B. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

C. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

D. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

E. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

ANSWER: B · choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

Why this is best: In this context, the relevant notebook finding is **Awake upper airway collapsibility index**: choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal. Higher = more collapsible, correlating with higher sleeping Pcrit. Takes ~10 min; easier and faster The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0011 §1.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on anatomical/non-anatomical sdb mechanisms, which description is specifically associated with **Awake upper airway collapsibility index**?

A. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

B. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

C. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

D. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

E. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

ANSWER: B · choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

Why this is best: Awake upper airway collapsibility index: choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal. Higher = more collapsible, correlating with higher sleeping Pcrit. Takes ~10 min; easier and faster The distractors correspond to other named concepts in this learning area (Obesity, CPAP dial-down, Soft tissue, Craniofacial), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0012 §1.8 · Integrated clinical vignette

An employer reports absenteeism and reduced work performance in staff with untreated sleep disorders. Which domain is being illustrated?

A. Diagnostic specificity of STOP-BANG

B. Evidence that OAT never helps quality of life

C. A mandatory indication for UPPP

D. An isolated dental occlusion problem

E. The economic and productivity impact of disrupted sleep

ANSWER: E · The economic and productivity impact of disrupted sleep

Why this is best: The notes include lost productivity, sick leave, disability and greater healthcare use among societal consequences.

Notebook anchor: *Integrated notebook, Topic 1, §1.8*

Q0013 §1.4 · Clinical-context application

Clinical context. A dental clinic plans risk-based sleep apnoea screening for its adult patients. During this case discussion, how should the candidate interpret **Gender**?

A. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

B. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

C. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

D. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

E. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

ANSWER: D · ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

Why this is best: In this context, the relevant notebook finding is **Gender**: ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution. Women present atypically (insomnia, fatigue, mood) and are under-diagnosed The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0014 §1.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on prevalence and comorbidities, which description is specifically associated with **Gender**?

A. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

B. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

C. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

D. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

E. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

ANSWER: E · ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

Why this is best: Gender: ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution. Women present atypically (insomnia, fatigue, mood) and are under-diagnosed The distractors correspond to other named concepts in this learning area (Predictive comorbidities, Others, Prevalence, CPAP dial-down), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0015 §1.2 · Integrated clinical vignette

An adult's overnight study shows REM periods lengthening towards morning. What is the best physiological explanation?

A. REM disappears in the latter half

B. N1 replaces REM during the last cycle

C. Normal REM episodes become longer in later sleep cycles

D. Sleep becomes exclusively N2 after midnight

E. N3 dominates the final third

ANSWER: C · Normal REM episodes become longer in later sleep cycles

Why this is best: Normal sleep is cyclical, with slow-wave N3 concentrated earlier and progressively longer REM periods later; REM-related OSA may therefore be prominent before waking.

Notebook anchor: *Integrated notebook, Topic 1, §1.2*

Q0016 §1.5 · Clinical-context application

Clinical context. A new referral has several risk modifiers and a non-classic presentation of sleep-related breathing problems. During this case discussion, how should the candidate interpret **Women more often present atypically**?

A. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

B. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

C. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

D. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

E. roughly a 2:1 to 3:1 ratio at any given threshold

ANSWER: B · insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

Why this is best: In this context, the relevant notebook finding is **Women more often present atypically**: insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0017 §1.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on age, sex and other risk factors, which description is specifically associated with **Women more often present atypically**?

A. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

B. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

C. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

D. roughly a 2:1 to 3:1 ratio at any given threshold

E. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

ANSWER: B · insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

Why this is best: Women more often present atypically: insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors The distractors correspond to other named concepts in this learning area (In children the dominant risk factor shifts with age, Smoking, SDB is substantially more prevalent in men, Gender), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0018 §1.3 · Integrated clinical vignette

A patient has severe anatomic collapsibility despite adequate tongue-muscle recruitment. Which PALM component primarily describes the problem?

A. A: low arousal threshold

B. M: poor muscle responsiveness

C. C: circadian phase delay

D. P: passive anatomy / critical closing pressure

E. L: high loop gain

ANSWER: D · P: passive anatomy / critical closing pressure

Why this is best: Pcrit describes the pressure at which the upper airway collapses. A more positive Pcrit indicates a more collapsible airway; A, L and M describe non-anatomical traits.

Notebook anchor: *Integrated notebook, Topic 1, §1.3*

Q0019 §1.7 · Clinical-context application

Clinical context. A symptomatic patient asks how chronic untreated sleep problems affect health and functioning. During this case discussion, how should the candidate interpret **Public safety**?

A. impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents

B. 33–50% of the adult population; about one third report dissatisfaction with sleep

C. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

D. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

E. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

ANSWER: A · impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents

Why this is best: In this context, the relevant notebook finding is **Public safety**: impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents. Suspected OSA in a commercial driver or safety-critical worker, or sleepiness while driving, needs urgent assessment and explicit driving advice. Sleep, pain and TMD — the dentally relevant impact The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 12, §1.7*

Q0020 §1.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on impact on the individual, which description is specifically associated with **Public safety**?

A. 33–50% of the adult population; about one third report dissatisfaction with sleep

B. impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents

C. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

D. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

E. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

ANSWER: B · impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents

Why this is best: Public safety: impaired reflexes and judgement → higher risk of serious and fatal crashes; workplace accidents. Suspected OSA in a commercial driver or safety-critical worker, or sleepiness while driving, needs urgent assessment and explicit driving advice. Sleep, pain and TMD — the dentally relevant impact The distractors correspond to other named concepts in this learning area (Risk factors, Treatment, Insomnia symptoms, Smoking), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 12, §1.7*

Q0021 §1.3 · Integrated clinical vignette

A patient awakens repeatedly before upper-airway dilator muscles can recruit adequately. Which endotype is most implicated?

A. Reduced loop gain alone

B. High threshold for arousal

C. Severe nasal septal deviation alone

D. Low respiratory arousal threshold

E. Increased REM atonia during wake

ANSWER: D · Low respiratory arousal threshold

Why this is best: A low arousal threshold precipitates waking before recruitment of compensatory dilator activity, perpetuating fragmentation and ventilatory instability.

Notebook anchor: *Integrated notebook, Topic 1, §1.3*

Q0022 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Short-term insomnia disorder**?

A. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

B. sleep-onset and/or maintenance complaints with daytime impairment, exceeding the minimal frequency and duration thresholds associated with clinically significant morbidity: ≥3 nights/week for ≥3 months

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

E. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

ANSWER: E · clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

Why this is best: In this context, the relevant notebook finding is **Short-term insomnia disorder**: clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 12, §1.9*

Q0023 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Short-term insomnia disorder**?

A. sleep-onset and/or maintenance complaints with daytime impairment, exceeding the minimal frequency and duration thresholds associated with clinically significant morbidity: ≥3 nights/week for ≥3 months

B. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

ANSWER: B · clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

Why this is best: Short-term insomnia disorder: clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention The distractors correspond to other named concepts in this learning area (Chronic insomnia disorder, Chronic insomnia disorder, Prevalence, Effect on outcomes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 12, §1.9*

Q0024 §1.3 · Integrated clinical vignette

An obese adult has recurrent obstructive events with ventilatory overshoot and undershoot despite moderate anatomy. Which non-anatomical mechanism is implicated?

A. Reduced closing pressure

B. Excessive jaw protrusion

C. High loop gain

D. Normal chemoreflex gain

E. Isolated macroglossia

ANSWER: C · High loop gain

Why this is best: High loop gain reflects unstable ventilatory control with overcorrection, hypocapnia and subsequent undershoot. It can contribute to incomplete oral-appliance response.

Notebook anchor: *Integrated notebook, Topic 1, §1.3*

Q0025 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Complementary**?

A. nasal valve dilators, lubricating sprays, herbal remedies

B. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

C. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

D. sustained desaturation during sleep without a comparable rise in CO2

E. sustained desaturation without comparable CO2 rise

ANSWER: A · nasal valve dilators, lubricating sprays, herbal remedies

Why this is best: In this context, the relevant notebook finding is **Complementary**: nasal valve dilators, lubricating sprays, herbal remedies. Dilator therapy is effective and safe in mild–moderate OSA but without long-term data; theoretical lipoid pneumonia concern with oil-based products. Not advised, given uncertain long- term outcome and cost The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0026 §1.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other non-respiratory sleep disorders, which description is specifically associated with **Circadian rhythm sleep–wake disorders**?

A. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

B. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

C. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

D. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

E. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

ANSWER: E · Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

Why this is best: Circadian rhythm sleep–wake disorders: Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag The distractors correspond to other named concepts in this learning area (Central disorders of hypersomnolence, Sleep-related movement disorders, OSA and TMD, Catathrenia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0027 §1.9 · Integrated clinical vignette

An adult complains of difficulty sleeping four nights per week for four months with impaired daytime function. How should the duration criterion be classified?

A. Insomnia cannot coexist with OSA

B. The pattern meets the chronic-insomnia duration and frequency criteria

C. A PSG AHI of 15 is necessary for diagnosis

D. It is acute because it lasts less than a year

E. Chronic insomnia requires nightly symptoms for six months

ANSWER: B · The pattern meets the chronic-insomnia duration and frequency criteria

Why this is best: The notebook specifies chronic insomnia at least three nights weekly for at least three months; daytime consequences also require clinical consideration.

Notebook anchor: *Integrated notebook, Topic 1, §1.9*

Q0028 §1.11 · Clinical-context application

Clinical context. A patient with daytime impairment may have another coexisting non-respiratory sleep disorder. During this case discussion, how should the candidate interpret **Circadian rhythm sleep–wake disorders**?

A. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

B. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

C. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

D. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

E. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

ANSWER: A · Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

Why this is best: In this context, the relevant notebook finding is **Circadian rhythm sleep–wake disorders**: Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0029 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Alcohol**?

A. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

B. reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring

C. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

D. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

E. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

ANSWER: B · reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring

Why this is best: Alcohol: reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring. Advise avoidance in the hours before bed The distractors correspond to other named concepts in this learning area (OSA prevalence, Non-anatomical traits, Caffeine, OSA and TMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 32, §1.12*

Q0030 §1.5 · Integrated clinical vignette

A post-menopausal woman reports fatigue and insomnia rather than loud snoring. Which approach best avoids under-recognition of OSA?

A. Exclude OSA because snoring is absent

B. Use male-only screening thresholds

C. Treat with a mandibular device before medical review

D. Diagnose depression without assessment

E. Screen for OSA despite an atypical symptom presentation

ANSWER: E · Screen for OSA despite an atypical symptom presentation

Why this is best: Women may present with fatigue, insomnia and mood complaints. Apparent absence of classic symptoms does not safely rule out OSA.

Notebook anchor: *Integrated notebook, Topic 1, §1.5*

Q0031 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Alcohol**?

A. reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring

B. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

C. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

D. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

E. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

ANSWER: A · reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring

Why this is best: In this context, the relevant notebook finding is **Alcohol**: reduces sleep latency but suppresses REM in the first half of the night with rebound in the second; fragments sleep; reduces pharyngeal dilator muscle tone and blunts arousal responses, worsening OSA and snoring. Advise avoidance in the hours before bed The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 32, §1.12*

Q0032 §1.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on benefits and functions of normal sleep, which description is specifically associated with **Memory consolidation**?

A. declarative in NREM (sleep spindles), procedural and emotional in REM

B. protein synthesis, cell and tissue repair, anabolic dominance

C. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

D. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

E. REM-dependent processing of affect

ANSWER: A · declarative in NREM (sleep spindles), procedural and emotional in REM

Why this is best: Memory consolidation: declarative in NREM (sleep spindles), procedural and emotional in REM The distractors correspond to other named concepts in this learning area (Physical restoration and repair, Emotional regulation, Biochemical refreshment, Hormone regulation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0033 §1.11 · Integrated clinical vignette

A sleepy patient suddenly loses muscle tone when laughing and enters REM unusually quickly. Which disorder must be considered by a sleep physician?

A. Primary snoring

B. Positional OSA

C. Narcolepsy

D. Idiopathic tooth grinding

E. Sleep-related hypoventilation alone

ANSWER: C · Narcolepsy

Why this is best: Cataplexy and abnormal REM onset are characteristic clues to narcolepsy, a central hypersomnolence disorder requiring medical assessment.

Notebook anchor: *Integrated notebook, Topic 1, §1.11*

Q0034 §1.1 · Clinical-context application

Clinical context. During a sleep-medicine consultation, an adult with unrefreshing sleep asks about the biological value of sleep. During this case discussion, how should the candidate interpret **Memory consolidation**?

A. REM-dependent processing of affect

B. declarative in NREM (sleep spindles), procedural and emotional in REM

C. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

D. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

E. protein synthesis, cell and tissue repair, anabolic dominance

ANSWER: B · declarative in NREM (sleep spindles), procedural and emotional in REM

Why this is best: In this context, the relevant notebook finding is **Memory consolidation**: declarative in NREM (sleep spindles), procedural and emotional in REM The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0035 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **Process S**?

A. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

B. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

C. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

D. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

E. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

ANSWER: C · homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

Why this is best: Process S: homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep The distractors correspond to other named concepts in this learning area (Wake-promoting, N3 (old stages 3+4), REM atonia applies to the pharyngeal dilators too, Process C (circadian)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 18, §1.2*

Q0036 §1.12 · Integrated clinical vignette

A patient with suspected OSA takes an opioid at bedtime and develops more complex events. What is the key concern?

A. Opioids are first-line OSA treatment

B. Opioids can depress ventilatory drive and exacerbate sleep-disordered breathing

C. Opioids reliably restore REM sleep

D. Opioids abolish central apnoea

E. Opioids increase pharyngeal muscle tone

ANSWER: B · Opioids can depress ventilatory drive and exacerbate sleep-disordered breathing

Why this is best: Opioids suppress respiration and can worsen sleep-related breathing disturbances, including central events; medication review is necessary.

Notebook anchor: *Integrated notebook, Topic 1, §1.12*

Q0037 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **Process S**?

A. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

B. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

C. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

D. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

E. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

ANSWER: E · homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

Why this is best: In this context, the relevant notebook finding is **Process S**: homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 18, §1.2*

Q0038 §1.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on anatomical/non-anatomical sdb mechanisms, which description is specifically associated with **Craniofacial**?

A. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

B. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

C. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

D. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

E. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

ANSWER: C · retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

Why this is best: Craniofacial: retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular length The distractors correspond to other named concepts in this learning area (Soft tissue, CPAP dial-down, Awake upper airway collapsibility index, Obesity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 10, §1.3*

Q0039 §1.7 · Integrated clinical vignette

A professional driver with suspected OSA reports dozing at traffic lights. What is the most important immediate concern?

A. Rely on the absence of witnessed apnoeas

B. Delay all action until appliance fabrication

C. Urgent assessment of driving safety and medical referral

D. Advise further caffeine without assessment

E. Treat snoring only

ANSWER: C · Urgent assessment of driving safety and medical referral

Why this is best: Untreated sleepiness creates an immediate public-safety risk. The notebook emphasises explicit driving advice and prompt specialist evaluation for safety-critical workers.

Notebook anchor: *Integrated notebook, Topic 1, §1.7*

Q0040 §1.3 · Clinical-context application

Clinical context. An adult with confirmed OSA has incomplete benefit from mandibular advancement despite good use. During this case discussion, how should the candidate interpret **Craniofacial**?

A. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

B. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

C. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

D. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

E. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

ANSWER: E · retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

Why this is best: In this context, the relevant notebook finding is **Craniofacial**: retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular length The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 10, §1.3*

Q0041 §1.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on prevalence and comorbidities, which description is specifically associated with **Predictive comorbidities**?

A. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

B. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

C. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

D. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

E. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

ANSWER: E · obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

Why this is best: Predictive comorbidities: obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease The distractors correspond to other named concepts in this learning area (Others, Gender, Prevalence, Craniofacial), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0042 §1.3 · Integrated clinical vignette

A patient with severe pharyngeal collapsibility and high loop gain does not normalise on maximal comfortable mandibular advancement. Why might further protrusion fail?

A. A device directly raises arousal threshold

B. Protrusion always corrects loop gain

C. The non-anatomical ventilatory instability remains uncorrected

D. A negative Pcrit necessarily indicates severe anatomy

E. Mandibular advancement treats all OSA phenotypes equally

ANSWER: C · The non-anatomical ventilatory instability remains uncorrected

Why this is best: Oral appliances primarily improve airway anatomy; they do not systematically correct unstable ventilatory control, explaining selected non-responders and combined treatment approaches.

Notebook anchor: *Integrated notebook, Topic 1, §1.3*

Q0043 §1.4 · Clinical-context application

Clinical context. A dental clinic plans risk-based sleep apnoea screening for its adult patients. During this case discussion, how should the candidate interpret **Predictive comorbidities**?

A. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

B. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

C. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

D. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

E. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

ANSWER: B · obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

Why this is best: In this context, the relevant notebook finding is **Predictive comorbidities**: obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0044 §1.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on age, sex and other risk factors, which description is specifically associated with **SDB is substantially more prevalent in men**?

A. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

B. roughly a 2:1 to 3:1 ratio at any given threshold

C. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

D. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

E. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

ANSWER: B · roughly a 2:1 to 3:1 ratio at any given threshold

Why this is best: SDB is substantially more prevalent in men: roughly a 2:1 to 3:1 ratio at any given threshold The distractors correspond to other named concepts in this learning area (Smoking, Women more often present atypically, In children the dominant risk factor shifts with age, Prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0045 §1.5 · Clinical-context application

Clinical context. A new referral has several risk modifiers and a non-classic presentation of sleep-related breathing problems. During this case discussion, how should the candidate interpret **SDB is substantially more prevalent in men**?

A. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

B. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

C. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

D. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

E. roughly a 2:1 to 3:1 ratio at any given threshold

ANSWER: E · roughly a 2:1 to 3:1 ratio at any given threshold

Why this is best: In this context, the relevant notebook finding is **SDB is substantially more prevalent in men**: roughly a 2:1 to 3:1 ratio at any given threshold The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0046 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Chronic insomnia disorder**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

C. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

D. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

E. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

ANSWER: B · sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

Why this is best: Chronic insomnia disorder: sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity. Under ICSD-3/ICSD-3-TR this means symptoms at least three nights per week for at least three months The distractors correspond to other named concepts in this learning area (Short-term insomnia disorder, Prevalence, Effect on outcomes, Risk factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 26, §1.9*

Q0047 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Chronic insomnia disorder**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

E. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

ANSWER: B · sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity

Why this is best: In this context, the relevant notebook finding is **Chronic insomnia disorder**: sleep-onset and/or sleep-maintenance complaints with associated daytime impairment, exceeding minimal frequency and duration thresholds shown to be associated with clinically significant morbidity. Under ICSD-3/ICSD-3-TR this means symptoms at least three nights per week for at least three months The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 26, §1.9*

Q0048 §1.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other non-respiratory sleep disorders, which description is specifically associated with **Central disorders of hypersomnolence**?

A. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

B. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

C. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

D. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

E. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

ANSWER: E · Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

Why this is best: Central disorders of hypersomnolence: Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. MSLT: mean sleep latency ≤8 min with ≥2 SOREMPs; patients may enter sleep directly into REM. Also idiopathic hypersomnia, Kleine–Levin syndrome, insufficient sleep syndrome The distractors correspond to other named concepts in this learning area (Circadian rhythm sleep–wake disorders, Sleep-related movement disorders, Complications, Hypoventilation syndromes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0049 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Sleep-related hypoxaemia disorder**?

A. sustained desaturation during sleep without a comparable rise in CO2

B. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

C. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

D. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

E. nasal valve dilators, lubricating sprays, herbal remedies

ANSWER: A · sustained desaturation during sleep without a comparable rise in CO2

Why this is best: In this context, the relevant notebook finding is **Sleep-related hypoxaemia disorder**: sustained desaturation during sleep without a comparable rise in CO2 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 29, §1.10*

Q0050 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Caffeine**?

A. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

B. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

C. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

D. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

E. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

ANSWER: E · an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

Why this is best: Caffeine: an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours. Shortens sleep duration, prolongs sleep onset latency, increases WASO and daytime sleepiness, and reduces slow-wave sleep in a dose-related manner (see Topic 8 for the adolescent vicious cycle) The distractors correspond to other named concepts in this learning area (Non-anatomical traits, OSA and TMD, OSA prevalence, Insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 32, §1.12*

Q0051 §1.11 · Clinical-context application

Clinical context. A patient with daytime impairment may have another coexisting non-respiratory sleep disorder. During this case discussion, how should the candidate interpret **Central disorders of hypersomnolence**?

A. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

B. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

C. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

D. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

E. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

ANSWER: E · Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

Why this is best: In this context, the relevant notebook finding is **Central disorders of hypersomnolence**: Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without. MSLT: mean sleep latency ≤8 min with ≥2 SOREMPs; patients may enter sleep directly into REM. Also idiopathic hypersomnia, Kleine–Levin syndrome, insufficient sleep syndrome The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0052 §1.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on benefits and functions of normal sleep, which description is specifically associated with **Biochemical refreshment**?

A. protein synthesis, cell and tissue repair, anabolic dominance

B. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

C. declarative in NREM (sleep spindles), procedural and emotional in REM

D. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

E. REM-dependent processing of affect

ANSWER: D · synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

Why this is best: Biochemical refreshment: synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis The distractors correspond to other named concepts in this learning area (Hormone regulation, Memory consolidation, Physical restoration and repair, Emotional regulation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0053 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Caffeine**?

A. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

B. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

C. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

D. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

E. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

ANSWER: E · an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

Why this is best: In this context, the relevant notebook finding is **Caffeine**: an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours. Shortens sleep duration, prolongs sleep onset latency, increases WASO and daytime sleepiness, and reduces slow-wave sleep in a dose-related manner (see Topic 8 for the adolescent vicious cycle) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 32, §1.12*

Q0054 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **NREM-promoting**?

A. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

B. GABA and galanin from the ventrolateral preoptic area

C. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

D. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

E. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

ANSWER: B · GABA and galanin from the ventrolateral preoptic area

Why this is best: NREM-promoting: GABA and galanin from the ventrolateral preoptic area The distractors correspond to other named concepts in this learning area (REM atonia applies to the pharyngeal dilators too, Process C (circadian), Wake-promoting, Process S), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0055 §1.1 · Clinical-context application

Clinical context. During a sleep-medicine consultation, an adult with unrefreshing sleep asks about the biological value of sleep. During this case discussion, how should the candidate interpret **Biochemical refreshment**?

A. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

B. protein synthesis, cell and tissue repair, anabolic dominance

C. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

D. declarative in NREM (sleep spindles), procedural and emotional in REM

E. REM-dependent processing of affect

ANSWER: C · synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

Why this is best: In this context, the relevant notebook finding is **Biochemical refreshment**: synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0056 §1.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on anatomical/non-anatomical sdb mechanisms, which description is specifically associated with **CPAP dial-down**?

A. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

B. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

C. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

D. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

E. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

ANSWER: A · CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

Why this is best: CPAP dial-down: CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit The distractors correspond to other named concepts in this learning area (Soft tissue, Craniofacial, Awake upper airway collapsibility index, Obesity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0057 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **NREM-promoting**?

A. GABA and galanin from the ventrolateral preoptic area

B. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

C. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

D. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

E. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

ANSWER: A · GABA and galanin from the ventrolateral preoptic area

Why this is best: In this context, the relevant notebook finding is **NREM-promoting**: GABA and galanin from the ventrolateral preoptic area The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0058 §1.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on prevalence and comorbidities, which description is specifically associated with **Prevalence**?

A. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

B. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

C. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

D. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

E. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

ANSWER: C · AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

Why this is best: Prevalence: AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993). Moderate–severe 6% / 13% (US 2007–2010). ~17% of US adults, up to ~38% in some estimates. Globally 936 million any, 425 million moderate–severe, aged 30–69 The distractors correspond to other named concepts in this learning area (Gender, Predictive comorbidities, Others, Obesity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0059 §1.3 · Clinical-context application

Clinical context. An adult with confirmed OSA has incomplete benefit from mandibular advancement despite good use. During this case discussion, how should the candidate interpret **CPAP dial-down**?

A. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

B. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

C. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

D. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

E. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

ANSWER: E · CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

Why this is best: In this context, the relevant notebook finding is **CPAP dial-down**: CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0060 §1.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on age, sex and other risk factors, which description is specifically associated with **In children the dominant risk factor shifts with age**?

A. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

B. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

C. roughly a 2:1 to 3:1 ratio at any given threshold

D. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

E. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

ANSWER: D · adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

Why this is best: In children the dominant risk factor shifts with age: adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender The distractors correspond to other named concepts in this learning area (Women more often present atypically, Smoking, SDB is substantially more prevalent in men, Predictive comorbidities), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0061 §1.4 · Clinical-context application

Clinical context. A dental clinic plans risk-based sleep apnoea screening for its adult patients. During this case discussion, how should the candidate interpret **Prevalence**?

A. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

B. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

C. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

D. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine…

E. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

ANSWER: B · AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

Why this is best: In this context, the relevant notebook finding is **Prevalence**: AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993). Moderate–severe 6% / 13% (US 2007–2010). ~17% of US adults, up to ~38% in some estimates. Globally 936 million any, 425 million moderate–severe, aged 30–69 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0062 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Insomnia symptoms**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. 33–50% of the adult population; about one third report dissatisfaction with sleep

C. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

D. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

E. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

ANSWER: B · 33–50% of the adult population; about one third report dissatisfaction with sleep

Why this is best: Insomnia symptoms: 33–50% of the adult population; about one third report dissatisfaction with sleep The distractors correspond to other named concepts in this learning area (Treatment, Risk factors, Effect on outcomes, Prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 27, §1.9*

Q0063 §1.5 · Clinical-context application

Clinical context. A new referral has several risk modifiers and a non-classic presentation of sleep-related breathing problems. During this case discussion, how should the candidate interpret **In children the dominant risk factor shifts with age**?

A. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

B. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

C. roughly a 2:1 to 3:1 ratio at any given threshold

D. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

E. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

ANSWER: B · adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

Why this is best: In this context, the relevant notebook finding is **In children the dominant risk factor shifts with age**: adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0064 §1.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other non-respiratory sleep disorders, which description is specifically associated with **Sleep-related movement disorders**?

A. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

B. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

C. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

D. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

E. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

ANSWER: E · Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

Why this is best: Sleep-related movement disorders: Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction. PLMD — PLM index >15/h adults (>5/h children) is abnormal; the disorder requires associated sleep disturbance. Also sleep bruxism and rhythmic movement disorder The distractors correspond to other named concepts in this learning area (Circadian rhythm sleep–wake disorders, Central disorders of hypersomnolence, Relationship to OSA, Positional therapy in supine-dominant snorers), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0065 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Insomnia symptoms**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

C. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

D. 33–50% of the adult population; about one third report dissatisfaction with sleep

E. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

ANSWER: D · 33–50% of the adult population; about one third report dissatisfaction with sleep

Why this is best: In this context, the relevant notebook finding is **Insomnia symptoms**: 33–50% of the adult population; about one third report dissatisfaction with sleep The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 27, §1.9*

Q0066 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Non-anatomical traits**?

A. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

B. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

C. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

D. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

E. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

ANSWER: C · ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

Why this is best: Non-anatomical traits: ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23% The distractors correspond to other named concepts in this learning area (OSA and TMD, Caffeine, OSA prevalence, Suggested for chronic insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0067 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Aetiologies**?

A. frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

B. high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

C. slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia

D. a lack of drive to breathe during sleep → repetitive insufficient ventilation and compromised gas exchange

E. insufficient alveolar ventilation during sleep causing CO2 retention and hypox- aemia

ANSWER: B · high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

Why this is best: In this context, the relevant notebook finding is **Aetiologies**: high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia); treatment- emergent CSA; opioid-induced; Cheyne–Stokes breathing (heart failure, stroke); obesity hypoventilation syndrome; idiopathic CSA — uncommon, possibly <5% of patients The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.10*

Q0068 §1.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on benefits and functions of normal sleep, which description is specifically associated with **Hormone regulation**?

A. protein synthesis, cell and tissue repair, anabolic dominance

B. declarative in NREM (sleep spindles), procedural and emotional in REM

C. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

D. REM-dependent processing of affect

E. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

ANSWER: C · growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

Why this is best: Hormone regulation: growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control The distractors correspond to other named concepts in this learning area (Biochemical refreshment, Memory consolidation, Physical restoration and repair, Emotional regulation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0069 §1.11 · Clinical-context application

Clinical context. A patient with daytime impairment may have another coexisting non-respiratory sleep disorder. During this case discussion, how should the candidate interpret **Sleep-related movement disorders**?

A. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

B. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

C. Narcolepsy type 1 — hypocretin/orexin deficiency with cataplexy; type 2 without

D. Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

E. Delayed phase (adolescents), advanced phase (older adults), irregular and non-24-hour rhythms, shift work disorder, jet lag

ANSWER: D · Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction

Why this is best: In this context, the relevant notebook finding is **Sleep-related movement disorders**: Restless legs syndrome (Willis–Ekbom) — urge to move with creeping/crawling sensations, worse in the evening and at rest, relieved by movement; associated with iron deficiency and dopaminergic dysfunction. PLMD — PLM index >15/h adults (>5/h children) is abnormal; the disorder requires associated sleep disturbance. Also sleep bruxism and rhythmic movement disorder The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.11*

Q0070 §1.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on anatomical/non-anatomical sdb mechanisms, which description is specifically associated with **Obesity**?

A. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

B. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

C. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

D. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

E. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

ANSWER: A · peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

Why this is best: Obesity: peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; reduces functional residual capacity and chest wall compliance, so less caudal tracheal traction on the pharynx; disturbs the relationship between respiratory drive and load compensation The distractors correspond to other named concepts in this learning area (Awake upper airway collapsibility index, CPAP dial-down, Soft tissue, Craniofacial), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0071 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Non-anatomical traits**?

A. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

B. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

C. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

D. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

E. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

ANSWER: A · ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

Why this is best: In this context, the relevant notebook finding is **Non-anatomical traits**: ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0072 §1.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on prevalence and comorbidities, which description is specifically associated with **Others**?

A. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

B. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

C. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

D. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

E. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture,…

ANSWER: E · narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture,…

Why this is best: Others: narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture, hypothyroidism, acromegaly, PCOS, heart failure, type 2 diabetes The distractors correspond to other named concepts in this learning area (Predictive comorbidities, Gender, Prevalence, Craniofacial), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0073 §1.1 · Clinical-context application

Clinical context. During a sleep-medicine consultation, an adult with unrefreshing sleep asks about the biological value of sleep. During this case discussion, how should the candidate interpret **Hormone regulation**?

A. protein synthesis, cell and tissue repair, anabolic dominance

B. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

C. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

D. declarative in NREM (sleep spindles), procedural and emotional in REM

E. REM-dependent processing of affect

ANSWER: C · growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

Why this is best: In this context, the relevant notebook finding is **Hormone regulation**: growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0074 §1.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on age, sex and other risk factors, which description is specifically associated with **Smoking**?

A. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

B. roughly a 2:1 to 3:1 ratio at any given threshold

C. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

D. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

E. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

ANSWER: A · chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

Why this is best: Smoking: chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease The distractors correspond to other named concepts in this learning area (Women more often present atypically, In children the dominant risk factor shifts with age, SDB is substantially more prevalent in men, Awake upper airway collapsibility index), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0075 §1.3 · Clinical-context application

Clinical context. An adult with confirmed OSA has incomplete benefit from mandibular advancement despite good use. During this case discussion, how should the candidate interpret **Obesity**?

A. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

B. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

C. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

D. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

E. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

ANSWER: E · peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

Why this is best: In this context, the relevant notebook finding is **Obesity**: peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep; reduces functional residual capacity and chest wall compliance, so less caudal tracheal traction on the pharynx; disturbs the relationship between respiratory drive and load compensation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.3*

Q0076 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Insomnia**?

A. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

B. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

C. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

D. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

E. stimulant with additional nocturnal withdrawal-related arousals; smoking also causes upper airway in- flammation

ANSWER: D · symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

Why this is best: Insomnia: symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months The distractors correspond to other named concepts in this learning area (Opioids, Caffeine, Suggested for chronic insomnia, Nicotine), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0077 §1.4 · Clinical-context application

Clinical context. A dental clinic plans risk-based sleep apnoea screening for its adult patients. During this case discussion, how should the candidate interpret **Others**?

A. AHI>5 — 9% women / 24% men; AHI>5 with sleepiness — 2% / 4%; AHI>15 — 4% / 9% (Wisconsin 1993)

B. narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture,...

C. obesity, hypertension, cardiovascular disease (especially ischaemic heart disease), type 2 diabetes. Long-term: neurocognitive impairment, metabolic syndrome, systemic hypertension, cardiovascular disease

D. ~2:1–3:1 male predominance; the gap narrows with age and after the menopause — loss of oestrogen's protective effect plus post-menopausal weight gain and central fat redistribution

E. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular...

ANSWER: B · narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture,...

Why this is best: In this context, the relevant notebook finding is **Others**: narrowed airway, craniofacial abnormality, large neck circumference, large tongue, family history, smoking, alcohol/ sedatives/hypnotics/opioids, nasal congestion, rhinitis, sinusitis, inflammation, supine posture, hypothyroidism, acromegaly, PCOS, heart failure, type 2 diabetes The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 11, §1.4*

Q0078 §1.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on benefits and functions of normal sleep, which description is specifically associated with **Emotional regulation**?

A. declarative in NREM (sleep spindles), procedural and emotional in REM

B. protein synthesis, cell and tissue repair, anabolic dominance

C. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

D. REM-dependent processing of affect

E. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

ANSWER: D · REM-dependent processing of affect

Why this is best: Emotional regulation: REM-dependent processing of affect The distractors correspond to other named concepts in this learning area (Physical restoration and repair, Memory consolidation, Biochemical refreshment, Hormone regulation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0079 §1.5 · Clinical-context application

Clinical context. A new referral has several risk modifiers and a non-classic presentation of sleep-related breathing problems. During this case discussion, how should the candidate interpret **Smoking**?

A. roughly a 2:1 to 3:1 ratio at any given threshold

B. chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

C. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

D. adenotonsillar hypertrophy predominates in younger children (peak 2–8 years, when tonsillar tissue is proportionally largest), while overweight becomes the major factor in ad- olescence. Gender

E. insomnia, fatigue, mood disturbance rather than classic witnessed apnoea — and are therefore under-diagnosed. This is a good point to raise in a viva about screening. Other risk factors

ANSWER: B · chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease

Why this is best: In this context, the relevant notebook finding is **Smoking**: chronic upper airway inflammation and failure, type 2 diabetes, hypothyroidism, acromegaly, oedema PCOS, neurological disease The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 24, §1.5*

Q0080 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **Wake-promoting**?

A. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

B. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

C. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

D. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

E. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

ANSWER: C · orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

Why this is best: Wake-promoting: orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine The distractors correspond to other named concepts in this learning area (Process S, Process C (circadian), REM atonia applies to the pharyngeal dilators too, N3 (old stages 3+4)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0081 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **CPAP in those already committed or willing**?

A. insufficient alveolar ventilation during sleep causing CO2 retention and hypox- aemia

B. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

C. sustained desaturation during sleep without a comparable rise in CO2

D. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

E. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

ANSWER: D · effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

Why this is best: In this context, the relevant notebook finding is **CPAP in those already committed or willing**: effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0082 §1.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on anatomical/non-anatomical sdb mechanisms, which description is specifically associated with **Soft tissue**?

A. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

B. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

C. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

D. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

E. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

ANSWER: E · macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

Why this is best: Soft tissue: macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening The distractors correspond to other named concepts in this learning area (CPAP dial-down, Craniofacial, Awake upper airway collapsibility index, Obesity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 10, §1.3*

Q0083 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Insomnia**?

A. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

B. stimulant with additional nocturnal withdrawal-related arousals; smoking also causes upper airway in- flammation

C. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

D. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

E. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

ANSWER: A · symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

Why this is best: In this context, the relevant notebook finding is **Insomnia**: symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0084 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Effect on outcomes**?

A. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

B. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

C. sleep–wake difficulties with clinically significant sleep dissatisfaction or waking impairment that fail to meet the frequency and duration criteria of chronic insomnia disorder

D. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

E. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

ANSWER: E · marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

Why this is best: Effect on outcomes: marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time. Sleep-diary measures are the most consistently used outcome because of ease of assessment The distractors correspond to other named concepts in this learning area (Prevalence, Risk factors, Short-term insomnia disorder, Treatment), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0085 §1.1 · Clinical-context application

Clinical context. During a sleep-medicine consultation, an adult with unrefreshing sleep asks about the biological value of sleep. During this case discussion, how should the candidate interpret **Emotional regulation**?

A. REM-dependent processing of affect

B. synaptic and neuronal function; glial-mediated glymphatic clearance of metabolites; glycogen restoration; neurogenesis

C. growth hormone in early SWS; prolactin, testosterone, PTH; thyroid hormone late evening; melatonin under light–dark control

D. protein synthesis, cell and tissue repair, anabolic dominance

E. declarative in NREM (sleep spindles), procedural and emotional in REM

ANSWER: A · REM-dependent processing of affect

Why this is best: In this context, the relevant notebook finding is **Emotional regulation**: REM-dependent processing of affect The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.1*

Q0086 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Not benign**?

A. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

B. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

E. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome

ANSWER: E · heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome

Why this is best: Not benign: heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome. Also bed-partner sleep deprivation, depression and fatigue. Graded management of primary snoring The distractors correspond to other named concepts in this learning area (Relationship to OSA, Positional therapy in supine-dominant snorers, Classification by awake CO2, Bidirectional relationship with OSA), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0087 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **Wake-promoting**?

A. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

B. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

C. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

D. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

E. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

ANSWER: B · orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

Why this is best: In this context, the relevant notebook finding is **Wake-promoting**: orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0088 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **OSA–TMD**?

A. 9% F / 24% M at AHI>5; 6% / 13% moderate–severe; 936 M / 425 M globally

B. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

C. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

D. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

E. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

ANSWER: C · ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

Why this is best: OSA–TMD: ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent The distractors correspond to other named concepts in this learning area (Nicotine, Caffeine, Insomnia, Prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0089 §1.3 · Clinical-context application

Clinical context. An adult with confirmed OSA has incomplete benefit from mandibular advancement despite good use. During this case discussion, how should the candidate interpret **Soft tissue**?

A. peripharyngeal fat mechanically loads the airway, offsetting dilator activity most markedly in sleep

B. CPAP withdrawn for 5 breaths, repeated 5–10×. Peak inspiratory flow as % of CPAP-supported baseline; PIF ≥30% = only mildly collapsible airway; higher PIF = more negative Pcrit

C. choanal and epiglottic pressure catheters during negative-pressure inspiration; % collapsibility = (choanal − epiglottic)/choanal

D. retrognathia, micrognathia, maxillary constriction with high-arched/narrow palate, steep mandibular plane angle, excessive vertical facial growth, inferiorly positioned hyoid, reduced maxillary/mandibular…

E. macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

ANSWER: E · macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening

Why this is best: In this context, the relevant notebook finding is **Soft tissue**: macroglossia, elongated soft palate, enlarged uvula, adenotonsillar hypertrophy (dominant in children), parapharyngeal fat pads, large mandibular tori, lateral pharyngeal wall thickening The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 10, §1.3*

Q0090 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **N3 (old stages 3+4)**?

A. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

B. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

C. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

D. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

E. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

ANSWER: E · ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

Why this is best: N3 (old stages 3+4): ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here The distractors correspond to other named concepts in this learning area (Process S, Wake-promoting, Process C (circadian), REM atonia applies to the pharyngeal dilators too), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0091 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Effect on outcomes**?

A. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

B. sleep–wake difficulties with clinically significant sleep dissatisfaction or waking impairment that fail to meet the frequency and duration criteria of chronic insomnia disorder

C. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

ANSWER: E · marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

Why this is best: In this context, the relevant notebook finding is **Effect on outcomes**: marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time. Sleep-diary measures are the most consistently used outcome because of ease of assessment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0092 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Treatment**?

A. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

B. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

C. 33–50% of the adult population; about one third report dissatisfaction with sleep

D. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

E. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

ANSWER: B · CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

Why this is best: Treatment: CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy. Five components, more effective combined: cognitive therapy, stimulus control, sleep restriction, sleep hygiene, relaxation training (stimulus control and sleep restriction have the strongest evidence) The distractors correspond to other named concepts in this learning area (Insomnia symptoms, Risk factors, Effect on outcomes, Prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0093 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Not benign**?

A. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome

B. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

C. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

D. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

E. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

ANSWER: A · heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome

Why this is best: In this context, the relevant notebook finding is **Not benign**: heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic syndrome. Also bed-partner sleep deprivation, depression and fatigue. Graded management of primary snoring The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0094 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Nasal surgery**?

A. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

B. frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

C. speech-pathologist-delivered over ~3 months, isotonic and isometric exercises for tongue, soft palate and lateral pharyngeal walls

D. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

E. snoring affects up to 50% of the population; the prevalence of primary snoring is hard to establish because OSA must first be excluded

ANSWER: D · septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

Why this is best: Nasal surgery: septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction; benefit as a primary anti-snoring procedure is uncertain and inconsistent, but useful as a preliminary or adjunctive step The distractors correspond to other named concepts in this learning area (Catathrenia, Myofunctional therapy, Prevalence, Complications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0095 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **OSA–TMD**?

A. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

B. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

C. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

D. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

E. 9% F / 24% M at AHI>5; 6% / 13% moderate–severe; 936 M / 425 M globally

ANSWER: B · ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

Why this is best: In this context, the relevant notebook finding is **OSA–TMD**: ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0096 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Opioids**?

A. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

B. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

C. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

D. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

E. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

ANSWER: C · dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

Why this is best: Opioids: dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA. Nuance: they may improve subjective sleep quality where pain disturbs sleep, but impair architecture and respiratory control, so the net effect in SDB is adverse The distractors correspond to other named concepts in this learning area (Insomnia, Insomnia, Caffeine, Suggested for chronic insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 15, §1.12*

Q0097 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **N3 (old stages 3+4)**?

A. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

B. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

C. ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

D. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

E. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

ANSWER: C · ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here

Why this is best: In this context, the relevant notebook finding is **N3 (old stages 3+4)**: ~13–23%, 20–40 min first cycle. High-voltage slow delta; highest arousal threshold of all NREM stages; concentrated in the first third of the night; GH pulse here; disorders of arousal arise here The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0098 §1.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on sleep physiology and architecture, which description is specifically associated with **Process C (circadian)**?

A. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

B. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

C. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

D. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

E. GABA and galanin from the ventrolateral preoptic area; overall reduction in neuronal firing

ANSWER: B · endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

Why this is best: Process C (circadian): endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract. Melatonin (pineal) induces sleepiness partly by opposing the SCN alerting signal, and is suppressed by light The distractors correspond to other named concepts in this learning area (REM atonia applies to the pharyngeal dilators too, NREM-promoting, Wake-promoting, Process S), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0099 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Treatment**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. 33–50% of the adult population; about one third report dissatisfaction with sleep

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

ANSWER: E · CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

Why this is best: In this context, the relevant notebook finding is **Treatment**: CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy. Five components, more effective combined: cognitive therapy, stimulus control, sleep restriction, sleep hygiene, relaxation training (stimulus control and sleep restriction have the strongest evidence) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0100 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Combination therapy**?

A. speech-pathologist-delivered over ~3 months, isotonic and isometric exercises for tongue, soft palate and lateral pharyngeal walls

B. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

C. pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

D. snoring affects up to 50% of the population; the prevalence of primary snoring is hard to establish because OSA must first be excluded

E. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

ANSWER: C · pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

Why this is best: Combination therapy: pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders The distractors correspond to other named concepts in this learning area (Prevalence, Nasal EPAP, Myofunctional therapy, Nasal surgery), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0101 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Nasal surgery**?

A. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

B. speech-pathologist-delivered over ~3 months, isotonic and isometric exercises for tongue, soft palate and lateral pharyngeal walls

C. frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

D. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

E. snoring affects up to 50% of the population; the prevalence of primary snoring is hard to establish because OSA must first be excluded

ANSWER: D · septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

Why this is best: In this context, the relevant notebook finding is **Nasal surgery**: septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction; benefit as a primary anti-snoring procedure is uncertain and inconsistent, but useful as a preliminary or adjunctive step The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0102 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Risk factors**?

A. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

B. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. 33–50% of the adult population; about one third report dissatisfaction with sleep

ANSWER: B · increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

Why this is best: Risk factors: increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status. Pathophysiology — hyperarousal The unifying concept is physiological, cognitive and cortical hyperarousal present during both sleep and wakeful- ness, with heightened activity of the sympathetic nervous system and the hypothalamic–pituitary–adrenal axis thought to perpetuate sleep–wake dysfunction. Objective correlates include: The distractors correspond to other named concepts in this learning area (Insomnia symptoms, Treatment, Effect on outcomes, Prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 27, §1.9*

Q0103 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Opioids**?

A. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

B. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

C. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

D. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

E. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

ANSWER: C · dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

Why this is best: In this context, the relevant notebook finding is **Opioids**: dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA. Nuance: they may improve subjective sleep quality where pain disturbs sleep, but impair architecture and respiratory control, so the net effect in SDB is adverse The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 15, §1.12*

Q0104 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Weight**?

A. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

B. 10% gain → +32% AHI; 10% loss → −26% AHI

C. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

D. 9% F / 24% M at AHI>5; 6% / 13% moderate–severe; 936 M / 425 M globally

E. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

ANSWER: B · 10% gain → +32% AHI; 10% loss → −26% AHI

Why this is best: Weight: 10% gain → +32% AHI; 10% loss → −26% AHI The distractors correspond to other named concepts in this learning area (Prevalence, OSA–TMD, Nicotine, Caffeine), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0105 §1.2 · Clinical-context application

Clinical context. A sleep specialist reviews a full-night polysomnogram with a candidate and discusses normal neurophysiology. During this case discussion, how should the candidate interpret **Process C (circadian)**?

A. homeostatic sleep drive. Sleep pressure accumulates with time awake (adenosine accumulation is the principal candidate mediator) and dissipates during sleep

B. This is why REM-predominant OSA exists and why it responds relatively poorly to mandibular advancement (Topic 7)

C. GABA and galanin from the ventrolateral preoptic area; overall reduction in neuronal firing

D. endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

E. orexin/hypocretin (lateral hypothalamus), noradrenaline (locus coeruleus), serotonin (raphe), histamine (tuberomammillary), acetylcholine, dopamine

ANSWER: D · endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract

Why this is best: In this context, the relevant notebook finding is **Process C (circadian)**: endogenous ~24 h oscillator in the suprachiasmatic nucleus, entrained by light via the retinohypothalamic tract. Melatonin (pineal) induces sleepiness partly by opposing the SCN alerting signal, and is suppressed by light The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 9, §1.2*

Q0106 §1.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on insomnia, which description is specifically associated with **Prevalence**?

A. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

B. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

C. sleep–wake difficulties with clinically significant sleep dissatisfaction or waking impairment that fail to meet the frequency and duration criteria of chronic insomnia disorder

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

ANSWER: D · symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

Why this is best: Prevalence: symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%. Risk factors: increasing age, female sex, comorbidity, shift work, unemployment, lower socioeconomic status The distractors correspond to other named concepts in this learning area (Effect on outcomes, Short-term insomnia disorder, Risk factors, Short-term insomnia disorder), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0107 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Combination therapy**?

A. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

B. pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

C. speech-pathologist-delivered over ~3 months, isotonic and isometric exercises for tongue, soft palate and lateral pharyngeal walls

D. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

E. snoring affects up to 50% of the population; the prevalence of primary snoring is hard to establish because OSA must first be excluded

ANSWER: B · pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

Why this is best: In this context, the relevant notebook finding is **Combination therapy**: pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0108 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Catathrenia**?

A. slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia

B. high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

C. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

D. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

E. insufficient alveolar ventilation during sleep causing CO2 retention and hypox- aemia

ANSWER: A · slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia

Why this is best: Catathrenia: slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia The distractors correspond to other named concepts in this learning area (Hypoventilation syndromes, Aetiologies, CPAP in those already committed or willing, Complications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 13, §1.10*

Q0109 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Risk factors**?

A. 33–50% of the adult population; about one third report dissatisfaction with sleep

B. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

C. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

D. CBT-I is first line, without tolerance or the adverse effects of pharmacotherapy

E. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

ANSWER: C · increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

Why this is best: In this context, the relevant notebook finding is **Risk factors**: increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status. Pathophysiology — hyperarousal The unifying concept is physiological, cognitive and cortical hyperarousal present during both sleep and wakeful- ness, with heightened activity of the sympathetic nervous system and the hypothalamic–pituitary–adrenal axis thought to perpetuate sleep–wake dysfunction. Objective correlates include: The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 27, §1.9*

Q0110 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **OSA prevalence**?

A. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

B. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

C. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

D. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

E. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

ANSWER: B · AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

Why this is best: OSA prevalence: AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe The distractors correspond to other named concepts in this learning area (Caffeine, Non-anatomical traits, OSA and TMD, Insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0111 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Weight**?

A. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

B. 10% gain → +32% AHI; 10% loss → −26% AHI

C. 9% F / 24% M at AHI>5; 6% / 13% moderate–severe; 936 M / 425 M globally

D. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

E. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

ANSWER: B · 10% gain → +32% AHI; 10% loss → −26% AHI

Why this is best: In this context, the relevant notebook finding is **Weight**: 10% gain → +32% AHI; 10% loss → −26% AHI The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 16, §1.12*

Q0112 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Classification by awake CO2**?

A. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

B. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

C. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

D. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

E. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

ANSWER: E · hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

Why this is best: Classification by awake CO2: hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe"); and non-hypercapnic CSA, typically a high-loop-gain disorder The distractors correspond to other named concepts in this learning area (Positional therapy in supine-dominant snorers, Bidirectional relationship with OSA, Nasal EPAP, Relationship to OSA), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 28, §1.10*

Q0113 §1.9 · Clinical-context application

Clinical context. A patient with OSA also reports chronic difficulty falling asleep and recurrent night-time awakening. During this case discussion, how should the candidate interpret **Prevalence**?

A. sleep–wake difficulties with clinically significant sleep dissatisfaction or waking impairment that fail to meet the frequency and duration criteria of chronic insomnia disorder

B. clinically significant dissatisfaction or impairment failing those frequency/duration criteria. Other insomnia disorder — rare cases not meeting short-term criteria but warranting attention

C. marked, durable improvement in SOL, WASO and sleep efficiency; only a small improvement in total sleep time, augmenting over time

D. symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

E. increasing age, female sex, comorbid disorders, shift work, unemployment, lower socioeconomic status

ANSWER: D · symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%

Why this is best: In this context, the relevant notebook finding is **Prevalence**: symptoms 33–50%; ~⅓ report dissatisfaction with sleep; with distress/impairment 10–15%; chronic 5–15%; with medical/psychiatric comorbidity 50–75%. Risk factors: increasing age, female sex, comorbidity, shift work, unemployment, lower socioeconomic status The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.9*

Q0114 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **Nicotine**?

A. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

B. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

C. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

D. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

E. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

ANSWER: A · stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

Why this is best: Nicotine: stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation The distractors correspond to other named concepts in this learning area (OSA–TMD, Caffeine, Insomnia, Opioids), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 15, §1.12*

Q0115 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Catathrenia**?

A. slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia

B. insufficient alveolar ventilation during sleep causing CO2 retention and hypox- aemia

C. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

D. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

E. high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

ANSWER: A · slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia

Why this is best: In this context, the relevant notebook finding is **Catathrenia**: slow, monotone, groaning expiratory vocalisation; benign, often mistaken for a parasomnia The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.10*

Q0116 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Complications**?

A. a lack of drive to breathe during sleep → repetitive insufficient ventilation and compromised gas exchange

B. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

C. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

D. frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

E. high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

ANSWER: D · frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

Why this is best: Complications: frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes The distractors correspond to other named concepts in this learning area (Definition, Nasal surgery, Catathrenia, Aetiologies), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 28, §1.10*

Q0117 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **OSA prevalence**?

A. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

B. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

C. symptoms 33–50%; disorder 10–15%; chronic 5–15%; with comorbidity 50–75%. Chronic = ≥3 nights/week for ≥3 months

D. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

E. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

ANSWER: D · AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

Why this is best: In this context, the relevant notebook finding is **OSA prevalence**: AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0118 §1.12 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on medications and sleep, which description is specifically associated with **OSA and TMD**?

A. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

B. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

C. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

D. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

E. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

ANSWER: D · ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

Why this is best: OSA and TMD: ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults The distractors correspond to other named concepts in this learning area (Non-anatomical traits, Caffeine, OSA prevalence, Suggested for chronic insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0119 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Classification by awake CO2**?

A. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

B. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

E. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

ANSWER: C · hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

Why this is best: In this context, the relevant notebook finding is **Classification by awake CO2**: hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe"); and non-hypercapnic CSA, typically a high-loop-gain disorder The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 28, §1.10*

Q0120 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Hypoventilation syndromes**?

A. sustained desaturation during sleep without a comparable rise in CO2

B. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

C. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

D. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

E. nasal valve dilators, lubricating sprays, herbal remedies

ANSWER: D · insufficient alveolar ventilation causing CO2 retention and hypoxaemia

Why this is best: Hypoventilation syndromes: insufficient alveolar ventilation causing CO2 retention and hypoxaemia. Includes obesity hypoventilation syndrome (obesity with awake hypercapnia not otherwise explained), medication/substance-induced, and disorder-related (Duchenne muscular dystrophy, kyphoscoliosis, COPD) The distractors correspond to other named concepts in this learning area (Sleep-related hypoxaemia disorder, Complications, CPAP in those already committed or willing, Complementary), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 13, §1.10*

Q0121 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **Nicotine**?

A. dose-dependent respiratory depression with central apnoeas and ataxic breathing; a recognised cause of CSA

B. stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

C. symptoms 33–50% · disorder 10–15% · chronic 5–15% · comorbid 50–75% · chronic = ≥3 nights/week × ≥3 months

D. ≥2 signs/symptoms → 73% greater first-onset TMD incidence; chronic TMD >3× more frequent

E. adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure), half-life ~5 h

ANSWER: B · stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation

Why this is best: In this context, the relevant notebook finding is **Nicotine**: stimulant plus nocturnal withdrawal arousals; smoking also causes upper airway inflammation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 15, §1.12*

Q0122 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Nasal EPAP**?

A. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

B. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

C. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

D. pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

E. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

ANSWER: B · one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

Why this is best: Nasal EPAP: one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term The distractors correspond to other named concepts in this learning area (Bidirectional relationship with OSA, Combination therapy, Classification by awake CO2, Positional therapy in supine-dominant snorers), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0123 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Complications**?

A. frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

B. a lack of drive to breathe during sleep → repetitive insufficient ventilation and compromised gas exchange

C. septoplasty with turbinate reduction only where there are both symptoms and objective nasendoscopic evidence of obstruction

D. high-altitude periodic breathing (which most healthy people develop given sufficient alveolar hypoxia)

E. slow, monotone, groaning expiratory vocalisation during sleep; benign but often mistaken for a parasomnia by the bed partner

ANSWER: A · frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes

Why this is best: In this context, the relevant notebook finding is **Complications**: frequent nocturnal awakenings, excessive daytime sleepiness and increased risk of adverse cardiovascular outcomes The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 28, §1.10*

Q0124 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Positional therapy in supine-dominant snorers**?

A. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic…

B. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

E. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

ANSWER: B · physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

Why this is best: Positional therapy in supine-dominant snorers: physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence The distractors correspond to other named concepts in this learning area (Relationship to OSA, Classification by awake CO2, Bidirectional relationship with OSA, Not benign), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0125 §1.12 · Clinical-context application

Clinical context. The medication history reveals agents that may alter sleep and breathing. During this case discussion, how should the candidate interpret **OSA and TMD**?

A. AHI>5 — 9% F / 24% M (Wisconsin 1993); moderate–severe — 6% F / 13% M (2007–2010); global 936 million / 425 million moderate–severe

B. ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

C. an adenosine receptor antagonist (blocking the mediator of homeostatic sleep pressure) with a half-life of roughly five hours

D. ~70% have ≥1; loop gain ~36%, low arousal threshold ~37%, poor muscle responsiveness ~36%; Pcrit distribution ~19% / 58% / 23%

E. short/intermediate-acting benzodiazepine receptor agonists (zolpidem, temazepam, eszopiclone, zaleplon, triazolam)

ANSWER: B · ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults

Why this is best: In this context, the relevant notebook finding is **OSA and TMD**: ≥2 OSA signs/symptoms → 73% greater incidence of first-onset TMD; chronic TMD >3× more frequent in high-OSA-likelihood adults The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 33, §1.12*

Q0126 §1.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 1**, in the revision material on other respiratory sleep disorders, which description is specifically associated with **Relationship to OSA**?

A. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

B. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

E. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic…

ANSWER: B · approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

Why this is best: Relationship to OSA: approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject The distractors correspond to other named concepts in this learning area (Not benign, Positional therapy in supine-dominant snorers, Classification by awake CO2, Bidirectional relationship with OSA), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 29, §1.10*

Q0127 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Hypoventilation syndromes**?

A. nasal valve dilators, lubricating sprays, herbal remedies

B. frequent awakenings, excessive sleepiness, adverse cardiovascular outcomes

C. sustained desaturation during sleep without a comparable rise in CO2

D. insufficient alveolar ventilation causing CO2 retention and hypoxaemia

E. effective and safe, compliance highly variable; 4–6 cmH2O eliminates snoring

ANSWER: D · insufficient alveolar ventilation causing CO2 retention and hypoxaemia

Why this is best: In this context, the relevant notebook finding is **Hypoventilation syndromes**: insufficient alveolar ventilation causing CO2 retention and hypoxaemia. Includes obesity hypoventilation syndrome (obesity with awake hypercapnia not otherwise explained), medication/substance-induced, and disorder-related (Duchenne muscular dystrophy, kyphoscoliosis, COPD) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 13, §1.10*

Q0128 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Nasal EPAP**?

A. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

B. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

C. one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

D. pre-phase + adjunctive, adjunctive + mainstream, or multiple mainstream. Higher efficacy, limited evidence. 1.11 Other non-respiratory sleep disorders

E. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

ANSWER: C · one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term

Why this is best: In this context, the relevant notebook finding is **Nasal EPAP**: one-way resistor valves creating positive end-expiratory pressure to stabilise the airway; overall improvement but less than CPAP; minimally harmful, costly long term The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0129 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Positional therapy in supine-dominant snorers**?

A. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

B. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic…

E. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

ANSWER: E · physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

Why this is best: In this context, the relevant notebook finding is **Positional therapy in supine-dominant snorers**: physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 14, §1.10*

Q0130 §1.10 · Clinical-context application

Clinical context. The physician asks the dentist to recognise a respiratory sleep disorder other than straightforward OSA. During this case discussion, how should the candidate interpret **Relationship to OSA**?

A. approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

B. physical barrier or a position-sensing vibratory device. Reduces snoring frequency and rate in most non-apnoeic snorers (AHI<5) but not in those with OSA; older devices had poor long-term adherence

C. hypercapnic CSA, reflecting either abnormal central pattern generator output ("won't breathe") or impairment of respiratory motor output caudal to the pattern generator ("can't breathe")

D. heavy snoring (>50% of sleep time) may induce endothelial injury at the carotid bifurcation given its proximity to the hypopharyngeal walls → increased intima-media thickness, carotid atherosclerosis, metabolic…

E. central events provoke obstructive events in patients with vulnerable pharyngeal anatomy and vice versa; this mixed picture is present in the majority of sleep apnoea patients

ANSWER: A · approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject

Why this is best: In this context, the relevant notebook finding is **Relationship to OSA**: approximately one third of patients with OSA do not snore, and about one third of snorers have no OSA (AHI <5). Snoring therefore neither rules OSA in nor out — the single most useful sentence on this subject The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 29, §1.10*

TOPIC 02
Diagnostic sleep testing and interpretation

115 questions | Full notebook pages 36–60 | Each question has a justification and linked learning code.

Q0131 §2.2 · Integrated clinical vignette

A respiratory event shows a 95% airflow drop lasting 14 seconds while thoracoabdominal effort continues. What is it?

A. Obstructive apnoea

B. RERA

C. Central apnoea

D. Hypopnoea

E. Mixed apnoea without effort

ANSWER: A · Obstructive apnoea

Why this is best: An apnoea involves at least 90% airflow reduction for at least 10 seconds; persistent effort indicates obstruction.

Notebook anchor: *Integrated notebook, Topic 2, §2.2*

Q0132 §2.1 · All-of-the-above integrated judgement

For a patient undergoing attended PSG, which is the MOST COMPLETE description of a standard comprehensive recording?

A. EEG, EOG and EMG for stages

B. Airflow for respiratory events

C. Effort and oxygenation monitoring

D. Body position and cardiac rhythm

E. All of the above

ANSWER: E · All of the above

Why this is best: All these channels are used for comprehensive physiological interpretation, including stages, event type and treatment planning.

Notebook anchor: *Integrated notebook, Topic 2, §2.1*

Q0133 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **Snoring**?

A. positional dependence; directly relevant to appliance and positional therapy decisions

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. The minimum oxygen level contributes information on hypoxaemic severity

D. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

E. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

ANSWER: E · microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

Why this is best: In this context, the relevant notebook finding is **Snoring**: microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0134 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **Snoring**?

A. positional dependence; directly relevant to appliance and positional therapy decisions

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

D. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

E. The minimum oxygen level contributes information on hypoxaemic severity

ANSWER: C · microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

Why this is best: Snoring: microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB The distractors correspond to other named concepts in this learning area (Airflow, Body position, EEG + EOG + EMG together determine sleep stage, Oxygen saturation nadir), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0135 §2.1 · None-of-the-above discrimination

A patient asks which of these tests independently establishes OSA severity from a dental visit. Which is sufficient?

A. ESS alone

B. STOP-BANG alone

C. Mallampati grade alone

D. A panoramic dental radiograph alone

E. None of the above

ANSWER: E · None of the above

Why this is best: Screening scores and dental anatomy are not diagnostic sleep tests; event severity requires medical sleep evaluation.

Notebook anchor: *Integrated notebook, Topic 2, §2.1*

Q0136 §2.2 · Integrated clinical vignette

A 12-second airflow reduction of 40% is associated with a cortical arousal but no 4% oxygen fall. How is it scored under the recommended AASM rule?

A. Central apnoea automatically

B. Only a RERA can be scored

C. Normal breathing

D. An apnoea requiring a 90% fall

E. Hypopnoea

ANSWER: E · Hypopnoea

Why this is best: The recommended hypopnoea rule requires at least a 30% airflow reduction for at least 10 seconds plus 3% desaturation or arousal; 4% is not universally necessary.

Notebook anchor: *Integrated notebook, Topic 2, §2.2*

Q0137 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Chronic insomnia on PSG**?

A. Total scored apnoeas and hypopnoeas divided by hours of sleep

B. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

C. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

D. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

E. AHI plus respiratory effort-related arousals per hour of sleep

ANSWER: D · increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

Why this is best: In this context, the relevant notebook finding is **Chronic insomnia on PSG**: increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; increased N1, decreased slow-wave sleep; greater high-frequency (beta/gamma) EEG power; and a characteristic reverse first-night effect The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0138 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Chronic insomnia on PSG**?

A. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

B. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

C. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

D. AHI plus respiratory effort-related arousals per hour of sleep

E. Total scored apnoeas and hypopnoeas divided by hours of sleep

ANSWER: C · increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

Why this is best: Chronic insomnia on PSG: increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age; increased N1, decreased slow-wave sleep; greater high-frequency (beta/gamma) EEG power; and a characteristic reverse first-night effect The distractors correspond to other named concepts in this learning area (Severity, Respiratory effort-related arousal (RERA), Apnoea-hypopnoea index, Respiratory disturbance index), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0139 §2.2 · Integrated clinical vignette

An adult has a PSG AHI of 22 events per hour. Which severity category applies?

A. Moderate OSA

B. Severe OSA

C. Not classifiable without ESS

D. Mild OSA

E. Normal

ANSWER: A · Moderate OSA

Why this is best: For adults, 5–<15 is mild, 15–<30 moderate and at least 30 severe; AHI 22 is moderate.

Notebook anchor: *Integrated notebook, Topic 2, §2.2*

Q0140 §2.3 · Clinical-context application

Clinical context. A diagnosed OSA patient is being considered for oral appliance therapy based on the sleep-study report. During this case discussion, how should the candidate interpret **Surgery**?

A. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

B. positional therapy where the study shows supine dependence; weight loss where obesity is present

C. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

D. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

E. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

ANSWER: A · considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

Why this is best: In this context, the relevant notebook finding is **Surgery**: considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0141 §2.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on psg interpretation and treatment choices, which description is specifically associated with **Surgery**?

A. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

B. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

C. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

D. positional therapy where the study shows supine dependence; weight loss where obesity is present

E. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

ANSWER: E · considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

Why this is best: Surgery: considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists The distractors correspond to other named concepts in this learning area (Oral appliance therapy, Pharmacotherapy, Behavioural therapy, Apnoea), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0142 §2.5 · Integrated clinical vignette

A patient has significant heart failure, possible central apnoea and severe daytime sleepiness. Which diagnostic test is most appropriate?

A. Attended in-laboratory polysomnography under physician care

B. A trial appliance without diagnostic testing

C. A symptom questionnaire as definitive testing

D. A dentist-interpreted portable monitor for diagnosis

E. Unsupervised single-channel oximetry

ANSWER: A · Attended in-laboratory polysomnography under physician care

Why this is best: Comorbid cardiopulmonary disease and suspected non-obstructive events limit uncomplicated home testing; attended PSG is the appropriate comprehensive test.

Notebook anchor: *Integrated notebook, Topic 2, §2.5*

Q0143 §2.4 · Clinical-context application

Clinical context. A patient returns after appliance therapy with a new sleep study and a record of symptom changes. During this case discussion, how should the candidate interpret **Treatment failure**?

A. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

B. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

C. ongoing clinical symptoms, with or without <50% AHI reduction

D. resolution of OSA symptoms and treatment AHI <5/h

E. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

ANSWER: C · ongoing clinical symptoms, with or without <50% AHI reduction

Why this is best: In this context, the relevant notebook finding is **Treatment failure**: ongoing clinical symptoms, with or without <50% AHI reduction The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0144 §2.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on assessment of therapeutic response, which description is specifically associated with **Treatment failure**?

A. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

B. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

C. resolution of OSA symptoms and treatment AHI <5/h

D. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

E. ongoing clinical symptoms, with or without <50% AHI reduction

ANSWER: E · ongoing clinical symptoms, with or without <50% AHI reduction

Why this is best: Treatment failure: ongoing clinical symptoms, with or without <50% AHI reduction The distractors correspond to other named concepts in this learning area (Partial response, Complete response, Signature of a good outcome, Type 4), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0145 §2.5 · Integrated clinical vignette

A type 3 home sleep study reports REI 12 with long wake periods. What limitation should the dentist explain?

A. Recording time rather than verified sleep time can underestimate event frequency

B. REI always exceeds true AHI

C. Type 3 always includes EEG staging

D. Home testing is mandatory in suspected narcolepsy

E. A type 3 test diagnoses insomnia

ANSWER: A · Recording time rather than verified sleep time can underestimate event frequency

Why this is best: Without EEG staging, home devices use monitoring time as the denominator, which can underestimate sleep event burden.

Notebook anchor: *Integrated notebook, Topic 2, §2.5*

Q0146 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 1**?

A. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

B. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

C. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

D. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

E. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

ANSWER: D · attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

Why this is best: In this context, the relevant notebook finding is **Type 1**: attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0147 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 1**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

C. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

D. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

E. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

ANSWER: C · attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

Why this is best: Type 1: attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately The distractors correspond to other named concepts in this learning area (Type 3, Type 1 indications, Type 1 advantages, Type 2 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0148 §2.6 · Integrated clinical vignette

A patient is being investigated for narcolepsy after an overnight PSG. Which next-day test quantifies propensity to fall asleep and SOREMPs?

A. Overnight oximetry alone

B. NoSAS

C. Multiple Sleep Latency Test

D. Maintenance of Wakefulness Test

E. STOP-BANG

ANSWER: C · Multiple Sleep Latency Test

Why this is best: The MSLT quantifies mean sleep latency and sleep-onset REM periods; MWT primarily examines ability to remain awake.

Notebook anchor: *Integrated notebook, Topic 2, §2.6*

Q0149 §2.6 · Clinical-context application

Clinical context. Daytime sleepiness persists and the sleep physician orders additional vigilance testing. During this case discussion, how should the candidate interpret **MWT interpretation**?

A. mean sleep latency averaged across naps — shorter = more severe sleepiness

B. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

C. follows an overnight PSG to confirm adequate prior sleep

D. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

E. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

ANSWER: B · a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

Why this is best: In this context, the relevant notebook finding is **MWT interpretation**: a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0150 §2.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on vigilance testing: mslt and mwt, which description is specifically associated with **MWT interpretation**?

A. mean sleep latency averaged across naps — shorter = more severe sleepiness

B. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

C. follows an overnight PSG to confirm adequate prior sleep

D. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

E. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

ANSWER: B · a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

Why this is best: MWT interpretation: a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity The distractors correspond to other named concepts in this learning area (MWT method, MSLT indications, MSLT interpretation, MSLT method), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0151 §2.7 · Integrated clinical vignette

A STOP-BANG score of five is recorded at dental assessment. How should it be used?

A. As a replacement for objective testing

B. As a screen prompting physician evaluation, not as a diagnosis

C. As proof of severe OSA

D. As proof an appliance will succeed

E. As a measurement of treatment AHI

ANSWER: B · As a screen prompting physician evaluation, not as a diagnosis

Why this is best: STOP-BANG is a screening questionnaire. Positive risk does not establish the diagnosis, severity or treatment efficacy.

Notebook anchor: *Integrated notebook, Topic 2, §2.7*

Q0152 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Macroglossia**?

A. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

B. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

C. scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage

D. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

E. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

ANSWER: C · scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage

Why this is best: In this context, the relevant notebook finding is **Macroglossia**: scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage. Digital morphometry correlates increased tongue width and thickness with higher AHI The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0153 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Macroglossia**?

A. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

B. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

C. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

D. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

E. scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage

ANSWER: E · scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage

Why this is best: Macroglossia: scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding; often with inferior-posterior hyoid position placing tongue muscles at mechanical disadvantage. Digital morphometry correlates increased tongue width and thickness with higher AHI The distractors correspond to other named concepts in this learning area (Modified Mallampati, Large mandibular tori reduce oral volume, displacing the tongue, High palatal vault / narrow arch, Mouth breathing with dry mouth), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0154 §2.4 · Integrated clinical vignette

Baseline AHI is 32; with the appliance it is 8 and symptoms improve. Which label is most appropriate in the notebook?

A. Complete response

B. Partial treatment response

C. No change

D. Treatment failure by definition

E. Central sleep apnoea

ANSWER: B · Partial treatment response

Why this is best: The AHI fell by 75% but remains at least 5; improved symptoms plus at least 50% AHI reduction is a partial response, not complete normalisation.

Notebook anchor: *Integrated notebook, Topic 2, §2.4*

Q0155 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **EEG + EOG + EMG together determine sleep stage**?

A. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. The minimum oxygen level contributes information on hypoxaemic severity

D. positional dependence; directly relevant to appliance and positional therapy decisions

E. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

ANSWER: A · No staging is possible without them — the fundamental limitation of level 3 and 4 studies

Why this is best: In this context, the relevant notebook finding is **EEG + EOG + EMG together determine sleep stage**: No staging is possible without them — the fundamental limitation of level 3 and 4 studies The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0156 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **EEG + EOG + EMG together determine sleep stage**?

A. positional dependence; directly relevant to appliance and positional therapy decisions

B. The minimum oxygen level contributes information on hypoxaemic severity

C. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

D. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

E. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

ANSWER: C · No staging is possible without them — the fundamental limitation of level 3 and 4 studies

Why this is best: EEG + EOG + EMG together determine sleep stage: No staging is possible without them — the fundamental limitation of level 3 and 4 studies The distractors correspond to other named concepts in this learning area (Airflow, Snoring, Body position, Oxygen saturation nadir), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0157 §2.7 · Integrated clinical vignette

A patient has a high ESS but a low respiratory event index. What is the best interpretation?

A. Low AHI proves adequate alertness

B. Sleepiness requires assessment; ESS alone neither diagnoses nor grades OSA

C. The dentist should raise the appliance to maximum immediately

D. Sleepiness excludes narcolepsy

E. An ESS above ten proves severe OSA

ANSWER: B · Sleepiness requires assessment; ESS alone neither diagnoses nor grades OSA

Why this is best: ESS reflects subjective sleep propensity and correlates poorly with AHI. Discordance warrants evaluation of other causes and objective findings.

Notebook anchor: *Integrated notebook, Topic 2, §2.7*

Q0158 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Respiratory effort-related arousal (RERA)**?

A. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

B. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

C. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

D. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

E. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

ANSWER: D · Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

Why this is best: In this context, the relevant notebook finding is **Respiratory effort-related arousal (RERA)**: The RERA is part of the respiratory disturbance index, but not the AHI numerator. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0159 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Respiratory effort-related arousal (RERA)**?

A. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

B. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

C. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

D. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

E. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

ANSWER: C · Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

Why this is best: Respiratory effort-related arousal (RERA): The RERA is part of the respiratory disturbance index, but not the AHI numerator. The distractors correspond to other named concepts in this learning area (Severity, Hypopnoea, Chronic insomnia on PSG, Fragmentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0160 §2.3 · Integrated clinical vignette

A dentist reviews a study revealing substantial central apnoeas. What is the next step?

A. Discuss with the sleep physician before choosing an OSA appliance

B. Use a snoring questionnaire to replace the PSG

C. Diagnose central apnoea independently

D. Treat all events by protrusion alone

E. Ignore central events if dentition is sound

ANSWER: A · Discuss with the sleep physician before choosing an OSA appliance

Why this is best: Event type changes management: central apnoeas need physician-led interpretation, and the dentist must not extrapolate from OSA appliance efficacy.

Notebook anchor: *Integrated notebook, Topic 2, §2.3*

Q0161 §2.3 · Clinical-context application

Clinical context. A diagnosed OSA patient is being considered for oral appliance therapy based on the sleep-study report. During this case discussion, how should the candidate interpret **Oral appliance therapy**?

A. positional therapy where the study shows supine dependence; weight loss where obesity is present

B. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

C. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

D. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

E. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

ANSWER: E · for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

Why this is best: In this context, the relevant notebook finding is **Oral appliance therapy**: for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0162 §2.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on psg interpretation and treatment choices, which description is specifically associated with **Oral appliance therapy**?

A. positional therapy where the study shows supine dependence; weight loss where obesity is present

B. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

C. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

D. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

E. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

ANSWER: E · for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

Why this is best: Oral appliance therapy: for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success The distractors correspond to other named concepts in this learning area (Surgery, Pharmacotherapy, Behavioural therapy, OSA diagnosis), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0163 §2.4 · Clinical-context application

Clinical context. A patient returns after appliance therapy with a new sleep study and a record of symptom changes. During this case discussion, how should the candidate interpret **Partial response**?

A. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

B. resolution of OSA symptoms and treatment AHI <5/h

C. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

D. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

E. ongoing clinical symptoms, with or without <50% AHI reduction

ANSWER: A · improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

Why this is best: In this context, the relevant notebook finding is **Partial response**: improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0164 §2.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on assessment of therapeutic response, which description is specifically associated with **Partial response**?

A. ongoing clinical symptoms, with or without <50% AHI reduction

B. resolution of OSA symptoms and treatment AHI <5/h

C. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

D. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

E. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

ANSWER: E · improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

Why this is best: Partial response: improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h The distractors correspond to other named concepts in this learning area (Treatment failure, Complete response, Signature of a good outcome, Type 4), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0165 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 3**?

A. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

B. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

C. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

D. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

E. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

ANSWER: C · home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

Why this is best: In this context, the relevant notebook finding is **Type 3**: home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0166 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 3**?

A. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

B. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

C. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

D. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

E. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

ANSWER: C · home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

Why this is best: Type 3: home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG The distractors correspond to other named concepts in this learning area (Type 1, Type 1 indications, Type 1 advantages, Type 2 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0167 §2.6 · Clinical-context application

Clinical context. Daytime sleepiness persists and the sleep physician orders additional vigilance testing. During this case discussion, how should the candidate interpret **MSLT indications**?

A. follows an overnight PSG to confirm adequate prior sleep

B. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

C. mean sleep latency averaged across naps — shorter = more severe sleepiness

D. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

E. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

ANSWER: E · diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

Why this is best: In this context, the relevant notebook finding is **MSLT indications**: diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; evaluating persistent sleepiness after treatment; objectively confirming reported improvement after intervention; assessing the effects of hypnotics The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0168 §2.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on vigilance testing: mslt and mwt, which description is specifically associated with **MSLT indications**?

A. follows an overnight PSG to confirm adequate prior sleep

B. mean sleep latency averaged across naps — shorter = more severe sleepiness

C. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

D. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

E. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

ANSWER: D · diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

Why this is best: MSLT indications: diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia; evaluating persistent sleepiness after treatment; objectively confirming reported improvement after intervention; assessing the effects of hypnotics The distractors correspond to other named concepts in this learning area (MSLT interpretation, MWT method, MSLT method, MWT interpretation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0169 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Modified Mallampati**?

A. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

B. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

C. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

D. scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding

E. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

ANSWER: B · describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

Why this is best: In this context, the relevant notebook finding is **Modified Mallampati**: describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic Mallampati) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0170 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Modified Mallampati**?

A. scalloping (crenations) of the lateral tongue borders suggests the tongue is too wide for the arch, causing oropharyngeal crowding

B. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

C. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

D. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

E. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

ANSWER: B · describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

Why this is best: Modified Mallampati: describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic Mallampati) The distractors correspond to other named concepts in this learning area (Macroglossia, Large mandibular tori reduce oral volume, displacing the tongue, High palatal vault / narrow arch, Mouth breathing with dry mouth), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0171 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **Effort**?

A. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

B. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

C. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

D. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

E. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

ANSWER: C · thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

Why this is best: In this context, the relevant notebook finding is **Effort**: thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0172 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **Effort**?

A. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

B. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

C. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

D. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

E. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

ANSWER: A · thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

Why this is best: Effort: thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction The distractors correspond to other named concepts in this learning area (End-tidal or transcutaneous CO2, EEG + EOG + EMG together determine sleep stage, Airflow, Snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0173 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Fragmentation**?

A. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

B. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

C. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

D. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

E. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

ANSWER: A · increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

Why this is best: In this context, the relevant notebook finding is **Fragmentation**: increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0174 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Fragmentation**?

A. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

B. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

C. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

D. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

E. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

ANSWER: B · increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

Why this is best: Fragmentation: increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA The distractors correspond to other named concepts in this learning area (Hypopnoea, OSA diagnosis, Respiratory effort-related arousal (RERA), Severity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0175 §2.3 · Clinical-context application

Clinical context. A diagnosed OSA patient is being considered for oral appliance therapy based on the sleep-study report. During this case discussion, how should the candidate interpret **Pharmacotherapy**?

A. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

B. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

C. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

D. positional therapy where the study shows supine dependence; weight loss where obesity is present

E. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

ANSWER: B · used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

Why this is best: In this context, the relevant notebook finding is **Pharmacotherapy**: used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0176 §2.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on psg interpretation and treatment choices, which description is specifically associated with **Pharmacotherapy**?

A. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

B. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

C. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

D. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

E. positional therapy where the study shows supine dependence; weight loss where obesity is present

ANSWER: B · used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

Why this is best: Pharmacotherapy: used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself The distractors correspond to other named concepts in this learning area (Oral appliance therapy, Surgery, Behavioural therapy, Fragmentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0177 §2.4 · Clinical-context application

Clinical context. A patient returns after appliance therapy with a new sleep study and a record of symptom changes. During this case discussion, how should the candidate interpret **Signature of a good outcome**?

A. resolution of OSA symptoms and treatment AHI <5/h

B. ongoing clinical symptoms, with or without <50% AHI reduction

C. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

D. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

E. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

ANSWER: E · Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

Why this is best: In this context, the relevant notebook finding is **Signature of a good outcome**: Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases. Respiratory: AHI reduces, minimum SaO2 increases, snoring frequency and intensity reduce The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0178 §2.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on assessment of therapeutic response, which description is specifically associated with **Signature of a good outcome**?

A. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

B. ongoing clinical symptoms, with or without <50% AHI reduction

C. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

D. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

E. resolution of OSA symptoms and treatment AHI <5/h

ANSWER: D · Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

Why this is best: Signature of a good outcome: Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases. Respiratory: AHI reduces, minimum SaO2 increases, snoring frequency and intensity reduce The distractors correspond to other named concepts in this learning area (Partial response, Treatment failure, Complete response, Surgery), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0179 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 2 indications**?

A. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

B. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

C. The physician should consider attended polysomnography when clinical suspicion remains high

D. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

E. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

ANSWER: E · high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

Why this is best: In this context, the relevant notebook finding is **Type 2 indications**: high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0180 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 2 indications**?

A. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

B. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

C. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

D. The physician should consider attended polysomnography when clinical suspicion remains high

E. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

ANSWER: C · high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

Why this is best: Type 2 indications: high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder The distractors correspond to other named concepts in this learning area (High-risk patient with negative HSAT, Type 2, Type 4, Type 1 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0181 §2.6 · Clinical-context application

Clinical context. Daytime sleepiness persists and the sleep physician orders additional vigilance testing. During this case discussion, how should the candidate interpret **MSLT method**?

A. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

B. follows an overnight PSG to confirm adequate prior sleep

C. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

D. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

E. mean sleep latency averaged across naps — shorter = more severe sleepiness

ANSWER: B · follows an overnight PSG to confirm adequate prior sleep

Why this is best: In this context, the relevant notebook finding is **MSLT method**: follows an overnight PSG to confirm adequate prior sleep. Four to five nap opportunities at ~2 h intervals; in a dark quiet room the patient lies down and is told to try to fall asleep. Each trial terminates after 20 min if no sleep. The clinical version lets each nap continue to 15 min after sleep onset to permit REM to emerge; the research version terminates as soon as a sleep-onset criterion is met, limiting total sleep obtained The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0182 §2.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on vigilance testing: mslt and mwt, which description is specifically associated with **MSLT method**?

A. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

B. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

C. mean sleep latency averaged across naps — shorter = more severe sleepiness

D. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

E. follows an overnight PSG to confirm adequate prior sleep

ANSWER: E · follows an overnight PSG to confirm adequate prior sleep

Why this is best: MSLT method: follows an overnight PSG to confirm adequate prior sleep. Four to five nap opportunities at ~2 h intervals; in a dark quiet room the patient lies down and is told to try to fall asleep. Each trial terminates after 20 min if no sleep. The clinical version lets each nap continue to 15 min after sleep onset to permit REM to emerge; the research version terminates as soon as a sleep-onset criterion is met, limiting total sleep obtained The distractors correspond to other named concepts in this learning area (MSLT interpretation, MSLT indications, MWT method, MWT interpretation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0183 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Sensors**?

A. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

B. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

C. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

D. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

E. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

ANSWER: C · type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

Why this is best: In this context, the relevant notebook finding is **Sensors**: type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 43, §2.7*

Q0184 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Sensors**?

A. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

B. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

C. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

D. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

E. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

ANSWER: C · type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

Why this is best: Sensors: type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10% The distractors correspond to other named concepts in this learning area (OSA diagnosis, Epworth Sleepiness Scale threshold, STOP-BANG, Screening thresholds), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 43, §2.7*

Q0185 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **End-tidal or transcutaneous CO2**?

A. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

D. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

E. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

ANSWER: D · hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

Why this is best: In this context, the relevant notebook finding is **End-tidal or transcutaneous CO2**: hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0186 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **End-tidal or transcutaneous CO2**?

A. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

B. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

C. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

D. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

E. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

ANSWER: C · hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

Why this is best: End-tidal or transcutaneous CO2: hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation The distractors correspond to other named concepts in this learning area (Effort, EEG + EOG + EMG together determine sleep stage, Airflow, Snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0187 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Respiratory disturbance index**?

A. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

B. Total scored apnoeas and hypopnoeas divided by hours of sleep

C. AHI plus respiratory effort-related arousals per hour of sleep

D. AHI at least 30 events per hour

E. narcolepsy patients may enter sleep directly into REM

ANSWER: C · AHI plus respiratory effort-related arousals per hour of sleep

Why this is best: In this context, the relevant notebook finding is **Respiratory disturbance index**: RDI cannot be lower than AHI when events are counted consistently. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0188 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Respiratory disturbance index**?

A. Total scored apnoeas and hypopnoeas divided by hours of sleep

B. AHI at least 30 events per hour

C. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

D. narcolepsy patients may enter sleep directly into REM

E. AHI plus respiratory effort-related arousals per hour of sleep

ANSWER: E · AHI plus respiratory effort-related arousals per hour of sleep

Why this is best: Respiratory disturbance index: RDI cannot be lower than AHI when events are counted consistently. The distractors correspond to other named concepts in this learning area (Apnoea-hypopnoea index, Irregular or absent stages point to specific disorders, Chronic insomnia on PSG, Adult severe OSA threshold), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0189 §2.3 · Clinical-context application

Clinical context. A diagnosed OSA patient is being considered for oral appliance therapy based on the sleep-study report. During this case discussion, how should the candidate interpret **Behavioural therapy**?

A. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

B. positional therapy where the study shows supine dependence; weight loss where obesity is present

C. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

D. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

E. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

ANSWER: B · positional therapy where the study shows supine dependence; weight loss where obesity is present

Why this is best: In this context, the relevant notebook finding is **Behavioural therapy**: positional therapy where the study shows supine dependence; weight loss where obesity is present The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0190 §2.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on psg interpretation and treatment choices, which description is specifically associated with **Behavioural therapy**?

A. positional therapy where the study shows supine dependence; weight loss where obesity is present

B. considered for severe disease where CPAP and appliances are not tolerated or effective, particularly where a specific correctable anatomical obstruction exists

C. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

D. for mild to moderate OSA, or severe OSA where CPAP is refused, not tolerated or ineffective. Positional dependence increases the likelihood of success

E. used for other disorders diagnosed on the study (e.g. RBD) or for residual sleepiness; it is not a primary treatment for OSA itself

ANSWER: A · positional therapy where the study shows supine dependence; weight loss where obesity is present

Why this is best: Behavioural therapy: positional therapy where the study shows supine dependence; weight loss where obesity is present The distractors correspond to other named concepts in this learning area (Pharmacotherapy, Oral appliance therapy, Surgery, Severity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 48, §2.3*

Q0191 §2.4 · Clinical-context application

Clinical context. A patient returns after appliance therapy with a new sleep study and a record of symptom changes. During this case discussion, how should the candidate interpret **Complete response**?

A. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

B. resolution of OSA symptoms and treatment AHI <5/h

C. ongoing clinical symptoms, with or without <50% AHI reduction

D. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

E. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

ANSWER: B · resolution of OSA symptoms and treatment AHI <5/h

Why this is best: In this context, the relevant notebook finding is **Complete response**: resolution of OSA symptoms and treatment AHI <5/h The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0192 §2.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on assessment of therapeutic response, which description is specifically associated with **Complete response**?

A. ongoing clinical symptoms, with or without <50% AHI reduction

B. Sleep: REM increases, NREM decreases; TST typically unchanged — significant redistribution to more REM (REM rebound), not more sleep; arousal index decreases

C. resolution of OSA symptoms and treatment AHI <5/h

D. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

E. improved symptoms and ≥50% AHI reduction, but AHI remains ≥5/h

ANSWER: C · resolution of OSA symptoms and treatment AHI <5/h

Why this is best: Complete response: resolution of OSA symptoms and treatment AHI <5/h The distractors correspond to other named concepts in this learning area (Treatment failure, Partial response, Signature of a good outcome, Type 4), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 38, §2.4*

Q0193 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 2**?

A. The physician should consider attended polysomnography when clinical suspicion remains high

B. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

C. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

D. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

E. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

ANSWER: E · unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

Why this is best: In this context, the relevant notebook finding is **Type 2**: unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels. Full montage without a technician. High agreement with type 1 for diagnosing OSA, but greater signal loss and study failure (~7%) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0194 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 2**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. The physician should consider attended polysomnography when clinical suspicion remains high

C. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

D. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

E. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

ANSWER: D · unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

Why this is best: Type 2: unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels. Full montage without a technician. High agreement with type 1 for diagnosing OSA, but greater signal loss and study failure (~7%) The distractors correspond to other named concepts in this learning area (High-risk patient with negative HSAT, Type 4, Type 2 indications, Type 3), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0195 §2.6 · Clinical-context application

Clinical context. Daytime sleepiness persists and the sleep physician orders additional vigilance testing. During this case discussion, how should the candidate interpret **MWT method**?

A. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

B. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

C. follows an overnight PSG to confirm adequate prior sleep

D. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

E. mean sleep latency averaged across naps — shorter = more severe sleepiness

ANSWER: B · the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

Why this is best: In this context, the relevant notebook finding is **MWT method**: the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement. Four or five trials beginning 1.5–3 h after awakening, repeating two-hourly. Each ends after 20 min if no sleep (modern protocols use 40 min) or immediately after sleep onset; patients stay awake and occupied between trials The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0196 §2.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on vigilance testing: mslt and mwt, which description is specifically associated with **MWT method**?

A. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

B. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

C. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

D. mean sleep latency averaged across naps — shorter = more severe sleepiness

E. follows an overnight PSG to confirm adequate prior sleep

ANSWER: C · the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

Why this is best: MWT method: the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement. Four or five trials beginning 1.5–3 h after awakening, repeating two-hourly. Each ends after 20 min if no sleep (modern protocols use 40 min) or immediately after sleep onset; patients stay awake and occupied between trials The distractors correspond to other named concepts in this learning area (MSLT indications, MSLT interpretation, MSLT method, MWT interpretation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0197 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Australian direct-referral pathway**?

A. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

B. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

C. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

D. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

E. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

ANSWER: D · positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

Why this is best: In this context, the relevant notebook finding is **Australian direct-referral pathway**: positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0198 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Australian direct-referral pathway**?

A. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

B. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

C. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

D. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

E. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

ANSWER: E · positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

Why this is best: Australian direct-referral pathway: positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8 The distractors correspond to other named concepts in this learning area (Screening thresholds, Epworth Sleepiness Scale threshold, OSA diagnosis, Sensors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0199 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **Oxygen saturation nadir**?

A. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

B. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

C. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

D. The minimum oxygen level contributes information on hypoxaemic severity

E. positional dependence; directly relevant to appliance and positional therapy decisions

ANSWER: D · The minimum oxygen level contributes information on hypoxaemic severity

Why this is best: In this context, the relevant notebook finding is **Oxygen saturation nadir**: Desaturation and event burden are related but distinct dimensions of sleep-study interpretation. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0200 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **Oxygen saturation nadir**?

A. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

B. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

C. The minimum oxygen level contributes information on hypoxaemic severity

D. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

E. positional dependence; directly relevant to appliance and positional therapy decisions

ANSWER: C · The minimum oxygen level contributes information on hypoxaemic severity

Why this is best: Oxygen saturation nadir: Desaturation and event burden are related but distinct dimensions of sleep-study interpretation. The distractors correspond to other named concepts in this learning area (EEG + EOG + EMG together determine sleep stage, Airflow, Body position, Snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0201 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Apnoea-hypopnoea index**?

A. AHI plus respiratory effort-related arousals per hour of sleep

B. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

C. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

D. narcolepsy patients may enter sleep directly into REM

E. Total scored apnoeas and hypopnoeas divided by hours of sleep

ANSWER: E · Total scored apnoeas and hypopnoeas divided by hours of sleep

Why this is best: In this context, the relevant notebook finding is **Apnoea-hypopnoea index**: AHI is the conventional adult severity index on polysomnography. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0202 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Apnoea-hypopnoea index**?

A. narcolepsy patients may enter sleep directly into REM

B. AHI plus respiratory effort-related arousals per hour of sleep

C. Total scored apnoeas and hypopnoeas divided by hours of sleep

D. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

E. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

ANSWER: C · Total scored apnoeas and hypopnoeas divided by hours of sleep

Why this is best: Apnoea-hypopnoea index: AHI is the conventional adult severity index on polysomnography. The distractors correspond to other named concepts in this learning area (Respiratory disturbance index, Irregular or absent stages point to specific disorders, Chronic insomnia on PSG, Severity), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0203 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 4**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. The physician should consider attended polysomnography when clinical suspicion remains high

C. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

D. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

E. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

ANSWER: D · home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

Why this is best: In this context, the relevant notebook finding is **Type 4**: home; 1–2 bioparameters, typically oximetry ± airflow or heart rate The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0204 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 4**?

A. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

B. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

C. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

D. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

E. The physician should consider attended polysomnography when clinical suspicion remains high

ANSWER: D · home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

Why this is best: Type 4: home; 1–2 bioparameters, typically oximetry ± airflow or heart rate The distractors correspond to other named concepts in this learning area (Type 2, High-risk patient with negative HSAT, Type 2 indications, Type 1 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0205 §2.6 · Clinical-context application

Clinical context. Daytime sleepiness persists and the sleep physician orders additional vigilance testing. During this case discussion, how should the candidate interpret **MSLT interpretation**?

A. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

B. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

C. follows an overnight PSG to confirm adequate prior sleep

D. mean sleep latency averaged across naps — shorter = more severe sleepiness

E. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

ANSWER: D · mean sleep latency averaged across naps — shorter = more severe sleepiness

Why this is best: In this context, the relevant notebook finding is **MSLT interpretation**: mean sleep latency averaged across naps — shorter = more severe sleepiness. Also a measure of "sleepability": high sleepability gives MSL 6–8 min even after two weeks with 10 h time in bed; low sleepability gives long MSL, as in insomnia. ≥2 SOREMPs with MSL ≤8 min supports narcolepsy — but SOREMPs are also frequent in untreated OSA. Negative correlation between prior-night TST and next-day MSL The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0206 §2.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on vigilance testing: mslt and mwt, which description is specifically associated with **MSLT interpretation**?

A. the patient sits up in a chair in a quiet dimly lit room, instructed to stay awake; no vocalisation or movement

B. diagnosing and quantifying physiological sleepiness in narcolepsy and idiopathic hypersomnia

C. a longer mean sleep latency indicates greater ability to maintain wakefulness; considered a more accurate reflection of the ability to function and maintain alertness in common situations of inactivity

D. follows an overnight PSG to confirm adequate prior sleep

E. mean sleep latency averaged across naps — shorter = more severe sleepiness

ANSWER: E · mean sleep latency averaged across naps — shorter = more severe sleepiness

Why this is best: MSLT interpretation: mean sleep latency averaged across naps — shorter = more severe sleepiness. Also a measure of "sleepability": high sleepability gives MSL 6–8 min even after two weeks with 10 h time in bed; low sleepability gives long MSL, as in insomnia. ≥2 SOREMPs with MSL ≤8 min supports narcolepsy — but SOREMPs are also frequent in untreated OSA. Negative correlation between prior-night TST and next-day MSL The distractors correspond to other named concepts in this learning area (MSLT method, MSLT indications, MWT method, MWT interpretation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.6*

Q0207 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Mouth breathing with dry mouth**?

A. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

B. 11 items in 3 categories (snoring severity; daytime sleepiness; hypertension or obesity); high risk = positive in ≥2

C. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

D. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

E. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

ANSWER: E · compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

Why this is best: In this context, the relevant notebook finding is **Mouth breathing with dry mouth**: compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0208 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Mouth breathing with dry mouth**?

A. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

B. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

C. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

D. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

E. 11 items in 3 categories (snoring severity; daytime sleepiness; hypertension or obesity); high risk = positive in ≥2

ANSWER: C · compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

Why this is best: Mouth breathing with dry mouth: compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists The distractors correspond to other named concepts in this learning area (High palatal vault / narrow arch, Large mandibular tori reduce oral volume, displacing the tongue, Berlin, STOP-BANG), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0209 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **Body position**?

A. The minimum oxygen level contributes information on hypoxaemic severity

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

D. positional dependence; directly relevant to appliance and positional therapy decisions

E. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

ANSWER: D · positional dependence; directly relevant to appliance and positional therapy decisions

Why this is best: In this context, the relevant notebook finding is **Body position**: positional dependence; directly relevant to appliance and positional therapy decisions The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0210 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **Body position**?

A. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

D. positional dependence; directly relevant to appliance and positional therapy decisions

E. The minimum oxygen level contributes information on hypoxaemic severity

ANSWER: D · positional dependence; directly relevant to appliance and positional therapy decisions

Why this is best: Body position: positional dependence; directly relevant to appliance and positional therapy decisions The distractors correspond to other named concepts in this learning area (Snoring, EEG + EOG + EMG together determine sleep stage, Airflow, Oxygen saturation nadir), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0211 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **High-risk patient with negative HSAT**?

A. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

B. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

C. The physician should consider attended polysomnography when clinical suspicion remains high

D. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

E. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

ANSWER: C · The physician should consider attended polysomnography when clinical suspicion remains high

Why this is best: In this context, the relevant notebook finding is **High-risk patient with negative HSAT**: Home monitoring can be technically inadequate and may underestimate events without sleep staging. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0212 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **High-risk patient with negative HSAT**?

A. The physician should consider attended polysomnography when clinical suspicion remains high

B. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

C. unattended, home (occasionally laboratory-applied then home); 10–15 sensors, ≥7 channels

D. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

E. home; 1–2 bioparameters, typically oximetry ± airflow or heart rate

ANSWER: A · The physician should consider attended polysomnography when clinical suspicion remains high

Why this is best: High-risk patient with negative HSAT: Home monitoring can be technically inadequate and may underestimate events without sleep staging. The distractors correspond to other named concepts in this learning area (Type 2, Type 2 indications, Type 4, Type 1 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0213 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Australian direct referral**?

A. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

B. positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

C. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

D. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

E. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

ANSWER: B · positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

Why this is best: In this context, the relevant notebook finding is **Australian direct referral**: positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 43, §2.7*

Q0214 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Australian direct referral**?

A. positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

B. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

C. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

D. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

E. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

ANSWER: A · positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

Why this is best: Australian direct referral: positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8 The distractors correspond to other named concepts in this learning area (Screening thresholds, Epworth Sleepiness Scale threshold, OSA diagnosis, Sensors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 43, §2.7*

Q0215 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **Airflow**?

A. The minimum oxygen level contributes information on hypoxaemic severity

B. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

C. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

D. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

E. positional dependence; directly relevant to appliance and positional therapy decisions

ANSWER: D · oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

Why this is best: In this context, the relevant notebook finding is **Airflow**: oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0216 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **Airflow**?

A. positional dependence; directly relevant to appliance and positional therapy decisions

B. No staging is possible without them — the fundamental limitation of level 3 and 4 studies

C. The minimum oxygen level contributes information on hypoxaemic severity

D. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

E. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

ANSWER: E · oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

Why this is best: Airflow: oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both The distractors correspond to other named concepts in this learning area (Snoring, EEG + EOG + EMG together determine sleep stage, Body position, Oxygen saturation nadir), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0217 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Severity**?

A. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

B. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

C. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

D. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

E. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

ANSWER: C · adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

Why this is best: In this context, the relevant notebook finding is **Severity**: adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0218 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Severity**?

A. Arousal following increasing effort or flattened airflow that does not meet apnoea or hypopnoea criteria

B. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

C. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

D. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

E. adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

ANSWER: E · adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10

Why this is best: Severity: adult: normal <5 · mild 5–<15 · moderate 15–<30 · severe ≥30. Paediatric: <1 · 1–<5 · 5–<10 · ≥10 The distractors correspond to other named concepts in this learning area (Chronic insomnia on PSG, Respiratory effort-related arousal (RERA), Hypopnoea, Fragmentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0219 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 1 indications**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

C. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

D. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

E. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

ANSWER: C · low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

Why this is best: In this context, the relevant notebook finding is **Type 1 indications**: low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0220 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 1 indications**?

A. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

B. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

C. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

D. most accurate and reliable reference standard; low failure rate including for non-OSA SDB

E. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

ANSWER: A · low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

Why this is best: Type 1 indications: low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders The distractors correspond to other named concepts in this learning area (Type 1, Type 3, Type 1 advantages, Type 2 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0221 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Screening thresholds**?

A. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

B. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

C. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

D. positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

E. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

ANSWER: A · ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

Why this is best: In this context, the relevant notebook finding is **Screening thresholds**: ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0222 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Screening thresholds**?

A. positive questionnaire (Berlin / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

B. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

C. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

D. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

E. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

ANSWER: B · ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

Why this is best: Screening thresholds: ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33 The distractors correspond to other named concepts in this learning area (Australian direct-referral pathway, Epworth Sleepiness Scale threshold, Australian direct referral, Sensors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0223 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **EEG (scalp)**?

A. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

B. thoracic and abdominal bands each contain a conductor forming a wire loop

C. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

D. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

E. sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

ANSWER: E · sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

Why this is best: In this context, the relevant notebook finding is **EEG (scalp)**: sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0224 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **EEG (scalp)**?

A. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

B. sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

C. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

D. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

E. thoracic and abdominal bands each contain a conductor forming a wire loop

ANSWER: B · sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

Why this is best: EEG (scalp): sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism The distractors correspond to other named concepts in this learning area (RIP mechanism, Effort, End-tidal or transcutaneous CO2, Airflow), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0225 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Apnoea**?

A. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

B. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

C. irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

D. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

E. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

ANSWER: A · cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

Why this is best: In this context, the relevant notebook finding is **Apnoea**: cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0226 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Apnoea**?

A. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

B. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

C. irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

D. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

E. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

ANSWER: D · cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

Why this is best: Apnoea: cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive The distractors correspond to other named concepts in this learning area (Irregular or absent stages, OSA diagnosis, Fragmentation, Fragmentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0227 §2.5 · Clinical-context application

Clinical context. A patient asks whether an unattended home test will provide the same information as attended PSG. During this case discussion, how should the candidate interpret **Type 1 advantages**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

C. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

D. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

E. most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention

ANSWER: E · most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention

Why this is best: In this context, the relevant notebook finding is **Type 1 advantages**: most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention. Disadvantages: expensive, technically demanding, labour-intensive; unfamiliar environment and strange technician stressful (particularly for children), impairing sleep quality and cooperation; limited accessibility The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0228 §2.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on attended vs unattended studies, which description is specifically associated with **Type 1 advantages**?

A. high probability of moderate–severe OSA without significant medical comorbidity or other suspected sleep disorder

B. most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention

C. home; 4–7 sensors. Limited channel monitoring of breathing parameters without sleep assessment: airflow (≥2 channels of respiratory movement, or movement and airflow), effort, oximetry, heart rate/ECG

D. low probability of moderate–severe OSA; unsuitable for a type 2 study; failed type 2 study. Also the standard for narcolepsy, central sleep apnoea, parasomnias and movement disorders

E. attended, in-laboratory; 15–20 sensors, ≥7 channels. Full montage: EEG, EOG, EMG, ECG, oximetry, airflow, effort, position, limb movements. Measures sleep stage and quantifies respiratory events accurately

ANSWER: B · most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention

Why this is best: Type 1 advantages: most accurate and reliable reference standard; low failure rate including for non-OSA SDB; accurate staging and event quantification; expert supervision permitting real-time sensor troubleshooting and immediate comfort intervention. Disadvantages: expensive, technically demanding, labour-intensive; unfamiliar environment and strange technician stressful (particularly for children), impairing sleep quality and cooperation; limited accessibility The distractors correspond to other named concepts in this learning area (Type 1, Type 3, Type 1 indications, Type 2 indications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 39, §2.5*

Q0229 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **High palatal vault / narrow arch**?

A. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

B. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

C. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

D. 11 items in 3 categories (snoring severity; daytime sleepiness; hypertension or obesity); high risk = positive in ≥2

E. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

ANSWER: C · maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

Why this is best: In this context, the relevant notebook finding is **High palatal vault / narrow arch**: maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction; frequently associated with chronic mouth breathing The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0230 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **High palatal vault / narrow arch**?

A. maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

B. describes oral airway patency. Technique: patient sits upright, opens widely, protrudes the tongue maximally without phonation (the "without phonation" distinguishes the modified from the original anaesthetic…

C. Pharyngeal crowding — enlarged uvula, elongated soft palate, excessive oropharyngeal soft tissue; an enlarged uvula >15 mm long and >10 mm wide is directly linked to SDB

D. 11 items in 3 categories (snoring severity; daytime sleepiness; hypertension or obesity); high risk = positive in ≥2

E. compensation for nasal obstruction. Bruxism — common in disrupted sleep and airway problems. Dental erosion — may indicate GORD, which frequently coexists

ANSWER: A · maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction

Why this is best: High palatal vault / narrow arch: maxillary constriction reduces oral cavity space; narrow tapered V-shaped maxillary arch, possibly with posterior crossbites or compensatory mandibular constriction; frequently associated with chronic mouth breathing The distractors correspond to other named concepts in this learning area (Large mandibular tori reduce oral volume, displacing the tongue, Mouth breathing with dry mouth, Modified Mallampati, Berlin), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 42, §2.7*

Q0231 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **RIP mechanism**?

A. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

B. sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

C. thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume

D. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

E. positional dependence; directly relevant to appliance and positional therapy decisions

ANSWER: C · thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume

Why this is best: In this context, the relevant notebook finding is **RIP mechanism**: thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume. Calibrated against a spirometer or pneumotachograph it gives quantitative tidal volume and flow. RIP is the recommended effort sensor for both laboratory PSG and portable monitoring The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0232 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **RIP mechanism**?

A. hypoventilation; a standard measurement in paediatric PSG. Video/audio — parasomnia, seizure, positional confirmation

B. positional dependence; directly relevant to appliance and positional therapy decisions

C. thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume

D. sleep staging and arousals. EOG (periorbital) — eye movements, identifies REM and sleep onset. EMG (submental, tibial) — muscle tone/REM atonia, leg movements; masseter EMG for bruxism

E. thoracic and abdominal RIP belts, diaphragm EMG. Distinguishes obstructive from central; paradoxical movement indicates obstruction

ANSWER: C · thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume

Why this is best: RIP mechanism: thoracic and abdominal bands each contain a conductor forming a wire loop; respiration changes circumference, so cross-sectional area and hence inductance change; a sensor converts this to a voltage reflecting thoracoabdominal volume. Calibrated against a spirometer or pneumotachograph it gives quantitative tidal volume and flow. RIP is the recommended effort sensor for both laboratory PSG and portable monitoring The distractors correspond to other named concepts in this learning area (EEG (scalp), Effort, End-tidal or transcutaneous CO2, Body position), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 36, §2.1*

Q0233 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Irregular or absent stages**?

A. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

B. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

C. irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

D. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

E. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

ANSWER: C · irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

Why this is best: In this context, the relevant notebook finding is **Irregular or absent stages**: irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 47, §2.2*

Q0234 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Irregular or absent stages**?

A. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

B. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

C. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

D. irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

E. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

ANSWER: D · irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period)

Why this is best: Irregular or absent stages: irregular cycling or missing stages point to specific disorders — for example, a patient with narcolepsy may enter sleep directly into REM (sleep-onset REM period) The distractors correspond to other named concepts in this learning area (Apnoea, OSA diagnosis, Fragmentation, Fragmentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 47, §2.2*

Q0235 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **OSA diagnosis**?

A. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

B. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

C. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

D. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

E. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

ANSWER: D · RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

Why this is best: In this context, the relevant notebook finding is **OSA diagnosis**: RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0236 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **OSA diagnosis**?

A. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

B. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

C. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

D. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

E. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

ANSWER: C · RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

Why this is best: OSA diagnosis: RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk The distractors correspond to other named concepts in this learning area (Sensors, Epworth Sleepiness Scale threshold, STOP-BANG, Screening thresholds), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 59, §2.7*

Q0237 §2.1 · Clinical-context application

Clinical context. A 57-year-old with witnessed apnoeas undergoes an attended overnight PSG, and the team reviews the monitoring channels. During this case discussion, how should the candidate interpret **NREM 75–80% dominating the first half, REM 20–25% dominating the second**?

A. positional dependence; directly relevant to appliance and positional therapy decisions

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

D. The minimum oxygen level contributes information on hypoxaemic severity

E. Arousal index low, scored by the ASDA 3-second rule

ANSWER: E · Arousal index low, scored by the ASDA 3-second rule

Why this is best: In this context, the relevant notebook finding is **NREM 75–80% dominating the first half, REM 20–25% dominating the second**: Arousal index low, scored by the ASDA 3-second rule The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.1*

Q0238 §2.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on normal psg architecture and parameters, which description is specifically associated with **NREM 75–80% dominating the first half, REM 20–25% dominating the second**?

A. microphone/sound level meter 1 m above the head; frequency per hour and intensity in dB

B. oronasal thermal sensor (apnoea) and nasal pressure transducer (hypopnoea); ideally both

C. The minimum oxygen level contributes information on hypoxaemic severity

D. Arousal index low, scored by the ASDA 3-second rule

E. positional dependence; directly relevant to appliance and positional therapy decisions

ANSWER: D · Arousal index low, scored by the ASDA 3-second rule

Why this is best: NREM 75–80% dominating the first half, REM 20–25% dominating the second: Arousal index low, scored by the ASDA 3-second rule The distractors correspond to other named concepts in this learning area (Oxygen saturation nadir, Airflow, Body position, Snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.1*

Q0239 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Hypopnoea**?

A. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

B. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

C. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

D. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

E. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

ANSWER: E · reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

Why this is best: In this context, the relevant notebook finding is **Hypopnoea**: reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal. (Historically expressed as >50% reduction; the ≥4% desaturation alternative became optional in Scoring Manual v3, 2023, though some payers still require 4%.) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0240 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Hypopnoea**?

A. increased arousal index, increased stage shifts, increased N1, reduced slow-wave and REM sleep: the signature of untreated OSA

B. reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

C. increased arousal index, increased stage shifts, increased N1 and reduced slow-wave and REM sleep — the signature of untreated OSA

D. obstructive events per hour of sleep (the RDI) >15/h, or >5/h in a patient reporting any of: unintentional sleep episodes during wakefulness

E. cessation of airflow (≥90% reduction) for ≥10 s. Obstructive if effort continues; central if effort absent; mixed if it begins central and ends obstructive

ANSWER: B · reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal

Why this is best: Hypopnoea: reduction in airflow and/or thoracoabdominal movement of ≥30% of baseline for ≥10 s, with ≥3% desaturation and/or an arousal. (Historically expressed as >50% reduction; the ≥4% desaturation alternative became optional in Scoring Manual v3, 2023, though some payers still require 4%.) The distractors correspond to other named concepts in this learning area (Fragmentation, Fragmentation, OSA diagnosis, Apnoea), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0241 §2.7 · Clinical-context application

Clinical context. At a dental screening visit, an adult reports snoring and tiredness, and the practitioner performs a structured risk assessment. During this case discussion, how should the candidate interpret **Epworth Sleepiness Scale threshold**?

A. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

B. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

C. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

D. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

E. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

ANSWER: D · More than ten points of the possible twenty-four suggests excessive daytime sleepiness

Why this is best: In this context, the relevant notebook finding is **Epworth Sleepiness Scale threshold**: This is not a severity classifier or diagnostic test for OSA. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 40, §2.7*

Q0242 §2.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on screening and dental findings, which description is specifically associated with **Epworth Sleepiness Scale threshold**?

A. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

B. ESS >10 · STOP-BANG ≥3 · OSA50 ≥5 · Berlin ≥2 categories · NoSAS ≥8 · PSQ >0.33

C. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

D. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

E. positive questionnaire (Berlin positive / OSA50 ≥5 / STOP-BANG ≥4) plus ESS ≥8

ANSWER: A · More than ten points of the possible twenty-four suggests excessive daytime sleepiness

Why this is best: Epworth Sleepiness Scale threshold: This is not a severity classifier or diagnostic test for OSA. The distractors correspond to other named concepts in this learning area (Australian direct-referral pathway, Sensors, OSA diagnosis, Screening thresholds), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 40, §2.7*

Q0243 §2.2 · Clinical-context application

Clinical context. An attended study shows repeated flow limitation and arousals; the report must be interpreted accurately. During this case discussion, how should the candidate interpret **Irregular or absent stages point to specific disorders**?

A. AHI at least 30 events per hour

B. narcolepsy patients may enter sleep directly into REM

C. Total scored apnoeas and hypopnoeas divided by hours of sleep

D. AHI plus respiratory effort-related arousals per hour of sleep

E. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

ANSWER: B · narcolepsy patients may enter sleep directly into REM

Why this is best: In this context, the relevant notebook finding is **Irregular or absent stages point to specific disorders**: narcolepsy patients may enter sleep directly into REM The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0244 §2.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 2**, in the revision material on abnormal respiratory and sleep indices, which description is specifically associated with **Irregular or absent stages point to specific disorders**?

A. narcolepsy patients may enter sleep directly into REM

B. AHI at least 30 events per hour

C. Total scored apnoeas and hypopnoeas divided by hours of sleep

D. increased sleep latency and/or WASO (often >30 min) with reduced sleep efficiency for age

E. AHI plus respiratory effort-related arousals per hour of sleep

ANSWER: A · narcolepsy patients may enter sleep directly into REM

Why this is best: Irregular or absent stages point to specific disorders: narcolepsy patients may enter sleep directly into REM The distractors correspond to other named concepts in this learning area (Apnoea-hypopnoea index, Respiratory disturbance index, Adult severe OSA threshold, Chronic insomnia on PSG), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 37, §2.2*

Q0245 §2.7 · Targeted knowledge / discriminative SBA

Which description does the notebook specifically associate with **STOP-BANG**?

A. RDI >15/h, or >5/h with symptoms. AHI ≥15 → material increase in metabolic and vascular risk

B. Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

C. type 1 = 15–20 · 2 = 10–15 · 3 = 4–7 · 4 = 1–2. Type 2 failure ~6.6–7%; underestimates AHI ~10%

D. 11 items in 3 categories (snoring severity; daytime sleepiness; hypertension or obesity); high risk = positive in ≥2

E. More than ten points of the possible twenty-four suggests excessive daytime sleepiness

ANSWER: B · Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male

Why this is best: STOP-BANG: Snoring, Tiredness, Observed apnoea, Pressure (hypertension), BMI >35, Age >50, Neck >40 cm, Gender male. ≥3 = increased risk; higher scores correlate with greater risk; high sensitivity for moderate–severe OSA; a higher cut-off reduces sensitivity and increases specificity Competing statements refer to other named topics: Sensors, OSA diagnosis, Berlin, Epworth Sleepiness Scale threshold.

Notebook anchor: *Integrated notebook, page 41, §2.7*

TOPIC 03
Oral appliance therapy and other treatments

125 questions | Full notebook pages 61–85 | Each question has a justification and linked learning code.

Q0246 §3.2 · Integrated clinical vignette

A patient asks why a protrusive device can reduce retropalatal obstruction although it does not directly contact the soft palate. What is the best explanation?

A. It prevents all arousal during REM

B. Mandibular advancement alters tongue position and tensions connected palatal and pharyngeal tissues

C. It abolishes loop gain directly

D. It delivers positive airway pressure via an oral pump

E. It reverses all adenotonsillar hypertrophy

ANSWER: B · Mandibular advancement alters tongue position and tensions connected palatal and pharyngeal tissues

Why this is best: Mandibular advancement can enlarge the velopharynx through connected palatoglossal/palatopharyngeal soft-tissue structures and improve passive anatomy.

Notebook anchor: *Integrated notebook, Topic 3, §3.2*

Q0247 §3.1 · All-of-the-above integrated judgement

A moderate-OSA patient wishes to discuss integrated treatment. Which is the MOST COMPLETE evidence-based approach?

A. Consider PAP as appropriate

B. Consider custom titratable OAT

C. Address weight and sleep position

D. Arrange objective medical efficacy review

E. All of the above

ANSWER: E · All of the above

Why this is best: Selection and follow-up depend on patient phenotype, preference, evidence, risk factors and medical verification.

Notebook anchor: *Integrated notebook, Topic 3, §3.1*

Q0248 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Meier-Ewert et al., 1985**?

A. advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

B. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

C. an intraoral protraction device; the 1980s work was the catalyst for the modern field

D. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

E. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

ANSWER: C · an intraoral protraction device; the 1980s work was the catalyst for the modern field

Why this is best: In this context, the relevant notebook finding is **Meier-Ewert et al., 1985**: an intraoral protraction device; the 1980s work was the catalyst for the modern field The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0249 §3.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on indications and contraindications, which description is specifically associated with **Advantages, including over CPAP**?

A. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

B. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

C. Reduces snoring and the harmful effects of OSA; less obtrusive

D. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

E. Around four centimetres of water pressure

ANSWER: C · Reduces snoring and the harmful effects of OSA; less obtrusive

Why this is best: Advantages, including over CPAP: Reduces snoring and the harmful effects of OSA; less obtrusive; more portable, especially travelling; silent; no power source; more acceptable to patients and families with higher acceptance and adherence; superior quality-of-life outcomes in some comparisons; reduces blood pressure comparably to CPAP in several trials The distractors correspond to other named concepts in this learning area (OSA and primary snoring distinction, Side effects are usually well tolerated once symptomatic relief is achieved, Indications, Initial adult PAP titration pressure), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.3*

Q0250 §3.1 · None-of-the-above discrimination

Which claim is a defensible universal rule about therapy for OSA?

A. MADs always reduce AHI more than CPAP

B. PAP has better adherence in every patient

C. Weight reduction guarantees cure

D. UPPP is always successful

E. None of the above

ANSWER: E · None of the above

Why this is best: All four statements overstate evidence. Treatment must be matched to the patient and outcomes objectively reassessed.

Notebook anchor: *Integrated notebook, Topic 3, §3.1*

Q0251 §3.2 · Integrated clinical vignette

Why can OAT fail in a patient with severe upper-airway collapsibility and high loop gain despite excellent nightly wear?

A. It always normalises upper-airway muscle responsiveness and loop gain

B. It raises the arousal threshold in every patient

C. It improves anatomy more than non-anatomical ventilatory instability

D. All effective devices must have maximal vertical opening

E. High loop gain can be measured from tooth wear

ANSWER: C · It improves anatomy more than non-anatomical ventilatory instability

Why this is best: The appliance advances jaw-related structures and lowers collapsibility but does not reliably normalise respiratory control traits.

Notebook anchor: *Integrated notebook, Topic 3, §3.2*

Q0252 §3.3 · Clinical-context application

Clinical context. An adult with physician-diagnosed OSA is weighing a custom device against positive airway pressure. During this case discussion, how should the candidate interpret **Advantages, including over CPAP**?

A. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

B. Reduces snoring and the harmful effects of OSA; less obtrusive

C. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

D. Around four centimetres of water pressure

E. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

ANSWER: B · Reduces snoring and the harmful effects of OSA; less obtrusive

Why this is best: In this context, the relevant notebook finding is **Advantages, including over CPAP**: Reduces snoring and the harmful effects of OSA; less obtrusive; more portable, especially travelling; silent; no power source; more acceptable to patients and families with higher acceptance and adherence; superior quality-of-life outcomes in some comparisons; reduces blood pressure comparably to CPAP in several trials The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.3*

Q0253 §3.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on surgery, which description is specifically associated with **Tongue base**?

A. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

B. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

C. More than fifty percent reduction in AHI with postoperative AHI below twenty

D. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

E. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

ANSWER: D · genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

Why this is best: Tongue base: genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil The distractors correspond to other named concepts in this learning area (Nasal (usually adjunctive/pre-phase), UPPP success convention, Titration, Monitoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 63, §3.5*

Q0254 §3.3 · Integrated clinical vignette

A 52-year-old with diagnosed moderate OSA cannot tolerate CPAP and has stable dentition. Which treatment is guideline-consistent?

A. A dentist-fitted custom titratable mandibular advancement device in shared care

B. No treatment because CPAP failed

C. Mandibular advancement without further physician involvement

D. A diagnosis made from snoring alone

E. A stock mouthguard as first choice

ANSWER: A · A dentist-fitted custom titratable mandibular advancement device in shared care

Why this is best: The guideline supports custom titratable oral appliance therapy for adults intolerant of CPAP or preferring an alternative, with physician verification of efficacy.

Notebook anchor: *Integrated notebook, Topic 3, §3.3*

Q0255 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Monitoring**?

A. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

B. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

C. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

D. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

E. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

ANSWER: B · suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

Why this is best: In this context, the relevant notebook finding is **Monitoring**: suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention. Improves adherence and lowers side-effect severity; trade-offs are privacy and cost The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 63, §3.4*

Q0256 §3.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on positional and behavioural therapies, which description is specifically associated with **Indication**?

A. positional data from the PSG establish the indication

B. More than fifty percent reduction in AHI with postoperative AHI below twenty

C. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

D. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

E. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

ANSWER: A · positional data from the PSG establish the indication

Why this is best: Indication: positional data from the PSG establish the indication. Strictly defined, positional OSA means a non-supine AHI <5; the therapeutic goal is to reduce total sleep time spent supine to zero. Also requires a motivated patient and financial resources The distractors correspond to other named concepts in this learning area (Sleep hygiene and behavioural therapy, Principle, Methods, UPPP success convention), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0257 §3.4 · Integrated clinical vignette

A patient has moderate-to-severe OSA and asks which treatment suppresses AHI most during use. Which is the strongest answer?

A. Positional therapy is superior in all phenotypes

B. Adherence and physiologic efficacy are identical

C. A mandibular device always eliminates more events

D. UPPP invariably outperforms CPAP

E. CPAP is generally more efficacious than an oral appliance

ANSWER: E · CPAP is generally more efficacious than an oral appliance

Why this is best: CPAP generally produces a larger AHI reduction when used; adherence and patient acceptance affect real-world effectiveness.

Notebook anchor: *Integrated notebook, Topic 3, §3.4*

Q0258 §3.5 · Clinical-context application

Clinical context. A CPAP-intolerant patient is being evaluated by a multidisciplinary airway-surgery team. During this case discussion, how should the candidate interpret **Tongue base**?

A. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

B. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

C. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

D. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

E. More than fifty percent reduction in AHI with postoperative AHI below twenty

ANSWER: D · genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

Why this is best: In this context, the relevant notebook finding is **Tongue base**: genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 63, §3.5*

Q0259 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **Hypoglossal stimulation anatomy**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

C. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

D. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

E. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

ANSWER: D · Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

Why this is best: Hypoglossal stimulation anatomy: Stimulation is a selected option for appropriate CPAP-intolerant patients, not all OSA phenotypes. The distractors correspond to other named concepts in this learning area (Acetazolamide, Orofacial myofunctional therapy, Modafinil / armodafinil, Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.7*

Q0260 §3.10 · Integrated clinical vignette

An appliance reduces untreated disease burden by 65% and is worn 80% of prescribed sleep time. What is the mean disease alleviation?

A. About 65%

B. About 52%

C. About 20%

D. About 80%

E. About 145%

ANSWER: B · About 52%

Why this is best: Mean disease alleviation approximates efficacy × adherence: 0.65 × 0.80 = 0.52, demonstrating why effective use matters.

Notebook anchor: *Integrated notebook, Topic 3, §3.10*

Q0261 §3.6 · Clinical-context application

Clinical context. A patient wants to combine dental treatment with behavioural and positional approaches. During this case discussion, how should the candidate interpret **Indication**?

A. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

B. More than fifty percent reduction in AHI with postoperative AHI below twenty

C. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

D. positional data from the PSG establish the indication

E. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

ANSWER: D · positional data from the PSG establish the indication

Why this is best: In this context, the relevant notebook finding is **Indication**: positional data from the PSG establish the indication. Strictly defined, positional OSA means a non- supine AHI <5; the therapeutic goal is to reduce total sleep time spent supine to zero. Also requires a motivated patient and financial resources The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0262 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **OAT + CPAP (connected or not)**?

A. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

B. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

C. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

D. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

E. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

ANSWER: E · for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

Why this is best: OAT + CPAP (connected or not): for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure. Custom masks and interfaces can be fabricated by the dentist to facilitate this The distractors correspond to other named concepts in this learning area (OAT + nasal or oral EPAP, OAT + CPAP, connected or unconnected, OAT and CPAP used interchangeably, OAT + positional therapy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0263 §3.6 · Integrated clinical vignette

A patient with positional OSA has events almost exclusively when supine. What adjunct is most anatomically relevant?

A. A positional treatment that reduces supine sleep

B. Insomnia medication as a definitive OSA cure

C. No reassessment after weight gain

D. Routine opioid at bedtime

E. Increased supine sleeping

ANSWER: A · A positional treatment that reduces supine sleep

Why this is best: Positional treatment targets the sleep posture contributing to the events and may be combined with other therapy where appropriate.

Notebook anchor: *Integrated notebook, Topic 3, §3.6*

Q0264 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **Hypoglossal stimulation anatomy**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

C. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

D. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: B · Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

Why this is best: In this context, the relevant notebook finding is **Hypoglossal stimulation anatomy**: Stimulation is a selected option for appropriate CPAP-intolerant patients, not all OSA phenotypes. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.7*

Q0265 §3.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on comparative efficacy, which description is specifically associated with **Adherence**?

A. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

B. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

C. OAT better for sleepiness; other measures similar in supine-dependent OSA

D. CPAP lower · OAT higher · positional therapy similar to OAT

E. incremental reduction in oxybutynin; acetazolamide plus eszopiclone

ANSWER: D · CPAP lower · OAT higher · positional therapy similar to OAT

Why this is best: Adherence: CPAP lower · OAT higher · positional therapy similar to OAT The distractors correspond to other named concepts in this learning area (Versus positional therapy, PAP versus oral appliance efficacy, AHI reduction, CPAP + weight loss), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0266 §3.5 · Integrated clinical vignette

Drug-induced sleep endoscopy identifies complete concentric palatal collapse in a patient considered for hypoglossal stimulation. What is the implication?

A. It guarantees successful stimulation

B. It proves a mandibular appliance cannot be used

C. This is an exclusion for the selected stimulation pathway described in the notebook

D. It rules out all surgical options

E. It is equivalent to a negative sleep study

ANSWER: C · This is an exclusion for the selected stimulation pathway described in the notebook

Why this is best: The notebook lists complete concentric retropalatal collapse as an exclusion for the described HNS selection approach.

Notebook anchor: *Integrated notebook, Topic 3, §3.5*

Q0267 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **OAT + CPAP (connected or not)**?

A. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

B. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

C. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

D. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: B · for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

Why this is best: In this context, the relevant notebook finding is **OAT + CPAP (connected or not)**: for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure. Custom masks and interfaces can be fabricated by the dentist to facilitate this The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0268 §3.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on mean disease alleviation, which description is specifically associated with **Mean disease alleviation = efficacy × compliance**?

A. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

B. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

C. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

D. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

E. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

ANSWER: B · the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

Why this is best: Mean disease alleviation = efficacy × compliance: the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness The distractors correspond to other named concepts in this learning area (Efficacy, Best practice in dental sleep medicine, UPPP success, AHI reduction), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0269 §3.8 · Integrated clinical vignette

A patient requires high CPAP pressure and reports mask intolerance but can tolerate a mandibular appliance. What combined strategy may reduce pressure needs?

A. Use an untitratable boil-and-bite mouthguard only

B. Increase vertical opening without limitation

C. Combine mandibular advancement with PAP under specialist supervision

D. Stop all medical follow-up

E. Use the appliance to diagnose treatment severity

ANSWER: C · Combine mandibular advancement with PAP under specialist supervision

Why this is best: OAT and CPAP can be combined in selected patients to improve airway patency and reduce required PAP pressure; efficacy should be confirmed medically.

Notebook anchor: *Integrated notebook, Topic 3, §3.8*

Q0270 §3.9 · Clinical-context application

Clinical context. A sleep team compares objective outcomes with PAP, oral appliances and other therapies. During this case discussion, how should the candidate interpret **Adherence**?

A. incremental reduction in oxybutynin; acetazolamide plus eszopiclone

B. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

C. CPAP lower · OAT higher · positional therapy similar to OAT

D. OAT better for sleepiness; other measures similar in supine-dependent OSA

E. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

ANSWER: C · CPAP lower · OAT higher · positional therapy similar to OAT

Why this is best: In this context, the relevant notebook finding is **Adherence**: CPAP lower · OAT higher · positional therapy similar to OAT The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0271 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Effective OA**?

A. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

B. increments ≤1 mm · range ≥5 mm · reversible · verifiable

C. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

D. 6 mm (limited protrusion <6 mm is a contraindication)

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: B · increments ≤1 mm · range ≥5 mm · reversible · verifiable

Why this is best: Effective OA: increments ≤1 mm · range ≥5 mm · reversible · verifiable The distractors correspond to other named concepts in this learning area (Minimum protrusive distance for MAD, Effective OA definition, Effective mandibular appliance adjustment, HNS selection), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0272 §3.6 · Integrated clinical vignette

An obese adult asks whether losing 10% of body weight is expected to cure OSA. What is the best counselling answer?

A. Weight is unrelated to upper-airway collapsibility

B. Only surgery can alter OSA severity

C. Every 10% loss guarantees AHI zero

D. Diet is contraindicated with oral appliances

E. Weight reduction often lowers AHI but does not guarantee cure or remove the need for objective reassessment

ANSWER: E · Weight reduction often lowers AHI but does not guarantee cure or remove the need for objective reassessment

Why this is best: Weight loss reduces OSA burden on average, but disease may persist; treatment decisions still depend on symptoms and repeat objective assessment.

Notebook anchor: *Integrated notebook, Topic 3, §3.6*

Q0273 §3.10 · Clinical-context application

Clinical context. A well-tolerated mandibular appliance is being compared with a less-used but more efficacious PAP device. During this case discussion, how should the candidate interpret **Mean disease alleviation = efficacy × compliance**?

A. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

B. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

C. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

D. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

E. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

ANSWER: C · the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

Why this is best: In this context, the relevant notebook finding is **Mean disease alleviation = efficacy × compliance**: the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0274 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **George Catlin (19th c.)**?

A. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

B. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

C. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

D. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

E. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

ANSWER: B · first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

Why this is best: George Catlin (19th c.): first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health The distractors correspond to other named concepts in this learning area (Custom OA, Modern era, Cartwright & Samelson, 1982, Titratable), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0275 §3.9 · Integrated clinical vignette

A patient has superior nightly use of a MAD despite a smaller per-hour AHI reduction than CPAP. Which concept best reconciles comparable outcomes?

A. Oral devices cure central sleep apnoea

B. Adherence cannot be measured

C. AHI has no relation to disease burden

D. CPAP always produces more nightly use

E. Real-world effectiveness depends on both efficacy and adherence

ANSWER: E · Real-world effectiveness depends on both efficacy and adherence

Why this is best: Mean disease alleviation combines the amount of disease reduced while worn with the amount of time the device is actually used.

Notebook anchor: *Integrated notebook, Topic 3, §3.9*

Q0276 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Effective OA**?

A. increments ≤1 mm · range ≥5 mm · reversible · verifiable

B. At most one-millimetre increments across a range of at least five millimetres

C. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

D. 6 mm (limited protrusion <6 mm is a contraindication)

E. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

ANSWER: A · increments ≤1 mm · range ≥5 mm · reversible · verifiable

Why this is best: In this context, the relevant notebook finding is **Effective OA**: increments ≤1 mm · range ≥5 mm · reversible · verifiable The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0277 §3.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on indications and contraindications, which description is specifically associated with **Indications**?

A. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis

B. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

C. Reduces snoring and the harmful effects of OSA; less obtrusive

D. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

E. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

ANSWER: A · Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis

Why this is best: Indications: Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis. Mild–moderate The distractors correspond to other named concepts in this learning area (Side effects are usually well tolerated once symptomatic relief is achieved, OSA and primary snoring distinction, Advantages, including over CPAP, Whom to treat), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.3*

Q0278 §3.11 · Integrated clinical vignette

A new device has appealing advertising but no independently published efficacy data. Which principle should guide selection?

A. Price is the only relevant criterion

B. Device choice can ignore dental conditions

C. Evidence-based appraisal and patient-specific decision-making

D. Manufacturer claims alone determine efficacy

E. Patient preference eliminates the need for evidence

ANSWER: C · Evidence-based appraisal and patient-specific decision-making

Why this is best: Evidence-based dental sleep medicine integrates valid research, clinical expertise and patient values rather than commercial claims alone.

Notebook anchor: *Integrated notebook, Topic 3, §3.11*

Q0279 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **George Catlin (19th c.)**?

A. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

B. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

C. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

D. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

E. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

ANSWER: C · first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

Why this is best: In this context, the relevant notebook finding is **George Catlin (19th c.)**: first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0280 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Good practice**?

A. treatment must be based on a diagnosis established by objective sleep apnoea testing

B. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

C. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

D. Around four centimetres of water pressure

E. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

ANSWER: A · treatment must be based on a diagnosis established by objective sleep apnoea testing

Why this is best: Good practice: treatment must be based on a diagnosis established by objective sleep apnoea testing; a new diagnosis requires home or laboratory testing before treatment — but a patient already on PAP with good symptom control, diagnosed without prior The distractors correspond to other named concepts in this learning area (Advantages, Disadvantages, Education and behaviour, Initial adult PAP titration pressure), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0281 §3.3 · Clinical-context application

Clinical context. An adult with physician-diagnosed OSA is weighing a custom device against positive airway pressure. During this case discussion, how should the candidate interpret **Indications**?

A. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis

B. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

C. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

D. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

E. Reduces snoring and the harmful effects of OSA; less obtrusive

ANSWER: A · Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis

Why this is best: In this context, the relevant notebook finding is **Indications**: Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA; snoring cannot be distinguished from OSA clinically, so patients who snore must not be treated without a diagnosis. Mild–moderate The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.3*

Q0282 §3.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on surgery, which description is specifically associated with **UPPP success convention**?

A. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

B. More than fifty percent reduction in AHI with postoperative AHI below twenty

C. treatment must be based on a diagnosis established by objective sleep apnoea testing

D. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

E. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

ANSWER: B · More than fifty percent reduction in AHI with postoperative AHI below twenty

Why this is best: UPPP success convention: This surgical response measure is not the same as complete normalisation of AHI below five. The distractors correspond to other named concepts in this learning area (Nasal (usually adjunctive/pre-phase), Tongue base, Advantages, Good practice), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.5*

Q0283 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Good practice**?

A. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

B. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

C. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

D. Around four centimetres of water pressure

E. treatment must be based on a diagnosis established by objective sleep apnoea testing

ANSWER: E · treatment must be based on a diagnosis established by objective sleep apnoea testing

Why this is best: In this context, the relevant notebook finding is **Good practice**: treatment must be based on a diagnosis established by objective sleep apnoea testing; a new diagnosis requires home or laboratory testing before treatment — but a patient already on PAP with good symptom control, diagnosed without prior The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0284 §3.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on positional and behavioural therapies, which description is specifically associated with **Sleep hygiene and behavioural therapy**?

A. positional data from the PSG establish the indication

B. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

C. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

D. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

E. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

ANSWER: B · Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

Why this is best: Sleep hygiene and behavioural therapy: Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol. Provide written information on all of these plus the device considered and all alternatives. Evidence quantifying the specific effect of sleep hygiene on OSA remains limited The distractors correspond to other named concepts in this learning area (Principle, Methods, Indication, Orofacial myofunctional therapy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.6*

Q0285 §3.5 · Clinical-context application

Clinical context. A CPAP-intolerant patient is being evaluated by a multidisciplinary airway-surgery team. During this case discussion, how should the candidate interpret **UPPP success convention**?

A. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

B. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

C. treatment must be based on a diagnosis established by objective sleep apnoea testing

D. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

E. More than fifty percent reduction in AHI with postoperative AHI below twenty

ANSWER: E · More than fifty percent reduction in AHI with postoperative AHI below twenty

Why this is best: In this context, the relevant notebook finding is **UPPP success convention**: This surgical response measure is not the same as complete normalisation of AHI below five. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.5*

Q0286 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **Modafinil / armodafinil**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

C. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

D. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: B · recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

Why this is best: Modafinil / armodafinil: recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA The distractors correspond to other named concepts in this learning area (GLP-1 receptor agonists (e.g. tirzepatide), Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin), Orofacial myofunctional therapy, Acetazolamide), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0287 §3.6 · Clinical-context application

Clinical context. A patient wants to combine dental treatment with behavioural and positional approaches. During this case discussion, how should the candidate interpret **Sleep hygiene and behavioural therapy**?

A. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

B. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

C. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

D. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

E. positional data from the PSG establish the indication

ANSWER: D · Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

Why this is best: In this context, the relevant notebook finding is **Sleep hygiene and behavioural therapy**: Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol. Provide written information on all of these plus the device considered and all alternatives. Evidence quantifying the specific effect of sleep hygiene on OSA remains limited The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.6*

Q0288 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **OAT + nasal or oral EPAP**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

C. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

D. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

E. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

ANSWER: E · significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

Why this is best: OAT + nasal or oral EPAP: significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination The distractors correspond to other named concepts in this learning area (OAT + CPAP, connected or unconnected, OAT + CPAP (connected or not), OAT and CPAP used interchangeably, OAT + positional therapy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0289 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **Modafinil / armodafinil**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: B · recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

Why this is best: In this context, the relevant notebook finding is **Modafinil / armodafinil**: recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0290 §3.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on comparative efficacy, which description is specifically associated with **AHI reduction**?

A. CPAP lower · OAT higher · positional therapy similar to OAT

B. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

C. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

D. OAT better for sleepiness; other measures similar in supine-dependent OSA

E. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

ANSWER: C · CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

Why this is best: AHI reduction: CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD The distractors correspond to other named concepts in this learning area (PAP versus oral appliance efficacy, Versus positional therapy, Adherence, Mean disease alleviation = efficacy × compliance), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0291 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **OAT + nasal or oral EPAP**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

C. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

D. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

E. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

ANSWER: D · significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

Why this is best: In this context, the relevant notebook finding is **OAT + nasal or oral EPAP**: significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0292 §3.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on mean disease alleviation, which description is specifically associated with **Efficacy**?

A. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

B. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

C. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

D. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: A · the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

Why this is best: Efficacy: the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit The distractors correspond to other named concepts in this learning area (Mean disease alleviation = efficacy × compliance, Purpose of EBD, OAT + CPAP (connected or not), OAT + CPAP, connected or unconnected), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0293 §3.9 · Clinical-context application

Clinical context. A sleep team compares objective outcomes with PAP, oral appliances and other therapies. During this case discussion, how should the candidate interpret **AHI reduction**?

A. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

B. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

C. OAT better for sleepiness; other measures similar in supine-dependent OSA

D. CPAP lower · OAT higher · positional therapy similar to OAT

E. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

ANSWER: A · CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

Why this is best: In this context, the relevant notebook finding is **AHI reduction**: CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0294 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **UPPP success**?

A. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

B. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

C. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

D. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

E. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

ANSWER: A · >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

Why this is best: UPPP success: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate The distractors correspond to other named concepts in this learning area (Best practice in dental sleep medicine, AHI reduction, Weight, Purpose of EBD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0295 §3.10 · Clinical-context application

Clinical context. A well-tolerated mandibular appliance is being compared with a less-used but more efficacious PAP device. During this case discussion, how should the candidate interpret **Efficacy**?

A. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

B. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

C. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

D. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

E. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

ANSWER: B · the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

Why this is best: In this context, the relevant notebook finding is **Efficacy**: the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0296 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Pierre Robin, 1903**?

A. advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

B. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

C. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

D. an intraoral protraction device; the 1980s work was the catalyst for the modern field

E. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

ANSWER: B · the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

Why this is best: Pierre Robin, 1903: the monobloc, for glossoptosis. ~30 years later used to reposition the mandible The distractors correspond to other named concepts in this learning area (Effective oral appliance (AADSM 2019), Meier-Ewert et al., 1985, Oral appliance, Cartwright & Samelson, 1982), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0297 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **UPPP success**?

A. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

D. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

E. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

ANSWER: C · >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

Why this is best: In this context, the relevant notebook finding is **UPPP success**: >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0298 §3.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on indications and contraindications, which description is specifically associated with **OSA and primary snoring distinction**?

A. Reduces snoring and the harmful effects of OSA; less obtrusive

B. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

C. treatment must be based on a diagnosis established by objective sleep apnoea testing

D. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

E. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

ANSWER: D · OSA must first be excluded by a physician before oral appliance therapy for primary snoring

Why this is best: OSA and primary snoring distinction: Symptoms and snoring alone do not establish the absence of obstructive sleep apnoea. The distractors correspond to other named concepts in this learning area (Side effects are usually well tolerated once symptomatic relief is achieved, Advantages, including over CPAP, Indications, Good practice), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 63, §3.3*

Q0299 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Pierre Robin, 1903**?

A. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

B. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

C. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

D. an intraoral protraction device; the 1980s work was the catalyst for the modern field

E. advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

ANSWER: A · the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

Why this is best: In this context, the relevant notebook finding is **Pierre Robin, 1903**: the monobloc, for glossoptosis. ~30 years later used to reposition the mandible The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0300 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Heated humidification is suggested to reduce side effects**?

A. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

B. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

C. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

D. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

E. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

ANSWER: A · dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

Why this is best: Heated humidification is suggested to reduce side effects: dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell. The trade-offs are "rain- out" (condensation in the circuit and on the face), increased ongoing costs and maintenance The distractors correspond to other named concepts in this learning area (Titration, Education and behaviour, Monitoring, Disadvantages), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 73, §3.4*

Q0301 §3.3 · Clinical-context application

Clinical context. An adult with physician-diagnosed OSA is weighing a custom device against positive airway pressure. During this case discussion, how should the candidate interpret **OSA and primary snoring distinction**?

A. treatment must be based on a diagnosis established by objective sleep apnoea testing

B. Reduces snoring and the harmful effects of OSA; less obtrusive

C. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

D. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

E. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

ANSWER: E · OSA must first be excluded by a physician before oral appliance therapy for primary snoring

Why this is best: In this context, the relevant notebook finding is **OSA and primary snoring distinction**: Symptoms and snoring alone do not establish the absence of obstructive sleep apnoea. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 63, §3.3*

Q0302 §3.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on surgery, which description is specifically associated with **Nasal (usually adjunctive/pre-phase)**?

A. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

B. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

C. More than fifty percent reduction in AHI with postoperative AHI below twenty

D. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

E. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

ANSWER: B · septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

Why this is best: Nasal (usually adjunctive/pre-phase): septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy. Improves airflow and CPAP/OAT tolerance; rarely curative alone. Intranasal corticosteroid for allergic nasopharyngeal oedema — treat allergic rhinitis before appliance therapy (refer to ENT) without waiting for complete resolution The distractors correspond to other named concepts in this learning area (Tongue base, UPPP success convention, Heated humidification is suggested to reduce side effects, Principle), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 63, §3.5*

Q0303 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Heated humidification is suggested to reduce side effects**?

A. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

B. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

C. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

D. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

E. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

ANSWER: A · dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

Why this is best: In this context, the relevant notebook finding is **Heated humidification is suggested to reduce side effects**: dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell. The trade-offs are "rain- out" (condensation in the circuit and on the face), increased ongoing costs and maintenance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 73, §3.4*

Q0304 §3.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on positional and behavioural therapies, which description is specifically associated with **Principle**?

A. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

B. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

C. positional data from the PSG establish the indication

D. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

E. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

ANSWER: A · promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

Why this is best: Principle: promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone The distractors correspond to other named concepts in this learning area (Sleep hygiene and behavioural therapy, Methods, Indication, Nasal (usually adjunctive/pre-phase)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0305 §3.5 · Clinical-context application

Clinical context. A CPAP-intolerant patient is being evaluated by a multidisciplinary airway-surgery team. During this case discussion, how should the candidate interpret **Nasal (usually adjunctive/pre-phase)**?

A. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

B. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

C. More than fifty percent reduction in AHI with postoperative AHI below twenty

D. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

E. genioglossus advancement; midline glossectomy; lingual tonsillectomy; transoral robotic surgery; tongue coblation for retroglossal prolapse or hypertrophic lingual tonsil

ANSWER: B · septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

Why this is best: In this context, the relevant notebook finding is **Nasal (usually adjunctive/pre-phase)**: septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy. Improves airflow and CPAP/OAT tolerance; rarely curative alone. Intranasal corticosteroid for allergic nasopharyngeal oedema — treat allergic rhinitis before appliance therapy (refer to ENT) without waiting for complete resolution The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 63, §3.5*

Q0306 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **Acetazolamide**?

A. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

B. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

C. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

D. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

E. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

ANSWER: E · carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

Why this is best: Acetazolamide: carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. Eszopiclone ~3 mg — raises the arousal threshold in the low arousal threshold endotype. Supplemental oxygen — also targets high loop gain The distractors correspond to other named concepts in this learning area (Hypoglossal stimulation anatomy, Orofacial myofunctional therapy, Modafinil / armodafinil, GLP-1 receptor agonists (e.g. tirzepatide)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0307 §3.6 · Clinical-context application

Clinical context. A patient wants to combine dental treatment with behavioural and positional approaches. During this case discussion, how should the candidate interpret **Principle**?

A. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

B. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

C. septoplasty; inferior turbinectomy or turbinoplasty; nasal valve surgery for lateral wall collapse; endoscopic sinus surgery; polypectomy

D. positional data from the PSG establish the indication

E. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

ANSWER: B · promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

Why this is best: In this context, the relevant notebook finding is **Principle**: promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0308 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **OAT and CPAP used interchangeably**?

A. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

B. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

C. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

D. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: B · improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

Why this is best: OAT and CPAP used interchangeably: improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects. Patients are willing to trade symptom-free days for fewer adverse events, and most are willing to pay for that The distractors correspond to other named concepts in this learning area (OAT + CPAP, connected or unconnected, OAT + nasal or oral EPAP, OAT + CPAP (connected or not), OAT + positional therapy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0309 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **Acetazolamide**?

A. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

B. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: A · carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

Why this is best: In this context, the relevant notebook finding is **Acetazolamide**: carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype. Eszopiclone ~3 mg — raises the arousal threshold in the low arousal threshold endotype. Supplemental oxygen — also targets high loop gain The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0310 §3.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on comparative efficacy, which description is specifically associated with **PAP versus oral appliance efficacy**?

A. CPAP lower · OAT higher · positional therapy similar to OAT

B. Physiological efficacy multiplied by the proportion of treatment actually used

C. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

D. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

E. OAT better for sleepiness; other measures similar in supine-dependent OSA

ANSWER: C · PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

Why this is best: PAP versus oral appliance efficacy: Real-world effectiveness depends on both event reduction and sustained use. The distractors correspond to other named concepts in this learning area (AHI reduction, Versus positional therapy, Adherence, Mean disease alleviation calculation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 67, §3.9*

Q0311 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **OAT and CPAP used interchangeably**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

C. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

D. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

E. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

ANSWER: E · improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

Why this is best: In this context, the relevant notebook finding is **OAT and CPAP used interchangeably**: improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects. Patients are willing to trade symptom-free days for fewer adverse events, and most are willing to pay for that The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.8*

Q0312 §3.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on mean disease alleviation, which description is specifically associated with **Compliance**?

A. the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

B. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

C. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

D. Physiological efficacy multiplied by the proportion of treatment actually used

E. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

ANSWER: A · the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

Why this is best: Compliance: the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised The distractors correspond to other named concepts in this learning area (Efficacy, Mean disease alleviation = efficacy × compliance, Mean disease alleviation calculation, Purpose of EBD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0313 §3.9 · Clinical-context application

Clinical context. A sleep team compares objective outcomes with PAP, oral appliances and other therapies. During this case discussion, how should the candidate interpret **PAP versus oral appliance efficacy**?

A. Physiological efficacy multiplied by the proportion of treatment actually used

B. CPAP lower · OAT higher · positional therapy similar to OAT

C. OAT better for sleepiness; other measures similar in supine-dependent OSA

D. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

E. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

ANSWER: E · PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

Why this is best: In this context, the relevant notebook finding is **PAP versus oral appliance efficacy**: Real-world effectiveness depends on both event reduction and sustained use. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 67, §3.9*

Q0314 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Weight**?

A. increments ≤1 mm · range ≥5 mm · reversible · verifiable

B. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

C. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

D. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: B · 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

Why this is best: Weight: 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35 The distractors correspond to other named concepts in this learning area (Effective OA definition, HNS selection, Effective mandibular appliance adjustment, Effective OA), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0315 §3.10 · Clinical-context application

Clinical context. A well-tolerated mandibular appliance is being compared with a less-used but more efficacious PAP device. During this case discussion, how should the candidate interpret **Compliance**?

A. the overall percentage reduction in disease severity achieved in real- world conditions. Reflects true effectiveness

B. the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

C. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

D. Physiological efficacy multiplied by the proportion of treatment actually used

E. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

ANSWER: B · the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

Why this is best: In this context, the relevant notebook finding is **Compliance**: the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.10*

Q0316 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Cartwright & Samelson, 1982**?

A. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

B. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

C. an intraoral protraction device; the 1980s work was the catalyst for the modern field

D. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

E. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

ANSWER: A · the tongue retaining device, holding the tongue forward by suction; historically the second most common type

Why this is best: Cartwright & Samelson, 1982: the tongue retaining device, holding the tongue forward by suction; historically the second most common type The distractors correspond to other named concepts in this learning area (Titratable, Oral appliance, Meier-Ewert et al., 1985, Pierre Robin, 1903), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0317 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Weight**?

A. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

B. increments ≤1 mm · range ≥5 mm · reversible · verifiable

C. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

D. At most one-millimetre increments across a range of at least five millimetres

E. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

ANSWER: E · 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

Why this is best: In this context, the relevant notebook finding is **Weight**: 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35 The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0318 §3.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on indications and contraindications, which description is specifically associated with **Side effects are usually well tolerated once symptomatic relief is achieved**?

A. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

B. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

C. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

D. Reduces snoring and the harmful effects of OSA; less obtrusive

E. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

ANSWER: C · Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

Why this is best: Side effects are usually well tolerated once symptomatic relief is achieved: Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner; the dentist assesses dental suitability, treats oral disease first, chooses the device, supervises implementation, and follows up to prevent, recognise and manage orofacial complications. 3.4 Positive airway pressure THE FOUR MODALITIES The distractors correspond to other named concepts in this learning area (OSA and primary snoring distinction, Advantages, including over CPAP, Indications, Education and behaviour), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.3*

Q0319 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Cartwright & Samelson, 1982**?

A. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

B. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

C. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

D. an intraoral protraction device; the 1980s work was the catalyst for the modern field

E. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

ANSWER: B · the tongue retaining device, holding the tongue forward by suction; historically the second most common type

Why this is best: In this context, the relevant notebook finding is **Cartwright & Samelson, 1982**: the tongue retaining device, holding the tongue forward by suction; historically the second most common type The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0320 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Initial adult PAP titration pressure**?

A. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

B. Around four centimetres of water pressure

C. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

D. treatment must be based on a diagnosis established by objective sleep apnoea testing

E. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

ANSWER: B · Around four centimetres of water pressure

Why this is best: Initial adult PAP titration pressure: The notebook describes increasing to the lowest effective pressure, commonly within the range to twenty. The distractors correspond to other named concepts in this learning area (Advantages, Good practice, Disadvantages, Education and behaviour), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 64, §3.4*

Q0321 §3.3 · Clinical-context application

Clinical context. An adult with physician-diagnosed OSA is weighing a custom device against positive airway pressure. During this case discussion, how should the candidate interpret **Side effects are usually well tolerated once symptomatic relief is achieved**?

A. OSA must first be excluded by a physician before oral appliance therapy for primary snoring

B. Reduces snoring and the harmful effects of OSA; less obtrusive

C. Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

D. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

E. Primary snoring without apnoea — first-line for a confirmed diagnosis in the confirmed absence of OSA

ANSWER: C · Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner

Why this is best: In this context, the relevant notebook finding is **Side effects are usually well tolerated once symptomatic relief is achieved**: Division of responsibility: diagnosis, prescription and monitoring of effectiveness rest with the medical practitioner; the dentist assesses dental suitability, treats oral disease first, chooses the device, supervises implementation, and follows up to prevent, recognise and manage orofacial complications. 3.4 Positive airway pressure THE FOUR MODALITIES The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.3*

Q0322 §3.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on positional and behavioural therapies, which description is specifically associated with **Methods**?

A. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

B. positional data from the PSG establish the indication

C. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

D. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

E. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

ANSWER: C · older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

Why this is best: Methods: older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness. Newer smart devices (e.g. NightShift) detect body position and deliver a vibratory stimulus; note that these generally provide no objective compliance data The distractors correspond to other named concepts in this learning area (Principle, Sleep hygiene and behavioural therapy, Indication, GLP-1 receptor agonists (e.g. tirzepatide)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0323 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Initial adult PAP titration pressure**?

A. treatment must be based on a diagnosis established by objective sleep apnoea testing

B. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

C. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

D. Around four centimetres of water pressure

E. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

ANSWER: D · Around four centimetres of water pressure

Why this is best: In this context, the relevant notebook finding is **Initial adult PAP titration pressure**: The notebook describes increasing to the lowest effective pressure, commonly within the range to twenty. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 64, §3.4*

Q0324 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **GLP-1 receptor agonists (e.g. tirzepatide)**?

A. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

B. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

C. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

D. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: A · reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

Why this is best: GLP-1 receptor agonists (e.g. tirzepatide): reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development. Orexin agonists/antagonists also under investigation The distractors correspond to other named concepts in this learning area (Modafinil / armodafinil, Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin), Orofacial myofunctional therapy, Acetazolamide), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0325 §3.6 · Clinical-context application

Clinical context. A patient wants to combine dental treatment with behavioural and positional approaches. During this case discussion, how should the candidate interpret **Methods**?

A. promotes side sleeping instead of supine sleeping. Snoring severity and AHI are greater supine; in children the lowest AHI is often prone

B. older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. positional data from the PSG establish the indication

E. Sleep behavioural therapy = sleep hygiene, sleeping position, weight control, avoidance of sedatives including alcohol

ANSWER: B · older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness

Why this is best: In this context, the relevant notebook finding is **Methods**: older techniques — tennis or golf ball in a sock or a bra worn backwards, body pillows, special shirts and vests — with poor long-term adherence due to discomfort and ineffectiveness. Newer smart devices (e.g. NightShift) detect body position and deliver a vibratory stimulus; note that these generally provide no objective compliance data The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 78, §3.6*

Q0326 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **CPAP + weight loss**?

A. incremental reduction in oxybutynin; acetazolamide plus eszopiclone

B. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

C. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

D. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: A · incremental reduction in oxybutynin; acetazolamide plus eszopiclone

Why this is best: CPAP + weight loss: incremental reduction in oxybutynin; acetazolamide plus eszopiclone; insulin resistance, serum triglycerides and blood supplemental oxygen with a hypnotic — targeting pressure compared with either alone specific physiological profiles The distractors correspond to other named concepts in this learning area (OAT and CPAP used interchangeably, OAT + CPAP, connected or unconnected, OAT + nasal or oral EPAP, OAT + CPAP (connected or not)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 80, §3.8*

Q0327 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **GLP-1 receptor agonists (e.g. tirzepatide)**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: C · reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

Why this is best: In this context, the relevant notebook finding is **GLP-1 receptor agonists (e.g. tirzepatide)**: reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development. Orexin agonists/antagonists also under investigation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0328 §3.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on comparative efficacy, which description is specifically associated with **Versus positional therapy**?

A. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

B. OAT better for sleepiness; other measures similar in supine-dependent OSA

C. Physiological efficacy multiplied by the proportion of treatment actually used

D. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

E. CPAP lower · OAT higher · positional therapy similar to OAT

ANSWER: B · OAT better for sleepiness; other measures similar in supine-dependent OSA

Why this is best: Versus positional therapy: OAT better for sleepiness; other measures similar in supine-dependent OSA The distractors correspond to other named concepts in this learning area (Adherence, PAP versus oral appliance efficacy, AHI reduction, Mean disease alleviation calculation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0329 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **CPAP + weight loss**?

A. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

B. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

C. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

D. incremental reduction in oxybutynin; acetazolamide plus eszopiclone

E. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

ANSWER: D · incremental reduction in oxybutynin; acetazolamide plus eszopiclone

Why this is best: In this context, the relevant notebook finding is **CPAP + weight loss**: incremental reduction in oxybutynin; acetazolamide plus eszopiclone; insulin resistance, serum triglycerides and blood supplemental oxygen with a hypnotic — targeting pressure compared with either alone specific physiological profiles The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 80, §3.8*

Q0330 §3.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on mean disease alleviation, which description is specifically associated with **Mean disease alleviation calculation**?

A. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

B. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

C. the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

D. At most one-millimetre increments across a range of at least five millimetres

E. Physiological efficacy multiplied by the proportion of treatment actually used

ANSWER: E · Physiological efficacy multiplied by the proportion of treatment actually used

Why this is best: Mean disease alleviation calculation: This illustrates why a less efficacious device with greater adherence can give comparable real-world reduction. The distractors correspond to other named concepts in this learning area (Efficacy, Compliance, Effective mandibular appliance adjustment, Effective OA definition), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 67, §3.10*

Q0331 §3.9 · Clinical-context application

Clinical context. A sleep team compares objective outcomes with PAP, oral appliances and other therapies. During this case discussion, how should the candidate interpret **Versus positional therapy**?

A. OAT better for sleepiness; other measures similar in supine-dependent OSA

B. Physiological efficacy multiplied by the proportion of treatment actually used

C. CPAP highest (~75%) · OAT moderate (~42%) · UPPP variable (~30%) · exercise and weight loss similar to MAD

D. CPAP lower · OAT higher · positional therapy similar to OAT

E. PAP typically has higher physiological suppression of AHI, while OAT can be better tolerated

ANSWER: A · OAT better for sleepiness; other measures similar in supine-dependent OSA

Why this is best: In this context, the relevant notebook finding is **Versus positional therapy**: OAT better for sleepiness; other measures similar in supine-dependent OSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.9*

Q0332 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Purpose of EBD**?

A. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

B. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

C. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

D. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

E. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

ANSWER: C · provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

Why this is best: Purpose of EBD: provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making The distractors correspond to other named concepts in this learning area (UPPP success, Best practice in dental sleep medicine, AHI reduction, Weight), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 67, §3.11*

Q0333 §3.10 · Clinical-context application

Clinical context. A well-tolerated mandibular appliance is being compared with a less-used but more efficacious PAP device. During this case discussion, how should the candidate interpret **Mean disease alleviation calculation**?

A. the extent to which patients correctly follow medical advice, often quantified as the percentage of prescribed therapy actually used. Determines how much of the efficacy is realised

B. Physiological efficacy multiplied by the proportion of treatment actually used

C. the maximum potential benefit of a treatment, typically measured in controlled trials as the reduction in a disease marker such as AHI. Sets the upper limit of benefit

D. At most one-millimetre increments across a range of at least five millimetres

E. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

ANSWER: B · Physiological efficacy multiplied by the proportion of treatment actually used

Why this is best: In this context, the relevant notebook finding is **Mean disease alleviation calculation**: This illustrates why a less efficacious device with greater adherence can give comparable real-world reduction. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 67, §3.10*

Q0334 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Custom OA**?

A. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

B. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

C. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

D. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

E. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

ANSWER: C · made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

Why this is best: Custom OA: made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified The distractors correspond to other named concepts in this learning area (George Catlin (19th c.), Modern era, Titratable, Cartwright & Samelson, 1982), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0335 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Purpose of EBD**?

A. >50% AHI reduction and AHI <20; Friedman stage I best, III mostly unsuccessful; ~4% surgical adverse event rate

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

D. written informed consent before treatment; satisfy regulator and association policies and codes of conduct

E. provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

ANSWER: E · provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making

Why this is best: In this context, the relevant notebook finding is **Purpose of EBD**: provide appropriate care, reduce inappropriate care, increase effectiveness of care, by promoting best available evidence in decision-making The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 67, §3.11*

Q0336 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Titration**?

A. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

B. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

C. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

D. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

E. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

ANSWER: E · establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

Why this is best: Titration: establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O The distractors correspond to other named concepts in this learning area (Monitoring, Heated humidification is suggested to reduce side effects, Heated humidification suggested, Modality), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0337 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Custom OA**?

A. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

B. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

C. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

D. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

E. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

ANSWER: E · made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

Why this is best: In this context, the relevant notebook finding is **Custom OA**: made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0338 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **Orofacial myofunctional therapy**?

A. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

B. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

E. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

ANSWER: A · targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

Why this is best: Orofacial myofunctional therapy: targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct. Other devices — high-flow nasal cannula, oral negative pressure devices, under study. 3.8 Combination therapy The distractors correspond to other named concepts in this learning area (Acetazolamide, Hypoglossal stimulation anatomy, GLP-1 receptor agonists (e.g. tirzepatide), Modafinil / armodafinil), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0339 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Titration**?

A. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

B. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

C. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

D. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

E. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

ANSWER: D · establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

Why this is best: In this context, the relevant notebook finding is **Titration**: establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0340 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **OAT + CPAP, connected or unconnected**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

C. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

D. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

E. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

ANSWER: D · for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

Why this is best: OAT + CPAP, connected or unconnected: for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while The distractors correspond to other named concepts in this learning area (OAT + nasal or oral EPAP, OAT + CPAP (connected or not), OAT and CPAP used interchangeably, OAT + positional therapy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 79, §3.8*

Q0341 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **Orofacial myofunctional therapy**?

A. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

B. Electrical stimulation of the hypoglossal nerve recruits tongue dilator function

C. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

D. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: E · targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

Why this is best: In this context, the relevant notebook finding is **Orofacial myofunctional therapy**: targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct. Other devices — high-flow nasal cannula, oral negative pressure devices, under study. 3.8 Combination therapy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0342 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **AHI reduction**?

A. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

B. At most one-millimetre increments across a range of at least five millimetres

C. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

D. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

E. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

ANSWER: E · CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

Why this is best: AHI reduction: CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night The distractors correspond to other named concepts in this learning area (Weight, Effective OA definition, Effective mandibular appliance adjustment, Weight), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0343 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **OAT + CPAP, connected or unconnected**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

C. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

D. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: E · for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

Why this is best: In this context, the relevant notebook finding is **OAT + CPAP, connected or unconnected**: for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 79, §3.8*

Q0344 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Titratable**?

A. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

B. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

C. an intraoral protraction device; the 1980s work was the catalyst for the modern field

D. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

E. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

ANSWER: D · allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

Why this is best: Titratable: allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible The distractors correspond to other named concepts in this learning area (Cartwright & Samelson, 1982, Oral appliance, Meier-Ewert et al., 1985, Modern era), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0345 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **AHI reduction**?

A. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

D. CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: D · CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night

Why this is best: In this context, the relevant notebook finding is **AHI reduction**: CPAP ~75% · OA ~42% · UPPP ~30%. MDA: OA ~51% vs CPAP ~50%. MAD 6.5 vs CPAP 5.2 h/night The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 68, §3.11*

Q0346 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Heated humidification suggested**?

A. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

B. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

C. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

D. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

E. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

ANSWER: B · reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

Why this is best: Heated humidification suggested: reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance The distractors correspond to other named concepts in this learning area (Monitoring, Modality, Whom to treat, Titration), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0347 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Titratable**?

A. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

B. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

C. an intraoral protraction device; the 1980s work was the catalyst for the modern field

D. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

E. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

ANSWER: A · allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

Why this is best: In this context, the relevant notebook finding is **Titratable**: allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0348 §3.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on emerging therapies, which description is specifically associated with **Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

C. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

D. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

E. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

ANSWER: A · target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

Why this is best: Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin): target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation The distractors correspond to other named concepts in this learning area (GLP-1 receptor agonists (e.g. tirzepatide), Modafinil / armodafinil, Orofacial myofunctional therapy, Acetazolamide), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0349 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Heated humidification suggested**?

A. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

B. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

C. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

D. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

E. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

ANSWER: D · reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

Why this is best: In this context, the relevant notebook finding is **Heated humidification suggested**: reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0350 §3.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on combination therapies, which description is specifically associated with **OAT + positional therapy**?

A. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

B. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

C. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

D. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

E. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

ANSWER: E · CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

Why this is best: OAT + positional therapy: CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone. CPAP + modafinil for residual sleepiness. MAS + surgery; multilevel surgical combinations. Pharmacological — atomoxetine + oxybutynin; acetazolamide + eszopiclone; oxygen with a hypnotic. 3.9 Comparative efficacy The distractors correspond to other named concepts in this learning area (OAT and CPAP used interchangeably, OAT + nasal or oral EPAP, OAT + CPAP, connected or unconnected, OAT + CPAP (connected or not)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 66, §3.8*

Q0351 §3.7 · Clinical-context application

Clinical context. A clinician discusses novel or adjunctive therapies while clarifying their evidence and limitations. During this case discussion, how should the candidate interpret **Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)**?

A. target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

B. reduce both AHI and body weight in OSA with obesity, addressing metabolic and respiratory components together; the most significant recent pharmacological development

C. recommended for residual excessive sleepiness in patients who remain sleepy despite effective PAP and lack any other identifiable cause. Not a treatment for the OSA

D. carbonic anhydrase inhibitor stabilising ventilatory control in the high loop gain endotype

E. targeted exercises strengthening upper airway muscles; reduces snoring and AHI in mild– moderate OSA or as an adjunct

ANSWER: A · target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation

Why this is best: In this context, the relevant notebook finding is **Noradrenergic–antimuscarinic combinations (atomoxetine + oxybutynin)**: target poor muscle responsiveness; significant AHI reduction and improved sleep quality, particularly in combination; long-term safety and selection under investigation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 65, §3.7*

Q0352 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Effective OA definition**?

A. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

D. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: C · increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

Why this is best: Effective OA definition: increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting The distractors correspond to other named concepts in this learning area (Effective mandibular appliance adjustment, Weight, Weight, HNS selection), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0353 §3.8 · Clinical-context application

Clinical context. A partially responsive MAD user wants to reduce residual OSA with another modality. During this case discussion, how should the candidate interpret **OAT + positional therapy**?

A. for severe OSA, reduces AHI, ODI and required CPAP pressure versus either alone, and can improve PAP adherence by permitting a lower, more comfortable pressure

B. significantly reduces AHI in incomplete responders to OAT alone; larger AHI reduction than MAS alone, though sleep efficiency is lower with the combination

C. CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

D. improves overall effectiveness and adherence by increasing time on treatment while reducing the burden of CPAP use and of appliance side effects

E. for overall treatment effectiveness and adherence by severe OSA, reduces AHI, ODI and the required increasing the percentage of time on treatment, while

ANSWER: C · CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone

Why this is best: In this context, the relevant notebook finding is **OAT + positional therapy**: CPAP + weight loss — incremental reduction in insulin resistance, triglycerides and blood pressure versus either alone. CPAP + modafinil for residual sleepiness. MAS + surgery; multilevel surgical combinations. Pharmacological — atomoxetine + oxybutynin; acetazolamide + eszopiclone; oxygen with a hypnotic. 3.9 Comparative efficacy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 66, §3.8*

Q0354 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Oral appliance**?

A. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

B. an intraoral protraction device; the 1980s work was the catalyst for the modern field

C. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

D. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

E. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

ANSWER: D · a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

Why this is best: Oral appliance: a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep The distractors correspond to other named concepts in this learning area (Meier-Ewert et al., 1985, Cartwright & Samelson, 1982, Pierre Robin, 1903, Titratable), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0355 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Effective OA definition**?

A. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

D. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: D · increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

Why this is best: In this context, the relevant notebook finding is **Effective OA definition**: increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0356 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Modality**?

A. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

B. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

C. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

D. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

E. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

ANSWER: B · recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

Why this is best: Modality: recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change). Suggest CPAP or APAP over BPAP routinely — similar benefit but BPAP risks a sub-optimally low expiratory pressure failing to prevent obstructive events, plus higher cost. Reserve BPAP for the specific indications above The distractors correspond to other named concepts in this learning area (Heated humidification suggested, Whom to treat, Monitoring, Titration), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 63, §3.4*

Q0357 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Oral appliance**?

A. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

B. a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

C. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

D. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

E. an intraoral protraction device; the 1980s work was the catalyst for the modern field

ANSWER: B · a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep

Why this is best: In this context, the relevant notebook finding is **Oral appliance**: a device intended to protrude and stabilise the mandible to maintain a patent airway during sleep The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0358 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Minimum protrusive distance for MAD**?

A. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

B. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

C. increments ≤1 mm · range ≥5 mm · reversible · verifiable

D. 6 mm (limited protrusion <6 mm is a contraindication)

E. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

ANSWER: D · 6 mm (limited protrusion <6 mm is a contraindication)

Why this is best: Minimum protrusive distance for MAD: 6 mm (limited protrusion <6 mm is a contraindication) The distractors correspond to other named concepts in this learning area (Effective OA, HNS selection, Weight, Effective OA definition), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0359 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Modality**?

A. reduces dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice, reduced smell. Trade-offs: "rain-out", cost, maintenance

B. suggest telemonitoring-guided intervention during the initial period — remote monitoring of usage, residual severity, unintentional leak and settings, triggering early intervention

C. establish the minimum pressure at which snoring ceases and apnoeic events are absent. Start 4 cmH2O, titrate to the lowest effective pressure, maximum ~20 cmH2O

D. recommend PAP over no therapy in excessive sleepiness; suggest PAP in impaired sleep-related quality of life (assessed by SAQLI and FOSQ); suggest PAP in comorbid hypertension to reduce blood pressure

E. recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

ANSWER: E · recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change)

Why this is best: In this context, the relevant notebook finding is **Modality**: recommend either CPAP or APAP for ongoing treatment (no difference in adherence, sleepiness or quality of life; APAP's advantage is automatic adjustment for alcohol, position and weight change). Suggest CPAP or APAP over BPAP routinely — similar benefit but BPAP risks a sub-optimally low expiratory pressure failing to prevent obstructive events, plus higher cost. Reserve BPAP for the specific indications above The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 63, §3.4*

Q0360 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Modern era**?

A. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

B. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

C. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

D. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

E. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

ANSWER: A · focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

Why this is best: Modern era: focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA. Soft palate lifters obsolete; TRDs used only rarely. DEFINITIONS — AADSM TERMINOLOGY The distractors correspond to other named concepts in this learning area (Titratable, Cartwright & Samelson, 1982, George Catlin (19th c.), Custom OA), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0361 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Minimum protrusive distance for MAD**?

A. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

B. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

C. 6 mm (limited protrusion <6 mm is a contraindication)

D. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

E. increments ≤1 mm · range ≥5 mm · reversible · verifiable

ANSWER: C · 6 mm (limited protrusion <6 mm is a contraindication)

Why this is best: In this context, the relevant notebook finding is **Minimum protrusive distance for MAD**: 6 mm (limited protrusion <6 mm is a contraindication) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 82, §3.11*

Q0362 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Disadvantages**?

A. treatment must be based on a diagnosis established by objective sleep apnoea testing

B. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

C. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

D. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

E. Around four centimetres of water pressure

ANSWER: D · lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

Why this is best: Disadvantages: lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; similar health outcomes to a MAD because greater efficacy is offset by inferior compliance; not curative (must be used nightly); cost, noise, equipment maintenance and follow-up burden; loss of intimacy; difficult in retrognathia because of mask leakage The distractors correspond to other named concepts in this learning area (Good practice, Education and behaviour, Heated humidification is suggested to reduce side effects, Initial adult PAP titration pressure), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0363 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Modern era**?

A. first suggested breathing affects sleep quality: nasal rather than mouth breathing gives more restful, better quality sleep, better daytime function and better general health

B. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

C. focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

D. made from digital or physical impressions/models; not prefabricated; biocompatible material; engages both arches. Non-custom ("boil-and-bite") — prefabricated, only partially modified

E. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

ANSWER: C · focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA

Why this is best: In this context, the relevant notebook finding is **Modern era**: focus on mandibular advancement appliances; case reports gave way to RCTs confirming efficacy in mild– moderate and more recently severe OSA. Soft palate lifters obsolete; TRDs used only rarely. DEFINITIONS — AADSM TERMINOLOGY The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0364 §3.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on history of oat, which description is specifically associated with **Effective oral appliance (AADSM 2019)**?

A. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

B. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

C. an intraoral protraction device; the 1980s work was the catalyst for the modern field

D. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

E. advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

ANSWER: E · advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

Why this is best: Effective oral appliance (AADSM 2019): advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm; reversal possible; setting verifiable; suitable for placement and removal by patient or caregiver; maintains a stable retentive relationship to teeth, implants or edentulous ridge and retains the prescribed setting during use; FDA-cleared or equivalent. 3.2 Mechanism of action The distractors correspond to other named concepts in this learning area (Pierre Robin, 1903, Meier-Ewert et al., 1985, Cartwright & Samelson, 1982, Titratable), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0365 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Disadvantages**?

A. Around four centimetres of water pressure

B. dry mouth and throat, nasal discharge and congestion, dry or bleeding nose, sinus pain, headache, sore throat, hoarse voice and reduced smell

C. treatment must be based on a diagnosis established by objective sleep apnoea testing

D. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

E. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

ANSWER: D · lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

Why this is best: In this context, the relevant notebook finding is **Disadvantages**: lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%; similar health outcomes to a MAD because greater efficacy is offset by inferior compliance; not curative (must be used nightly); cost, noise, equipment maintenance and follow-up burden; loss of intimacy; difficult in retrognathia because of mask leakage The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0366 §3.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on pap therapies, which description is specifically associated with **Advantages**?

A. treatment must be based on a diagnosis established by objective sleep apnoea testing

B. Around four centimetres of water pressure

C. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

D. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

E. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

ANSWER: D · more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

Why this is best: Advantages: more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation; prevents collapse and relieves daytime sleepiness; favourable cardiovascular outcomes, with significant reduction in nocturnal, daytime and 24-h blood pressure (largest effect nocturnally); decreases pain sensitivity and increases pain tolerance; almost 100% successful if tolerated The distractors correspond to other named concepts in this learning area (Good practice, Disadvantages, Initial adult PAP titration pressure, Education and behaviour), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0367 §3.1 · Clinical-context application

Clinical context. A dental sleep medicine trainee is discussing the origins and development of appliance-based therapy. During this case discussion, how should the candidate interpret **Effective oral appliance (AADSM 2019)**?

A. an intraoral protraction device; the 1980s work was the catalyst for the modern field

B. the monobloc, for glossoptosis. ~30 years later used to reposition the mandible

C. advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

D. the tongue retaining device, holding the tongue forward by suction; historically the second most common type

E. allows varying protrusion (the analogue of CPAP titration). Non-titratable — single position, no change possible

ANSWER: C · advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm

Why this is best: In this context, the relevant notebook finding is **Effective oral appliance (AADSM 2019)**: advancement in increments of 1 mm or less; protrusive adjustment range ≥5 mm; reversal possible; setting verifiable; suitable for placement and removal by patient or caregiver; maintains a stable retentive relationship to teeth, implants or edentulous ridge and retains the prescribed setting during use; FDA-cleared or equivalent. 3.2 Mechanism of action The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 61, §3.1*

Q0368 §3.11 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 3**, in the revision material on guidelines and evidence-based care, which description is specifically associated with **Effective mandibular appliance adjustment**?

A. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

B. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

C. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

D. At most one-millimetre increments across a range of at least five millimetres

E. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

ANSWER: D · At most one-millimetre increments across a range of at least five millimetres

Why this is best: Effective mandibular appliance adjustment: The setting must be reversible, verifiable, retentive and removable by the user. The distractors correspond to other named concepts in this learning area (Effective OA definition, HNS selection, Weight, Weight), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 62, §3.11*

Q0369 §3.4 · Clinical-context application

Clinical context. A patient with severe OSA is reviewing PAP choices, comfort and adherence barriers. During this case discussion, how should the candidate interpret **Advantages**?

A. recommend educational interventions before initiation (what OSA is, its consequences, what PAP is, potential benefits)

B. more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

C. Around four centimetres of water pressure

D. treatment must be based on a diagnosis established by objective sleep apnoea testing

E. lower compliance and tolerance, especially in mild OSA — 40–60% at 12 months, failure rates 29–83%

ANSWER: B · more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation

Why this is best: In this context, the relevant notebook finding is **Advantages**: more efficacious than a MAD for AHI, arousal index, ODI and oxygen saturation; prevents collapse and relieves daytime sleepiness; favourable cardiovascular outcomes, with significant reduction in nocturnal, daytime and 24-h blood pressure (largest effect nocturnally); decreases pain sensitivity and increases pain tolerance; almost 100% successful if tolerated The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.4*

Q0370 §3.11 · Clinical-context application

Clinical context. At a case conference, a dentist must justify treatment choice under the published OAT guidance. During this case discussion, how should the candidate interpret **Effective mandibular appliance adjustment**?

A. increments ≤1 mm · adjustment range ≥5 mm · reversible · verifiable setting

B. 10% gain → +32% AHI; 10% loss → −26% AHI; BMI target <30; bariatric threshold BMI ≥35

C. BMI <35, AHI 20–50, non-concentric retropalatal collapse; adherence ~86%

D. 10% gain → +32% AHI; 10% loss → −26%; BMI target <30; bariatric BMI ≥35

E. At most one-millimetre increments across a range of at least five millimetres

ANSWER: E · At most one-millimetre increments across a range of at least five millimetres

Why this is best: In this context, the relevant notebook finding is **Effective mandibular appliance adjustment**: The setting must be reversible, verifiable, retentive and removable by the user. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 62, §3.11*

TOPIC 04
History, dental examination, imaging and patient selection

100 questions | Full notebook pages 86–105 | Each question has a justification and linked learning code.

Q0371 §4.4 · Integrated clinical vignette

A patient with OSA has resistant hypertension and type 2 diabetes. Which interpretation is most appropriate?

A. They establish the AHI without testing

B. They are unrelated to OSA burden

C. These are recognised comorbidities relevant to risk and medical coordination

D. They prove that oral appliance treatment is contraindicated

E. They remove the need to assess cardiovascular risk

ANSWER: C · These are recognised comorbidities relevant to risk and medical coordination

Why this is best: OSA is associated with cardiovascular and metabolic disease. Documentation and communication with the medical team matter.

Notebook anchor: *Integrated notebook, Topic 4, §4.4*

Q0372 §4.1 · All-of-the-above integrated judgement

For a new MAD candidate with OSA and possible TMD, which is the MOST COMPLETE baseline assessment?

A. Dental and periodontal support

B. Occlusion and range of jaw movement

C. Medical, sleep and drug history

D. Patient goals and physician correspondence

E. All of the above

ANSWER: E · All of the above

Why this is best: A good baseline dental sleep medicine assessment integrates oral suitability, comorbid factors, expectations and medical shared care.

Notebook anchor: *Integrated notebook, Topic 4, §4.1*

Q0373 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Teeth and restorations**?

A. attachment loss, mobility, prognosis of the teeth that will bear the appliance

B. Occlusion — examine and record the existing condition; this is your medicolegal baseline

C. sleep hygiene, sleeping position, weight control, alcohol intake

D. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

E. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

ANSWER: E · number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

Why this is best: In this context, the relevant notebook finding is **Teeth and restorations**: number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0374 §4.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical and lifestyle modifiers, which description is specifically associated with **Obesity**?

A. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

B. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

C. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

D. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

E. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

ANSWER: D · increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

Why this is best: Obesity: increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management The distractors correspond to other named concepts in this learning area (Alcohol, Drugs and medications, Smoking, Other sleep disorders and comorbidities), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0375 §4.1 · None-of-the-above discrimination

A patient with bruxism and snoring wants a new splint. Which single finding proves sleep apnoea?

A. Tooth wear

B. Masseter tenderness

C. Morning headache

D. TMD clicking

E. None of the above

ANSWER: E · None of the above

Why this is best: These are possible associated findings, not a stand-alone diagnosis or severity measure for OSA.

Notebook anchor: *Integrated notebook, Topic 4, §4.1*

Q0376 §4.1 · Integrated clinical vignette

A patient requests a mouthguard for severe tooth wear and also reports gasping during sleep. What should be addressed before choosing the splint?

A. Prescribe a flat-plane splint without considering breathing

B. Advise that occlusal protection cures OSA

C. Assume all grinding is due to OSA

D. Screen for OSA and refer for physician evaluation when indicated

E. Diagnose sleep apnoea from tooth wear alone

ANSWER: D · Screen for OSA and refer for physician evaluation when indicated

Why this is best: Sleep bruxism and OSA can coexist but causation is unproven; some stabilisation splints may worsen airway obstruction, so breathing symptoms require assessment.

Notebook anchor: *Integrated notebook, Topic 4, §4.1*

Q0377 §4.2 · Clinical-context application

Clinical context. A snoring patient has obesity, a complex medicine list and evening alcohol use. During this case discussion, how should the candidate interpret **Obesity**?

A. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

B. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

C. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

D. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

E. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

ANSWER: E · increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

Why this is best: In this context, the relevant notebook finding is **Obesity**: increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0378 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **The relationship appears mutual rather than cause-and-effect**?

A. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

B. (i) fragmentation and hypoxia as a cause of depression; (ii) serotonin — abnormalities in central and peripheral serotonergic neurotransmission are a potential factor in depression, and serotonin also affects upper airway…

C. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

D. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

E. The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to…

ANSWER: E · The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to…

Why this is best: The relationship appears mutual rather than cause-and-effect: The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to both — insulin resistance, for example, contributes to depression as it does in OSA The distractors correspond to other named concepts in this learning area (Proposed mechanisms, Mood disorders, Insufficient sleep and shift work disorder, RLS and PLMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 95, §4.3*

Q0379 §4.1 · Integrated clinical vignette

Before a MAD is delivered, several posterior teeth show mobility and untreated periodontitis. What is the priority?

A. Use no baseline dental records

B. Treat periodontal disease and assess whether retention is safe

C. Measure only neck circumference

D. Ignore mobility once PSG is positive

E. Advance to maximum because loose teeth increase comfort

ANSWER: B · Treat periodontal disease and assess whether retention is safe

Why this is best: A device loads supporting teeth: periodontal health, mobility and retention are central suitability criteria.

Notebook anchor: *Integrated notebook, Topic 4, §4.1*

Q0380 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **The relationship appears mutual rather than cause-and-effect**?

A. (i) fragmentation and hypoxia as a cause of depression; (ii) serotonin — abnormalities in central and peripheral serotonergic neurotransmission are a potential factor in depression, and serotonin also affects upper airway…

B. The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to…

C. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

D. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

E. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

ANSWER: B · The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to…

Why this is best: In this context, the relevant notebook finding is **The relationship appears mutual rather than cause-and-effect**: The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate to both — insulin resistance, for example, contributes to depression as it does in OSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 95, §4.3*

Q0381 §4.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical comorbidities, which description is specifically associated with **Depression, anxiety, neurocognitive decline**?

A. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

D. Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

E. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

ANSWER: D · Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

Why this is best: Depression, anxiety, neurocognitive decline: Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels The distractors correspond to other named concepts in this learning area (Metabolic syndrome and diabetes, Insufficient sleep and shift work disorder, Mood disorders, RLS and PLMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 88, §4.4*

Q0382 §4.1 · Integrated clinical vignette

A clinician proposes CBCT solely to calculate a static airway volume before a MAD. Which response best matches the notes?

A. Cephalometry replaces physician diagnosis

B. Imaging has no radiation considerations

C. CBCT is mandatory for every OSA appliance

D. A static airway slice proves disease severity

E. Routine CBCT is not justified by airway-volume measurement alone

ANSWER: E · Routine CBCT is not justified by airway-volume measurement alone

Why this is best: Upper-airway obstruction is dynamic; static CBCT airway measurements are limited and radiation must be justified by clinical need.

Notebook anchor: *Integrated notebook, Topic 4, §4.1*

Q0383 §4.4 · Clinical-context application

Clinical context. A person with OSA also has metabolic and cardiovascular risk factors. During this case discussion, how should the candidate interpret **Depression, anxiety, neurocognitive decline**?

A. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

B. Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

C. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

D. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

E. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

ANSWER: B · Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

Why this is best: In this context, the relevant notebook finding is **Depression, anxiety, neurocognitive decline**: Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 88, §4.4*

Q0384 §4.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on ethics and patient consent, which description is specifically associated with **Informed dental treatment choice**?

A. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

B. Align the outcome with the patient and the physician, including symptom and objective disease endpoints

C. Explain risks, benefits, alternatives including no treatment, and expected objective verification

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. written professional communication between GP, sleep physician and dentist

ANSWER: C · Explain risks, benefits, alternatives including no treatment, and expected objective verification

Why this is best: Informed dental treatment choice: Consent and shared decision-making must precede appliance fitting. The distractors correspond to other named concepts in this learning area (Patient-specific goals, Borderline cases, Shared, Prevalence of TMD in OSA populations), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.5*

Q0385 §4.6 · Integrated clinical vignette

A patient presents with a diagnostic PSG but no physician treatment communication. What should the dentist obtain?

A. Medical report, treatment recommendation and a shared-care plan

B. A verbal diagnosis from the patient alone

C. Only a panoramic radiograph

D. An appliance prescription generated by the receptionist

E. Permission to bypass efficacy testing

ANSWER: A · Medical report, treatment recommendation and a shared-care plan

Why this is best: Shared care requires documented physician diagnosis, treatment context, subsequent objective verification and two-way communication.

Notebook anchor: *Integrated notebook, Topic 4, §4.6*

Q0386 §4.5 · Clinical-context application

Clinical context. A patient asks for a mandibular device but remains unsure of the benefits and irreversible dental effects. During this case discussion, how should the candidate interpret **Informed dental treatment choice**?

A. Align the outcome with the patient and the physician, including symptom and objective disease endpoints

B. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

C. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

D. Explain risks, benefits, alternatives including no treatment, and expected objective verification

E. written professional communication between GP, sleep physician and dentist

ANSWER: D · Explain risks, benefits, alternatives including no treatment, and expected objective verification

Why this is best: In this context, the relevant notebook finding is **Informed dental treatment choice**: Consent and shared decision-making must precede appliance fitting. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.5*

Q0387 §4.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on multidisciplinary communication, which description is specifically associated with **Severity and outcome in the shared-care setting**?

A. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

B. 6–91%, depending on assessment method

C. Adult mild 5–14 · moderate 15–30 · severe >30

D. written professional communication between GP, sleep physician and dentist

E. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

ANSWER: C · Adult mild 5–14 · moderate 15–30 · severe >30

Why this is best: Severity and outcome in the shared-care setting: Adult mild 5–14 · moderate 15–30 · severe >30. Children mild 1–5 · moderate 5–10 · severe >10. Successful outcome = reduction of AHI, respiratory disturbance index and respiratory event index to <5 and a >50% reduction in these indices. 4.7 Sleep bruxism and SDB DEFINITIONS — VERBATIM The distractors correspond to other named concepts in this learning area (Shared, Borderline cases, Portable monitor levels, Wide reported range), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.6*

Q0388 §4.7 · Integrated clinical vignette

A patient with both sleep bruxism and OSA asks whether one causes the other. What is the best answer?

A. Both are defined by the same PSG event

B. They are associated, but a causal relationship has not been established

C. The conditions cannot coexist

D. Bruxism invariably causes OSA

E. OSA invariably causes all bruxism

ANSWER: B · They are associated, but a causal relationship has not been established

Why this is best: The notebook stresses distinguishing association from causation; arousal-related mechanisms are proposed but not proof of directionality.

Notebook anchor: *Integrated notebook, Topic 4, §4.7*

Q0389 §4.6 · Clinical-context application

Clinical context. A dentist and sleep physician coordinate a complex patient's management pathway. During this case discussion, how should the candidate interpret **Severity and outcome in the shared-care setting**?

A. written professional communication between GP, sleep physician and dentist

B. Adult mild 5–14 · moderate 15–30 · severe >30

C. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

D. 6–91%, depending on assessment method

E. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

ANSWER: B · Adult mild 5–14 · moderate 15–30 · severe >30

Why this is best: In this context, the relevant notebook finding is **Severity and outcome in the shared-care setting**: Adult mild 5–14 · moderate 15–30 · severe >30. Children mild 1–5 · moderate 5–10 · severe >10. Successful outcome = reduction of AHI, respiratory disturbance index and respiratory event index to <5 and a >50% reduction in these indices. 4.7 Sleep bruxism and SDB DEFINITIONS — VERBATIM The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.6*

Q0390 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Wide reported range**?

A. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

B. ~8% of the adult population

C. both commoner supine; both involve oropharyngeal and masticatory muscle activation

D. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

E. 6–91%, depending on assessment method

ANSWER: E · 6–91%, depending on assessment method

Why this is best: Wide reported range: 6–91%, depending on assessment method The distractors correspond to other named concepts in this learning area (Self-reported sleep bruxism, Shared clinical features, Prevalence of TMD in OSA populations, Corollary), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 99, §4.7*

Q0391 §4.8 · Integrated clinical vignette

A patient with TMD is being considered for oral appliance therapy. Which assessment is essential?

A. Assume successful AHI reduction eliminates TMD

B. Record baseline pain, joint function, movements and clinical findings

C. Avoid asking about pain

D. Skip occlusal examination

E. Treat all previous disc displacements as absolute exclusions

ANSWER: B · Record baseline pain, joint function, movements and clinical findings

Why this is best: TMD is not invariably an absolute contraindication, but active symptoms and baseline function influence risk, counselling and monitoring.

Notebook anchor: *Integrated notebook, Topic 4, §4.8*

Q0392 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Wide reported range**?

A. both commoner supine; both involve oropharyngeal and masticatory muscle activation

B. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

C. 6–91%, depending on assessment method

D. ~8% of the adult population

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

ANSWER: C · 6–91%, depending on assessment method

Why this is best: In this context, the relevant notebook finding is **Wide reported range**: 6–91%, depending on assessment method The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 99, §4.7*

Q0393 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Fibromyalgia (ACR 1990)**?

A. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

B. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

C. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

D. AHI/RDI/REI <5 and >50% reduction

E. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

ANSWER: C · pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

Why this is best: Fibromyalgia (ACR 1990): pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5% The distractors correspond to other named concepts in this learning area (Conversely, SB prevalence, Bidirectional and cyclical, Successful treatment outcome), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 91, §4.8*

Q0394 §4.2 · Integrated clinical vignette

A sleep patient takes sedatives, drinks alcohol at bedtime and has a BMI of 36. Which medical-history interpretation is best?

A. Alcohol reduces pharyngeal collapse

B. BMI does not matter after fitting

C. These factors guarantee oral appliance success

D. Multiple modifiable factors may increase airway collapsibility and reduce treatment response

E. The dentist should independently stop prescribed medicines

ANSWER: D · Multiple modifiable factors may increase airway collapsibility and reduce treatment response

Why this is best: These are relevant risk modifiers to document and address in coordination with the prescriber and sleep physician.

Notebook anchor: *Integrated notebook, Topic 4, §4.2*

Q0395 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Fibromyalgia (ACR 1990)**?

A. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

B. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

C. AHI/RDI/REI <5 and >50% reduction

D. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

E. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

ANSWER: B · pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

Why this is best: In this context, the relevant notebook finding is **Fibromyalgia (ACR 1990)**: pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 91, §4.8*

Q0396 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Periodontal evaluation**?

A. attachment loss, mobility, prognosis of the teeth that will bear the appliance

B. age, weight, height; neck and belt circumference as indicated

C. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

D. sleep hygiene, sleeping position, weight control, alcohol intake

E. Occlusion — examine and record the existing condition; this is your medicolegal baseline

ANSWER: A · attachment loss, mobility, prognosis of the teeth that will bear the appliance

Why this is best: Periodontal evaluation: attachment loss, mobility, prognosis of the teeth that will bear the appliance The distractors correspond to other named concepts in this learning area (Bruxism, orofacial pain, headaches, Provide explanation and written information on sleep behavioural therapy, Medical–dental history, Teeth and restorations), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0397 §4.3 · Integrated clinical vignette

A person treated for OSA continues to have unrefreshing sleep and an urge to move the legs at night. What should be considered?

A. Leg movements diagnose OSA

B. ESS cannot be used in follow-up

C. All sleep disorders are corrected by mandibular advancement

D. Coexisting restless legs syndrome needs its own assessment

E. Persisting symptoms prove appliance fracture

ANSWER: D · Coexisting restless legs syndrome needs its own assessment

Why this is best: SDB may coexist with RLS/PLMD and other sleep disorders; symptom persistence should not automatically trigger further protrusion.

Notebook anchor: *Integrated notebook, Topic 4, §4.3*

Q0398 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Periodontal evaluation**?

A. age, weight, height; neck and belt circumference as indicated

B. sleep hygiene, sleeping position, weight control, alcohol intake

C. Occlusion — examine and record the existing condition; this is your medicolegal baseline

D. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

E. attachment loss, mobility, prognosis of the teeth that will bear the appliance

ANSWER: E · attachment loss, mobility, prognosis of the teeth that will bear the appliance

Why this is best: In this context, the relevant notebook finding is **Periodontal evaluation**: attachment loss, mobility, prognosis of the teeth that will bear the appliance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0399 §4.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical and lifestyle modifiers, which description is specifically associated with **Drugs and medications**?

A. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

B. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

C. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

D. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

E. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

ANSWER: D · anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

Why this is best: Drugs and medications: anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively. Also drug classes altering sleep architecture and inducing bruxism or RLS (1.12) The distractors correspond to other named concepts in this learning area (Alcohol, Obesity, Smoking, Other sleep disorders and comorbidities), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0400 §4.5 · Integrated clinical vignette

A patient values minimal dental side effects over complete numerical normalisation. What is the most appropriate planning approach?

A. Promise AHI zero in every case

B. Ignore the patient's goals

C. Shared goal setting with clear discussion of risks and physician-agreed success criteria

D. Define success exclusively by loudness of snoring

E. Avoid describing alternatives

ANSWER: C · Shared goal setting with clear discussion of risks and physician-agreed success criteria

Why this is best: Treatment aims should reflect realistic efficacy, patient values, tolerability and medical verification of remaining disease.

Notebook anchor: *Integrated notebook, Topic 4, §4.5*

Q0401 §4.2 · Clinical-context application

Clinical context. A snoring patient has obesity, a complex medicine list and evening alcohol use. During this case discussion, how should the candidate interpret **Drugs and medications**?

A. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

B. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

C. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

D. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

E. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

ANSWER: D · anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

Why this is best: In this context, the relevant notebook finding is **Drugs and medications**: anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively. Also drug classes altering sleep architecture and inducing bruxism or RLS (1.12) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0402 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **Narcolepsy**?

A. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

B. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

C. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

D. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

E. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

ANSWER: A · May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

Why this is best: Narcolepsy: May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed. SOREMPs on the MSLT — a narcolepsy criterion — are also frequent in untreated OSA The distractors correspond to other named concepts in this learning area (RLS and PLMD, Insufficient sleep and shift work disorder, CPAP's effect on depression is not conclusive, Insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0403 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **Narcolepsy**?

A. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

B. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

C. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

D. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

E. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

ANSWER: E · May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

Why this is best: In this context, the relevant notebook finding is **Narcolepsy**: May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed. SOREMPs on the MSLT — a narcolepsy criterion — are also frequent in untreated OSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0404 §4.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical comorbidities, which description is specifically associated with **Metabolic syndrome and diabetes**?

A. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

B. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

C. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

D. Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

E. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

ANSWER: E · Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

Why this is best: Metabolic syndrome and diabetes: Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk. Intermittent hypoxia and fragmentation contribute to insulin resistance, dyslipidaemia and systemic inflammation The distractors correspond to other named concepts in this learning area (Depression, anxiety, neurocognitive decline, Narcolepsy, RLS and PLMD, Insufficient sleep and shift work disorder), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.4*

Q0405 §4.4 · Clinical-context application

Clinical context. A person with OSA also has metabolic and cardiovascular risk factors. During this case discussion, how should the candidate interpret **Metabolic syndrome and diabetes**?

A. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

D. Higher rates particularly in women and post-menopausal populations. Neurocognitive decline — executive function, attention, memory — in adults and children, even at mild disease levels

E. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

ANSWER: C · Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk

Why this is best: In this context, the relevant notebook finding is **Metabolic syndrome and diabetes**: Hypertension, diabetes and metabolic syndrome are independently associated with moderate– severe SDB, and coexistence with obesity further increases risk. Intermittent hypoxia and fragmentation contribute to insulin resistance, dyslipidaemia and systemic inflammation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.4*

Q0406 §4.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on ethics and patient consent, which description is specifically associated with **Patient-specific goals**?

A. Align the outcome with the patient and the physician, including symptom and objective disease endpoints

B. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

C. written professional communication between GP, sleep physician and dentist

D. Explain risks, benefits, alternatives including no treatment, and expected objective verification

E. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

ANSWER: A · Align the outcome with the patient and the physician, including symptom and objective disease endpoints

Why this is best: Patient-specific goals: Clinical success can balance quality of life, objective disease burden and tolerability. The distractors correspond to other named concepts in this learning area (Informed dental treatment choice, Borderline cases, Shared, Portable monitor levels), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.5*

Q0407 §4.5 · Clinical-context application

Clinical context. A patient asks for a mandibular device but remains unsure of the benefits and irreversible dental effects. During this case discussion, how should the candidate interpret **Patient-specific goals**?

A. written professional communication between GP, sleep physician and dentist

B. Explain risks, benefits, alternatives including no treatment, and expected objective verification

C. Align the outcome with the patient and the physician, including symptom and objective disease endpoints

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

ANSWER: C · Align the outcome with the patient and the physician, including symptom and objective disease endpoints

Why this is best: In this context, the relevant notebook finding is **Patient-specific goals**: Clinical success can balance quality of life, objective disease burden and tolerability. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.5*

Q0408 §4.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on multidisciplinary communication, which description is specifically associated with **Portable monitor levels**?

A. Adult mild 5–14 · moderate 15–30 · severe >30

B. written professional communication between GP, sleep physician and dentist

C. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

ANSWER: E · Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

Why this is best: Portable monitor levels: Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation. Level 4 = 1–2 bioparameters, oxygen saturation or airflow. Both should be interpreted by a physician. The dentist assesses the result and observes change in respiratory sleep parameters with treatment; the dentist does not use these data for diagnosis or as a formal final follow-up assessment The distractors correspond to other named concepts in this learning area (Borderline cases, Shared, Severity and outcome in the shared-care setting, SB + TMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0409 §4.6 · Clinical-context application

Clinical context. A dentist and sleep physician coordinate a complex patient's management pathway. During this case discussion, how should the candidate interpret **Portable monitor levels**?

A. written professional communication between GP, sleep physician and dentist

B. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

C. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. Adult mild 5–14 · moderate 15–30 · severe >30

ANSWER: B · Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

Why this is best: In this context, the relevant notebook finding is **Portable monitor levels**: Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation. Level 4 = 1–2 bioparameters, oxygen saturation or airflow. Both should be interpreted by a physician. The dentist assesses the result and observes change in respiratory sleep parameters with treatment; the dentist does not use these data for diagnosis or as a formal final follow-up assessment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0410 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Corollary**?

A. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

B. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

C. both commoner supine; both involve oropharyngeal and masticatory muscle activation

D. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

E. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

ANSWER: A · SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

Why this is best: Corollary: SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences. Classified by consequence as: not a risk or protective factor (harmless); a risk factor (≥1 negative outcome); a protective factor (≥1 positive outcome). Also primary or secondary The distractors correspond to other named concepts in this learning area (SB + TMD, Prevalence of TMD in OSA populations, Shared clinical features, Assessment grading), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0411 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Corollary**?

A. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

B. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

C. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

D. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

E. both commoner supine; both involve oropharyngeal and masticatory muscle activation

ANSWER: B · SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

Why this is best: In this context, the relevant notebook finding is **Corollary**: SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences. Classified by consequence as: not a risk or protective factor (harmless); a risk factor (≥1 negative outcome); a protective factor (≥1 positive outcome). Also primary or secondary The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0412 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Bruxism, orofacial pain, headaches**?

A. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

B. attachment loss, mobility, prognosis of the teeth that will bear the appliance

C. sleep hygiene, sleeping position, weight control, alcohol intake

D. Occlusion — examine and record the existing condition; this is your medicolegal baseline

E. age, weight, height; neck and belt circumference as indicated

ANSWER: D · Occlusion — examine and record the existing condition; this is your medicolegal baseline

Why this is best: Bruxism, orofacial pain, headaches: Occlusion — examine and record the existing condition; this is your medicolegal baseline The distractors correspond to other named concepts in this learning area (Periodontal evaluation, Provide explanation and written information on sleep behavioural therapy, Teeth and restorations, Medical–dental history), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0413 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Bruxism, orofacial pain, headaches**?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline

B. sleep hygiene, sleeping position, weight control, alcohol intake

C. attachment loss, mobility, prognosis of the teeth that will bear the appliance

D. age, weight, height; neck and belt circumference as indicated

E. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

ANSWER: A · Occlusion — examine and record the existing condition; this is your medicolegal baseline

Why this is best: In this context, the relevant notebook finding is **Bruxism, orofacial pain, headaches**: Occlusion — examine and record the existing condition; this is your medicolegal baseline The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0414 §4.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical and lifestyle modifiers, which description is specifically associated with **Smoking**?

A. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

B. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

C. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

D. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

E. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

ANSWER: E · chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

Why this is best: Smoking: chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality. Passive smoking and family smoking history increase perioperative respiratory adverse events in children The distractors correspond to other named concepts in this learning area (Other sleep disorders and comorbidities, Obesity, Alcohol, Drugs and medications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0415 §4.2 · Clinical-context application

Clinical context. A snoring patient has obesity, a complex medicine list and evening alcohol use. During this case discussion, how should the candidate interpret **Smoking**?

A. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

B. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

C. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

D. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

E. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

ANSWER: C · chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

Why this is best: In this context, the relevant notebook finding is **Smoking**: chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality. Passive smoking and family smoking history increase perioperative respiratory adverse events in children The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0416 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **Symptoms of OSA and depression overlap**?

A. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

B. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

C. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

D. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

E. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

ANSWER: B · tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

Why this is best: Symptoms of OSA and depression overlap: tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each. OSA may be responsible for antidepressant treatment failure, and depression is a risk factor for an incomplete response to CPAP, so it may need to be independently addressed The distractors correspond to other named concepts in this learning area (Insomnia, CPAP's effect on depression is not conclusive, Narcolepsy, RLS and PLMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 95, §4.3*

Q0417 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **Symptoms of OSA and depression overlap**?

A. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

B. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

C. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

D. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

E. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

ANSWER: E · tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

Why this is best: In this context, the relevant notebook finding is **Symptoms of OSA and depression overlap**: tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each. OSA may be responsible for antidepressant treatment failure, and depression is a risk factor for an incomplete response to CPAP, so it may need to be independently addressed The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 95, §4.3*

Q0418 §4.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on multidisciplinary communication, which description is specifically associated with **Shared**?

A. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

B. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

C. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

D. Adult mild 5–14 · moderate 15–30 · severe >30

E. written professional communication between GP, sleep physician and dentist

ANSWER: E · written professional communication between GP, sleep physician and dentist

Why this is best: Shared: written professional communication between GP, sleep physician and dentist; a written report to the referring practitioner detailing dental treatment and symptomatic feedback; a systematic follow-up programme agreed between practitioners; onward referral to periodontics, orthodontics or prosthodontics for complications and coincidental conditions The distractors correspond to other named concepts in this learning area (Borderline cases, Severity and outcome in the shared-care setting, Portable monitor levels, Prevalence of TMD in OSA populations), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0419 §4.6 · Clinical-context application

Clinical context. A dentist and sleep physician coordinate a complex patient's management pathway. During this case discussion, how should the candidate interpret **Shared**?

A. Adult mild 5–14 · moderate 15–30 · severe >30

B. written professional communication between GP, sleep physician and dentist

C. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

D. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

E. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

ANSWER: B · written professional communication between GP, sleep physician and dentist

Why this is best: In this context, the relevant notebook finding is **Shared**: written professional communication between GP, sleep physician and dentist; a written report to the referring practitioner detailing dental treatment and symptomatic feedback; a systematic follow-up programme agreed between practitioners; onward referral to periodontics, orthodontics or prosthodontics for complications and coincidental conditions The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0420 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Sleep bruxism**?

A. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

B. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

C. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

D. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

E. a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

ANSWER: E · a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

Why this is best: Sleep bruxism: a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals The distractors correspond to other named concepts in this learning area (The "reactive–protective" hypothesis, Prevalence, Assessment grading, SB + TMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0421 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Sleep bruxism**?

A. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

B. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

C. a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

D. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

E. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

ANSWER: C · a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

Why this is best: In this context, the relevant notebook finding is **Sleep bruxism**: a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0422 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Conversely**?

A. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

B. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

C. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

D. AHI/RDI/REI <5 and >50% reduction

E. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

ANSWER: B · hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

Why this is best: Conversely: hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation; spontaneous muscle pain and headache develop or worsen; pain tolerance falls with increased fatigue and more severe subjective pain; the antinociceptive potency of morphine falls The distractors correspond to other named concepts in this learning area (Fibromyalgia (ACR 1990), SB prevalence, Fibromyalgia (ACR 1990), Successful outcome), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0423 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Conversely**?

A. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

B. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

C. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

D. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

E. AHI/RDI/REI <5 and >50% reduction

ANSWER: B · hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

Why this is best: In this context, the relevant notebook finding is **Conversely**: hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation; spontaneous muscle pain and headache develop or worsen; pain tolerance falls with increased fatigue and more severe subjective pain; the antinociceptive potency of morphine falls The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0424 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Imaging**?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline

B. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

C. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

D. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

E. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

ANSWER: D · initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

Why this is best: Imaging: initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage The distractors correspond to other named concepts in this learning area (Soft tissue/intraoral, Teeth and restorations, Reason for attendance, Bruxism, orofacial pain, headaches), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0425 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Imaging**?

A. Occlusion — examine and record the existing condition; this is your medicolegal baseline

B. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

C. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

D. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

E. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

ANSWER: B · initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

Why this is best: In this context, the relevant notebook finding is **Imaging**: initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0426 §4.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical and lifestyle modifiers, which description is specifically associated with **Alcohol**?

A. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

B. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

C. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

D. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

E. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

ANSWER: C · impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

Why this is best: Alcohol: impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use The distractors correspond to other named concepts in this learning area (Drugs and medications, Obesity, Smoking, Other sleep disorders and comorbidities), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0427 §4.2 · Clinical-context application

Clinical context. A snoring patient has obesity, a complex medicine list and evening alcohol use. During this case discussion, how should the candidate interpret **Alcohol**?

A. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

B. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

C. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

D. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

E. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

ANSWER: E · impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

Why this is best: In this context, the relevant notebook finding is **Alcohol**: impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0428 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **Insomnia**?

A. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

B. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

C. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

D. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

E. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

ANSWER: E · COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

Why this is best: Insomnia: COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life. Both must be treated — reducing AHI alone will not help. CBT-I plus SDB treatment The distractors correspond to other named concepts in this learning area (CPAP's effect on depression is not conclusive, Symptoms of OSA and depression overlap, Narcolepsy, RLS and PLMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0429 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **Insomnia**?

A. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

D. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

E. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

ANSWER: E · COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

Why this is best: In this context, the relevant notebook finding is **Insomnia**: COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life. Both must be treated — reducing AHI alone will not help. CBT-I plus SDB treatment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0430 §4.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on multidisciplinary communication, which description is specifically associated with **Borderline cases**?

A. Adult mild 5–14 · moderate 15–30 · severe >30

B. written professional communication between GP, sleep physician and dentist

C. Explain risks, benefits, alternatives including no treatment, and expected objective verification

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

ANSWER: D · Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

Why this is best: Borderline cases: Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance. Sleepiness with suspected OSA → refer. Sleep bruxism with orofacial pain or TMD → may be diagnosed by the dentist — but if accompanied by sleepiness, snoring or sleep apnoea, a sleep physician is required and the patient should consult a professional trained in those fields The distractors correspond to other named concepts in this learning area (Shared, Portable monitor levels, Severity and outcome in the shared-care setting, Informed dental treatment choice), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0431 §4.6 · Clinical-context application

Clinical context. A dentist and sleep physician coordinate a complex patient's management pathway. During this case discussion, how should the candidate interpret **Borderline cases**?

A. written professional communication between GP, sleep physician and dentist

B. Level 3 = ≥4 channels including airflow (≥2 channels of respiratory movement, or movement and airflow), heart rate/ECG, oxygen saturation

C. Explain risks, benefits, alternatives including no treatment, and expected objective verification

D. Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

E. Adult mild 5–14 · moderate 15–30 · severe >30

ANSWER: D · Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance

Why this is best: In this context, the relevant notebook finding is **Borderline cases**: Primary snoring without sleepiness → still refer to exclude OSA before providing an appliance. Sleepiness with suspected OSA → refer. Sleep bruxism with orofacial pain or TMD → may be diagnosed by the dentist — but if accompanied by sleepiness, snoring or sleep apnoea, a sleep physician is required and the patient should consult a professional trained in those fields The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 88, §4.6*

Q0432 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **The "reactive–protective" hypothesis**?

A. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

B. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

C. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

D. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

E. a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

ANSWER: C · Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

Why this is best: The "reactive–protective" hypothesis: Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure. Supporting: muscle tone is inversely related to The distractors correspond to other named concepts in this learning area (Prevalence, Assessment grading, SB + TMD, Sleep bruxism), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0433 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **The "reactive–protective" hypothesis**?

A. a masticatory muscle activity during sleep, characterised as rhythmic (phasic — grinding) or non-rhythmic (tonic — clenching), and not a movement disorder or a sleep disorder in otherwise healthy individuals

B. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

C. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

D. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

E. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

ANSWER: C · Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

Why this is best: In this context, the relevant notebook finding is **The "reactive–protective" hypothesis**: Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure. Supporting: muscle tone is inversely related to The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0434 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Implications**?

A. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

B. dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

C. chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

D. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

E. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

ANSWER: C · chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

Why this is best: Implications: chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life The distractors correspond to other named concepts in this learning area (Mechanisms, Bidirectional and cyclical, Fibromyalgia (ACR 1990), SB prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0435 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Implications**?

A. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

B. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

C. dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

D. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

E. chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

ANSWER: E · chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

Why this is best: In this context, the relevant notebook finding is **Implications**: chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0436 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Results vary, and some side effects**?

A. sleep hygiene, sleeping position, weight control, alcohol intake

B. age, weight, height; neck and belt circumference as indicated

C. attachment loss, mobility, prognosis of the teeth that will bear the appliance

D. CPAP, surgery, positional therapy

E. dental changes, discomfort — may occur

ANSWER: E · dental changes, discomfort — may occur

Why this is best: Results vary, and some side effects: dental changes, discomfort — may occur The distractors correspond to other named concepts in this learning area (Explain the type of device being considered and all alternatives, Medical–dental history, Provide explanation and written information on sleep behavioural therapy, Periodontal evaluation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 92, §4.1*

Q0437 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Results vary, and some side effects**?

A. attachment loss, mobility, prognosis of the teeth that will bear the appliance

B. age, weight, height; neck and belt circumference as indicated

C. CPAP, surgery, positional therapy

D. dental changes, discomfort — may occur

E. sleep hygiene, sleeping position, weight control, alcohol intake

ANSWER: D · dental changes, discomfort — may occur

Why this is best: In this context, the relevant notebook finding is **Results vary, and some side effects**: dental changes, discomfort — may occur The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 92, §4.1*

Q0438 §4.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on medical and lifestyle modifiers, which description is specifically associated with **Other sleep disorders and comorbidities**?

A. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

B. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

C. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

D. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

E. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

ANSWER: C · chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

Why this is best: Other sleep disorders and comorbidities: chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management; recent upper respiratory infection significantly raises perioperative airway risk, especially in children. 4.3 SDB and other sleep problems The distractors correspond to other named concepts in this learning area (Smoking, Obesity, Alcohol, Drugs and medications), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0439 §4.2 · Clinical-context application

Clinical context. A snoring patient has obesity, a complex medicine list and evening alcohol use. During this case discussion, how should the candidate interpret **Other sleep disorders and comorbidities**?

A. anaesthetics, sedatives, opioids and neuromuscular blockers impair dilator function, increasing obstruction and respiratory failure risk, particularly perioperatively

B. impairs airway muscle tone and increases obstruction risk, especially with sedatives; even moderate intake may cause transient desaturation despite appliance use

C. chronic inflammation, upper airway pathology, susceptibility to infection and cancer, higher morbidity and mortality

D. increases collapse and respiratory muscle dysfunction, contributing to OSA and postoperative respiratory failure; requires tailored perioperative management

E. chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

ANSWER: E · chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management

Why this is best: In this context, the relevant notebook finding is **Other sleep disorders and comorbidities**: chronic rhinosinusitis, allergic rhinitis, asthma or COPD worsen airway function and complicate management; recent upper respiratory infection significantly raises perioperative airway risk, especially in children. 4.3 SDB and other sleep problems The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.2*

Q0440 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **Insufficient sleep and shift work disorder**?

A. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

B. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

C. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

D. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

E. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

ANSWER: A · SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

Why this is best: Insufficient sleep and shift work disorder: SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents. A patient simply not spending enough time in bed will not respond as expected to an appliance The distractors correspond to other named concepts in this learning area (RLS and PLMD, Narcolepsy, Mood disorders, Insomnia), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0441 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **Insufficient sleep and shift work disorder**?

A. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

B. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

C. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

D. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

E. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

ANSWER: B · SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

Why this is best: In this context, the relevant notebook finding is **Insufficient sleep and shift work disorder**: SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents. A patient simply not spending enough time in bed will not respond as expected to an appliance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0442 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Assessment grading**?

A. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

B. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

C. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

D. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

E. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

ANSWER: C · possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

Why this is best: Assessment grading: possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG). Standard cut-offs should not be applied in otherwise healthy individuals; assess along a continuum of the behaviour The distractors correspond to other named concepts in this learning area (Prevalence, The "reactive–protective" hypothesis, SB + TMD, Corollary), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0443 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Assessment grading**?

A. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

B. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

C. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

D. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

ANSWER: A · possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

Why this is best: In this context, the relevant notebook finding is **Assessment grading**: possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG). Standard cut-offs should not be applied in otherwise healthy individuals; assess along a continuum of the behaviour The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0444 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Mechanisms**?

A. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

B. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

C. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

D. chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

E. dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

ANSWER: E · dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

Why this is best: Mechanisms: dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction The distractors correspond to other named concepts in this learning area (Bidirectional and cyclical, Implications, Fibromyalgia (ACR 1990), SB prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0445 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Mechanisms**?

A. chronic pain management must address sleep simultaneously, usually multidisciplinary; psychological input for the mood and anxiety components; improving sleep improves pain tolerance and quality of life

B. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

C. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

D. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

E. dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

ANSWER: E · dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction

Why this is best: In this context, the relevant notebook finding is **Mechanisms**: dysregulated serotonin, noradrenaline and dopamine (regulating both pain and sleep); pro-inflammatory TNF-α and IL-6 in both pathways; HPA-axis dysfunction The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 90, §4.8*

Q0446 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Reason for attendance**?

A. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

B. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

C. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

D. Occlusion — examine and record the existing condition; this is your medicolegal baseline

E. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

ANSWER: E · snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

Why this is best: Reason for attendance: snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being The distractors correspond to other named concepts in this learning area (Soft tissue/intraoral, Imaging, Teeth and restorations, Bruxism, orofacial pain, headaches), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0447 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Reason for attendance**?

A. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

B. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

C. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

D. Occlusion — examine and record the existing condition; this is your medicolegal baseline

E. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

ANSWER: B · snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

Why this is best: In this context, the relevant notebook finding is **Reason for attendance**: snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0448 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **Mood disorders**?

A. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

B. (i) fragmentation and hypoxia as a cause of depression; (ii) serotonin — abnormalities in central and peripheral serotonergic neurotransmission are a potential factor in depression, and serotonin also affects upper airway…

C. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

D. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

E. The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate…

ANSWER: D · SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

Why this is best: Mood disorders: SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance The distractors correspond to other named concepts in this learning area (Insufficient sleep and shift work disorder, The relationship appears mutual rather than cause-and-effect, RLS and PLMD, Proposed mechanisms), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0449 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **Mood disorders**?

A. SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. The pathophysiological relationship is not fully understood: it may be coincidental, or due to sleep fragmentation or repeated hypoxia, and confounders such as obesity, hypertension, diabetes and cardiovascular disease relate…

D. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

E. (i) fragmentation and hypoxia as a cause of depression; (ii) serotonin — abnormalities in central and peripheral serotonergic neurotransmission are a potential factor in depression, and serotonin also affects upper airway…

ANSWER: A · SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance

Why this is best: In this context, the relevant notebook finding is **Mood disorders**: SDB is associated with mood disorders, depression and anxiety, which may present with insomnia or hypersomnia, creating a cycle of worsening sleep and mental health. OSA and depression — the nuance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0450 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **SB + TMD**?

A. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

B. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

C. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

D. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

E. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

ANSWER: E · assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

Why this is best: SB + TMD: assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated. If high likelihood of OSA, refer to a sleep physician and consider a MAD thereafter; if no obvious OSA signs, do not refer for a sleep study and avoid a MAD The distractors correspond to other named concepts in this learning area (Prevalence, Assessment grading, Corollary, The "reactive–protective" hypothesis), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 90, §4.7*

Q0451 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **SB + TMD**?

A. Sequence: muscle tone falls → obstruction → more negative intraluminal pressure → bruxism triggered, increasing tone and restoring patency → less negative pressure

B. reported range 6–91%; self-reported adults ~8%; children and adolescents 14–46%; 30–50% of adults with OSA also exhibit SB. TMD in OSA 2–52%; OSA in TMD ~28%

C. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

D. possible (self-report), probable (self-report + clinical examination), definite (self-report + examination + instrumental assessment, notably PSG with EMG)

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

ANSWER: C · assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

Why this is best: In this context, the relevant notebook finding is **SB + TMD**: assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated. If high likelihood of OSA, refer to a sleep physician and consider a MAD thereafter; if no obvious OSA signs, do not refer for a sleep study and avoid a MAD The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 90, §4.7*

Q0452 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **SB prevalence**?

A. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

B. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

C. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

D. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

E. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

ANSWER: B · reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

Why this is best: SB prevalence: reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients The distractors correspond to other named concepts in this learning area (Fibromyalgia (ACR 1990), Fibromyalgia (ACR 1990), Conversely, Bidirectional and cyclical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 103, §4.8*

Q0453 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **SB prevalence**?

A. Pain prevents onset and causes frequent awakenings; acute experimental painful stimuli induce arousal and wakefulness in healthy people

B. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

C. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

D. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

E. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

ANSWER: E · reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

Why this is best: In this context, the relevant notebook finding is **SB prevalence**: reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 103, §4.8*

Q0454 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Soft tissue/intraoral**?

A. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

B. Occlusion — examine and record the existing condition; this is your medicolegal baseline

C. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

D. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

E. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

ANSWER: D · modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

Why this is best: Soft tissue/intraoral: modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency The distractors correspond to other named concepts in this learning area (Reason for attendance, Imaging, Teeth and restorations, Bruxism, orofacial pain, headaches), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0455 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Soft tissue/intraoral**?

A. initial dental radiographs as indicated; panoramic and lateral cephalometric as a pre-treatment record. CBCT and other CT are not indicated at this stage

B. number of functional teeth, mobility, spacing, curve of Spee, heavily restored teeth, anticipated restorative needs

C. Occlusion — examine and record the existing condition; this is your medicolegal baseline

D. modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

E. snoring, suspected or diagnosed OSA, sleep bruxism, or orofacial pain related to sleep disorders. Ask about sleep symptoms, previous diagnoses, comorbidities, and impact on daily life and well-being

ANSWER: D · modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency

Why this is best: In this context, the relevant notebook finding is **Soft tissue/intraoral**: modified Mallampati, tonsil grade, tongue size and scalloping, palatal vault, posterior pharyngeal crowding, signs of mouth breathing, signs of GORD such as enamel erosion, nasal patency The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 86, §4.1*

Q0456 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **RLS and PLMD**?

A. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

B. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

C. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

D. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

E. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

ANSWER: E · Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

Why this is best: RLS and PLMD: Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness. Some antidepressants induce or worsen RLS and periodic limb movements The distractors correspond to other named concepts in this learning area (Narcolepsy, Insufficient sleep and shift work disorder, Insomnia, CPAP's effect on depression is not conclusive), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0457 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **RLS and PLMD**?

A. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. SDB worsens both by further disrupting architecture, increasing cognitive and cardiovascular risk; shift workers with SDB are at higher risk of sleepiness and accidents

D. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

E. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

ANSWER: B · Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

Why this is best: In this context, the relevant notebook finding is **RLS and PLMD**: Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness. Some antidepressants induce or worsen RLS and periodic limb movements The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0458 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Self-reported sleep bruxism**?

A. both commoner supine; both involve oropharyngeal and masticatory muscle activation

B. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

C. ~8% of the adult population

D. 6–91%, depending on assessment method

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

ANSWER: C · ~8% of the adult population

Why this is best: Self-reported sleep bruxism: ~8% of the adult population The distractors correspond to other named concepts in this learning area (Wide reported range, Shared clinical features, Prevalence of TMD in OSA populations, Corollary), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 100, §4.7*

Q0459 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Self-reported sleep bruxism**?

A. ~8% of the adult population

B. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

C. both commoner supine; both involve oropharyngeal and masticatory muscle activation

D. 6–91%, depending on assessment method

E. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

ANSWER: A · ~8% of the adult population

Why this is best: In this context, the relevant notebook finding is **Self-reported sleep bruxism**: ~8% of the adult population The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 100, §4.7*

Q0460 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Successful treatment outcome**?

A. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

B. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

C. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

D. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

E. AHI/RDI/REI <5 and >50% reduction

ANSWER: E · AHI/RDI/REI <5 and >50% reduction

Why this is best: Successful treatment outcome: AHI/RDI/REI <5 and >50% reduction The distractors correspond to other named concepts in this learning area (Conversely, Fibromyalgia (ACR 1990), SB prevalence, Fibromyalgia (ACR 1990)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 103, §4.8*

Q0461 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Successful treatment outcome**?

A. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

B. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

C. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

D. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

E. AHI/RDI/REI <5 and >50% reduction

ANSWER: E · AHI/RDI/REI <5 and >50% reduction

Why this is best: In this context, the relevant notebook finding is **Successful treatment outcome**: AHI/RDI/REI <5 and >50% reduction The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 103, §4.8*

Q0462 §4.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on history, objectives and examination, which description is specifically associated with **Explain the type of device being considered and all alternatives**?

A. CPAP, surgery, positional therapy

B. attachment loss, mobility, prognosis of the teeth that will bear the appliance

C. sleep hygiene, sleeping position, weight control, alcohol intake

D. dental changes, discomfort — may occur

E. age, weight, height; neck and belt circumference as indicated

ANSWER: A · CPAP, surgery, positional therapy

Why this is best: Explain the type of device being considered and all alternatives: CPAP, surgery, positional therapy The distractors correspond to other named concepts in this learning area (Results vary, and some side effects, Medical–dental history, Provide explanation and written information on sleep behavioural therapy, Periodontal evaluation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 93, §4.1*

Q0463 §4.1 · Clinical-context application

Clinical context. A new OAT referral attends for history, intraoral examination and baseline records. During this case discussion, how should the candidate interpret **Explain the type of device being considered and all alternatives**?

A. attachment loss, mobility, prognosis of the teeth that will bear the appliance

B. dental changes, discomfort — may occur

C. sleep hygiene, sleeping position, weight control, alcohol intake

D. CPAP, surgery, positional therapy

E. age, weight, height; neck and belt circumference as indicated

ANSWER: D · CPAP, surgery, positional therapy

Why this is best: In this context, the relevant notebook finding is **Explain the type of device being considered and all alternatives**: CPAP, surgery, positional therapy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 93, §4.1*

Q0464 §4.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on other sleep disorders, which description is specifically associated with **CPAP's effect on depression is not conclusive**?

A. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

B. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

C. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

D. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

E. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

ANSWER: C · evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

Why this is best: CPAP's effect on depression is not conclusive: evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities The distractors correspond to other named concepts in this learning area (Insomnia, Symptoms of OSA and depression overlap, Narcolepsy, RLS and PLMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0465 §4.3 · Clinical-context application

Clinical context. A patient remains sleepy despite treated OSA; a second sleep disorder is suspected. During this case discussion, how should the candidate interpret **CPAP's effect on depression is not conclusive**?

A. COMISA. Highly prevalent in SDB; the overlap produces more severe fragmentation, increased sleepiness and poorer quality of life

B. tiredness, fatigue, sleepiness, poor motivation and poor concentration — complicating the diagnosis and treatment of each

C. May coexist, compounding sleepiness and cognitive impairment; symptom overlap makes clinical differentiation unreliable, so careful sleep studies are needed

D. evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

E. Frequently co-occur; SDB exacerbates limb movements and vice versa, causing further disruption and arousals; both contribute to sleepiness and impaired alertness

ANSWER: D · evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities

Why this is best: In this context, the relevant notebook finding is **CPAP's effect on depression is not conclusive**: evidence is limited; some studies report benefit for both sleepiness and depression, others no effect. 4.4 Medical comorbidities The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 87, §4.3*

Q0466 §4.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on sleep bruxism, which description is specifically associated with **Shared clinical features**?

A. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

B. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

C. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

D. both commoner supine; both involve oropharyngeal and masticatory muscle activation

E. 6–91%, depending on assessment method

ANSWER: D · both commoner supine; both involve oropharyngeal and masticatory muscle activation

Why this is best: Shared clinical features: both commoner supine; both involve oropharyngeal and masticatory muscle activation; both relate to arousals (micro-arousals in SB, major arousals in OSA); both associated with GORD, TMD and headache; both may respond to CPAP (case reports of CPAP treating SB), UPPP (treating SB in children with enlarged tonsils) and mandibular advancement (decreasing SB short-term). Shared risk factors: age, obesity, smoking, alcohol The distractors correspond to other named concepts in this learning area (Prevalence of TMD in OSA populations, Corollary, Wide reported range, SB + TMD), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0467 §4.7 · Clinical-context application

Clinical context. A patient with tooth wear and reported grinding is being screened for possible coexisting OSA. During this case discussion, how should the candidate interpret **Shared clinical features**?

A. 2–52%. Prevalence of OSA in TMD populations: ~28%. Pathophysiology of sleep bruxism

B. both commoner supine; both involve oropharyngeal and masticatory muscle activation

C. assess risk factors including behavioural; treat the TMD (often self-limiting, non-surgical) and avoid a MAD until the TMD is treated

D. 6–91%, depending on assessment method

E. SB is not a disorder but a behaviour that may be a risk factor or a protective factor for clinical consequences

ANSWER: B · both commoner supine; both involve oropharyngeal and masticatory muscle activation

Why this is best: In this context, the relevant notebook finding is **Shared clinical features**: both commoner supine; both involve oropharyngeal and masticatory muscle activation; both relate to arousals (micro-arousals in SB, major arousals in OSA); both associated with GORD, TMD and headache; both may respond to CPAP (case reports of CPAP treating SB), UPPP (treating SB in children with enlarged tonsils) and mandibular advancement (decreasing SB short-term). Shared risk factors: age, obesity, smoking, alcohol The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 89, §4.7*

Q0468 §4.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 4**, in the revision material on pain, tmd and sleep, which description is specifically associated with **Successful outcome**?

A. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

B. AHI/RDI/REI <5 and >50% reduction

C. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

D. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

E. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

ANSWER: B · AHI/RDI/REI <5 and >50% reduction

Why this is best: Successful outcome: AHI/RDI/REI <5 and >50% reduction The distractors correspond to other named concepts in this learning area (Conversely, Fibromyalgia (ACR 1990), SB prevalence, Fibromyalgia (ACR 1990)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 91, §4.8*

Q0469 §4.8 · Clinical-context application

Clinical context. A patient with facial pain and TMD is being assessed for sleep-related pain amplification. During this case discussion, how should the candidate interpret **Successful outcome**?

A. reported range 6–91%; self-reported adult ~8%; children and adolescents 14–46%; 30–50% of OSA pa- tients

B. widespread pain ≥3 months + tenderness at ≥11 of 18 sites at 4 kg pressure; prevalence 1– 2%, 80% female

C. AHI/RDI/REI <5 and >50% reduction

D. pain ≥3 months + ≥11 of 18 tender sites at 4 kg; prevalence 1–2%, 80% female. CFS 0.4–1.5%

E. hyperalgesia — short or disturbed sleep increases sensitivity to painful stimulation

ANSWER: C · AHI/RDI/REI <5 and >50% reduction

Why this is best: In this context, the relevant notebook finding is **Successful outcome**: AHI/RDI/REI <5 and >50% reduction The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 91, §4.8*

Q0470 §4.1 · Targeted knowledge / discriminative SBA

Which description does the notebook specifically associate with **Medical–dental history**?

A. attachment loss, mobility, prognosis of the teeth that will bear the appliance

B. sleep hygiene, sleeping position, weight control, alcohol intake

C. age, weight, height; neck and belt circumference as indicated

D. CPAP, surgery, positional therapy

E. dental changes, discomfort — may occur

ANSWER: C · age, weight, height; neck and belt circumference as indicated

Why this is best: Medical–dental history: age, weight, height; neck and belt circumference as indicated; presence of tonsils; tongue size; palate width and depth; malocclusion classification with overjet and overbite measurements; tooth erosion or wear; medications; other disease; family history of sleep disorders; chief complaint and current sleep problems including snoring, witnessed apnoeas, nocturia, morning headaches, hypersomnolence Competing statements refer to other named topics: Provide explanation and written information on sleep behavioural therapy, Periodontal evaluation, Results vary, and some side effects, Explain the type of device being considered and all alternatives.

Notebook anchor: *Integrated notebook, page 86, §4.1*

TOPIC 05
Appliance design, registration, manufacture and fitting

130 questions | Full notebook pages 106–131 | Each question has a justification and linked learning code.

Q0471 §5.7 · Integrated clinical vignette

A lab says the impression tore at a retentive molar undercut. Which material property is most relevant?

A. No need for stable working models

B. Avoiding periodontal assessment

C. Higher AHI before treatment

D. Adequate tear strength and recovery without distortion

E. Greater maximum mandibular protrusion

ANSWER: D · Adequate tear strength and recovery without distortion

Why this is best: Impression materials must reproduce fine detail and tolerate removal without tearing or permanent deformation.

Notebook anchor: *Integrated notebook, Topic 5, §5.7*

Q0472 §5.1 · All-of-the-above integrated judgement

At delivery of a custom MAD, which is the MOST COMPLETE safety check?

A. Stable retention and fit

B. Comfortable protrusion and opening

C. Ability to insert and remove

D. Care instructions and follow-up plan

E. All of the above

ANSWER: E · All of the above

Why this is best: Delivery must establish clinical fit, a tolerable starting configuration, competent home use and monitoring.

Notebook anchor: *Integrated notebook, Topic 5, §5.1*

Q0473 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Diagnosis first**?

A. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

B. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

C. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

D. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

E. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

ANSWER: C · Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

Why this is best: In this context, the relevant notebook finding is **Diagnosis first**: Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established. Severity must be established to make an appropriate decision, and severity predicts appliance response The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0474 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Diagnosis first**?

A. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

B. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

C. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

D. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

E. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

ANSWER: A · Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

Why this is best: Diagnosis first: Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established. Severity must be established to make an appropriate decision, and severity predicts appliance response The distractors correspond to other named concepts in this learning area (Severe OSA should have an initial trial of nasal CPAP, Dentition, Nasopharyngeal assessment, Use a custom, titratable appliance over non-custom devices), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0475 §5.1 · None-of-the-above discrimination

Which design is automatically the best device for every OSA patient?

A. One specific lateral-push design

B. One fixed non-titratable monobloc

C. An over-the-counter boil-and-bite guard

D. The device with the greatest opening

E. None of the above

ANSWER: E · None of the above

Why this is best: No single configuration is uniformly best; custom titratable selection must account for oral anatomy, tolerance and evidence.

Notebook anchor: *Integrated notebook, Topic 5, §5.1*

Q0476 §5.8 · Integrated clinical vignette

A laboratory produces a device at an arbitrary mandibular advancement because the prescription lacked a construction bite. What error occurred?

A. An unavoidable sleep-study scoring error

B. Failure to communicate the measured horizontal and vertical bite relationship

C. A confirmed medical diagnosis error

D. Proof that every device must be non-titratable

E. A fault corrected solely by prescribing more sleep hours

ANSWER: B · Failure to communicate the measured horizontal and vertical bite relationship

Why this is best: Laboratory requests should specify verified records, protrusion, vertical opening and chosen design so clinical treatment can be reproduced.

Notebook anchor: *Integrated notebook, Topic 5, §5.8*

Q0477 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Mild OSA suits the mechanism better**?

A. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

B. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

C. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

D. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

E. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

ANSWER: D · less collapsible airways, and the pharynx increases more along its whole length than in severe disease

Why this is best: In this context, the relevant notebook finding is **Mild OSA suits the mechanism better**: less collapsible airways, and the pharynx increases more along its whole length than in severe disease. The mechanism is less predictable than CPAP's, even long term. Can be used alone or combined with CPAP or positional therapy, and can reduce CPAP side effects in patients troubled by high pressures The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0478 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Mild OSA suits the mechanism better**?

A. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

B. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

C. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

D. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

E. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

ANSWER: D · less collapsible airways, and the pharynx increases more along its whole length than in severe disease

Why this is best: Mild OSA suits the mechanism better: less collapsible airways, and the pharynx increases more along its whole length than in severe disease. The mechanism is less predictable than CPAP's, even long term. Can be used alone or combined with CPAP or positional therapy, and can reduce CPAP side effects in patients troubled by high pressures The distractors correspond to other named concepts in this learning area (Polysomnographic, Patient traits, Endotype, Essential caveat), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0479 §5.1 · Integrated clinical vignette

A patient seeks an appliance for 'primary snoring' without any medical assessment. What is required first?

A. AHI can be inferred from ESS

B. A physician must exclude OSA before the label and treatment pathway are confirmed

C. A dental photograph confirms the absence of OSA

D. The dentist may diagnose primary snoring from sound alone

E. A stock mouthguard is an adequate diagnostic test

ANSWER: B · A physician must exclude OSA before the label and treatment pathway are confirmed

Why this is best: The notebook requires a physician diagnosis or exclusion of OSA before treating even apparently simple habitual snoring.

Notebook anchor: *Integrated notebook, Topic 5, §5.1*

Q0480 §5.3 · Clinical-context application

Clinical context. The dentist evaluates whether the teeth, periodontium and joints can tolerate a mandibular device. During this case discussion, how should the candidate interpret **Severe OSA is a relative contraindication only**?

A. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

B. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

C. anterior screw or strap repositioning the mandible against the opposing overlay

D. mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

E. CPAP should be trialled first, but an appliance is appropriate if CPAP fails

ANSWER: E · CPAP should be trialled first, but an appliance is appropriate if CPAP fails

Why this is best: In this context, the relevant notebook finding is **Severe OSA is a relative contraindication only**: CPAP should be trialled first, but an appliance is appropriate if CPAP fails The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 119, §5.3*

Q0481 §5.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on indications and contraindications, which description is specifically associated with **Severe OSA is a relative contraindication only**?

A. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

B. CPAP should be trialled first, but an appliance is appropriate if CPAP fails

C. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

D. mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

E. anterior screw or strap repositioning the mandible against the opposing overlay

ANSWER: B · CPAP should be trialled first, but an appliance is appropriate if CPAP fails

Why this is best: Severe OSA is a relative contraindication only: CPAP should be trialled first, but an appliance is appropriate if CPAP fails The distractors correspond to other named concepts in this learning area (Indications, Selection factors, Lateral Pull, Anterior Pull), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 119, §5.3*

Q0482 §5.3 · Integrated clinical vignette

A patient has advanced active periodontal disease with mobile anchor teeth. Which appliance decision is most appropriate?

A. Use maximal protrusion to splint teeth

B. Stabilise dental disease and reassess suitability before a tooth-retained MAD

C. Ignore anchorage because the appliance is removable

D. Treat as an ideal candidate without modification

E. Use no maintenance programme

ANSWER: B · Stabilise dental disease and reassess suitability before a tooth-retained MAD

Why this is best: Adequate periodontal support and retention are prerequisites for a predictable device; tooth mobility increases risk.

Notebook anchor: *Integrated notebook, Topic 5, §5.3*

Q0483 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Selection factors**?

A. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

B. There are clinically insignificant differences between appliances in symptoms or AHI

C. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

D. anterior screw or strap repositioning the mandible against the opposing overlay

E. overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin)

ANSWER: C · number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

Why this is best: In this context, the relevant notebook finding is **Selection factors**: number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0484 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Nasal patency**?

A. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

B. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

C. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

D. sufficient healthy teeth and a suitable bite are crucial

E. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

ANSWER: A · assess and treat first. It does not itself treat the OSA but improves the treatment outcome

Why this is best: Nasal patency: assess and treat first. It does not itself treat the OSA but improves the treatment outcome. Ask specifically about allergic rhinitis and treat it (referring to ENT) before appliance therapy; treatment may take time but you need not wait for complete resolution The distractors correspond to other named concepts in this learning area (Dexterity, cognition, visual acuity, range of motion, Sleep study findings, Mouth and tongue, Dental factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0485 §5.6 · Integrated clinical vignette

Maximum measured protrusive range is 10 mm. A cautious initial construction bite is planned at 50% of range. What advancement is registered?

A. 10 mm

B. 0 mm regardless of symptoms

C. 15 mm

D. 2 mm

E. 5 mm from the reference position

ANSWER: E · 5 mm from the reference position

Why this is best: The notebook recommends a commonly used initial target around 50–70% of measured maximum rather than starting at the end range.

Notebook anchor: *Integrated notebook, Topic 5, §5.6*

Q0486 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Nasal patency**?

A. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

B. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

C. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

D. sufficient healthy teeth and a suitable bite are crucial

E. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

ANSWER: C · assess and treat first. It does not itself treat the OSA but improves the treatment outcome

Why this is best: In this context, the relevant notebook finding is **Nasal patency**: assess and treat first. It does not itself treat the OSA but improves the treatment outcome. Ask specifically about allergic rhinitis and treat it (referring to ENT) before appliance therapy; treatment may take time but you need not wait for complete resolution The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0487 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Phonetic**?

A. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

B. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

C. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

D. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

E. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

ANSWER: C · uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

Why this is best: Phonetic: uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening The distractors correspond to other named concepts in this learning area (Classical practice, Vertical (bite opening), Lateral (midline), Clinical measurement and simulation tools), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0488 §5.6 · Integrated clinical vignette

A patient develops discomfort when the construction bite is held at near-maximum protrusion. What adjustment is best justified?

A. Abandon all treatment without discussing alternatives

B. Reduce toward a comfortable submaximal initial setting

C. Increase vertical opening without limit

D. Do not measure the range

E. Increase beyond maximum protrusion

ANSWER: B · Reduce toward a comfortable submaximal initial setting

Why this is best: Higher protrusion may improve airway patency but increases dental and joint loading; titration balances efficacy and tolerance.

Notebook anchor: *Integrated notebook, Topic 5, §5.6*

Q0489 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Phonetic**?

A. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

B. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

C. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

D. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: A · uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

Why this is best: In this context, the relevant notebook finding is **Phonetic**: uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0490 §5.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on impression accuracy, which description is specifically associated with **Requirements**?

A. The acceptable range for an individual is not known

B. sufficient healthy teeth and a suitable bite are crucial

C. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

D. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

E. accuracy and detail reproduction (some materials to 20–25 µm)

ANSWER: E · accuracy and detail reproduction (some materials to 20–25 µm)

Why this is best: Requirements: accuracy and detail reproduction (some materials to 20–25 µm); dimensional stability from removal to pouring; elasticity and tear strength for undercuts; wettability on moist tissues; biocompatibility; ease of handling with suitable working time but short setting time; appropriate viscosity and thixotropy; a rigid tray; moisture control; and operator technique, which matters as much as the material The distractors correspond to other named concepts in this learning area (Why not maximum protrusion, Dental factors, Laboratory prescription contents, Mouth and tongue), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.7*

Q0491 §5.6 · Integrated clinical vignette

Two otherwise equivalent devices differ mainly in vertical opening. Which choice aligns with the notebook?

A. Use an open-mouth appliance by default

B. Use the least vertical opening compatible with adequate function and fit

C. A 10 mm opening is routinely required

D. Vertical opening has no airway effect

E. Prefer maximal vertical opening in everyone

ANSWER: B · Use the least vertical opening compatible with adequate function and fit

Why this is best: Increasing vertical dimension may worsen upper-airway mechanics and mouth breathing; minimal necessary opening is favoured.

Notebook anchor: *Integrated notebook, Topic 5, §5.6*

Q0492 §5.7 · Clinical-context application

Clinical context. A laboratory reports that the latest impression did not reproduce critical tooth surfaces. During this case discussion, how should the candidate interpret **Requirements**?

A. sufficient healthy teeth and a suitable bite are crucial

B. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

C. The acceptable range for an individual is not known

D. accuracy and detail reproduction (some materials to 20–25 µm)

E. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

ANSWER: D · accuracy and detail reproduction (some materials to 20–25 µm)

Why this is best: In this context, the relevant notebook finding is **Requirements**: accuracy and detail reproduction (some materials to 20–25 µm); dimensional stability from removal to pouring; elasticity and tear strength for undercuts; wettability on moist tissues; biocompatibility; ease of handling with suitable working time but short setting time; appropriate viscosity and thixotropy; a rigid tray; moisture control; and operator technique, which matters as much as the material The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.7*

Q0493 §5.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on laboratory prescription, which description is specifically associated with **Laboratory prescription contents**?

A. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

B. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

C. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

D. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

E. Teach insertion, removal, cleaning, adjustment and the need for further appointments

ANSWER: A · Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

Why this is best: Laboratory prescription contents: A complete prescription allows accurate fabrication and communication of relevant dental limitations. The distractors correspond to other named concepts in this learning area (Delivery check, Device instruction at fit, Vertical, Impression), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.8*

Q0494 §5.4 · Integrated clinical vignette

An appliance cannot be adjusted after manufacture and has no verifiable setting. What is the key problem?

A. It removes the need for testing

B. It does not meet the preferred custom titratable design criteria

C. It automatically gives higher efficacy

D. It is inherently safer for every TMJ

E. It is the preferred guideline device for moderate OSA

ANSWER: B · It does not meet the preferred custom titratable design criteria

Why this is best: Guidelines favour custom titratable devices capable of gradual measurable reversal and adjustment.

Notebook anchor: *Integrated notebook, Topic 5, §5.4*

Q0495 §5.8 · Clinical-context application

Clinical context. A technician requests clarification before fabricating a prescribed mandibular appliance. During this case discussion, how should the candidate interpret **Laboratory prescription contents**?

A. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

B. Teach insertion, removal, cleaning, adjustment and the need for further appointments

C. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

D. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

E. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

ANSWER: A · Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

Why this is best: In this context, the relevant notebook finding is **Laboratory prescription contents**: A complete prescription allows accurate fabrication and communication of relevant dental limitations. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.8*

Q0496 §5.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on appliance delivery, which description is specifically associated with **Delivery check**?

A. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

B. Teach insertion, removal, cleaning, adjustment and the need for further appointments

C. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

D. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

E. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

ANSWER: D · Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

Why this is best: Delivery check: An appliance must fit correctly before adjustment and efficacy assessment. The distractors correspond to other named concepts in this learning area (Device instruction at fit, Vertical, Impression, Laboratory prescription contents), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.9*

Q0497 §5.8 · Integrated clinical vignette

A dental laboratory receives an appliance order without the protrusive registration, design or material. What is the best corrective action?

A. Proceed without a verified bite

B. Select an arbitrary vertical dimension

C. Ask the technician to guess maximum protrusion

D. Send a complete laboratory prescription with records, design, bite setting and material instructions

E. Avoid documenting allergies or dental limitations

ANSWER: D · Send a complete laboratory prescription with records, design, bite setting and material instructions

Why this is best: Laboratory communication must specify material, design, adequate records and construction bite so the fabricated device matches clinical planning.

Notebook anchor: *Integrated notebook, Topic 5, §5.8*

Q0498 §5.9 · Clinical-context application

Clinical context. A patient attends delivery and the dentist must assess fit, comfort and stability. During this case discussion, how should the candidate interpret **Delivery check**?

A. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

B. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

C. Teach insertion, removal, cleaning, adjustment and the need for further appointments

D. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

E. Specify the chosen design, impressions or scans, bite registration, advancement and vertical settings

ANSWER: A · Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

Why this is best: In this context, the relevant notebook finding is **Delivery check**: An appliance must fit correctly before adjustment and efficacy assessment. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.9*

Q0499 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Vertical**?

A. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

B. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

C. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

D. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

E. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

ANSWER: C · 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

Why this is best: Vertical: 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm The distractors correspond to other named concepts in this learning area (Cleaning, Handling and storage, Oral health, Impression), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0500 §5.9 · Integrated clinical vignette

At delivery a mandibular device dislodges easily when the patient opens. What should the dentist assess first?

A. Refer only after several years

B. Tell the patient to bite harder overnight

C. Assume the device works during sleep

D. Increase protrusion automatically

E. Seating, retention, stability and whether adjustment/remake is needed

ANSWER: E · Seating, retention, stability and whether adjustment/remake is needed

Why this is best: A therapeutic device must remain stable and retentive; fit is evaluated at delivery before efficacy titration.

Notebook anchor: *Integrated notebook, Topic 5, §5.9*

Q0501 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Vertical**?

A. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

B. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

C. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

D. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

E. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

ANSWER: C · 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

Why this is best: In this context, the relevant notebook finding is **Vertical**: 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0502 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Severe OSA should have an initial trial of nasal CPAP**?

A. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

B. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

C. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

D. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

E. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

ANSWER: D · greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

Why this is best: Severe OSA should have an initial trial of nasal CPAP: greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective The distractors correspond to other named concepts in this learning area (Diagnosis first, Dentition, Nasopharyngeal assessment, Use a custom, titratable appliance over non-custom devices), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0503 §5.10 · Integrated clinical vignette

A patient reports cleaning a nylon appliance with hot water and abrasive paste. What advice is appropriate?

A. Use abrasive grit to improve retention

B. Store it permanently in direct sun

C. Use the device-specific approved cleaning and storage protocol, avoiding heat-related distortion

D. Boil it each morning regardless of material

E. Never inspect for wear or biofilm

ANSWER: C · Use the device-specific approved cleaning and storage protocol, avoiding heat-related distortion

Why this is best: Home care should protect material stability, hygiene and device fit; heat may deform thermoplastic components.

Notebook anchor: *Integrated notebook, Topic 5, §5.10*

Q0504 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Severe OSA should have an initial trial of nasal CPAP**?

A. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

B. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

C. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

D. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

E. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

ANSWER: C · greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

Why this is best: In this context, the relevant notebook finding is **Severe OSA should have an initial trial of nasal CPAP**: greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0505 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Trial-based**?

A. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…

B. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

C. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

D. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve…

E. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

ANSWER: A · sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…

Why this is best: Trial-based: sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to good") The distractors correspond to other named concepts in this learning area (Dose dependence extends beyond anatomy, Cephalometric, Dose dependence extends beyond anatomy, The airway enlarges), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0506 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Trial-based**?

A. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

B. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve…

C. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

D. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…

E. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

ANSWER: D · sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to…

Why this is best: In this context, the relevant notebook finding is **Trial-based**: sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair to good") The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0507 §5.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on indications and contraindications, which description is specifically associated with **Indications**?

A. increasing vertical opening increases pharyngeal collapsibility and is associated with reduced efficacy

B. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

C. CPAP should be trialled first, but an appliance is appropriate if CPAP fails

D. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

E. mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

ANSWER: E · mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

Why this is best: Indications: mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to; snoring — often the only reason the patient attended a sleep clinic, and non-apnoeic snorers without daytime symptoms tolerate CPAP poorly (most agree to try but the majority give up), so an appliance is a good alternative The distractors correspond to other named concepts in this learning area (Severe OSA is a relative contraindication only, Mild OSA suits the mechanism better, Larger advancement = larger efficacy; smaller vertical opening preferred, Polysomnographic), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.3*

Q0508 §5.3 · Clinical-context application

Clinical context. The dentist evaluates whether the teeth, periodontium and joints can tolerate a mandibular device. During this case discussion, how should the candidate interpret **Indications**?

A. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

B. mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

C. increasing vertical opening increases pharyngeal collapsibility and is associated with reduced efficacy

D. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

E. CPAP should be trialled first, but an appliance is appropriate if CPAP fails

ANSWER: B · mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to

Why this is best: In this context, the relevant notebook finding is **Indications**: mild–moderate OSA as a primary option; moderate–severe OSA where CPAP cannot be tolerated or adhered to; snoring — often the only reason the patient attended a sleep clinic, and non-apnoeic snorers without daytime symptoms tolerate CPAP poorly (most agree to try but the majority give up), so an appliance is a good alternative The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.3*

Q0509 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Arch relationship**?

A. severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

B. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

C. class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

D. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

E. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

ANSWER: C · class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

Why this is best: Arch relationship: class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all The distractors correspond to other named concepts in this learning area (Skeletal pattern, Sleep study findings, Nasal patency, Dexterity, cognition, visual acuity, range of motion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0510 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Mono/Twin Block**?

A. There are clinically insignificant differences between appliances in symptoms or AHI

B. anterior screw or strap repositioning the mandible against the opposing overlay

C. overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin)

D. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

E. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

ANSWER: C · overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin)

Why this is best: In this context, the relevant notebook finding is **Mono/Twin Block**: overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin). Titrated by swapping overlays. Mono-block can beat prefabricated devices and gives good advancement but restricts jaw movement and can cause TMJ discomfort The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0511 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Horizontal (protrusion)**?

A. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

B. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

C. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

D. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

E. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

ANSWER: A · the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

Why this is best: Horizontal (protrusion): the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially The distractors correspond to other named concepts in this learning area (Clinical measurement and simulation tools, Construction bite devices, Neuromuscular (TENS), Lateral (midline)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0512 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Arch relationship**?

A. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

B. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

C. class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

D. severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

E. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

ANSWER: C · class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

Why this is best: In this context, the relevant notebook finding is **Arch relationship**: class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0513 §5.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on appliance delivery, which description is specifically associated with **Device instruction at fit**?

A. Teach insertion, removal, cleaning, adjustment and the need for further appointments

B. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

C. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

D. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

E. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

ANSWER: A · Teach insertion, removal, cleaning, adjustment and the need for further appointments

Why this is best: Device instruction at fit: The delivery appointment also establishes safe patient self-care and expectations. The distractors correspond to other named concepts in this learning area (Delivery check, Dentition, Initial protrusion 50–70% of maximum (AADSM 2025, Impression), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 113, §5.9*

Q0514 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Horizontal (protrusion)**?

A. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

B. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

C. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

D. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: C · the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

Why this is best: In this context, the relevant notebook finding is **Horizontal (protrusion)**: the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0515 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Oral health**?

A. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

B. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

C. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

D. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

E. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

ANSWER: A · Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

Why this is best: Oral health: Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if provided The distractors correspond to other named concepts in this learning area (Handling and storage, Cleaning, Vertical, Vertical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0516 §5.9 · Clinical-context application

Clinical context. A patient attends delivery and the dentist must assess fit, comfort and stability. During this case discussion, how should the candidate interpret **Device instruction at fit**?

A. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

B. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

C. Assess complete seating, patient-removable retention, stability, comfort and demonstrated insertion/removal

D. Teach insertion, removal, cleaning, adjustment and the need for further appointments

E. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

ANSWER: D · Teach insertion, removal, cleaning, adjustment and the need for further appointments

Why this is best: In this context, the relevant notebook finding is **Device instruction at fit**: The delivery appointment also establishes safe patient self-care and expectations. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 113, §5.9*

Q0517 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Dentition**?

A. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

B. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

C. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

D. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

E. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

ANSWER: E · generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

Why this is best: Dentition: generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status The distractors correspond to other named concepts in this learning area (Nasopharyngeal assessment, Diagnosis first, Severe OSA should have an initial trial of nasal CPAP, Use a custom, titratable appliance over non-custom devices), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 107, §5.1*

Q0518 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Oral health**?

A. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

B. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

C. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

D. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

E. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

ANSWER: A · Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

Why this is best: In this context, the relevant notebook finding is **Oral health**: Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if provided The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0519 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Dose dependence extends beyond anatomy**?

A. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

B. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

C. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

D. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair…

E. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in…

ANSWER: E · there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in…

Why this is best: Dose dependence extends beyond anatomy: there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in a dose-dependent way The distractors correspond to other named concepts in this learning area (Trial-based, Cephalometric, The airway enlarges, Adverse), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 118, §5.2*

Q0520 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Dentition**?

A. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

B. greater effectiveness demonstrated. Upper airway surgery (tonsillectomy and adenoidectomy, craniofacial operations, tracheostomy) may also supersede appliance use where predicted to be highly effective

C. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

D. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

E. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

ANSWER: C · generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

Why this is best: In this context, the relevant notebook finding is **Dentition**: generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 107, §5.1*

Q0521 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Precision Post**?

A. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

B. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

C. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

D. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

E. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

ANSWER: D · dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

Why this is best: Precision Post: dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm) The distractors correspond to other named concepts in this learning area (Between-device differences are clinically small in symptoms or AHI, Fabrication, Interlocking Dorsal, Nylon / polyamide (PA12), CAD-CAM), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0522 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Dose dependence extends beyond anatomy**?

A. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair…

B. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

C. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in…

D. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

E. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

ANSWER: C · there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in…

Why this is best: In this context, the relevant notebook finding is **Dose dependence extends beyond anatomy**: there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve in a dose-dependent way The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 118, §5.2*

Q0523 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Dental factors**?

A. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

B. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

C. sufficient healthy teeth and a suitable bite are crucial

D. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

E. perceived comfort, ease of use, durability, cost

ANSWER: C · sufficient healthy teeth and a suitable bite are crucial

Why this is best: Dental factors: sufficient healthy teeth and a suitable bite are crucial. Assess number of functional teeth, mobility (should not be mobile), spacing, curve of Spee, restorative status. A deep curve of Spee means insufficient vertical dimension at the second molars — consider not covering the sevens and tell the laboratory The distractors correspond to other named concepts in this learning area (Patient preference, Mouth and tongue, Nasal patency, Dexterity, cognition, visual acuity, range of motion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0524 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Precision Post**?

A. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

B. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

C. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

D. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

E. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

ANSWER: A · dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

Why this is best: In this context, the relevant notebook finding is **Precision Post**: dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0525 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Clinical measurement and simulation tools**?

A. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

B. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

C. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

D. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: C · bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

Why this is best: Clinical measurement and simulation tools: bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency The distractors correspond to other named concepts in this learning area (Horizontal (protrusion), Construction bite devices, Neuromuscular (TENS), Lateral (midline)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0526 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Dental factors**?

A. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

B. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

C. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

D. perceived comfort, ease of use, durability, cost

E. sufficient healthy teeth and a suitable bite are crucial

ANSWER: E · sufficient healthy teeth and a suitable bite are crucial

Why this is best: In this context, the relevant notebook finding is **Dental factors**: sufficient healthy teeth and a suitable bite are crucial. Assess number of functional teeth, mobility (should not be mobile), spacing, curve of Spee, restorative status. A deep curve of Spee means insufficient vertical dimension at the second molars — consider not covering the sevens and tell the laboratory The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0527 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Handling and storage**?

A. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

B. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

C. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

D. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

E. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

ANSWER: E · Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

Why this is best: Handling and storage: Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children The distractors correspond to other named concepts in this learning area (Oral health, Cleaning, Vertical, Vertical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0528 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Clinical measurement and simulation tools**?

A. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

B. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

C. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

D. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

E. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

ANSWER: C · bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

Why this is best: In this context, the relevant notebook finding is **Clinical measurement and simulation tools**: bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0529 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Treatment objectives**?

A. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

B. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

C. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

D. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

E. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

ANSWER: C · Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

Why this is best: Treatment objectives: Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. OSA: resolution of clinical signs and symptoms and normalisation of the AHI and oxyhaemoglobin saturation. The six recommendations, with rationale The distractors correspond to other named concepts in this learning area (Use a custom, titratable appliance over non-custom devices, Nasopharyngeal assessment, Dentition, Diagnosis first), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0530 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Handling and storage**?

A. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

B. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

C. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

D. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

E. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

ANSWER: D · Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

Why this is best: In this context, the relevant notebook finding is **Handling and storage**: Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0531 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Between-device differences are clinically small in symptoms or AHI**?

A. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

B. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

C. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

D. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

E. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

ANSWER: B · The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

Why this is best: Between-device differences are clinically small in symptoms or AHI: The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design The distractors correspond to other named concepts in this learning area (Fabrication, Precision Post, Interlocking Dorsal, Nylon / polyamide (PA12), CAD-CAM), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0532 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Treatment objectives**?

A. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

B. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

C. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

D. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

E. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

ANSWER: D · Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

Why this is best: In this context, the relevant notebook finding is **Treatment objectives**: Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level. OSA: resolution of clinical signs and symptoms and normalisation of the AHI and oxyhaemoglobin saturation. The six recommendations, with rationale The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0533 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Patient preference**?

A. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

B. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

C. sufficient healthy teeth and a suitable bite are crucial

D. perceived comfort, ease of use, durability, cost

E. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

ANSWER: D · perceived comfort, ease of use, durability, cost

Why this is best: Patient preference: perceived comfort, ease of use, durability, cost. The appliance must not be selected solely because of insurer requirements — the choice belongs to the qualified dentist and the patient. 5.6 Bite registration THE THREE COMPONENTS OF THE CONSTRUCTION BITE The distractors correspond to other named concepts in this learning area (Dental factors, Mouth and tongue, Dexterity, cognition, visual acuity, range of motion, Nasal patency), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0534 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Between-device differences are clinically small in symptoms or AHI**?

A. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

B. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

C. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

D. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

E. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

ANSWER: C · The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

Why this is best: In this context, the relevant notebook finding is **Between-device differences are clinically small in symptoms or AHI**: The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0535 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Neuromuscular (TENS)**?

A. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

B. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

C. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

D. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

E. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

ANSWER: E · transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

Why this is best: Neuromuscular (TENS): transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy The distractors correspond to other named concepts in this learning area (Construction bite devices, Horizontal (protrusion), Clinical measurement and simulation tools, Lateral (midline)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0536 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Patient preference**?

A. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

B. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

C. perceived comfort, ease of use, durability, cost

D. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

E. sufficient healthy teeth and a suitable bite are crucial

ANSWER: C · perceived comfort, ease of use, durability, cost

Why this is best: In this context, the relevant notebook finding is **Patient preference**: perceived comfort, ease of use, durability, cost. The appliance must not be selected solely because of insurer requirements — the choice belongs to the qualified dentist and the patient. 5.6 Bite registration THE THREE COMPONENTS OF THE CONSTRUCTION BITE The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0537 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Impression**?

A. every 6 months in year 1, then at least annually

B. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

C. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

D. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

E. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

ANSWER: C · dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

Why this is best: Impression: dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick The distractors correspond to other named concepts in this learning area (Dentition, Initial protrusion 50–70% of maximum (AADSM 2025, Follow-up, Vertical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0538 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Neuromuscular (TENS)**?

A. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

B. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

C. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

D. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: C · transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

Why this is best: In this context, the relevant notebook finding is **Neuromuscular (TENS)**: transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0539 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Nasopharyngeal assessment**?

A. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

B. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

C. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

D. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

E. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

ANSWER: C · nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

Why this is best: Nasopharyngeal assessment: nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre The distractors correspond to other named concepts in this learning area (Dentition, Use a custom, titratable appliance over non-custom devices, Treatment objectives, Diagnosis first), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 107, §5.1*

Q0540 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Impression**?

A. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

B. every 6 months in year 1, then at least annually

C. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

D. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

E. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

ANSWER: D · dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

Why this is best: In this context, the relevant notebook finding is **Impression**: dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0541 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Polysomnographic**?

A. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

B. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

C. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

D. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

E. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

ANSWER: A · supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

Why this is best: Polysomnographic: supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; a lower required CPAP pressure (higher pressure implies a more collapsible airway and poorer appliance response) The distractors correspond to other named concepts in this learning area (Mild OSA suits the mechanism better, Endotype, Patient traits, Primary mode of action is improvement in passive pharyngeal anatomy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0542 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Nasopharyngeal assessment**?

A. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

B. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

C. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

D. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

E. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

ANSWER: A · nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

Why this is best: In this context, the relevant notebook finding is **Nasopharyngeal assessment**: nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 107, §5.1*

Q0543 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Not recommended**?

A. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

B. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

C. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

D. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

E. designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable

ANSWER: E · designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable

Why this is best: Not recommended: designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable. Materials The distractors correspond to other named concepts in this learning area (Between-device differences are clinically small in symptoms or AHI, Precision Post, Fabrication, Interlocking Dorsal), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0544 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Polysomnographic**?

A. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

B. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

C. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

D. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

E. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

ANSWER: E · supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

Why this is best: In this context, the relevant notebook finding is **Polysomnographic**: supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant; a lower required CPAP pressure (higher pressure implies a more collapsible airway and poorer appliance response) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0545 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Skeletal pattern**?

A. class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

B. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

C. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

D. severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

E. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

ANSWER: D · severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

Why this is best: Skeletal pattern: severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage The distractors correspond to other named concepts in this learning area (Arch relationship, Dexterity, cognition, visual acuity, range of motion, Sleep study findings, Nasal patency), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0546 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Not recommended**?

A. traditional (casts and bite registrations — well established, precise custom fit) or 3D printing (high customisation, potentially reduced time and cost, in-house printing for fast replacement

B. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

C. The influence of design on efficacy and side effects is almost unknown, and side effects are a product of protruding the mandible to achieve a therapeutic effect rather than of specific design

D. designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable

E. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

ANSWER: D · designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable

Why this is best: In this context, the relevant notebook finding is **Not recommended**: designs marketed on theories of airway and bone remodelling, promoted by manufacturers, with no peer- reviewed publication, no demonstrated superiority, and non-adherence to AADSM guidance — typically bulky and not titratable. Materials The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0547 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Lateral (midline)**?

A. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

B. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

C. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

D. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

E. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

ANSWER: B · accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

Why this is best: Lateral (midline): accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible The distractors correspond to other named concepts in this learning area (Phonetic, Clinical measurement and simulation tools, Horizontal (protrusion), Classical practice), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0548 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Skeletal pattern**?

A. class II vs III alters mechanics and the direction of unwanted movement. An anterior open bite precludes a MAD — no posterior clearance at all

B. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

C. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

D. severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

E. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

ANSWER: D · severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage

Why this is best: In this context, the relevant notebook finding is **Skeletal pattern**: severe maxillomandibular deformity may need specialised treatment; retrognathia favours an appliance and is a specific problem for CPAP because of mask leakage The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0549 §5.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on clinical guideline and diagnosis, which description is specifically associated with **Use a custom, titratable appliance over non-custom devices**?

A. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

B. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

C. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

D. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

E. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

ANSWER: C · Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

Why this is best: Use a custom, titratable appliance over non-custom devices: Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent. Neither significantly affects sleep architecture or efficiency — but custom appliances improve other physiological sleep parameters, thereby improving daily function and quality of life, and improve daytime sleepiness not inferior to CPAP The distractors correspond to other named concepts in this learning area (Treatment objectives, Nasopharyngeal assessment, Dentition, Diagnosis first), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0550 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Lateral (midline)**?

A. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

B. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

C. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

D. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

E. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

ANSWER: C · accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

Why this is best: In this context, the relevant notebook finding is **Lateral (midline)**: accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0551 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **The airway enlarges**?

A. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

B. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

C. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

D. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

E. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

ANSWER: D · particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

Why this is best: The airway enlarges: particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase. Loop gain, arousal threshold and dilator muscle activity are unchanged The distractors correspond to other named concepts in this learning area (Dose dependence extends beyond anatomy, Adverse, Cephalometric, Primary mode of action is improvement in passive pharyngeal anatomy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0552 §5.1 · Clinical-context application

Clinical context. A newly diagnosed OSA patient has been referred to a qualified dentist for appliance therapy. During this case discussion, how should the candidate interpret **Use a custom, titratable appliance over non-custom devices**?

A. nasolaryngoscopy to evaluate pharyngeal stenosis through the naso-, velo-, oro- and laryngopharynx, with jaw thrust and Müller manoeuvre

B. Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

C. Presence or absence of OSA must be determined before initiating treatment — to identify patients at risk from complications, and to provide a baseline against which effectiveness can be established

D. generally ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support); assess mobility, spacing, curve of Spee, restorative status

E. Primary snoring without OSA or UARS: reduce snoring to a subjectively acceptable level

ANSWER: B · Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent

Why this is best: In this context, the relevant notebook finding is **Use a custom, titratable appliance over non-custom devices**: Custom titratable devices reduce AHI, arousal index and ODI and increase oxygen saturation to a greater extent. Neither significantly affects sleep architecture or efficiency — but custom appliances improve other physiological sleep parameters, thereby improving daily function and quality of life, and improve daytime sleepiness not inferior to CPAP The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 106, §5.1*

Q0553 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Acrylic (PMMA)**?

A. increasing the vertical open- ing increases pharyngeal collapsibility and has been associated with reduced appliance efficacy

B. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

C. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

D. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

E. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

ANSWER: E · good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

Why this is best: Acrylic (PMMA): good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing. More breakage than nylon in bruxers; FDM-printed PMMA is weaker; needs ~5 mm clearance The distractors correspond to other named concepts in this learning area (Traditional, 3D printing / additive manufacturing, Larger advancement gives larger efficacy; smaller vertical opening is preferred, Interlocking Dorsal), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0554 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **The airway enlarges**?

A. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

B. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

C. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

D. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

E. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

ANSWER: A · particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

Why this is best: In this context, the relevant notebook finding is **The airway enlarges**: particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase. Loop gain, arousal threshold and dilator muscle activity are unchanged The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0555 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Mouth and tongue**?

A. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

B. sufficient healthy teeth and a suitable bite are crucial

C. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

D. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

E. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

ANSWER: A · a TRD is an option for missing teeth or where jaw alignment precludes a MAD

Why this is best: Mouth and tongue: a TRD is an option for missing teeth or where jaw alignment precludes a MAD. Bruxism favours nylon or cobalt-chrome and a two-piece design. TMD and gagging favour traction rather than compression mechanisms, with minimal vertical opening and bulk The distractors correspond to other named concepts in this learning area (Nasal patency, Dexterity, cognition, visual acuity, range of motion, Dental factors, Sleep study findings), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0556 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Acrylic (PMMA)**?

A. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

B. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

C. increasing the vertical open- ing increases pharyngeal collapsibility and has been associated with reduced appliance efficacy

D. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

E. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

ANSWER: E · good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

Why this is best: In this context, the relevant notebook finding is **Acrylic (PMMA)**: good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing. More breakage than nylon in bruxers; FDM-printed PMMA is weaker; needs ~5 mm clearance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0557 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Construction bite devices**?

A. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

B. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

C. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

D. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

E. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

ANSWER: C · mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

Why this is best: Construction bite devices: mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype The distractors correspond to other named concepts in this learning area (Neuromuscular (TENS), Clinical measurement and simulation tools, Horizontal (protrusion), Lateral (midline)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0558 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Mouth and tongue**?

A. sufficient healthy teeth and a suitable bite are crucial

B. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

C. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

D. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

E. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

ANSWER: B · a TRD is an option for missing teeth or where jaw alignment precludes a MAD

Why this is best: In this context, the relevant notebook finding is **Mouth and tongue**: a TRD is an option for missing teeth or where jaw alignment precludes a MAD. Bruxism favours nylon or cobalt-chrome and a two-piece design. TMD and gagging favour traction rather than compression mechanisms, with minimal vertical opening and bulk The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0559 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Patient traits**?

A. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

B. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

C. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

D. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

E. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

ANSWER: B · younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

Why this is best: Patient traits: younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI The distractors correspond to other named concepts in this learning area (Endotype, Mild OSA suits the mechanism better, Polysomnographic, Essential caveat), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0560 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Construction bite devices**?

A. mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

B. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

C. bite gauges and mandibular positioning simulators for reproducibility. Digital/ virtual — scanning and CAD, potentially outperforming traditional methods in accuracy and efficiency

D. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

E. transcutaneous electrical nerve stimulation relaxes the muscles, allowing a relaxed physiological mandibular position before recording, reducing nociceptive input and potentially improving accuracy

ANSWER: A · mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype

Why this is best: In this context, the relevant notebook finding is **Construction bite devices**: mechanical devices giving precise control and measurement in three dimensions, with guides and rulers documenting advancement and vertical opening numerically. The George Gauge is the archetype The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 112, §5.6*

Q0561 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Interlocking Dorsal**?

A. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

B. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

C. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

D. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

E. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

ANSWER: C · angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

Why this is best: Interlocking Dorsal: angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism The distractors correspond to other named concepts in this learning area (Nylon / polyamide (PA12), CAD-CAM, 3D printing / additive manufacturing, Precision Post, Traditional), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0562 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Patient traits**?

A. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

B. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

C. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

D. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

E. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

ANSWER: E · younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

Why this is best: In this context, the relevant notebook finding is **Patient traits**: younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0563 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Dexterity, cognition, visual acuity, range of motion**?

A. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

B. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

C. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

D. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

E. sufficient healthy teeth and a suitable bite are crucial

ANSWER: C · an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

Why this is best: Dexterity, cognition, visual acuity, range of motion: an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration The distractors correspond to other named concepts in this learning area (Nasal patency, Sleep study findings, Mouth and tongue, Dental factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0564 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Interlocking Dorsal**?

A. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

B. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

C. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

D. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

E. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

ANSWER: B · angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

Why this is best: In this context, the relevant notebook finding is **Interlocking Dorsal**: angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0565 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Vertical (bite opening)**?

A. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

B. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

C. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

D. The acceptable range for an individual is not known

E. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

ANSWER: E · provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

Why this is best: Vertical (bite opening): provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical). Aim for posterior clearance rather than a raised bite: nylon ~3 mm, acrylic ~5 mm, some designs up to 6 mm (very difficult) The distractors correspond to other named concepts in this learning area (Classical practice, Phonetic, Lateral (midline), Why not maximum protrusion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0566 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Dexterity, cognition, visual acuity, range of motion**?

A. sufficient healthy teeth and a suitable bite are crucial

B. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

C. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

D. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

E. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

ANSWER: D · an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

Why this is best: In this context, the relevant notebook finding is **Dexterity, cognition, visual acuity, range of motion**: an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0567 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Follow-up**?

A. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

B. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

C. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

D. absence predicts better response

E. every 6 months in year 1, then at least annually

ANSWER: E · every 6 months in year 1, then at least annually

Why this is best: Follow-up: every 6 months in year 1, then at least annually The distractors correspond to other named concepts in this learning area (Pterygomandibular raphe present in ~64% of people, Impression, Dentition, Initial protrusion 50–70% of maximum (AADSM 2025), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0568 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Vertical (bite opening)**?

A. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

B. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

C. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

D. The acceptable range for an individual is not known

E. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

ANSWER: A · provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

Why this is best: In this context, the relevant notebook finding is **Vertical (bite opening)**: provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical). Aim for posterior clearance rather than a raised bite: nylon ~3 mm, acrylic ~5 mm, some designs up to 6 mm (very difficult) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0569 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Endotype**?

A. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

B. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

C. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

D. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

E. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

ANSWER: E · less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

Why this is best: Endotype: less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal The distractors correspond to other named concepts in this learning area (Patient traits, Mild OSA suits the mechanism better, Polysomnographic, Essential caveat), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0570 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Follow-up**?

A. every 6 months in year 1, then at least annually

B. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

C. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

D. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

E. absence predicts better response

ANSWER: A · every 6 months in year 1, then at least annually

Why this is best: In this context, the relevant notebook finding is **Follow-up**: every 6 months in year 1, then at least annually The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0571 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Traditional**?

A. increasing the vertical open- ing increases pharyngeal collapsibility and has been associated with reduced appliance efficacy

B. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

C. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

D. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

E. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

ANSWER: D · dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

Why this is best: Traditional: dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy The distractors correspond to other named concepts in this learning area (3D printing / additive manufacturing, Acrylic (PMMA), Nylon / polyamide (PA12), CAD-CAM, Larger advancement gives larger efficacy; smaller vertical opening is preferred), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 123, §5.4*

Q0572 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Endotype**?

A. younger; female; lower BMI and non-obese; smaller neck circumference; lower baseline AHI

B. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

C. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

D. less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

E. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

ANSWER: D · less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal

Why this is best: In this context, the relevant notebook finding is **Endotype**: less collapsible airway; lower loop gain; higher arousal threshold; lower response to arousal The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0573 §5.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on individual device selection, which description is specifically associated with **Sleep study findings**?

A. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

B. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

C. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

D. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

E. sufficient healthy teeth and a suitable bite are crucial

ANSWER: A · severity and positional dependence; adjustable MADs have higher response rates than fixed devices

Why this is best: Sleep study findings: severity and positional dependence; adjustable MADs have higher response rates than fixed devices The distractors correspond to other named concepts in this learning area (Dexterity, cognition, visual acuity, range of motion, Nasal patency, Mouth and tongue, Dental factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0574 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Traditional**?

A. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

B. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

C. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

D. increasing the vertical open- ing increases pharyngeal collapsibility and has been associated with reduced appliance efficacy

E. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

ANSWER: C · dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

Why this is best: In this context, the relevant notebook finding is **Traditional**: dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 123, §5.4*

Q0575 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Why not maximum protrusion**?

A. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

B. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

C. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

D. The acceptable range for an individual is not known

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: D · The acceptable range for an individual is not known

Why this is best: Why not maximum protrusion: The acceptable range for an individual is not known. Maximum protrusion gives the greatest airway enlargement but is inadequate as a setting because it causes TMJ and muscle discomfort. Classical practice: registration at 60% of maximum with 2 mm vertical separation of the incisors; other conventions use ~75%, or a wax record in the most protrusive position the patient can comfortably maintain. Practical rule: start at 50%, do not go beyond 75% The distractors correspond to other named concepts in this learning area (Vertical (bite opening), Phonetic, Classical practice, Lateral (midline)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0576 §5.5 · Clinical-context application

Clinical context. Two patients have the same AHI but different oral anatomy and functional limitations. During this case discussion, how should the candidate interpret **Sleep study findings**?

A. assess and treat first. It does not itself treat the OSA but improves the treatment outcome

B. sufficient healthy teeth and a suitable bite are crucial

C. an anterior adjustment mechanism suits poor dexterity, accepting anterior force concentration

D. severity and positional dependence; adjustable MADs have higher response rates than fixed devices

E. a TRD is an option for missing teeth or where jaw alignment precludes a MAD

ANSWER: D · severity and positional dependence; adjustable MADs have higher response rates than fixed devices

Why this is best: In this context, the relevant notebook finding is **Sleep study findings**: severity and positional dependence; adjustable MADs have higher response rates than fixed devices The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.5*

Q0577 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Dentition**?

A. absence predicts better response

B. every 6 months in year 1, then at least annually

C. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

D. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

E. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

ANSWER: D · ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

Why this is best: Dentition: ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support) The distractors correspond to other named concepts in this learning area (Initial protrusion 50–70% of maximum (AADSM 2025, Vertical, Follow-up, Pterygomandibular raphe present in ~64% of people), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0578 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Why not maximum protrusion**?

A. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

B. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

C. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

D. The acceptable range for an individual is not known

E. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

ANSWER: D · The acceptable range for an individual is not known

Why this is best: In this context, the relevant notebook finding is **Why not maximum protrusion**: The acceptable range for an individual is not known. Maximum protrusion gives the greatest airway enlargement but is inadequate as a setting because it causes TMJ and muscle discomfort. Classical practice: registration at 60% of maximum with 2 mm vertical separation of the incisors; other conventions use ~75%, or a wax record in the most protrusive position the patient can comfortably maintain. Practical rule: start at 50%, do not go beyond 75% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 111, §5.6*

Q0579 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Adverse**?

A. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

B. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

C. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

D. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

E. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

ANSWER: C · weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

Why this is best: Adverse: weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events The distractors correspond to other named concepts in this learning area (Primary mode of action is improvement in passive pharyngeal anatomy, The airway enlarges, Essential caveat, Dose dependence extends beyond anatomy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0580 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Dentition**?

A. absence predicts better response

B. every 6 months in year 1, then at least annually

C. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

D. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

E. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

ANSWER: E · ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

Why this is best: In this context, the relevant notebook finding is **Dentition**: ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0581 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **3D printing / additive manufacturing**?

A. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

B. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

C. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

D. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

E. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

ANSWER: B · digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

Why this is best: 3D printing / additive manufacturing: digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement. DLP and SLA offer advantages in accuracy, resolution and speed; the choice depends on the application and the desired workflow The distractors correspond to other named concepts in this learning area (Traditional, Acrylic (PMMA), Nylon / polyamide (PA12), CAD-CAM, Interlocking Dorsal), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 123, §5.4*

Q0582 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Adverse**?

A. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

B. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

C. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

D. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

E. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

ANSWER: B · weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

Why this is best: In this context, the relevant notebook finding is **Adverse**: weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0583 §5.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on construction bite registration, which description is specifically associated with **Classical practice**?

A. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

B. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

C. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

D. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

E. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

ANSWER: D · occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

Why this is best: Classical practice: occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors. Other conventions advance the mandible to about 75% of maximum protrusion, or take a wax interocclusal record with the mandible in the most protrusive position the patient can comfortably maintain. A practical rule is to start at 50% and do not go beyond 75% The distractors correspond to other named concepts in this learning area (Phonetic, Vertical (bite opening), Lateral (midline), Horizontal (protrusion)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 124, §5.6*

Q0584 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **3D printing / additive manufacturing**?

A. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

B. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

C. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

D. good mechanical properties, well established; retention by frictional resistance; can be relined and repaired; used in DLP and SLA printing

E. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

ANSWER: A · digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

Why this is best: In this context, the relevant notebook finding is **3D printing / additive manufacturing**: digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement. DLP and SLA offer advantages in accuracy, resolution and speed; the choice depends on the application and the desired workflow The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 123, §5.4*

Q0585 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Initial protrusion 50–70% of maximum (AADSM 2025**?

A. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

B. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

C. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

D. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

E. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

ANSWER: E · 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

Why this is best: Initial protrusion 50–70% of maximum (AADSM 2025: 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept The distractors correspond to other named concepts in this learning area (Dentition, Impression, Impression, Vertical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0586 §5.6 · Clinical-context application

Clinical context. At the construction bite appointment, protrusion and interincisal opening are being measured. During this case discussion, how should the candidate interpret **Classical practice**?

A. provides interocclusal space without discomfort or fatigue. ~2 mm interincisal (moderate 1–3 mm increase typical)

B. occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

C. accommodates asymmetry or deviation. The midline should be reproducible at both centric and protrusion; it need not be identical, but it must be reproducible

D. the principal therapeutic variable. Start at 50–70% of maximum protrusion; AADSM 2025 states 25–75% lies within both the comfort and therapeutic range. Do not exceed ~75% initially

E. uses speech (phonation of particular sounds) to find a position that keeps the airway open without excessive protrusion or opening

ANSWER: B · occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors

Why this is best: In this context, the relevant notebook finding is **Classical practice**: occlusal registration at 60% of maximum mandibular protrusion with 2 mm of vertical sep- aration of the upper and lower incisors. Other conventions advance the mandible to about 75% of maximum protrusion, or take a wax interocclusal record with the mandible in the most protrusive position the patient can comfortably maintain. A practical rule is to start at 50% and do not go beyond 75% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 124, §5.6*

Q0587 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Primary mode of action is improvement in passive pharyngeal anatomy**?

A. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

B. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

C. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

D. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

E. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

ANSWER: B · Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

Why this is best: Primary mode of action is improvement in passive pharyngeal anatomy: Advancement reduces collapsibility dose-dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population The distractors correspond to other named concepts in this learning area (Adverse, Essential caveat, The airway enlarges, Polysomnographic), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0588 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Initial protrusion 50–70% of maximum (AADSM 2025**?

A. upper and lower dentition + ~3 mm gingiva; detail reproduction to 20–25 µm; acrylic plate 1.5–2 mm thick

B. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

C. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

D. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

E. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

ANSWER: B · 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

Why this is best: In this context, the relevant notebook finding is **Initial protrusion 50–70% of maximum (AADSM 2025**: 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0589 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Nylon / polyamide (PA12), CAD-CAM**?

A. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

B. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

C. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

D. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

E. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

ANSWER: D · lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

Why this is best: Nylon / polyamide (PA12), CAD-CAM: lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device. But stains easily so polishing is important; gross titration 0.5–1 mm; cannot be relined or repaired, so a large new restoration means a remake The distractors correspond to other named concepts in this learning area (Interlocking Dorsal, 3D printing / additive manufacturing, Traditional, Precision Post), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0590 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Primary mode of action is improvement in passive pharyngeal anatomy**?

A. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

B. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

C. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

D. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

E. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

ANSWER: D · Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

Why this is best: In this context, the relevant notebook finding is **Primary mode of action is improvement in passive pharyngeal anatomy**: Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 107, §5.2*

Q0591 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Pterygomandibular raphe present in ~64% of people**?

A. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

B. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

C. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

D. every 6 months in year 1, then at least annually

E. absence predicts better response

ANSWER: E · absence predicts better response

Why this is best: Pterygomandibular raphe present in ~64% of people: absence predicts better response The distractors correspond to other named concepts in this learning area (Follow-up, Impression, Dentition, Initial protrusion 50–70% of maximum (AADSM 2025), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0592 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Nylon / polyamide (PA12), CAD-CAM**?

A. lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

B. digital models built layer by layer. High customisation, potentially reduced production time and cost, and in-house printing offering fast replacement

C. angled dorsal posts from the mandibular overlay interlocking with maxillary blocks. Titrated by screws in the blocks (e.g. 10 turns = +1 mm). An unattached mechanism

D. dental casts and bite registrations used to create custom appliances. Well established, allowing precise custom fitting to the patient's anatomy

E. dual posts from both overlays interlocking perpendicularly to guide, reposition and stabilise the mandible. Titrated by swapping an overlay for one with a different setting (e.g. +1 mm)

ANSWER: A · lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device

Why this is best: In this context, the relevant notebook finding is **Nylon / polyamide (PA12), CAD-CAM**: lighter, smaller, robust, less breakage so better for bruxers; retention by flexion into undercuts; needs only ~3 mm clearance; titration scale visible on the device. But stains easily so polishing is important; gross titration 0.5–1 mm; cannot be relined or repaired, so a large new restoration means a remake The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 110, §5.4*

Q0593 §5.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on upper-airway mechanisms and predictors, which description is specifically associated with **Cephalometric**?

A. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

B. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve…

C. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

D. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

E. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair…

ANSWER: D · shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

Why this is best: Cephalometric: shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA The distractors correspond to other named concepts in this learning area (Dose dependence extends beyond anatomy, Trial-based, Dose dependence extends beyond anatomy, The airway enlarges), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0594 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Pterygomandibular raphe present in ~64% of people**?

A. absence predicts better response

B. 25–75% acceptable); start at 50%, do not exceed 75%; the challenged "70% threshold" concept

C. ≥6 teeth per arch (stricter: 8–10 sound teeth per arch with adequate bone support)

D. every 6 months in year 1, then at least annually

E. dentition + ~3 mm gingiva; detail to 20–25 µm; acrylic plate 1.5–2 mm thick

ANSWER: A · absence predicts better response

Why this is best: In this context, the relevant notebook finding is **Pterygomandibular raphe present in ~64% of people**: absence predicts better response The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 128, §5.10*

Q0595 §5.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on device designs and materials, which description is specifically associated with **Lateral Pull**?

A. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

B. anterior screw or strap repositioning the mandible against the opposing overlay

C. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

D. There are clinically insignificant differences between appliances in symptoms or AHI

E. overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin)

ANSWER: C · bilateral elastomeric or nylon straps pulling the mandibular overlay forward

Why this is best: Lateral Pull: bilateral elastomeric or nylon straps pulling the mandibular overlay forward. Titrated by swapping strap lengths. Small, well documented; but straps fracture or stretch, titration gross (0.5–1 mm), and poor retention lets the less retentive component be lifted by the strap The distractors correspond to other named concepts in this learning area (Selection factors, Anterior Pull, Between-device differences are clinically small, Mono/Twin Block), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0596 §5.2 · Clinical-context application

Clinical context. An OAT candidate asks what anatomical and phenotypic features affect the chance of success. During this case discussion, how should the candidate interpret **Cephalometric**?

A. particularly in its lateral dimension at the velopharyngeal level — and the tongue is displaced anteriorly; velopharyngeal, nasopharyngeal and anteroposterior dimensions all increase

B. dose-dependent airway enlargement, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing protrusion; respiratory and sleep variables also improve dose-dependently

C. shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

D. sufficient AHI reduction on an overnight test with a remotely controlled mandibular positioner — the best predictor available ("excellent"), against clinical/craniofacial prediction ("poor") and nasopharyngoscopy ("fair…

E. there is dose-dependent enlargement of the upper airway, augmentation of upper airway muscle activity and improvement in minimum SaO2 with increasing mandibular protrusion, and the respiratory and sleep variables also improve…

ANSWER: C · shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA

Why this is best: In this context, the relevant notebook finding is **Cephalometric**: shorter soft palate; longer maxilla; lower hyoid and decreased hyoid-to-mandibular-plane distance; greater angle between cranial base and mandibular plane; retrognathic mandible with narrow SNB and wider SNA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0597 §5.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 5**, in the revision material on home care and maintenance, which description is specifically associated with **Cleaning**?

A. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

B. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

C. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

D. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

E. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

ANSWER: E · Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

Why this is best: Cleaning: Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device). Avoid toothpaste — too abrasive, causing scratches that harbour bacteria, dull the appliance and cause odour. Disinfect weekly in a denture-safe soak The distractors correspond to other named concepts in this learning area (Vertical, Oral health, Handling and storage, Vertical), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 113, §5.10*

Q0598 §5.4 · Clinical-context application

Clinical context. A dentist compares custom mandibular device designs with laboratory and material alternatives. During this case discussion, how should the candidate interpret **Lateral Pull**?

A. There are clinically insignificant differences between appliances in symptoms or AHI

B. anterior screw or strap repositioning the mandible against the opposing overlay

C. bilateral elastomeric or nylon straps pulling the mandibular overlay forward

D. overlays fused at the occlusal surface (mono) or mated by interlocking blocks (twin)

E. number of teeth; arch relationship (class II vs III); bruxism; TMD; gagging

ANSWER: C · bilateral elastomeric or nylon straps pulling the mandibular overlay forward

Why this is best: In this context, the relevant notebook finding is **Lateral Pull**: bilateral elastomeric or nylon straps pulling the mandibular overlay forward. Titrated by swapping strap lengths. Small, well documented; but straps fracture or stretch, titration gross (0.5–1 mm), and poor retention lets the less retentive component be lifted by the strap The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 109, §5.4*

Q0599 §5.2 · Targeted knowledge / discriminative SBA

Which description does the notebook specifically associate with **Essential caveat**?

A. less collapsible airways, and the pharynx increases more along its whole length than in severe disease

B. Advancement reduces collapsibility dose- dependently without systematically changing genioglossus function, in a predominantly obese, severe-OSA population

C. patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

D. supine-position-dependent (positional) OSA; NREM-predominant or non-stage-dependent rather than REM-predominant

E. weight increase during treatment relates to failure. Differences between patients in the ability to advance the mandible modify the potential to eliminate events

ANSWER: C · patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy

Why this is best: Essential caveat: patient factors alone are not a good predictor, and demographic, anthropometric and polysomnographic data only weakly inform efficacy. ~⅔ achieve a clinically meaningful effect (OSA at least halved); ~⅓ are unlikely to benefit — hence the need for prediction methods and for objective verification rather than assumption. 5.3 Indications and contraindications Competing statements refer to other named topics: Polysomnographic, Primary mode of action is improvement in passive pharyngeal anatomy, Adverse, Mild OSA suits the mechanism better.

Notebook anchor: *Integrated notebook, page 108, §5.2*

Q0600 §5.10 · Clinical-context application

Clinical context. During appliance handover, the patient asks for clear home-care and follow-up instructions. During this case discussion, how should the candidate interpret **Cleaning**?

A. 2 mm grey fork (5 mm white too much); delivery 2–4 mm over habitual; clearance nylon ~3, acrylic ~5, some 6 mm

B. 2 mm grey George Gauge fork (5 mm white fork usually too much); interincisal increase 2–4 mm; posterior clearance nylon ~3 mm, acrylic ~5 mm, some designs 6 mm

C. Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

D. Remove over a soft surface or a basin of water. Air dry before placing in the ventilated case. Store dry in the case — never in tissue or loose in a pocket. Keep away from sunlight, heat, pets and children

E. Brush and floss regularly. Morning routine: gentle jaw exercises — resting the chin on a fist, curling the tongue back to find the retruded position — and the morning occlusal guide for 15–20 minutes if…

ANSWER: C · Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device)

Why this is best: In this context, the relevant notebook finding is **Cleaning**: Rinse immediately after removal. Clean each morning with a soft-bristled brush and mild soap or denture cleaner, using cool or warm — never hot — water (hot water warps the device). Avoid toothpaste — too abrasive, causing scratches that harbour bacteria, dull the appliance and cause odour. Disinfect weekly in a denture-safe soak The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 113, §5.10*

TOPIC 06
Titration, treatment effectiveness and adherence

80 questions | Full notebook pages 132–145 | Each question has a justification and linked learning code.

Q0601 §6.1 · Integrated clinical vignette

A patient reports less snoring two weeks after MAD fitting but has worsening bilateral jaw soreness. What is the best titration decision?

A. Classify complete OSA cure from snoring alone

B. Advance to maximum immediately

C. Reassess comfort and consider pausing or reducing advancement before further titration

D. Disregard jaw findings

E. Stop physician follow-up permanently

ANSWER: C · Reassess comfort and consider pausing or reducing advancement before further titration

Why this is best: Titration must balance symptomatic gain with tolerability; persistent symptoms and adverse effects require adjustment and monitoring.

Notebook anchor: *Integrated notebook, Topic 6, §6.1*

Q0602 §6.1 · All-of-the-above integrated judgement

A patient reports improvement while titrating but still has residual symptoms. Which is the MOST COMPLETE evaluation?

A. Return to the chief complaint

B. Check side effects and jaw function

C. Assess appliance setting and actual use

D. Arrange physician-led objective verification

E. All of the above

ANSWER: E · All of the above

Why this is best: Titration balances response, dose, safety, adherence and objectively verified remaining disease.

Notebook anchor: *Integrated notebook, Topic 6, §6.1*

Q0603 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Acclimatisation interval**?

A. Allow approximately two weeks at the initial setting

B. Start commonly around 50% of the measured maximum advancement

C. AHI improvement is dose-dependent on advancement

D. Reassess within 30 days of insertion

E. About six weeks of gradual self-adjustment may follow acclimatisation

ANSWER: A · Allow approximately two weeks at the initial setting

Why this is best: In this context, the relevant notebook finding is **Acclimatisation interval**: The notebook gives a two-week adjustment phase before patient-led incremental titration. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0604 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Acclimatisation interval**?

A. Reassess within 30 days of insertion

B. Allow approximately two weeks at the initial setting

C. About six weeks of gradual self-adjustment may follow acclimatisation

D. AHI improvement is dose-dependent on advancement

E. Start commonly around 50% of the measured maximum advancement

ANSWER: B · Allow approximately two weeks at the initial setting

Why this is best: Acclimatisation interval: The notebook gives a two-week adjustment phase before patient-led incremental titration. The distractors correspond to other named concepts in this learning area (Protocol, Initial protrusive range, Patient-led adjustment window, Early appointment timing), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0605 §6.1 · None-of-the-above discrimination

Which single observation by itself proves complete OAT success?

A. Reduced snoring

B. One night of comfortable use

C. Improved ESS

D. A bed-partner report of less movement

E. None of the above

ANSWER: E · None of the above

Why this is best: Symptomatic changes are useful but cannot replace objective confirmation of controlled respiratory events.

Notebook anchor: *Integrated notebook, Topic 6, §6.1*

Q0606 §6.3 · Integrated clinical vignette

A patient feels dramatically better using a MAD; baseline AHI was 38. How should treatment success be confirmed?

A. Refer to the physician for appropriate objective efficacy testing

B. Conclude residual AHI is zero

C. Declare cure solely from ESS improvement

D. Avoid further testing because snoring decreased

E. Use the dentist's portable monitor as definitive diagnosis

ANSWER: A · Refer to the physician for appropriate objective efficacy testing

Why this is best: Symptom improvement alone does not prove normalisation; the medical provider interprets objective follow-up testing.

Notebook anchor: *Integrated notebook, Topic 6, §6.3*

Q0607 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Dental discomfort and bite change**?

A. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

B. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

C. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

D. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

E. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

ANSWER: E · tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

Why this is best: In this context, the relevant notebook finding is **Dental discomfort and bite change**: tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0608 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Dental discomfort and bite change**?

A. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

B. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

C. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

D. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

E. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

ANSWER: B · tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

Why this is best: Dental discomfort and bite change: tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record The distractors correspond to other named concepts in this learning area (Salivation, Jaw and TMJ, Effectiveness, Approach), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0609 §6.4 · Integrated clinical vignette

A mandibular positioner identifies an effective setting during an overnight study. What is the next appropriate step?

A. Translate the setting into the device and verify final clinical efficacy through shared care

B. Assume the research device's setting proves permanent cure

C. Skip appliance fit examination

D. Use the result to diagnose OSA de novo

E. Ignore adverse effects

ANSWER: A · Translate the setting into the device and verify final clinical efficacy through shared care

Why this is best: An instrumented titration can guide protrusion but final device performance, tolerance and physician-led outcome assessment remain necessary.

Notebook anchor: *Integrated notebook, Topic 6, §6.4*

Q0610 §6.3 · Clinical-context application

Clinical context. A patient reports better sleep after several weeks of advancement but has not yet completed an efficacy study. During this case discussion, how should the candidate interpret **Not indicated**?

A. Used only to guide titration within local scope rules

B. comfort with nightly use, and whether side effects are reducing use

C. The treating physician using appropriate objective sleep testing

D. Incremental advancement commonly about 0.1–0.4 mm per turn

E. primary snoring, unless symptoms worsen or fail to resolve

ANSWER: E · primary snoring, unless symptoms worsen or fail to resolve

Why this is best: In this context, the relevant notebook finding is **Not indicated**: primary snoring, unless symptoms worsen or fail to resolve The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0611 §6.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on referral for objective assessment, which description is specifically associated with **Not indicated**?

A. primary snoring, unless symptoms worsen or fail to resolve

B. comfort with nightly use, and whether side effects are reducing use

C. Incremental advancement commonly about 0.1–0.4 mm per turn

D. Used only to guide titration within local scope rules

E. The treating physician using appropriate objective sleep testing

ANSWER: A · primary snoring, unless symptoms worsen or fail to resolve

Why this is best: Not indicated: primary snoring, unless symptoms worsen or fail to resolve The distractors correspond to other named concepts in this learning area (Who verifies efficacy, Dentist-operated portable data, Small mechanical adjustments, Tolerance and adherence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0612 §6.5 · Integrated clinical vignette

A patient's snoring resolves but follow-up AHI remains 24. Which statement is most accurate?

A. The dentist should independently diagnose central apnoea

B. An appliance is effective whenever it is worn

C. Subjective improvement does not establish adequate control of OSA

D. A high residual AHI requires no action

E. Snoring resolution always implies AHI <5

ANSWER: C · Subjective improvement does not establish adequate control of OSA

Why this is best: Residual objective disease requires review of adherence, fit, advancement and medical options.

Notebook anchor: *Integrated notebook, Topic 6, §6.5*

Q0613 §6.4 · Clinical-context application

Clinical context. A partially responsive OAT patient is undergoing dose adjustment and possible instrumented titration. During this case discussion, how should the candidate interpret **Multimethod positioning**?

A. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

B. initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…

C. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

D. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

E. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

ANSWER: B · initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…

Why this is best: In this context, the relevant notebook finding is **Multimethod positioning**: initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of efficacy by the physician The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0614 §6.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on clinical and instrumented titration, which description is specifically associated with **Multimethod positioning**?

A. initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…

B. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

C. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

D. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

E. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

ANSWER: A · initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of…

Why this is best: Multimethod positioning: initial positioning → resolution or improvement of signs and symptoms → pulse oximetry, high- resolution oximetry or HSAT administered by the qualified dentist, and/or HSAT or PSG ordered by the physician → verification of efficacy by the physician The distractors correspond to other named concepts in this learning area (Further titration, modification, redesign or remake as required, Signs and symptoms only, Remotely controlled mandibular positioner (RCMP), Drug-induced sleep endoscopy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0615 §6.6 · Integrated clinical vignette

An oral appliance appears ineffective but a temperature microsensor shows it is worn two hours nightly. What should be addressed?

A. Call the therapy a confirmed physiological failure

B. Assume objective adherence never matters

C. Increase protrusion by 10 mm without review

D. Replace medical efficacy testing with the adherence record

E. Poor adherence before concluding that the device itself cannot work

ANSWER: E · Poor adherence before concluding that the device itself cannot work

Why this is best: Effectiveness depends on both physiological efficacy and actual use; objective adherence helps distinguish an ineffective device from an unused one.

Notebook anchor: *Integrated notebook, Topic 6, §6.6*

Q0616 §6.5 · Clinical-context application

Clinical context. The specialist and dentist review the distinction between short-term event reduction and lasting clinical benefit. During this case discussion, how should the candidate interpret **Persistent snoring, no other symptoms**?

A. Persistent symptoms with insufficient objective improvement

B. Resolution of symptoms and AHI less than five per hour on treatment

C. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

D. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

E. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

ANSWER: D · may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

Why this is best: In this context, the relevant notebook finding is **Persistent snoring, no other symptoms**: may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it. With other symptoms — treat as inadequate control: re-titrate, verify objectively, consider combination or alternative therapy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0617 §6.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on efficacy and effectiveness, which description is specifically associated with **Persistent snoring, no other symptoms**?

A. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

B. Resolution of symptoms and AHI less than five per hour on treatment

C. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

D. Persistent symptoms with insufficient objective improvement

E. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

ANSWER: C · may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

Why this is best: Persistent snoring, no other symptoms: may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it. With other symptoms — treat as inadequate control: re-titrate, verify objectively, consider combination or alternative therapy The distractors correspond to other named concepts in this learning area (Long-term effectiveness, Partial treatment response, Complete treatment response, Ongoing clinical failure), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0618 §6.6 · Integrated clinical vignette

A patient wears a MAD for 6 hours on three nights a week. Does this meet the notebook's AADSM adherence definition?

A. Yes, because the 4-hour CPAP convention applies

B. Yes, if snoring improves on those nights

C. No, because a MAD must be used for 24 hours

D. Yes, because any three nights suffice

E. No, because five nights per week are specified

ANSWER: E · No, because five nights per week are specified

Why this is best: The notebook defines satisfactory use as at least 80% of the sleep period on at least five nights per week; CPAP research conventions are different.

Notebook anchor: *Integrated notebook, Topic 6, §6.6*

Q0619 §6.6 · Clinical-context application

Clinical context. The patient reports nightly use but objective wear-time and symptom data are available for comparison. During this case discussion, how should the candidate interpret **It distinguishes device inefficacy from poor compliance**?

A. ~80% at 1 month → ~67% at 6 months, then stable; 6.1–6.7 h per night; 55–68% still wearing at 33 months; ~9.8% discontinue; ~7.5% discontinue from side effects

B. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

C. start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

ANSWER: E · the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

Why this is best: In this context, the relevant notebook finding is **It distinguishes device inefficacy from poor compliance**: the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 142, §6.6*

Q0620 §6.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on adherence measurement, which description is specifically associated with **It distinguishes device inefficacy from poor compliance**?

A. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

B. the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

C. ~80% at 1 month → ~67% at 6 months, then stable; 6.1–6.7 h per night; 55–68% still wearing at 33 months; ~9.8% discontinue; ~7.5% discontinue from side effects

D. start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

E. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

ANSWER: B · the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

Why this is best: It distinguishes device inefficacy from poor compliance: the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour The distractors correspond to other named concepts in this learning area (Adherence, Titration, AADSM compliance, Efficacy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 142, §6.6*

Q0621 §6.2 · Integrated clinical vignette

A patient says the bite feels 'off' for an hour after removing the splint. Which question is most useful?

A. Should the patient never remove the appliance?

B. Has the bite returned to baseline after morning exercises or alignment?

C. Is the appliance blue?

D. Is snoring audible in the afternoon?

E. Has the patient stopped eating breakfast?

ANSWER: B · Has the bite returned to baseline after morning exercises or alignment?

Why this is best: Morning occlusal changes can occur after protrusion; assess their persistence, function and management rather than assuming harmlessness.

Notebook anchor: *Integrated notebook, Topic 6, §6.2*

Q0622 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Initial protrusive range**?

A. Start commonly around 50% of the measured maximum advancement

B. Allow approximately two weeks at the initial setting

C. AHI improvement is dose-dependent on advancement

D. About six weeks of gradual self-adjustment may follow acclimatisation

E. to resolve unacceptable side effects; to balance efficacy against tolerance

ANSWER: A · Start commonly around 50% of the measured maximum advancement

Why this is best: In this context, the relevant notebook finding is **Initial protrusive range**: Use a submaximal and comfortable initial setting, then titrate; avoid starting at maximal protrusion. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0623 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Initial protrusive range**?

A. About six weeks of gradual self-adjustment may follow acclimatisation

B. AHI improvement is dose-dependent on advancement

C. Start commonly around 50% of the measured maximum advancement

D. Allow approximately two weeks at the initial setting

E. to resolve unacceptable side effects; to balance efficacy against tolerance

ANSWER: C · Start commonly around 50% of the measured maximum advancement

Why this is best: Initial protrusive range: Use a submaximal and comfortable initial setting, then titrate; avoid starting at maximal protrusion. The distractors correspond to other named concepts in this learning area (Acclimatisation interval, Patient-led adjustment window, Protocol, Why revert), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0624 §6.4 · Integrated clinical vignette

After gradual protrusion the patient develops severe unilateral TMJ pain. What is best done?

A. Diagnose OSA severity from pain

B. Increase advancement until pain stops

C. Label it proof of therapeutic success

D. Ignore a unilateral change

E. Stop further advancement and evaluate the joint and device, with modification or referral as needed

ANSWER: E · Stop further advancement and evaluate the joint and device, with modification or referral as needed

Why this is best: Severe or persistent TMD symptoms require a safety response, not automatic further titration.

Notebook anchor: *Integrated notebook, Topic 6, §6.4*

Q0625 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Unexpected persistent snoring**?

A. comfort with nightly use, and whether side effects are reducing use

B. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

C. Reassess adherence, appliance stability, symptom change and residual OSA

D. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

E. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

ANSWER: C · Reassess adherence, appliance stability, symptom change and residual OSA

Why this is best: In this context, the relevant notebook finding is **Unexpected persistent snoring**: Snoring is a symptom, not an objective disease index; follow-up testing may be needed. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.2*

Q0626 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Unexpected persistent snoring**?

A. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

B. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

C. Reassess adherence, appliance stability, symptom change and residual OSA

D. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

E. comfort with nightly use, and whether side effects are reducing use

ANSWER: C · Reassess adherence, appliance stability, symptom change and residual OSA

Why this is best: Unexpected persistent snoring: Snoring is a symptom, not an objective disease index; follow-up testing may be needed. The distractors correspond to other named concepts in this learning area (Tolerance and adherence, Appliance condition, Effectiveness, Approach), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.2*

Q0627 §6.1 · Integrated clinical vignette

A baseline ESS was 16 but the dentist changes to a different questionnaire at follow-up. What is lost?

A. The ability to inspect an appliance

B. A directly comparable longitudinal symptom measure

C. The right to ask about comfort

D. All ability to discuss sleepiness

E. The diagnostic PSG's validity

ANSWER: B · A directly comparable longitudinal symptom measure

Why this is best: Using the same validated measure at baseline and follow-up enables interpretation of change over time.

Notebook anchor: *Integrated notebook, Topic 6, §6.1*

Q0628 §6.3 · Clinical-context application

Clinical context. A patient reports better sleep after several weeks of advancement but has not yet completed an efficacy study. During this case discussion, how should the candidate interpret **Who verifies efficacy**?

A. Reassess adherence, appliance stability, symptom change and residual OSA

B. primary snoring, unless symptoms worsen or fail to resolve

C. comfort with nightly use, and whether side effects are reducing use

D. The treating physician using appropriate objective sleep testing

E. Used only to guide titration within local scope rules

ANSWER: D · The treating physician using appropriate objective sleep testing

Why this is best: In this context, the relevant notebook finding is **Who verifies efficacy**: The dentist may use permitted objective information for titration but does not diagnose or independently certify therapeutic cure. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0629 §6.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on referral for objective assessment, which description is specifically associated with **Who verifies efficacy**?

A. The treating physician using appropriate objective sleep testing

B. Reassess adherence, appliance stability, symptom change and residual OSA

C. comfort with nightly use, and whether side effects are reducing use

D. Used only to guide titration within local scope rules

E. primary snoring, unless symptoms worsen or fail to resolve

ANSWER: A · The treating physician using appropriate objective sleep testing

Why this is best: Who verifies efficacy: The dentist may use permitted objective information for titration but does not diagnose or independently certify therapeutic cure. The distractors correspond to other named concepts in this learning area (Not indicated, Dentist-operated portable data, Tolerance and adherence, Unexpected persistent snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0630 §6.5 · Integrated clinical vignette

A patient has 50% AHI reduction from 40 to 20 but unchanged daytime symptoms. Which conclusion is most defensible?

A. The patient has no residual disease

B. Numerical improvement is insufficient to label a complete or clearly satisfactory response

C. It is a complete response because 50% fell

D. Snoring must be absent

E. A 20/h result is normal

ANSWER: B · Numerical improvement is insufficient to label a complete or clearly satisfactory response

Why this is best: The notebook requires symptom improvement plus objective reduction for partial response; residual AHI 20 and persisting symptoms demand review.

Notebook anchor: *Integrated notebook, Topic 6, §6.5*

Q0631 §6.4 · Clinical-context application

Clinical context. A partially responsive OAT patient is undergoing dose adjustment and possible instrumented titration. During this case discussion, how should the candidate interpret **Further titration, modification, redesign or remake as required**?

A. The dentist's objective data supplement the titration decision; they never replace physician verification

B. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

C. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

D. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

E. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

ANSWER: B · Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

Why this is best: In this context, the relevant notebook finding is **Further titration, modification, redesign or remake as required**: Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0632 §6.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on clinical and instrumented titration, which description is specifically associated with **Further titration, modification, redesign or remake as required**?

A. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

B. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

C. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

D. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

E. The dentist's objective data supplement the titration decision; they never replace physician verification

ANSWER: A · Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

Why this is best: Further titration, modification, redesign or remake as required: Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations The distractors correspond to other named concepts in this learning area (Signs and symptoms only, Drug-induced sleep endoscopy, Both end in the same place, Remotely controlled mandibular positioner (RCMP)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0633 §6.3 · Integrated clinical vignette

A patient has residual AHI 17 after comfortable titration. Who should determine whether the remaining disease is acceptably controlled?

A. The appliance manufacturer

B. The patient's bed partner alone

C. The dentist alone without testing

D. An insurer acting as clinical decision-maker

E. The sleep physician in discussion with the patient and dentist

ANSWER: E · The sleep physician in discussion with the patient and dentist

Why this is best: Ongoing abnormal sleep indices require medically supervised evaluation; the dentist supplies fit, tolerance and adherence information.

Notebook anchor: *Integrated notebook, Topic 6, §6.3*

Q0634 §6.5 · Clinical-context application

Clinical context. The specialist and dentist review the distinction between short-term event reduction and lasting clinical benefit. During this case discussion, how should the candidate interpret **Partial treatment response**?

A. Persistent symptoms with insufficient objective improvement

B. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

C. Resolution of symptoms and AHI less than five per hour on treatment

D. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

E. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

ANSWER: E · Improved symptoms with at least a 50% AHI fall but residual AHI at least five

Why this is best: In this context, the relevant notebook finding is **Partial treatment response**: A meaningful reduction may still leave residual disease requiring medical assessment. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0635 §6.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on efficacy and effectiveness, which description is specifically associated with **Partial treatment response**?

A. Persistent symptoms with insufficient objective improvement

B. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

C. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

D. Resolution of symptoms and AHI less than five per hour on treatment

E. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

ANSWER: C · Improved symptoms with at least a 50% AHI fall but residual AHI at least five

Why this is best: Partial treatment response: A meaningful reduction may still leave residual disease requiring medical assessment. The distractors correspond to other named concepts in this learning area (Complete treatment response, Ongoing clinical failure, Persistent snoring, no other symptoms, Long-term effectiveness), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0636 §6.4 · Integrated clinical vignette

A 55-year-old is comfortable at 5 mm advancement but develops jaw pain at 7 mm. What is the most appropriate therapeutic target?

A. A fixed 10 mm in all adults

B. 7 mm regardless of pain

C. No protrusion assessment

D. The largest measurable anatomical protrusion

E. The maximally comfortable setting with verified clinical efficacy

ANSWER: E · The maximally comfortable setting with verified clinical efficacy

Why this is best: Optimal titration seeks the effective dose the patient can tolerate; an excessive protrusive position increases adverse effects.

Notebook anchor: *Integrated notebook, Topic 6, §6.4*

Q0637 §6.6 · Clinical-context application

Clinical context. The patient reports nightly use but objective wear-time and symptom data are available for comparison. During this case discussion, how should the candidate interpret **Titration**?

A. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

B. ~80% at 1 month → ~67% at 6 months, then stable; 6.1–6.7 h per night; 55–68% still wearing at 33 months; ~9.8% discontinue; ~7.5% discontinue from side effects

C. start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

ANSWER: C · start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

Why this is best: In this context, the relevant notebook finding is **Titration**: start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0638 §6.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on adherence measurement, which description is specifically associated with **Titration**?

A. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

B. start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

C. the single most important diagnostic function of adherence data. Without it you cannot know whether to advance the appliance or address behaviour

D. ~80% at 1 month → ~67% at 6 months, then stable; 6.1–6.7 h per night; 55–68% still wearing at 33 months; ~9.8% discontinue; ~7.5% discontinue from side effects

E. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

ANSWER: B · start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant

Why this is best: Titration: start at 50% of maximum; 2-week acclimatisation, then 6-week self-adjustment; increments 0.2–0.4 mm per night; total titration ~2–5 months; vertical dimension held constant The distractors correspond to other named concepts in this learning area (Adherence, It distinguishes device inefficacy from poor compliance, AADSM compliance, Efficacy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0639 §6.6 · Integrated clinical vignette

A patient reports wearing the appliance every night, but sensor data show use only twice a week. What is the key clinical consequence?

A. A low ESS is sufficient to prove control

B. The physician study becomes irrelevant

C. The appliance is necessarily mismanufactured

D. Low actual use limits effectiveness despite potential physiological efficacy

E. Reported use always outranks objective data

ANSWER: D · Low actual use limits effectiveness despite potential physiological efficacy

Why this is best: Objective adherence data identify suboptimal usage that can explain inadequate real-world benefit.

Notebook anchor: *Integrated notebook, Topic 6, §6.6*

Q0640 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Early appointment timing**?

A. Start commonly around 50% of the measured maximum advancement

B. AHI improvement is dose-dependent on advancement

C. Approximately two to five months

D. Allow approximately two weeks at the initial setting

E. Reassess within 30 days of insertion

ANSWER: E · Reassess within 30 days of insertion

Why this is best: In this context, the relevant notebook finding is **Early appointment timing**: Early review of the chief complaint, comfort, side effects and baseline questionnaire allows timely adjustment. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0641 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Early appointment timing**?

A. Allow approximately two weeks at the initial setting

B. Approximately two to five months

C. Reassess within 30 days of insertion

D. AHI improvement is dose-dependent on advancement

E. Start commonly around 50% of the measured maximum advancement

ANSWER: C · Reassess within 30 days of insertion

Why this is best: Early appointment timing: Early review of the chief complaint, comfort, side effects and baseline questionnaire allows timely adjustment. The distractors correspond to other named concepts in this learning area (Typical overall titration time, Protocol, Acclimatisation interval, Initial protrusive range), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0642 §6.5 · Integrated clinical vignette

A patient reaches AHI 4 but continues to report major sleepiness. How should the dentist respond?

A. Seek medical assessment for other sleepiness causes while reviewing OAT use

B. Abandon documentation

C. Declare all sleep symptoms cured

D. Diagnose narcolepsy independently

E. Increase the appliance beyond comfortable range automatically

ANSWER: A · Seek medical assessment for other sleepiness causes while reviewing OAT use

Why this is best: The OSA index is normalised, but persistent sleepiness may reflect other disorders, inadequate sleep or medication effects.

Notebook anchor: *Integrated notebook, Topic 6, §6.5*

Q0643 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Jaw and TMJ**?

A. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

B. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

C. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

D. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

E. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

ANSWER: E · discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

Why this is best: In this context, the relevant notebook finding is **Jaw and TMJ**: discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0644 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Jaw and TMJ**?

A. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

B. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

C. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

D. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

E. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

ANSWER: A · discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

Why this is best: Jaw and TMJ: discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment The distractors correspond to other named concepts in this learning area (Salivation, Effectiveness, Approach, Dental discomfort and bite change), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0645 §6.1 · Integrated clinical vignette

At three weeks a MAD user asks how much to advance per turn. Which principle best limits early discomfort?

A. Gradual small increments with symptom- and TMJ-guided reassessment

B. Full end-range advancement in one night

C. Increasing vertical dimension to maximum

D. No follow-up until year five

E. Using pain as the treatment target

ANSWER: A · Gradual small increments with symptom- and TMJ-guided reassessment

Why this is best: The source describes small measurable increments during monitored titration rather than abrupt maximal protrusion.

Notebook anchor: *Integrated notebook, Topic 6, §6.1*

Q0646 §6.3 · Clinical-context application

Clinical context. A patient reports better sleep after several weeks of advancement but has not yet completed an efficacy study. During this case discussion, how should the candidate interpret **Dentist-operated portable data**?

A. Reassess adherence, appliance stability, symptom change and residual OSA

B. primary snoring, unless symptoms worsen or fail to resolve

C. Incremental advancement commonly about 0.1–0.4 mm per turn

D. Used only to guide titration within local scope rules

E. The treating physician using appropriate objective sleep testing

ANSWER: D · Used only to guide titration within local scope rules

Why this is best: In this context, the relevant notebook finding is **Dentist-operated portable data**: The source distinguishes permitted titration assistance from medical diagnosis and formal follow-up efficacy assessment. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0647 §6.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on referral for objective assessment, which description is specifically associated with **Dentist-operated portable data**?

A. The treating physician using appropriate objective sleep testing

B. Reassess adherence, appliance stability, symptom change and residual OSA

C. primary snoring, unless symptoms worsen or fail to resolve

D. Incremental advancement commonly about 0.1–0.4 mm per turn

E. Used only to guide titration within local scope rules

ANSWER: E · Used only to guide titration within local scope rules

Why this is best: Dentist-operated portable data: The source distinguishes permitted titration assistance from medical diagnosis and formal follow-up efficacy assessment. The distractors correspond to other named concepts in this learning area (Who verifies efficacy, Not indicated, Small mechanical adjustments, Unexpected persistent snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.3*

Q0648 §6.4 · Integrated clinical vignette

On a treatment study, the patient removes the appliance midway through recording because of pain. What should the report include?

A. Assumption that the device was worn continuously

B. An AHI classified as a definitive appliance failure without use context

C. Actual device use and treatment position, with the limitation on interpreting effectiveness

D. Only total recording time

E. No adverse effect history

ANSWER: C · Actual device use and treatment position, with the limitation on interpreting effectiveness

Why this is best: Interpreting efficacy requires knowing whether the device was present and at what position; incomplete use limits inference.

Notebook anchor: *Integrated notebook, Topic 6, §6.4*

Q0649 §6.4 · Clinical-context application

Clinical context. A partially responsive OAT patient is undergoing dose adjustment and possible instrumented titration. During this case discussion, how should the candidate interpret **Both end in the same place**?

A. The maximal comfortable therapeutic position, not maximal anatomical range

B. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

C. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

D. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

E. The dentist's objective data supplement the titration decision; they never replace physician verification

ANSWER: E · The dentist's objective data supplement the titration decision; they never replace physician verification

Why this is best: In this context, the relevant notebook finding is **Both end in the same place**: The dentist's objective data supplement the titration decision; they never replace physician verification. Advance until target values are attained, can no longer be improved, or the maximum comfortable position is reached The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0650 §6.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on clinical and instrumented titration, which description is specifically associated with **Both end in the same place**?

A. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

B. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

C. The maximal comfortable therapeutic position, not maximal anatomical range

D. The dentist's objective data supplement the titration decision; they never replace physician verification

E. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

ANSWER: D · The dentist's objective data supplement the titration decision; they never replace physician verification

Why this is best: Both end in the same place: The dentist's objective data supplement the titration decision; they never replace physician verification. Advance until target values are attained, can no longer be improved, or the maximum comfortable position is reached The distractors correspond to other named concepts in this learning area (Remotely controlled mandibular positioner (RCMP), Drug-induced sleep endoscopy, Signs and symptoms only, Limit of protrusion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0651 §6.2 · Integrated clinical vignette

A patient develops a sore gingival margin without recurrent snoring. What is the immediate device-focused assessment?

A. Prescribe greater mandibular advancement

B. Examine appliance edges, fit, retention and hygiene

C. Assume no problem because AHI was improved

D. Ignore tissue injury until the annual review

E. Use only a STOP-BANG score

ANSWER: B · Examine appliance edges, fit, retention and hygiene

Why this is best: Local adverse effects can stem from fit or material contact and should be managed while protecting treatment adherence.

Notebook anchor: *Integrated notebook, Topic 6, §6.2*

Q0652 §6.5 · Clinical-context application

Clinical context. The specialist and dentist review the distinction between short-term event reduction and lasting clinical benefit. During this case discussion, how should the candidate interpret **Complete treatment response**?

A. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

B. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

C. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

D. Resolution of symptoms and AHI less than five per hour on treatment

E. Persistent symptoms with insufficient objective improvement

ANSWER: D · Resolution of symptoms and AHI less than five per hour on treatment

Why this is best: In this context, the relevant notebook finding is **Complete treatment response**: Normalisation requires both subjective and objective response, not simply improved snoring. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0653 §6.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on efficacy and effectiveness, which description is specifically associated with **Complete treatment response**?

A. Persistent symptoms with insufficient objective improvement

B. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

C. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

D. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

E. Resolution of symptoms and AHI less than five per hour on treatment

ANSWER: E · Resolution of symptoms and AHI less than five per hour on treatment

Why this is best: Complete treatment response: Normalisation requires both subjective and objective response, not simply improved snoring. The distractors correspond to other named concepts in this learning area (Ongoing clinical failure, Partial treatment response, Persistent snoring, no other symptoms, Long-term effectiveness), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0654 §6.6 · Clinical-context application

Clinical context. The patient reports nightly use but objective wear-time and symptom data are available for comparison. During this case discussion, how should the candidate interpret **AADSM compliance**?

A. A device-embedded temperature sensor can objectively estimate nightly wear

B. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

C. At least 80% of the sleep period on at least five nights each week

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. Monthly clinical monitoring may be used during initial titration and adaptation

ANSWER: D · ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

Why this is best: In this context, the relevant notebook finding is **AADSM compliance**: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0655 §6.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on adherence measurement, which description is specifically associated with **AADSM compliance**?

A. A device-embedded temperature sensor can objectively estimate nightly wear

B. Monthly clinical monitoring may be used during initial titration and adaptation

C. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. At least 80% of the sleep period on at least five nights each week

ANSWER: D · ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

Why this is best: AADSM compliance: ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night The distractors correspond to other named concepts in this learning area (Efficacy, First six months of therapy, Recorded adherence technology, MAD adherence criterion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0656 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Why revert**?

A. AHI improvement is dose-dependent on advancement

B. Start commonly around 50% of the measured maximum advancement

C. About six weeks of gradual self-adjustment may follow acclimatisation

D. to resolve unacceptable side effects; to balance efficacy against tolerance

E. Allow approximately two weeks at the initial setting

ANSWER: D · to resolve unacceptable side effects; to balance efficacy against tolerance

Why this is best: In this context, the relevant notebook finding is **Why revert**: to resolve unacceptable side effects; to balance efficacy against tolerance; and to avoid prematurely abandoning titration over minor side effects, which would give a suboptimal outcome. 6.2 Reviewing signs and symptoms at follow-up The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0657 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Why revert**?

A. AHI improvement is dose-dependent on advancement

B. Allow approximately two weeks at the initial setting

C. to resolve unacceptable side effects; to balance efficacy against tolerance

D. Start commonly around 50% of the measured maximum advancement

E. About six weeks of gradual self-adjustment may follow acclimatisation

ANSWER: C · to resolve unacceptable side effects; to balance efficacy against tolerance

Why this is best: Why revert: to resolve unacceptable side effects; to balance efficacy against tolerance; and to avoid prematurely abandoning titration over minor side effects, which would give a suboptimal outcome. 6.2 Reviewing signs and symptoms at follow-up The distractors correspond to other named concepts in this learning area (Patient-led adjustment window, Initial protrusive range, Protocol, Acclimatisation interval), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0658 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Appliance condition**?

A. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

B. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

C. Reassess adherence, appliance stability, symptom change and residual OSA

D. comfort with nightly use, and whether side effects are reducing use

E. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

ANSWER: B · cracks, warping, wear affecting fit and efficacy; confirm retention and stability

Why this is best: In this context, the relevant notebook finding is **Appliance condition**: cracks, warping, wear affecting fit and efficacy; confirm retention and stability The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0659 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Appliance condition**?

A. Reassess adherence, appliance stability, symptom change and residual OSA

B. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

C. comfort with nightly use, and whether side effects are reducing use

D. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

E. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

ANSWER: D · cracks, warping, wear affecting fit and efficacy; confirm retention and stability

Why this is best: Appliance condition: cracks, warping, wear affecting fit and efficacy; confirm retention and stability The distractors correspond to other named concepts in this learning area (Unexpected persistent snoring, Tolerance and adherence, Approach, Effectiveness), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0660 §6.4 · Clinical-context application

Clinical context. A partially responsive OAT patient is undergoing dose adjustment and possible instrumented titration. During this case discussion, how should the candidate interpret **Signs and symptoms only**?

A. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

B. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

C. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

D. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

E. The dentist's objective data supplement the titration decision; they never replace physician verification

ANSWER: A · initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

Why this is best: In this context, the relevant notebook finding is **Signs and symptoms only**: initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0661 §6.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on clinical and instrumented titration, which description is specifically associated with **Signs and symptoms only**?

A. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

B. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

C. Monitor monthly for the first six months and at least annually thereafter for as long as the appliance is worn. 6.5 Efficacy, long-term effectiveness, limitations

D. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

E. The dentist's objective data supplement the titration decision; they never replace physician verification

ANSWER: D · initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

Why this is best: Signs and symptoms only: initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician The distractors correspond to other named concepts in this learning area (Remotely controlled mandibular positioner (RCMP), Drug-induced sleep endoscopy, Both end in the same place, Further titration, modification, redesign or remake as required), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.4*

Q0662 §6.5 · Clinical-context application

Clinical context. The specialist and dentist review the distinction between short-term event reduction and lasting clinical benefit. During this case discussion, how should the candidate interpret **Long-term effectiveness**?

A. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

B. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

C. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

D. Persistent symptoms with insufficient objective improvement

E. Resolution of symptoms and AHI less than five per hour on treatment

ANSWER: B · Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

Why this is best: In this context, the relevant notebook finding is **Long-term effectiveness**: Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function. Long-term The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 133, §6.5*

Q0663 §6.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on efficacy and effectiveness, which description is specifically associated with **Long-term effectiveness**?

A. Resolution of symptoms and AHI less than five per hour on treatment

B. Persistent symptoms with insufficient objective improvement

C. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

D. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

E. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

ANSWER: E · Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

Why this is best: Long-term effectiveness: Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function. Long-term The distractors correspond to other named concepts in this learning area (Persistent snoring, no other symptoms, Partial treatment response, Ongoing clinical failure, Complete treatment response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 133, §6.5*

Q0664 §6.6 · Clinical-context application

Clinical context. The patient reports nightly use but objective wear-time and symptom data are available for comparison. During this case discussion, how should the candidate interpret **Efficacy**?

A. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

B. Monthly clinical monitoring may be used during initial titration and adaptation

C. At least 80% of the sleep period on at least five nights each week

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. A device-embedded temperature sensor can objectively estimate nightly wear

ANSWER: A · AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

Why this is best: In this context, the relevant notebook finding is **Efficacy**: AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0665 §6.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on adherence measurement, which description is specifically associated with **Efficacy**?

A. At least 80% of the sleep period on at least five nights each week

B. A device-embedded temperature sensor can objectively estimate nightly wear

C. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

D. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

E. Monthly clinical monitoring may be used during initial titration and adaptation

ANSWER: D · AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

Why this is best: Efficacy: AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none The distractors correspond to other named concepts in this learning area (First six months of therapy, AADSM compliance, Recorded adherence technology, MAD adherence criterion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 143, §6.6*

Q0666 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Patient-led adjustment window**?

A. AHI improvement is dose-dependent on advancement

B. About six weeks of gradual self-adjustment may follow acclimatisation

C. Allow approximately two weeks at the initial setting

D. Start commonly around 50% of the measured maximum advancement

E. to resolve unacceptable side effects; to balance efficacy against tolerance

ANSWER: B · About six weeks of gradual self-adjustment may follow acclimatisation

Why this is best: In this context, the relevant notebook finding is **Patient-led adjustment window**: The representative protocol permits gradual advancement with clinical supervision, symptoms and tolerance determining the endpoint. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0667 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Patient-led adjustment window**?

A. AHI improvement is dose-dependent on advancement

B. Start commonly around 50% of the measured maximum advancement

C. to resolve unacceptable side effects; to balance efficacy against tolerance

D. Allow approximately two weeks at the initial setting

E. About six weeks of gradual self-adjustment may follow acclimatisation

ANSWER: E · About six weeks of gradual self-adjustment may follow acclimatisation

Why this is best: Patient-led adjustment window: The representative protocol permits gradual advancement with clinical supervision, symptoms and tolerance determining the endpoint. The distractors correspond to other named concepts in this learning area (Initial protrusive range, Why revert, Protocol, Acclimatisation interval), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0668 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Salivation**?

A. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

B. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

C. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

D. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

E. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

ANSWER: B · excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

Why this is best: In this context, the relevant notebook finding is **Salivation**: excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0669 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Salivation**?

A. excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

B. tooth pain, gingival irritation, soreness, the bite feeling "off" or teeth not fitting as before; long-term use decreases overbite and overjet — monitor and record

C. discomfort, stiffness, pain, clicking or sounds; usually improve, but persistent or severe symptoms need management or adjustment

D. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

E. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

ANSWER: A · excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage

Why this is best: Salivation: excessive salivation or dry mouth, common initially and often transient (salivation typically settles after a few weeks) but document and manage The distractors correspond to other named concepts in this learning area (Jaw and TMJ, Dental discomfort and bite change, Effectiveness, Approach), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0670 §6.4 · Clinical-context application

Clinical context. A partially responsive OAT patient is undergoing dose adjustment and possible instrumented titration. During this case discussion, how should the candidate interpret **Drug-induced sleep endoscopy**?

A. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

B. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

C. The maximal comfortable therapeutic position, not maximal anatomical range

D. The dentist's objective data supplement the titration decision; they never replace physician verification

E. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

ANSWER: A · a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

Why this is best: In this context, the relevant notebook finding is **Drug-induced sleep endoscopy**: a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 139, §6.4*

Q0671 §6.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on clinical and instrumented titration, which description is specifically associated with **Drug-induced sleep endoscopy**?

A. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

B. The dentist's objective data supplement the titration decision; they never replace physician verification

C. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

D. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

E. The maximal comfortable therapeutic position, not maximal anatomical range

ANSWER: D · a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

Why this is best: Drug-induced sleep endoscopy: a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation The distractors correspond to other named concepts in this learning area (Remotely controlled mandibular positioner (RCMP), Both end in the same place, Signs and symptoms only, Limit of protrusion), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 139, §6.4*

Q0672 §6.5 · Clinical-context application

Clinical context. The specialist and dentist review the distinction between short-term event reduction and lasting clinical benefit. During this case discussion, how should the candidate interpret **Ongoing clinical failure**?

A. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

B. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

C. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

D. Resolution of symptoms and AHI less than five per hour on treatment

E. Persistent symptoms with insufficient objective improvement

ANSWER: E · Persistent symptoms with insufficient objective improvement

Why this is best: In this context, the relevant notebook finding is **Ongoing clinical failure**: Persistent symptoms and residual disease prompt review of dose, use, device and alternative therapy. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0673 §6.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on efficacy and effectiveness, which description is specifically associated with **Ongoing clinical failure**?

A. may be acceptable to the patient; but persistent snoring on treatment relates to insufficient apnoea control and poor adherence, so verify objectively before accepting it

B. Improved symptoms with at least a 50% AHI fall but residual AHI at least five

C. Resolution of symptoms and AHI less than five per hour on treatment

D. Health outcomes are comparable to CPAP at least short-term, even in severe OSA, with greater adherence to the splint; appliances also benefit blood pressure, endothelial function and left ventricular function

E. Persistent symptoms with insufficient objective improvement

ANSWER: E · Persistent symptoms with insufficient objective improvement

Why this is best: Ongoing clinical failure: Persistent symptoms and residual disease prompt review of dose, use, device and alternative therapy. The distractors correspond to other named concepts in this learning area (Complete treatment response, Partial treatment response, Persistent snoring, no other symptoms, Long-term effectiveness), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 134, §6.5*

Q0674 §6.6 · Clinical-context application

Clinical context. The patient reports nightly use but objective wear-time and symptom data are available for comparison. During this case discussion, how should the candidate interpret **Recorded adherence technology**?

A. A device-embedded temperature sensor can objectively estimate nightly wear

B. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

C. Monthly clinical monitoring may be used during initial titration and adaptation

D. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

E. At least 80% of the sleep period on at least five nights each week

ANSWER: A · A device-embedded temperature sensor can objectively estimate nightly wear

Why this is best: In this context, the relevant notebook finding is **Recorded adherence technology**: Wear-time is distinct from physiological reduction of events while the appliance is worn. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 135, §6.6*

Q0675 §6.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on adherence measurement, which description is specifically associated with **Recorded adherence technology**?

A. ≥80% of the night, ≥5 nights per week, with ≥7 h sleep. CPAP convention ≥4 h per night

B. At least 80% of the sleep period on at least five nights each week

C. A device-embedded temperature sensor can objectively estimate nightly wear

D. AHI <5 ~37% · AHI <10 ~52% · ≥50% reduction ~64% · ⅔ meaningful benefit, ⅓ none

E. Monthly clinical monitoring may be used during initial titration and adaptation

ANSWER: C · A device-embedded temperature sensor can objectively estimate nightly wear

Why this is best: Recorded adherence technology: Wear-time is distinct from physiological reduction of events while the appliance is worn. The distractors correspond to other named concepts in this learning area (First six months of therapy, MAD adherence criterion, Efficacy, AADSM compliance), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 135, §6.6*

Q0676 §6.1 · Clinical-context application

Clinical context. Three weeks after delivery, the patient reports improved snoring but changing jaw tolerance. During this case discussion, how should the candidate interpret **Typical overall titration time**?

A. Approximately two to five months

B. Start commonly around 50% of the measured maximum advancement

C. Allow approximately two weeks at the initial setting

D. AHI improvement is dose-dependent on advancement

E. Reassess within 30 days of insertion

ANSWER: A · Approximately two to five months

Why this is best: In this context, the relevant notebook finding is **Typical overall titration time**: Titration can take longer than PAP setup because tissues and joints require accommodation. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0677 §6.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on chief complaint and adjustment, which description is specifically associated with **Typical overall titration time**?

A. Allow approximately two weeks at the initial setting

B. AHI improvement is dose-dependent on advancement

C. Approximately two to five months

D. Start commonly around 50% of the measured maximum advancement

E. Reassess within 30 days of insertion

ANSWER: C · Approximately two to five months

Why this is best: Typical overall titration time: Titration can take longer than PAP setup because tissues and joints require accommodation. The distractors correspond to other named concepts in this learning area (Early appointment timing, Protocol, Acclimatisation interval, Initial protrusive range), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.1*

Q0678 §6.2 · Clinical-context application

Clinical context. At an early recall, the dentist reviews symptoms, dental discomfort and joint function. During this case discussion, how should the candidate interpret **Tolerance and adherence**?

A. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

B. comfort with nightly use, and whether side effects are reducing use

C. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

D. Reassess adherence, appliance stability, symptom change and residual OSA

E. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

ANSWER: B · comfort with nightly use, and whether side effects are reducing use

Why this is best: In this context, the relevant notebook finding is **Tolerance and adherence**: comfort with nightly use, and whether side effects are reducing use The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0679 §6.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 6**, in the revision material on symptoms and side-effects monitoring, which description is specifically associated with **Tolerance and adherence**?

A. cracks, warping, wear affecting fit and efficacy; confirm retention and stability

B. record and discuss symptoms at each visit; reassure about transient side effects and educate on managing them

C. Reassess adherence, appliance stability, symptom change and residual OSA

D. persistence or return of snoring, apnoea symptoms, sleepiness or fatigue; follow-up testing may be warranted

E. comfort with nightly use, and whether side effects are reducing use

ANSWER: E · comfort with nightly use, and whether side effects are reducing use

Why this is best: Tolerance and adherence: comfort with nightly use, and whether side effects are reducing use The distractors correspond to other named concepts in this learning area (Unexpected persistent snoring, Appliance condition, Effectiveness, Approach), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 132, §6.2*

Q0680 §6.4 · Targeted knowledge / discriminative SBA

Which description does the notebook specifically associate with **Remotely controlled mandibular positioner (RCMP)**?

A. The maximal comfortable therapeutic position, not maximal anatomical range

B. initial mandibular positioning → resolution or improvement of signs and symptoms → verification of efficacy by the physician

C. The dentist's objective data supplement the titration decision; they never replace physician verification

D. a remotely controlled positioner can be used with DISE to titrate mandibular position under direct visualisation

E. allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

ANSWER: E · allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement

Why this is best: Remotely controlled mandibular positioner (RCMP): allows objective, single-night titration during polysom- nography to identify the optimal therapeutic advancement. This is the best-performing predictive method available, rated "excellent" against clinical and craniofacial prediction ("poor") and nasopharyngoscopy ("fair to good") Competing statements refer to other named topics: Drug-induced sleep endoscopy, Signs and symptoms only, Both end in the same place, Limit of protrusion.

Notebook anchor: *Integrated notebook, page 139, §6.4*

TOPIC 07
Long-term oral appliance follow-up

60 questions | Full notebook pages 146–155 | Each question has a justification and linked learning code.

Q0681 §7.1 · Integrated clinical vignette

Three years after fitting, a patient gains 12 kg and develops recurrent snoring despite unchanged appliance setting. What should be assessed?

A. Possible disease progression with weight-related increased airway burden

B. Diagnose the new severity by BMI alone

C. Stop all follow-up after the first year

D. Assume the appliance still normalises AHI

E. Replace the device without a medical review

ANSWER: A · Possible disease progression with weight-related increased airway burden

Why this is best: Weight gain is a major modifier of OSA and may reduce an appliance's effectiveness, prompting symptom, device and physician review.

Notebook anchor: *Integrated notebook, Topic 7, §7.1*

Q0682 §7.1 · All-of-the-above integrated judgement

At the annual recall of a previously successful MAD user, which is the MOST COMPLETE set of review tasks?

A. Ask about symptoms and weight

B. Review medicines and alcohol

C. Inspect fit, wear and occlusion

D. Coordinate medical testing if disease recurs

E. All of the above

ANSWER: E · All of the above

Why this is best: Long-term outcomes depend on progressive disease, lifestyle modifiers, device function and interdisciplinary assessment.

Notebook anchor: *Integrated notebook, Topic 7, §7.1*

Q0683 §7.1 · Clinical-context application

Clinical context. At a four-year review, a previously stable MAD user has changes in weight, medication and sleep habits. During this case discussion, how should the candidate interpret **Alcohol**?

A. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

B. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

C. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

D. younger; less obese with smaller neck circumference (men); lower baseline AHI

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: E · before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

Why this is best: In this context, the relevant notebook finding is **Alcohol**: before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0684 §7.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on age, weight, alcohol, drugs and sleep, which description is specifically associated with **Alcohol**?

A. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

B. younger; less obese with smaller neck circumference (men); lower baseline AHI

C. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

D. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

E. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

ANSWER: D · before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

Why this is best: Alcohol: before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed The distractors correspond to other named concepts in this learning area (Medication change, Sleep hygiene and quantity, Weight change, Better long-term response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0685 §7.1 · None-of-the-above discrimination

Which finding alone proves that an unchanged 5-year-old MAD remains effective?

A. The case is not cracked

B. The patient still owns it

C. The setting has never changed

D. The patient does not recall a new problem

E. None of the above

ANSWER: E · None of the above

Why this is best: None of these substitutes for assessment of symptoms, adherence, device status and objective response when indicated.

Notebook anchor: *Integrated notebook, Topic 7, §7.1*

Q0686 §7.1 · Integrated clinical vignette

A previously successful patient begins using a sedating muscle relaxant at night. What should be documented?

A. A promise that adjustment will correct every drug effect

B. An assumption that medication cannot affect OSA

C. Only the device's colour

D. Only the patient's dental shade

E. Medication change as a possible contributor to recurrent airway collapse

ANSWER: E · Medication change as a possible contributor to recurrent airway collapse

Why this is best: Sedatives and muscle relaxants may worsen respiratory control or muscle tone; coordinate with the prescribing clinician.

Notebook anchor: *Integrated notebook, Topic 7, §7.1*

Q0687 §7.2 · Clinical-context application

Clinical context. The dentist documents the evolution of original snoring and sleepiness complaints over several years. During this case discussion, how should the candidate interpret **Evolution tracking**?

A. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

B. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

C. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

D. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

E. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

ANSWER: E · regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

Why this is best: In this context, the relevant notebook finding is **Evolution tracking**: regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0688 §7.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on longitudinal symptom documentation, which description is specifically associated with **Evolution tracking**?

A. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

B. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

C. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

D. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: C · regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

Why this is best: Evolution tracking: regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry The distractors correspond to other named concepts in this learning area (Sleep-observer reports, Self-reported symptoms, Alcohol, Medication change), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0689 §7.2 · Integrated clinical vignette

The patient says there is no snoring, but a partner witnesses pauses and choking again. What is the best response?

A. Assume absence of self-reported symptoms proves efficacy

B. Set the device at maximal protrusion immediately

C. Ignore partner reports

D. Diagnose new events without testing

E. Consider observer evidence and seek medical reassessment where indicated

ANSWER: E · Consider observer evidence and seek medical reassessment where indicated

Why this is best: Long-term review combines self-report, bed-partner observations and objective testing when deterioration is suspected.

Notebook anchor: *Integrated notebook, Topic 7, §7.2*

Q0690 §7.3 · Clinical-context application

Clinical context. Despite continued appliance use, the patient develops recurrent respiratory events and seeks treatment modification. During this case discussion, how should the candidate interpret **Key considerations**?

A. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

B. the optimal position is unique to each patient and determined by careful repeated titration

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

E. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

ANSWER: B · the optimal position is unique to each patient and determined by careful repeated titration

Why this is best: In this context, the relevant notebook finding is **Key considerations**: the optimal position is unique to each patient and determined by careful repeated titration; construction, fitting and adjustment should be by a dentist with expertise; the process is always a balance of therapeutic effect against side effects The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 147, §7.3*

Q0691 §7.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on treatment modification with progression, which description is specifically associated with **Key considerations**?

A. the optimal position is unique to each patient and determined by careful repeated titration

B. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

E. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

ANSWER: A · the optimal position is unique to each patient and determined by careful repeated titration

Why this is best: Key considerations: the optimal position is unique to each patient and determined by careful repeated titration; construction, fitting and adjustment should be by a dentist with expertise; the process is always a balance of therapeutic effect against side effects The distractors correspond to other named concepts in this learning area (Progression of the disease, Long-term dental change, Poorer response, Self-reported symptoms), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 147, §7.3*

Q0692 §7.4 · Integrated clinical vignette

At an annual review, the device looks intact but the advancement mechanism slips during sleep. What is the priority?

A. Assume appearance proves function

B. Repair or remake the mechanism and verify the retained protrusive setting

C. Stop all cleaning instructions

D. Disregard slipping if the patient is happy

E. Document no changes

ANSWER: B · Repair or remake the mechanism and verify the retained protrusive setting

Why this is best: Hardware functionality and stable settings are essential; visible condition alone cannot prove continued delivery of therapy.

Notebook anchor: *Integrated notebook, Topic 7, §7.4*

Q0693 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Better response**?

A. ensure the appliance does not interfere with the bite or cause premature contacts

B. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

C. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

D. gently tug to verify the device resists displacement during normal oral function

E. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

ANSWER: E · younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

Why this is best: In this context, the relevant notebook finding is **Better response**: younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 147, §7.4*

Q0694 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Better response**?

A. gently tug to verify the device resists displacement during normal oral function

B. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

E. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

ANSWER: B · younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

Why this is best: Better response: younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant The distractors correspond to other named concepts in this learning area (Poorer response, Retention, Occlusal contacts, Follow-up), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 147, §7.4*

Q0695 §7.4 · Integrated clinical vignette

An old appliance has accumulated deposits, loss of retention and tooth migration. What should the review include?

A. No dental examination because OSA is medical

B. Hygiene, device stability, occlusion and treatment redesign if needed

C. An automatic prescription for CPAP without discussion

D. Only a snoring recording

E. Only an ESS score

ANSWER: B · Hygiene, device stability, occlusion and treatment redesign if needed

Why this is best: Yearly dental review examines wear, hygiene, fitting, occlusal change, adherence and evolving medical risk.

Notebook anchor: *Integrated notebook, Topic 7, §7.4*

Q0696 §7.1 · Clinical-context application

Clinical context. At a four-year review, a previously stable MAD user has changes in weight, medication and sleep habits. During this case discussion, how should the candidate interpret **Weight change**?

A. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

B. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

C. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

D. younger; less obese with smaller neck circumference (men); lower baseline AHI

E. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

ANSWER: B · gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

Why this is best: In this context, the relevant notebook finding is **Weight change**: gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes. Significant fluctuation requires reassessment of fit and efficacy, and is a specific trigger for repeat objective testing. Recall: 10% gain → ~32% AHI rise; 10% loss → ~26% fall The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0697 §7.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on age, weight, alcohol, drugs and sleep, which description is specifically associated with **Weight change**?

A. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

B. younger; less obese with smaller neck circumference (men); lower baseline AHI

C. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

D. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: D · gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

Why this is best: Weight change: gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes. Significant fluctuation requires reassessment of fit and efficacy, and is a specific trigger for repeat objective testing. Recall: 10% gain → ~32% AHI rise; 10% loss → ~26% fall The distractors correspond to other named concepts in this learning area (Sleep hygiene and quantity, Better long-term response, Medication change, Alcohol), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0698 §7.3 · Integrated clinical vignette

A patient with worsening OSA cannot tolerate additional protrusion. What is the best plan?

A. Discuss multidisciplinary alternatives or combination therapy rather than force further advancement

B. Declare long-term follow-up unnecessary

C. Increase beyond comfort indefinitely

D. Avoid discussing PAP

E. Assume positional measures cannot help anyone

ANSWER: A · Discuss multidisciplinary alternatives or combination therapy rather than force further advancement

Why this is best: Progressive disease may outstrip tolerable appliance settings; the team may consider PAP, positional measures, weight strategies, redesign or alternatives.

Notebook anchor: *Integrated notebook, Topic 7, §7.3*

Q0699 §7.2 · Clinical-context application

Clinical context. The dentist documents the evolution of original snoring and sleepiness complaints over several years. During this case discussion, how should the candidate interpret **Self-reported symptoms**?

A. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

B. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

C. the optimal position is unique to each patient and determined by careful repeated titration

D. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

E. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

ANSWER: E · the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

Why this is best: In this context, the relevant notebook finding is **Self-reported symptoms**: the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0700 §7.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on longitudinal symptom documentation, which description is specifically associated with **Self-reported symptoms**?

A. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

B. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

E. the optimal position is unique to each patient and determined by careful repeated titration

ANSWER: B · the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

Why this is best: Self-reported symptoms: the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications The distractors correspond to other named concepts in this learning area (Sleep-observer reports, Evolution tracking, Progression of the disease, Key considerations), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0701 §7.1 · Integrated clinical vignette

A patient now drinks substantial alcohol immediately before bed. What impact is most likely?

A. No effect on oral-appliance outcomes

B. Guaranteed reduction in AHI

C. Increased airway collapsibility and reduced control of sleep-disordered breathing

D. Prevention of REM-related events

E. Improved genioglossus responsiveness

ANSWER: C · Increased airway collapsibility and reduced control of sleep-disordered breathing

Why this is best: Evening alcohol can worsen upper-airway collapse, undermining established therapy; counselling is part of follow-up.

Notebook anchor: *Integrated notebook, Topic 7, §7.1*

Q0702 §7.3 · Clinical-context application

Clinical context. Despite continued appliance use, the patient develops recurrent respiratory events and seeks treatment modification. During this case discussion, how should the candidate interpret **Progression of the disease**?

A. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

B. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. the optimal position is unique to each patient and determined by careful repeated titration

E. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

ANSWER: C · Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

Why this is best: In this context, the relevant notebook finding is **Progression of the disease**: Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 151, §7.3*

Q0703 §7.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on treatment modification with progression, which description is specifically associated with **Progression of the disease**?

A. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

B. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

E. the optimal position is unique to each patient and determined by careful repeated titration

ANSWER: C · Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

Why this is best: Progression of the disease: Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase The distractors correspond to other named concepts in this learning area (Long-term dental change, Key considerations, Self-reported symptoms, Follow-up), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 151, §7.3*

Q0704 §7.2 · Integrated clinical vignette

Baseline symptoms were recorded verbally only; no instrument was completed. How can future monitoring be improved?

A. Stop recording dental changes

B. Avoid written communication with the physician

C. Begin consistent validated symptom and adherence records and track them serially

D. Use a different tool every appointment

E. Rely solely on the technician's memory

ANSWER: C · Begin consistent validated symptom and adherence records and track them serially

Why this is best: A reproducible longitudinal record supports recognition of recurrence and evaluates treatment modification.

Notebook anchor: *Integrated notebook, Topic 7, §7.2*

Q0705 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Handling**?

A. always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

B. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

C. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

D. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

E. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

ANSWER: A · always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

Why this is best: In this context, the relevant notebook finding is **Handling**: always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0706 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Handling**?

A. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

B. always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

C. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

D. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

E. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

ANSWER: B · always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

Why this is best: Handling: always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance The distractors correspond to other named concepts in this learning area (Storage, Hardware functionality, Better response, Seating accuracy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0707 §7.4 · Integrated clinical vignette

The patient asks why a dental appliance needs review once the AHI has been controlled. What is the best explanation?

A. OSA and oral structures change over time; both disease control and the appliance need ongoing surveillance

B. Only CPAP needs maintenance

C. The dentist may replace medical assessment entirely

D. Changes in occlusion never affect fit

E. Once fitted, the setting is lifelong without exception

ANSWER: A · OSA and oral structures change over time; both disease control and the appliance need ongoing surveillance

Why this is best: OSA is often progressive while dentition, periodontal support, retention and materials change; annual dental and periodic medical follow-up are needed.

Notebook anchor: *Integrated notebook, Topic 7, §7.4*

Q0708 §7.1 · Clinical-context application

Clinical context. At a four-year review, a previously stable MAD user has changes in weight, medication and sleep habits. During this case discussion, how should the candidate interpret **Sleep hygiene and quantity**?

A. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

B. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

C. younger; less obese with smaller neck circumference (men); lower baseline AHI

D. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: A · good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

Why this is best: In this context, the relevant notebook finding is **Sleep hygiene and quantity**: good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea. Poor habits blunt the daytime alertness gains and potentiate severity. A patient sleeping five hours will not feel the benefit of a well-titrated appliance, and may wrongly conclude it has failed. 7.2 Documenting the evolution of the complaint The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0709 §7.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on age, weight, alcohol, drugs and sleep, which description is specifically associated with **Sleep hygiene and quantity**?

A. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

B. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

C. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

D. younger; less obese with smaller neck circumference (men); lower baseline AHI

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: B · good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

Why this is best: Sleep hygiene and quantity: good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea. Poor habits blunt the daytime alertness gains and potentiate severity. A patient sleeping five hours will not feel the benefit of a well-titrated appliance, and may wrongly conclude it has failed. 7.2 Documenting the evolution of the complaint The distractors correspond to other named concepts in this learning area (Weight change, Better long-term response, Medication change, Alcohol), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0710 §7.3 · Integrated clinical vignette

A patient's current treatment AHI is markedly higher than the historical value despite good nightly use. What does this most strongly justify?

A. Ignore the change because use is high

B. Use only a morning aligner to treat residual AHI

C. Define snoring reduction as an objective cure

D. Physician-led repeat assessment of efficacy and evolving disease

E. Conclude that adherence is the cause

ANSWER: D · Physician-led repeat assessment of efficacy and evolving disease

Why this is best: Adequate use with worsening objective indices points toward disease progression, device failure or phenotype change requiring reassessment.

Notebook anchor: *Integrated notebook, Topic 7, §7.3*

Q0711 §7.2 · Clinical-context application

Clinical context. The dentist documents the evolution of original snoring and sleepiness complaints over several years. During this case discussion, how should the candidate interpret **Sleep-observer reports**?

A. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

B. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

C. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

D. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

E. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

ANSWER: B · the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

Why this is best: In this context, the relevant notebook finding is **Sleep-observer reports**: the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0712 §7.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on longitudinal symptom documentation, which description is specifically associated with **Sleep-observer reports**?

A. the patient's perception of severity and therapy effect: ESS, sleep diaries, mobile applications

B. the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

C. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

D. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

E. regular follow-ups, app-based tracking, and repeated administration of the same baseline questionnaires. Objective data integration — compliance-monitor data, CPAP usage logs, polysomnography, oximetry

ANSWER: B · the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items

Why this is best: Sleep-observer reports: the bed partner or family on snoring intensity, apnoeas and restless sleep, offering "objective-seeming" data that complement self-report: partner observations, STOP-BANG items The distractors correspond to other named concepts in this learning area (Evolution tracking, Self-reported symptoms, Medication change, Alcohol), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.2*

Q0713 §7.3 · Clinical-context application

Clinical context. Despite continued appliance use, the patient develops recurrent respiratory events and seeks treatment modification. During this case discussion, how should the candidate interpret **Long-term dental change**?

A. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

B. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. the optimal position is unique to each patient and determined by careful repeated titration

E. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

ANSWER: E · Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

Why this is best: In this context, the relevant notebook finding is **Long-term dental change**: Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position. Critically, a forward shift of the lower teeth relative to the upper reduces the effective degree of mandibular advancement, putting the efficacy of treatment at risk. This is why appliances in long-term series had to be continuously advanced simply to compensate for occlusal drift The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 151, §7.3*

Q0714 §7.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on treatment modification with progression, which description is specifically associated with **Long-term dental change**?

A. Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

B. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

C. Sleep apnoea worsens over time, and the optimal advancement needed to keep the airway open may increase

D. the optimal position is unique to each patient and determined by careful repeated titration

E. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

ANSWER: A · Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position

Why this is best: Long-term dental change: Prolonged appliance use produces dentoskeletal change — reduced overbite and overjet, altered incisor position. Critically, a forward shift of the lower teeth relative to the upper reduces the effective degree of mandibular advancement, putting the efficacy of treatment at risk. This is why appliances in long-term series had to be continuously advanced simply to compensate for occlusal drift The distractors correspond to other named concepts in this learning area (Progression of the disease, Key considerations, Seating accuracy, Follow-up), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 151, §7.3*

Q0715 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Poorer response**?

A. check for cracks, crazing or rough spots that could irritate the oral tissues

B. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. gently tug to verify the device resists displacement during normal oral function

E. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

ANSWER: B · supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

Why this is best: In this context, the relevant notebook finding is **Poorer response**: supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0716 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Poorer response**?

A. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

B. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. check for cracks, crazing or rough spots that could irritate the oral tissues

E. gently tug to verify the device resists displacement during normal oral function

ANSWER: A · supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

Why this is best: Poorer response: supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment The distractors correspond to other named concepts in this learning area (Better response, Surface integrity, Retention, Occlusal contacts), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0717 §7.1 · Clinical-context application

Clinical context. At a four-year review, a previously stable MAD user has changes in weight, medication and sleep habits. During this case discussion, how should the candidate interpret **Medication change**?

A. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

B. younger; less obese with smaller neck circumference (men); lower baseline AHI

C. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

D. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: D · drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

Why this is best: In this context, the relevant notebook finding is **Medication change**: drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy. Emerging research explores combining pharmacotherapy with appliance therapy, but changes should be monitored closely. Ask about new medications at every review The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0718 §7.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on age, weight, alcohol, drugs and sleep, which description is specifically associated with **Medication change**?

A. younger; less obese with smaller neck circumference (men); lower baseline AHI

B. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

C. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

D. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

E. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

ANSWER: D · drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

Why this is best: Medication change: drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy. Emerging research explores combining pharmacotherapy with appliance therapy, but changes should be monitored closely. Ask about new medications at every review The distractors correspond to other named concepts in this learning area (Alcohol, Sleep hygiene and quantity, Weight change, Better long-term response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0719 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Occlusal contacts**?

A. check for cracks, crazing or rough spots that could irritate the oral tissues

B. note discolouration, thinning or deformation of the acrylic or thermoplastic

C. gently tug to verify the device resists displacement during normal oral function

D. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

E. ensure the appliance does not interfere with the bite or cause premature contacts

ANSWER: E · ensure the appliance does not interfere with the bite or cause premature contacts

Why this is best: In this context, the relevant notebook finding is **Occlusal contacts**: ensure the appliance does not interfere with the bite or cause premature contacts The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0720 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Occlusal contacts**?

A. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

B. note discolouration, thinning or deformation of the acrylic or thermoplastic

C. check for cracks, crazing or rough spots that could irritate the oral tissues

D. gently tug to verify the device resists displacement during normal oral function

E. ensure the appliance does not interfere with the bite or cause premature contacts

ANSWER: E · ensure the appliance does not interfere with the bite or cause premature contacts

Why this is best: Occlusal contacts: ensure the appliance does not interfere with the bite or cause premature contacts The distractors correspond to other named concepts in this learning area (Retention, Material wear, Surface integrity, Poorer response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0721 §7.1 · Clinical-context application

Clinical context. At a four-year review, a previously stable MAD user has changes in weight, medication and sleep habits. During this case discussion, how should the candidate interpret **Better long-term response**?

A. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

B. younger; less obese with smaller neck circumference (men); lower baseline AHI

C. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

D. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

E. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

ANSWER: B · younger; less obese with smaller neck circumference (men); lower baseline AHI

Why this is best: In this context, the relevant notebook finding is **Better long-term response**: younger; less obese with smaller neck circumference (men); lower baseline AHI. Supine and REM sleep predict a lower response rate — the splint is relatively poor at alleviating REM-predominant OSA compared with NREM- predominant or non-stage-dependent disease; conversely positional OSA favours the appliance. Gender and polysomnographic phenotype are, in some analyses, unrelated to outcome — predictors are probabilistic, not deterministic The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0722 §7.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on age, weight, alcohol, drugs and sleep, which description is specifically associated with **Better long-term response**?

A. before sleep it exacerbates collapse and increases severity, undermining benefit; even moderate intake may cause transient desaturation despite appliance use. Advise avoidance in the hours before bed

B. gain worsens OSA and may reduce effectiveness by altering airway anatomy and appliance fit; loss improves outcomes

C. drugs affecting the CNS or muscle tone (sedatives, hypnotics, opioids, muscle relaxants) may reduce airway patency or alter architecture, potentially reducing efficacy

D. good hygiene and adequate duration support success by stabilising architecture and reducing factors that worsen apnoea

E. younger; less obese with smaller neck circumference (men); lower baseline AHI

ANSWER: E · younger; less obese with smaller neck circumference (men); lower baseline AHI

Why this is best: Better long-term response: younger; less obese with smaller neck circumference (men); lower baseline AHI. Supine and REM sleep predict a lower response rate — the splint is relatively poor at alleviating REM-predominant OSA compared with NREM- predominant or non-stage-dependent disease; conversely positional OSA favours the appliance. Gender and polysomnographic phenotype are, in some analyses, unrelated to outcome — predictors are probabilistic, not deterministic The distractors correspond to other named concepts in this learning area (Weight change, Sleep hygiene and quantity, Medication change, Alcohol), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 146, §7.1*

Q0723 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Weight**?

A. rinse immediately after removal to wash away saliva and debris

B. check for cracks, crazing or rough spots that could irritate the oral tissues

C. gently tug to verify the device resists displacement during normal oral function

D. 10% gain → +32% AHI; 10% loss → −26% AHI

E. ensure the appliance does not interfere with the bite or cause premature contacts

ANSWER: D · 10% gain → +32% AHI; 10% loss → −26% AHI

Why this is best: In this context, the relevant notebook finding is **Weight**: 10% gain → +32% AHI; 10% loss → −26% AHI The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 147, §7.4*

Q0724 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Weight**?

A. rinse immediately after removal to wash away saliva and debris

B. 10% gain → +32% AHI; 10% loss → −26% AHI

C. gently tug to verify the device resists displacement during normal oral function

D. check for cracks, crazing or rough spots that could irritate the oral tissues

E. ensure the appliance does not interfere with the bite or cause premature contacts

ANSWER: B · 10% gain → +32% AHI; 10% loss → −26% AHI

Why this is best: Weight: 10% gain → +32% AHI; 10% loss → −26% AHI The distractors correspond to other named concepts in this learning area (Cleaning, Surface integrity, Retention, Occlusal contacts), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 147, §7.4*

Q0725 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Follow-up**?

A. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

B. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

C. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

D. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

E. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

ANSWER: B · every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

Why this is best: In this context, the relevant notebook finding is **Follow-up**: every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0726 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Follow-up**?

A. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

B. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

C. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

D. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

E. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

ANSWER: D · every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

Why this is best: Follow-up: every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years The distractors correspond to other named concepts in this learning area (Seating accuracy, Better response, Better response, Poorer response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0727 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Surface integrity**?

A. gently tug to verify the device resists displacement during normal oral function

B. note discolouration, thinning or deformation of the acrylic or thermoplastic

C. rinse immediately after removal to wash away saliva and debris

D. check for cracks, crazing or rough spots that could irritate the oral tissues

E. ensure the appliance does not interfere with the bite or cause premature contacts

ANSWER: D · check for cracks, crazing or rough spots that could irritate the oral tissues

Why this is best: In this context, the relevant notebook finding is **Surface integrity**: check for cracks, crazing or rough spots that could irritate the oral tissues The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0728 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Surface integrity**?

A. note discolouration, thinning or deformation of the acrylic or thermoplastic

B. gently tug to verify the device resists displacement during normal oral function

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. rinse immediately after removal to wash away saliva and debris

E. check for cracks, crazing or rough spots that could irritate the oral tissues

ANSWER: E · check for cracks, crazing or rough spots that could irritate the oral tissues

Why this is best: Surface integrity: check for cracks, crazing or rough spots that could irritate the oral tissues The distractors correspond to other named concepts in this learning area (Retention, Occlusal contacts, Material wear, Cleaning), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0729 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Hardware functionality**?

A. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

B. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

C. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

D. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

E. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

ANSWER: A · where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

Why this is best: In this context, the relevant notebook finding is **Hardware functionality**: where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0730 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Hardware functionality**?

A. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

B. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

C. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

D. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

E. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

ANSWER: A · where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

Why this is best: Hardware functionality: where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding The distractors correspond to other named concepts in this learning area (Better response, Seating accuracy, Storage, Follow-up), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0731 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Material wear**?

A. note discolouration, thinning or deformation of the acrylic or thermoplastic

B. ensure the appliance does not interfere with the bite or cause premature contacts

C. check for cracks, crazing or rough spots that could irritate the oral tissues

D. gently tug to verify the device resists displacement during normal oral function

E. rinse immediately after removal to wash away saliva and debris

ANSWER: A · note discolouration, thinning or deformation of the acrylic or thermoplastic

Why this is best: In this context, the relevant notebook finding is **Material wear**: note discolouration, thinning or deformation of the acrylic or thermoplastic The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0732 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Material wear**?

A. gently tug to verify the device resists displacement during normal oral function

B. rinse immediately after removal to wash away saliva and debris

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. note discolouration, thinning or deformation of the acrylic or thermoplastic

E. check for cracks, crazing or rough spots that could irritate the oral tissues

ANSWER: D · note discolouration, thinning or deformation of the acrylic or thermoplastic

Why this is best: Material wear: note discolouration, thinning or deformation of the acrylic or thermoplastic The distractors correspond to other named concepts in this learning area (Retention, Occlusal contacts, Surface integrity, Cleaning), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0733 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Retention**?

A. gently tug to verify the device resists displacement during normal oral function

B. note discolouration, thinning or deformation of the acrylic or thermoplastic

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

E. check for cracks, crazing or rough spots that could irritate the oral tissues

ANSWER: A · gently tug to verify the device resists displacement during normal oral function

Why this is best: In this context, the relevant notebook finding is **Retention**: gently tug to verify the device resists displacement during normal oral function The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0734 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Retention**?

A. check for cracks, crazing or rough spots that could irritate the oral tissues

B. younger, less obese, smaller neck (men), lower baseline AHI, positional OSA, NREM-predominant

C. gently tug to verify the device resists displacement during normal oral function

D. ensure the appliance does not interfere with the bite or cause premature contacts

E. note discolouration, thinning or deformation of the acrylic or thermoplastic

ANSWER: C · gently tug to verify the device resists displacement during normal oral function

Why this is best: Retention: gently tug to verify the device resists displacement during normal oral function The distractors correspond to other named concepts in this learning area (Occlusal contacts, Material wear, Surface integrity, Better response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0735 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Seating accuracy**?

A. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

B. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

C. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

D. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

E. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

ANSWER: D · confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

Why this is best: In this context, the relevant notebook finding is **Seating accuracy**: confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0736 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Seating accuracy**?

A. supine-predominant sleep during the study, REM-predominant OSA, weight gain during treatment

B. every 6 months in year 1, at least annually thereafter; physician reassessment approximately every 5 years

C. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

D. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

E. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

ANSWER: D · confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

Why this is best: Seating accuracy: confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking The distractors correspond to other named concepts in this learning area (Follow-up, Better response, Hardware functionality, Poorer response), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 152, §7.4*

Q0737 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Cleaning**?

A. check for cracks, crazing or rough spots that could irritate the oral tissues

B. 10% gain → ~32% AHI increase; 10% loss → ~26% AHI decrease

C. ensure the appliance does not interfere with the bite or cause premature contacts

D. rinse immediately after removal to wash away saliva and debris

E. note discolouration, thinning or deformation of the acrylic or thermoplastic

ANSWER: D · rinse immediately after removal to wash away saliva and debris

Why this is best: In this context, the relevant notebook finding is **Cleaning**: rinse immediately after removal to wash away saliva and debris; brush gently with a soft toothbrush and a non-abrasive, fragrance-free cleanser, avoiding toothpaste; disinfect weekly in a denture-safe soak per manufacturer instructions The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0738 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Cleaning**?

A. ensure the appliance does not interfere with the bite or cause premature contacts

B. check for cracks, crazing or rough spots that could irritate the oral tissues

C. note discolouration, thinning or deformation of the acrylic or thermoplastic

D. rinse immediately after removal to wash away saliva and debris

E. 10% gain → ~32% AHI increase; 10% loss → ~26% AHI decrease

ANSWER: D · rinse immediately after removal to wash away saliva and debris

Why this is best: Cleaning: rinse immediately after removal to wash away saliva and debris; brush gently with a soft toothbrush and a non-abrasive, fragrance-free cleanser, avoiding toothpaste; disinfect weekly in a denture-safe soak per manufacturer instructions The distractors correspond to other named concepts in this learning area (Weight, Material wear, Surface integrity, Occlusal contacts), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0739 §7.4 · Clinical-context application

Clinical context. At an annual appliance appointment, the clinician inspects the device, occlusion and oral structures. During this case discussion, how should the candidate interpret **Storage**?

A. always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

B. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

C. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

D. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

E. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

ANSWER: B · store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

Why this is best: In this context, the relevant notebook finding is **Storage**: store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources. Emphasise: "Consistent daily care helps prevent odour, staining and bacterial growth. A few minutes each day will save you discomfort and costly repairs later." 4. Scheduling and next steps The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 153, §7.4*

Q0740 §7.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 7**, in the revision material on annual device and clinical review, which description is specifically associated with **Storage**?

A. store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

B. where metal components are present (clasps, hinges, screws), ensure they are free of corrosion and move smoothly without binding

C. always remove over a soft surface or a basin filled with water to prevent accidental drop damage; avoid hot water and direct sunlight, which warp the appliance

D. confirm the appliance fully engages the intended undercuts, or rests on the occlusal surfaces, without rocking

E. younger, less obese, smaller neck circumference (men), lower baseline AHI, positional OSA, NREM- predominant disease

ANSWER: A · store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources

Why this is best: Storage: store dry in the ventilated case when not in use — never wrapped in tissue or loose in a pocket; keep away from pets, children and heat sources. Emphasise: "Consistent daily care helps prevent odour, staining and bacterial growth. A few minutes each day will save you discomfort and costly repairs later." 4. Scheduling and next steps The distractors correspond to other named concepts in this learning area (Handling, Hardware functionality, Better response, Seating accuracy), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 153, §7.4*

TOPIC 08
Paediatric sleep-disordered breathing

100 questions | Full notebook pages 156–182 | Each question has a justification and linked learning code.

Q0741 §8.3 · Integrated clinical vignette

A child with paediatric OSA is irritable and underperforming in school but not sleepy by adult measures. Why is the presentation plausible?

A. Only adults can experience OSA

B. Tonsil size predicts the exact cognitive score

C. Normal ESS in children rules out OSA

D. Sleep disruption cannot affect learning

E. Paediatric SDB often presents with behavioural and cognitive disturbance

ANSWER: E · Paediatric SDB often presents with behavioural and cognitive disturbance

Why this is best: Children may manifest sleep fragmentation through hyperactivity, inattention, learning and behavioural problems.

Notebook anchor: *Integrated notebook, Topic 8, §8.3*

Q0742 §8.1 · All-of-the-above integrated judgement

A snoring child has behavioural problems and tonsillar enlargement. What is the MOST COMPLETE appropriate assessment?

A. Ask about sleep and daytime behaviour

B. Examine and grade the airway findings

C. Consider medical and syndromic risk

D. Refer for paediatric physician/ENT assessment

E. All of the above

ANSWER: E · All of the above

Why this is best: Paediatric SDB screening integrates symptom, anatomy, comorbidity and timely medical referral; PSG remains diagnostic.

Notebook anchor: *Integrated notebook, Topic 8, §8.1*

Q0743 §8.1 · Clinical-context application

Clinical context. The parents of a preschool child ask how common sleep-disordered breathing is at this age. During this case discussion, how should the candidate interpret **General prevalence of paediatric OSA**?

A. Attention, hyperactivity and behavioural or learning difficulties may predominate

B. Adenotonsillar hypertrophy is the predominant anatomical factor

C. 3–12% (some sources cite 10–20%)

D. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

E. Approximately 1–5% of children, with estimates near 2–3%

ANSWER: E · Approximately 1–5% of children, with estimates near 2–3%

Why this is best: In this context, the relevant notebook finding is **General prevalence of paediatric OSA**: Prevalence varies by population, scoring and diagnostic threshold. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0744 §8.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric prevalence, which description is specifically associated with **General prevalence of paediatric OSA**?

A. 3–12% (some sources cite 10–20%)

B. Approximately 1–5% of children, with estimates near 2–3%

C. Attention, hyperactivity and behavioural or learning difficulties may predominate

D. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

E. Adenotonsillar hypertrophy is the predominant anatomical factor

ANSWER: B · Approximately 1–5% of children, with estimates near 2–3%

Why this is best: General prevalence of paediatric OSA: Prevalence varies by population, scoring and diagnostic threshold. The distractors correspond to other named concepts in this learning area (Habitual snoring, The four contributing mechanisms, Common preschool cause, Paediatric daytime presentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0745 §8.1 · None-of-the-above discrimination

Which single dental finding definitively rules out paediatric OSA?

A. Normal incisor overjet

B. A normal dental arch width

C. No tooth grinding

D. No crowding

E. None of the above

ANSWER: E · None of the above

Why this is best: No isolated dental finding rules out paediatric OSA; assessment and testing are clinical and multidisciplinary.

Notebook anchor: *Integrated notebook, Topic 8, §8.1*

Q0746 §8.2 · Integrated clinical vignette

A preschool child develops obstructive breathing during sleep despite no obesity. The tonsils are markedly enlarged. Which aetiology is most typical?

A. A malformed dental retainer as the only explanation

B. Adult-type smoking exposure as the necessary cause

C. Fixed central sleep apnoea in all children

D. Isolated lower incisor crowding as the diagnosis

E. Adenotonsillar hypertrophy narrowing the pharyngeal airway

ANSWER: E · Adenotonsillar hypertrophy narrowing the pharyngeal airway

Why this is best: Adenotonsillar enlargement is the leading cause in younger children; aetiology changes with growth and comorbidity.

Notebook anchor: *Integrated notebook, Topic 8, §8.2*

Q0747 §8.2 · Clinical-context application

Clinical context. A child with chronic mouth breathing and enlarged tonsils is being assessed for upper-airway restriction. During this case discussion, how should the candidate interpret **Common preschool cause**?

A. Attention, hyperactivity and behavioural or learning difficulties may predominate

B. 3–12% (some sources cite 10–20%)

C. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

D. Approximately 1–5% of children, with estimates near 2–3%

E. Adenotonsillar hypertrophy is the predominant anatomical factor

ANSWER: E · Adenotonsillar hypertrophy is the predominant anatomical factor

Why this is best: In this context, the relevant notebook finding is **Common preschool cause**: Age modifies aetiology: obesity becomes increasingly relevant in adolescents. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 156, §8.2*

Q0748 §8.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on aetiology and pathophysiology, which description is specifically associated with **Common preschool cause**?

A. 3–12% (some sources cite 10–20%)

B. Adenotonsillar hypertrophy is the predominant anatomical factor

C. Approximately 1–5% of children, with estimates near 2–3%

D. Attention, hyperactivity and behavioural or learning difficulties may predominate

E. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

ANSWER: B · Adenotonsillar hypertrophy is the predominant anatomical factor

Why this is best: Common preschool cause: Age modifies aetiology: obesity becomes increasingly relevant in adolescents. The distractors correspond to other named concepts in this learning area (Prevalence, General prevalence of paediatric OSA, Paediatric daytime presentation, Habitual snoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 156, §8.2*

Q0749 §8.3 · Integrated clinical vignette

A six-year-old snores heavily and has hyperactivity and poor school concentration without daytime sleepiness. What should the dentist consider?

A. Adult ESS alone establishes severity

B. OSA is excluded because the child is hyperactive

C. Possible paediatric OSA with behavioural presentation

D. Treat with a mandibular splint without referral

E. Only depression can explain such behaviour

ANSWER: C · Possible paediatric OSA with behavioural presentation

Why this is best: Children often manifest SDB through hyperactivity, inattention, behavioural problems or growth concerns rather than classic adult sleepiness.

Notebook anchor: *Integrated notebook, Topic 8, §8.3*

Q0750 §8.3 · Clinical-context application

Clinical context. A school-age child snores and has new behavioural and attention difficulties. During this case discussion, how should the candidate interpret **Paediatric daytime presentation**?

A. Attention, hyperactivity and behavioural or learning difficulties may predominate

B. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

C. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

D. chronic medical illness; anxiety and stress; prescribed psychotropics

E. Adenotonsillar hypertrophy is the predominant anatomical factor

ANSWER: A · Attention, hyperactivity and behavioural or learning difficulties may predominate

Why this is best: In this context, the relevant notebook finding is **Paediatric daytime presentation**: Children may not show the same prominent daytime somnolence as adults. The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 157, §8.3*

Q0751 §8.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on signs and symptoms, which description is specifically associated with **Paediatric daytime presentation**?

A. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

B. Attention, hyperactivity and behavioural or learning difficulties may predominate

C. chronic medical illness; anxiety and stress; prescribed psychotropics

D. Adenotonsillar hypertrophy is the predominant anatomical factor

E. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

ANSWER: B · Attention, hyperactivity and behavioural or learning difficulties may predominate

Why this is best: Paediatric daytime presentation: Children may not show the same prominent daytime somnolence as adults. The distractors correspond to other named concepts in this learning area (Common preschool cause, School start times, Prevalence, Other causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 157, §8.3*

Q0752 §8.8 · Integrated clinical vignette

A child has an obstructive AHI of 6 per hour. Which severity category follows the notebook?

A. Unclassifiable because adults use AHI 6 as mild

B. Severe only above 30/h

C. Mild

D. Moderate paediatric OSA

E. Normal

ANSWER: D · Moderate paediatric OSA

Why this is best: The child thresholds differ from adult thresholds: 1–<5 mild, 5–<10 moderate, and at least 10 severe.

Notebook anchor: *Integrated notebook, Topic 8, §8.8*

Q0753 §8.4 · Clinical-context application

Clinical context. A child with inadequate sleep develops learning and attention problems. During this case discussion, how should the candidate interpret **Recommended duration**?

A. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

B. <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

C. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

D. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

E. attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

ANSWER: B · <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

Why this is best: In this context, the relevant notebook finding is **Recommended duration**: <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 156, §8.4*

Q0754 §8.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on insufficient sleep and development, which description is specifically associated with **Recommended duration**?

A. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

B. attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

C. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

D. <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

E. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

ANSWER: D · <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

Why this is best: Recommended duration: <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h The distractors correspond to other named concepts in this learning area (Less than recommended, Electronic media, Tongue posture and function, Jaw size and position), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 156, §8.4*

Q0755 §8.7 · Integrated clinical vignette

A five-year-old with large obstructive tonsils has PSG-confirmed OSA. What is generally first-line treatment?

A. ENT assessment for adenotonsillectomy

B. A panoramic radiograph as definitive management

C. Nasal EPAP alone without assessment

D. No treatment until adulthood

E. A dentist-issued mandibular advancement splint as standard first-line

ANSWER: A · ENT assessment for adenotonsillectomy

Why this is best: Adenotonsillar hypertrophy is common in younger children; adenotonsillectomy is typically the first-line intervention when appropriate.

Notebook anchor: *Integrated notebook, Topic 8, §8.7*

Q0756 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Mental health problems**?

A. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

B. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

C. anxiety, stress sleep, leading to residual daytime sleepiness

D. Sleep pressure builds more slowly with time awake in adolescents

E. chronic medical illness; anxiety and stress; prescribed psychotropics

ANSWER: C · anxiety, stress sleep, leading to residual daytime sleepiness

Why this is best: In this context, the relevant notebook finding is **Mental health problems**: anxiety, stress sleep, leading to residual daytime sleepiness The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 167, §8.5*

Q0757 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Jaw size and position**?

A. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

B. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

C. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

D. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

E. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

ANSWER: E · retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

Why this is best: Jaw size and position: retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length. Monitor jaw growth regularly in growing patients, as intervention may be considered if structural concerns persist The distractors correspond to other named concepts in this learning area (Tongue posture and function, Cognitive, behavioural and developmental assessment, Palate and arches, Medical, dental and family history), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0758 §8.9 · Integrated clinical vignette

A child with Down syndrome appears symptom-free but is approaching four years of age. Which plan accords with the notebook?

A. Use adult AHI severity cutoffs exclusively

B. Assume a questionnaire rules out OSA

C. Exclude OSA due to absent snoring

D. Wait for school failure before evaluation

E. Arrange physician-led PSG assessment even if symptoms are not reported

ANSWER: E · Arrange physician-led PSG assessment even if symptoms are not reported

Why this is best: OSA is common and may be clinically silent in Down syndrome; PSG screening by age four is highlighted in the notes.

Notebook anchor: *Integrated notebook, Topic 8, §8.9*

Q0759 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Jaw size and position**?

A. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

B. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

C. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

D. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

E. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

ANSWER: D · retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

Why this is best: In this context, the relevant notebook finding is **Jaw size and position**: retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length. Monitor jaw growth regularly in growing patients, as intervention may be considered if structural concerns persist The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0760 §8.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on differences from adult sdb, which description is specifically associated with **Diagnosis**?

A. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

B. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

C. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

D. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

E. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

ANSWER: A · Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

Why this is best: Diagnosis: Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement. Adults: clinical or home sleep studies; evaluation of coexisting cardiovascular disease or metabolic syndrome The distractors correspond to other named concepts in this learning area (Treatment, Thresholds, Presentation, Causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0761 §8.7 · Integrated clinical vignette

A parent requests rapid maxillary expansion specifically to cure their child's OSA. What is an evidence-consistent response?

A. All children worsen following expansion

B. Expansion cures every child

C. Expansion may help selected maxillary constriction but is not a reliable substitute for medical treatment or follow-up PSG

D. Physician assessment can be omitted

E. An orthodontic scan alone diagnoses OSA

ANSWER: C · Expansion may help selected maxillary constriction but is not a reliable substitute for medical treatment or follow-up PSG

Why this is best: Published improvements are variable and not definitive; RME may be adjunctive in selected cases under coordinated medical supervision.

Notebook anchor: *Integrated notebook, Topic 8, §8.7*

Q0762 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Indications**?

A. Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment

B. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

C. children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion

D. obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

E. first line and the most effective surgical therapy, improving polysomnographic parameters, behaviour and quality of life

ANSWER: C · children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion

Why this is best: In this context, the relevant notebook finding is **Indications**: children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion. Often used in children with neuromuscular or craniofacial disorders, or residual OSA after surgery The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 176, §8.7*

Q0763 §8.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on underlying syndromes and conditions, which description is specifically associated with **Clinical symptoms are unreliable**?

A. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

B. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

C. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

D. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

E. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

ANSWER: D · only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

Why this is best: Clinical symptoms are unreliable: only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms. This is the single strongest argument for objective screening in a syndromic population The distractors correspond to other named concepts in this learning area (OSA is highly prevalent, Genetic syndromes, Medical conditions, Prevalence by AHI), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 178, §8.9*

Q0764 §8.4 · Integrated clinical vignette

A 14-year-old sleeps 6.5 hours on school nights and catches up at weekends. Which issue should be assessed first?

A. Weekend sleep always cures school-night debt

B. Normal recommended sleep duration is six hours

C. Chronic insufficient sleep in the setting of adolescent circadian delay

D. A mandibular device is the first-line response

E. The pattern proves OSA

ANSWER: C · Chronic insufficient sleep in the setting of adolescent circadian delay

Why this is best: Teenagers generally require 8–10 hours; adolescent delayed phase and early school starts frequently create chronic sleep restriction.

Notebook anchor: *Integrated notebook, Topic 8, §8.4*

Q0765 §8.8 · Clinical-context application

Clinical context. A young child and adult both have AHI 6, prompting a discussion about age-dependent definitions. During this case discussion, how should the candidate interpret **Diagnosis**?

A. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

B. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

C. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

D. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

E. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

ANSWER: D · Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

Why this is best: In this context, the relevant notebook finding is **Diagnosis**: Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement. Adults: clinical or home sleep studies; evaluation of coexisting cardiovascular disease or metabolic syndrome The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0766 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **Adenotonsillectomy failure risk**?

A. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

B. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

C. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

D. overnight, attended, in-laboratory. It records EEG, EOG, EMG, ECG, respiratory airflow through oronasal sensors, respiratory effort via chest and abdominal belts, oxygen saturation, body position and (in children) end-tidal…

E. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

ANSWER: A · age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

Why this is best: Adenotonsillectomy failure risk: age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5–16% The distractors correspond to other named concepts in this learning area (Indications for PSG, Polysomnography is the diagnostic gold standard, Sleep duration, Sleep physician or ENT), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 162, §8.10*

Q0767 §8.6 · Integrated clinical vignette

A 7-year-old has loud snoring, mouth breathing and tonsils occupying more than half of the oropharyngeal space. What is the most useful action?

A. Infer AHI from tonsil grade alone

B. Start orthodontic expansion before medical evaluation

C. Dismiss the symptoms as normal growing pains

D. Document tonsil grade and refer for paediatric airway evaluation

E. Fit an adult CPAP setting independently

ANSWER: D · Document tonsil grade and refer for paediatric airway evaluation

Why this is best: Brodsky tonsil grading documents obstruction but does not itself diagnose OSA; symptoms plus hypertrophy justify specialist evaluation.

Notebook anchor: *Integrated notebook, Topic 8, §8.6*

Q0768 §8.9 · Clinical-context application

Clinical context. A child with a genetic syndrome has possible OSA despite few reported symptoms. During this case discussion, how should the candidate interpret **Clinical symptoms are unreliable**?

A. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

B. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

C. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

D. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

E. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

ANSWER: A · only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

Why this is best: In this context, the relevant notebook finding is **Clinical symptoms are unreliable**: only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms. This is the single strongest argument for objective screening in a syndromic population The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 178, §8.9*

Q0769 §8.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric prevalence, which description is specifically associated with **The four contributing mechanisms**?

A. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

B. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

C. Adenotonsillar hypertrophy is the predominant anatomical factor

D. Approximately 1–5% of children, with estimates near 2–3%

E. 3–12% (some sources cite 10–20%)

ANSWER: B · craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

Why this is best: The four contributing mechanisms: craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control The distractors correspond to other named concepts in this learning area (Prevalence, General prevalence of paediatric OSA, Habitual snoring, Common preschool cause), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0770 §8.9 · Integrated clinical vignette

A child with obesity and craniofacial syndrome remains symptomatic after adenotonsillectomy. What is the next principle?

A. Reassessment for residual OSA with a multidisciplinary team

B. Use adult screening exclusively

C. Treat only the bite

D. Ignore obesity and neuromuscular factors

E. Assume the surgery always cures disease

ANSWER: A · Reassessment for residual OSA with a multidisciplinary team

Why this is best: Residual OSA risk is higher with obesity, severe baseline OSA and syndromic or craniofacial factors; objective reassessment is important.

Notebook anchor: *Integrated notebook, Topic 8, §8.9*

Q0771 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **Adenotonsillectomy failure risk**?

A. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

B. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

C. overnight, attended, in-laboratory. It records EEG, EOG, EMG, ECG, respiratory airflow through oronasal sensors, respiratory effort via chest and abdominal belts, oxygen saturation, body position and (in children) end-tidal…

D. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

E. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

ANSWER: B · age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

Why this is best: In this context, the relevant notebook finding is **Adenotonsillectomy failure risk**: age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16% The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 162, §8.10*

Q0772 §8.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on insufficient sleep and development, which description is specifically associated with **Less than recommended**?

A. attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

B. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the…

C. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

D. <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

E. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

ANSWER: A · attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

Why this is best: Less than recommended: attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts. More than recommended: hypertension, diabetes, obesity, mental health problems. Sufficient: improved attention, behaviour, learning, memory, emotional regulation, quality of life, and mental and physical health The distractors correspond to other named concepts in this learning area (Recommended duration, Circadian phase delay, Weekend catch-up, Tonsils and adenoids), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 157, §8.4*

Q0773 §8.10 · Integrated clinical vignette

A parent reports habitual snoring in a preschool child and asks whether a dental examination diagnoses OSA. What is correct?

A. A flat-plane nightguard is mandatory before PSG

B. Dentists may declare OSA from snoring alone

C. A PSQ above threshold establishes definitive AHI

D. Clinical screening triggers referral; overnight PSG is the diagnostic gold standard

E. A Mallampati score diagnoses paediatric OSA

ANSWER: D · Clinical screening triggers referral; overnight PSG is the diagnostic gold standard

Why this is best: Paediatric screening supports suspicion, while diagnosis and severity require appropriate physician-led sleep evaluation.

Notebook anchor: *Integrated notebook, Topic 8, §8.10*

Q0774 §8.1 · Clinical-context application

Clinical context. The parents of a preschool child ask how common sleep-disordered breathing is at this age. During this case discussion, how should the candidate interpret **The four contributing mechanisms**?

A. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

B. Adenotonsillar hypertrophy is the predominant anatomical factor

C. 3–12% (some sources cite 10–20%)

D. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

E. Approximately 1–5% of children, with estimates near 2–3%

ANSWER: D · craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

Why this is best: In this context, the relevant notebook finding is **The four contributing mechanisms**: craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0775 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **Sleep–wake homeostasis**?

A. anxiety, stress sleep, leading to residual daytime sleepiness

B. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

C. chronic medical illness; anxiety and stress; prescribed psychotropics

D. Sleep pressure builds more slowly with time awake in adolescents

E. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

ANSWER: D · Sleep pressure builds more slowly with time awake in adolescents

Why this is best: Sleep–wake homeostasis: Sleep pressure builds more slowly with time awake in adolescents. Together with the phase delay this makes it a major challenge for young people to fall asleep in the early evening and to wake refreshed in the early morning The distractors correspond to other named concepts in this learning area (Mental health problems, Other causes, School start times, Electronic media), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0776 §8.5 · Integrated clinical vignette

An adolescent has late-night device use, delayed sleep onset and poor decisions during daytime. What is the most appropriate explanatory model?

A. Poor decisions are unrelated to sleep

B. An oral appliance fixes all adolescent sleep debt

C. Delayed phase means OSA is impossible

D. Media displacement, circadian phase delay and sleep debt can interact

E. Screen time guarantees central apnoea

ANSWER: D · Media displacement, circadian phase delay and sleep debt can interact

Why this is best: Adolescent SDB and insufficient sleep need separate assessment; media use and phase delay can impair cognition and impulse control.

Notebook anchor: *Integrated notebook, Topic 8, §8.5*

Q0777 §8.4 · Clinical-context application

Clinical context. A child with inadequate sleep develops learning and attention problems. During this case discussion, how should the candidate interpret **Less than recommended**?

A. attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

B. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the…

C. <4 months not specified (variation too large) · 4–12 months 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h

D. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

E. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

ANSWER: A · attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts

Why this is best: In this context, the relevant notebook finding is **Less than recommended**: attention, behaviour and learning problems; increased risk of accidents, injuries, hypertension, obesity, diabetes and depression; in teenagers, increased self-harm, suicidal thoughts and suicide attempts. More than recommended: hypertension, diabetes, obesity, mental health problems. Sufficient: improved attention, behaviour, learning, memory, emotional regulation, quality of life, and mental and physical health The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 157, §8.4*

Q0778 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Tonsils and adenoids**?

A. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

B. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

C. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

D. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

E. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

ANSWER: E · use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

Why this is best: Tonsils and adenoids: use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity The distractors correspond to other named concepts in this learning area (Medical, dental and family history, Jaw size and position, Tongue posture and function, Cognitive, behavioural and developmental assessment), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0779 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Sleep–wake homeostasis**?

A. chronic medical illness; anxiety and stress; prescribed psychotropics

B. anxiety, stress sleep, leading to residual daytime sleepiness

C. Sleep pressure builds more slowly with time awake in adolescents

D. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

E. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

ANSWER: C · Sleep pressure builds more slowly with time awake in adolescents

Why this is best: In this context, the relevant notebook finding is **Sleep–wake homeostasis**: Sleep pressure builds more slowly with time awake in adolescents. Together with the phase delay this makes it a major challenge for young people to fall asleep in the early evening and to wake refreshed in the early morning The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0780 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Requires objective assessment of adherence**?

A. first line and the most effective surgical therapy, improving polysomnographic parameters, behaviour and quality of life

B. children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion

C. obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

D. Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment

E. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

ANSWER: D · Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment

Why this is best: Requires objective assessment of adherence: Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment The distractors correspond to other named concepts in this learning area (Indications, Risk factors for failure, Adenotonsillectomy, Positional and other), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 176, §8.7*

Q0781 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Tonsils and adenoids**?

A. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

B. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

C. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

D. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

E. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

ANSWER: A · use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

Why this is best: In this context, the relevant notebook finding is **Tonsils and adenoids**: use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0782 §8.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on differences from adult sdb, which description is specifically associated with **Thresholds**?

A. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

B. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

C. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

D. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

E. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

ANSWER: E · Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

Why this is best: Thresholds: Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years The distractors correspond to other named concepts in this learning area (Diagnosis, Treatment, Causes, Presentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0783 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Requires objective assessment of adherence**?

A. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

B. obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

C. first line and the most effective surgical therapy, improving polysomnographic parameters, behaviour and quality of life

D. Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment

E. children who do not respond adequately to surgery, or in whom surgery is contraindicated — which can include patient and family preference, for example religious belief against surgery or blood transfusion

ANSWER: D · Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment

Why this is best: In this context, the relevant notebook finding is **Requires objective assessment of adherence**: Attempt to improve adherence by addressing adverse effects and using behaviour modification techniques; if the child remains non-compliant, use an alternative treatment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 176, §8.7*

Q0784 §8.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on underlying syndromes and conditions, which description is specifically associated with **OSA is highly prevalent**?

A. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

B. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

C. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

D. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

E. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

ANSWER: E · Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

Why this is best: OSA is highly prevalent: Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population The distractors correspond to other named concepts in this learning area (Clinical symptoms are unreliable, Medical conditions, Genetic syndromes, Prevalence by AHI), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 178, §8.9*

Q0785 §8.8 · Clinical-context application

Clinical context. A young child and adult both have AHI 6, prompting a discussion about age-dependent definitions. During this case discussion, how should the candidate interpret **Thresholds**?

A. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

B. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

C. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

D. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

E. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

ANSWER: B · Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

Why this is best: In this context, the relevant notebook finding is **Thresholds**: Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0786 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **Sleep duration**?

A. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

B. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

C. detailed history focusing on habitual snoring, observed apnoeas, laboured or paradoxical breathing during sleep, daytime sleepiness, behavioural problems and learning difficulties

D. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

E. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

ANSWER: E · 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

Why this is best: Sleep duration: 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping The distractors correspond to other named concepts in this learning area (Sleep physician or ENT, Clinical evaluation, Indications for PSG, Adenotonsillectomy failure risk), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 162, §8.10*

Q0787 §8.9 · Clinical-context application

Clinical context. A child with a genetic syndrome has possible OSA despite few reported symptoms. During this case discussion, how should the candidate interpret **OSA is highly prevalent**?

A. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

B. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

C. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

D. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

E. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

ANSWER: C · Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

Why this is best: In this context, the relevant notebook finding is **OSA is highly prevalent**: Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 178, §8.9*

Q0788 §8.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric prevalence, which description is specifically associated with **Habitual snoring**?

A. 3–12% (some sources cite 10–20%)

B. Attention, hyperactivity and behavioural or learning difficulties may predominate

C. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

D. Adenotonsillar hypertrophy is the predominant anatomical factor

E. Approximately 1–5% of children, with estimates near 2–3%

ANSWER: A · 3–12% (some sources cite 10–20%)

Why this is best: Habitual snoring: 3–12% (some sources cite 10–20%) The distractors correspond to other named concepts in this learning area (General prevalence of paediatric OSA, The four contributing mechanisms, Common preschool cause, Paediatric daytime presentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 163, §8.1*

Q0789 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **Sleep duration**?

A. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

B. detailed history focusing on habitual snoring, observed apnoeas, laboured or paradoxical breathing during sleep, daytime sleepiness, behavioural problems and learning difficulties

C. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

D. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

E. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

ANSWER: D · 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

Why this is best: In this context, the relevant notebook finding is **Sleep duration**: 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 162, §8.10*

Q0790 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **Electronic media**?

A. chronic medical illness; anxiety and stress; prescribed psychotropics

B. anxiety, stress sleep, leading to residual daytime sleepiness

C. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

D. Sleep pressure builds more slowly with time awake in adolescents

E. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

ANSWER: E · four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

Why this is best: Electronic media: four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night; device light suppresses melatonin and disrupts circadian rhythm, producing inability to fall asleep at a reasonable time; and use increases sleep-disrupting mental, emotional and physiological arousal The distractors correspond to other named concepts in this learning area (School start times, Other causes, Sleep–wake homeostasis, Mental health problems), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0791 §8.1 · Clinical-context application

Clinical context. The parents of a preschool child ask how common sleep-disordered breathing is at this age. During this case discussion, how should the candidate interpret **Habitual snoring**?

A. Attention, hyperactivity and behavioural or learning difficulties may predominate

B. Adenotonsillar hypertrophy is the predominant anatomical factor

C. Approximately 1–5% of children, with estimates near 2–3%

D. 3–12% (some sources cite 10–20%)

E. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

ANSWER: D · 3–12% (some sources cite 10–20%)

Why this is best: In this context, the relevant notebook finding is **Habitual snoring**: 3–12% (some sources cite 10–20%) The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 163, §8.1*

Q0792 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Medical, dental and family history**?

A. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

B. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

C. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

D. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

E. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

ANSWER: E · chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

Why this is best: Medical, dental and family history: chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk The distractors correspond to other named concepts in this learning area (Jaw size and position, Tonsils and adenoids, Tongue posture and function, Palate and arches), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 168, §8.6*

Q0793 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Electronic media**?

A. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

B. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

C. anxiety, stress sleep, leading to residual daytime sleepiness

D. Sleep pressure builds more slowly with time awake in adolescents

E. chronic medical illness; anxiety and stress; prescribed psychotropics

ANSWER: B · four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

Why this is best: In this context, the relevant notebook finding is **Electronic media**: four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night; device light suppresses melatonin and disrupts circadian rhythm, producing inability to fall asleep at a reasonable time; and use increases sleep-disrupting mental, emotional and physiological arousal The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0794 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Positional and other**?

A. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

B. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

C. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

D. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

E. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

ANSWER: E · AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

Why this is best: Positional and other: AHI is increased supine and lowest prone in children — but position is hard to maintain overnight. Weight management for obese children. Watchful waiting with close monitoring may suit mild snoring or mild OSA. Myofunctional therapy as an adjunct The distractors correspond to other named concepts in this learning area (Intranasal corticosteroid, Other surgery, Contraindication, Medication), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.7*

Q0795 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Medical, dental and family history**?

A. use the modified Mallampati, Friedman tongue classification, or the Brodsky scale. Hypertrophy obstructing >50% of the airway warrants further evaluation — but tonsil size does not correlate with severity

B. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

C. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

D. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

E. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

ANSWER: E · chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

Why this is best: In this context, the relevant notebook finding is **Medical, dental and family history**: chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 168, §8.6*

Q0796 §8.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on differences from adult sdb, which description is specifically associated with **Causes**?

A. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

B. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

C. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

D. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

E. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

ANSWER: A · Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

Why this is best: Causes: Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y. Adults: obesity, age, male sex; smoking and alcohol; often diagnosed as OSA or CSA The distractors correspond to other named concepts in this learning area (Thresholds, Diagnosis, Treatment, Presentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0797 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Positional and other**?

A. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

B. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

C. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

D. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

E. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

ANSWER: D · AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

Why this is best: In this context, the relevant notebook finding is **Positional and other**: AHI is increased supine and lowest prone in children — but position is hard to maintain overnight. Weight management for obese children. Watchful waiting with close monitoring may suit mild snoring or mild OSA. Myofunctional therapy as an adjunct The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.7*

Q0798 §8.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on underlying syndromes and conditions, which description is specifically associated with **Medical conditions**?

A. macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

B. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

C. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

D. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

E. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

ANSWER: D · obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

Why this is best: Medical conditions: obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone. Children with Duchenne muscular dystrophy and myasthenia gravis have weakened respiratory muscles, leading to hypoventilation and greater susceptibility The distractors correspond to other named concepts in this learning area (Clinical symptoms are unreliable, OSA is highly prevalent, Prevalence by AHI, Predisposing factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.9*

Q0799 §8.8 · Clinical-context application

Clinical context. A young child and adult both have AHI 6, prompting a discussion about age-dependent definitions. During this case discussion, how should the candidate interpret **Causes**?

A. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

B. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

C. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

D. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

E. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

ANSWER: A · Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

Why this is best: In this context, the relevant notebook finding is **Causes**: Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y. Adults: obesity, age, male sex; smoking and alcohol; often diagnosed as OSA or CSA The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0800 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **Sleep physician or ENT**?

A. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

B. detailed history focusing on habitual snoring, observed apnoeas, laboured or paradoxical breathing during sleep, daytime sleepiness, behavioural problems and learning difficulties

C. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

D. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

E. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

ANSWER: E · for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

Why this is best: Sleep physician or ENT: for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line The distractors correspond to other named concepts in this learning area (Sleep duration, Clinical evaluation, Indications for PSG, Adenotonsillectomy failure risk), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 161, §8.10*

Q0801 §8.9 · Clinical-context application

Clinical context. A child with a genetic syndrome has possible OSA despite few reported symptoms. During this case discussion, how should the candidate interpret **Medical conditions**?

A. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

B. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

C. macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

D. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

E. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

ANSWER: B · obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

Why this is best: In this context, the relevant notebook finding is **Medical conditions**: obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone. Children with Duchenne muscular dystrophy and myasthenia gravis have weakened respiratory muscles, leading to hypoventilation and greater susceptibility The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.9*

Q0802 §8.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric prevalence, which description is specifically associated with **Prevalence**?

A. Adenotonsillar hypertrophy is the predominant anatomical factor

B. Approximately 1–5% of children, with estimates near 2–3%

C. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

D. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

E. 3–12% (some sources cite 10–20%)

ANSWER: D · OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

Why this is best: Prevalence: OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). Peaks in younger children (2–8 y) and again in adolescents; most commonly affects ages 3–6, when tonsillar tissue is proportionally largest. Varies by population, gender and race The distractors correspond to other named concepts in this learning area (General prevalence of paediatric OSA, The four contributing mechanisms, Habitual snoring, Common preschool cause), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0803 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **Sleep physician or ENT**?

A. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

B. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

C. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

D. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

E. detailed history focusing on habitual snoring, observed apnoeas, laboured or paradoxical breathing during sleep, daytime sleepiness, behavioural problems and learning difficulties

ANSWER: C · for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

Why this is best: In this context, the relevant notebook finding is **Sleep physician or ENT**: for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 161, §8.10*

Q0804 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **Weekend catch-up**?

A. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological…

B. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

C. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

D. chronic medical illness; anxiety and stress; prescribed psychotropics

E. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

ANSWER: A · Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological…

Why this is best: Weekend catch-up: Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological night, perpetuating the cycle The distractors correspond to other named concepts in this learning area (Circadian phase delay, Electronic media, School start times, Other causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0805 §8.1 · Clinical-context application

Clinical context. The parents of a preschool child ask how common sleep-disordered breathing is at this age. During this case discussion, how should the candidate interpret **Prevalence**?

A. Approximately 1–5% of children, with estimates near 2–3%

B. Adenotonsillar hypertrophy is the predominant anatomical factor

C. 3–12% (some sources cite 10–20%)

D. craniofacial morphology, tonsillar hypertrophy, altered neuromuscular tone, ventilatory control

E. OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

ANSWER: E · OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%)

Why this is best: In this context, the relevant notebook finding is **Prevalence**: OSA 1–5% (typically 2–3%); habitual snoring 3–12% (some cite 10–20%). Peaks in younger children (2–8 y) and again in adolescents; most commonly affects ages 3–6, when tonsillar tissue is proportionally largest. Varies by population, gender and race The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 156, §8.1*

Q0806 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Cognitive, behavioural and developmental assessment**?

A. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

B. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

C. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

D. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

E. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

ANSWER: E · children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

Why this is best: Cognitive, behavioural and developmental assessment: children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD The distractors correspond to other named concepts in this learning area (Palate and arches, Tongue posture and function, Jaw size and position, Medical, dental and family history), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 168, §8.6*

Q0807 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Weekend catch-up**?

A. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

B. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

C. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological…

D. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

E. chronic medical illness; anxiety and stress; prescribed psychotropics

ANSWER: C · Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological…

Why this is best: In this context, the relevant notebook finding is **Weekend catch-up**: Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the biological night, perpetuating the cycle The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0808 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Risk factors for failure**?

A. obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

B. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

C. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

D. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

E. first line and the most effective surgical therapy, improving polysomnographic parameters, behaviour and quality of life

ANSWER: A · obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

Why this is best: Risk factors for failure: obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma. Post-operative PSG is suggested for these high-risk children even without persistent snoring — though persistent OSA is less likely if there is no post-operative snoring The distractors correspond to other named concepts in this learning area (Adenotonsillectomy, Positional and other, Intranasal corticosteroid, Other surgery), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0809 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Cognitive, behavioural and developmental assessment**?

A. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

B. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

C. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

D. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

E. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

ANSWER: B · children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

Why this is best: In this context, the relevant notebook finding is **Cognitive, behavioural and developmental assessment**: children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 168, §8.6*

Q0810 §8.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on differences from adult sdb, which description is specifically associated with **Presentation**?

A. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

B. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

C. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

D. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

E. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

ANSWER: E · Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

Why this is best: Presentation: Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance. Adults: loud chronic snoring; stopped breathing episodes; morning headache or dry mouth; daytime drowsiness; decreased libido and mood swings; impaired memory or concentration The distractors correspond to other named concepts in this learning area (Treatment, Diagnosis, Thresholds, Causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0811 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Risk factors for failure**?

A. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

B. obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

C. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

D. first line and the most effective surgical therapy, improving polysomnographic parameters, behaviour and quality of life

E. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

ANSWER: B · obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma

Why this is best: In this context, the relevant notebook finding is **Risk factors for failure**: obesity; high pre-operative AHI; age <3 or ≥7; nasal abnormality or craniofacial disproportion; family history of OSA; asthma. Post-operative PSG is suggested for these high-risk children even without persistent snoring — though persistent OSA is less likely if there is no post-operative snoring The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0812 §8.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on underlying syndromes and conditions, which description is specifically associated with **Genetic syndromes**?

A. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

B. only a weak correlation between parental perception and PSG findings, and 53.8% of affected children had no symptoms — the strongest argument for objective screening in a syndromic population

C. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

D. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

E. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

ANSWER: D · Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

Why this is best: Genetic syndromes: Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway. In Prader–Willi, hypersomnolence and varying airway obstruction contribute. DOWN SYNDROME The distractors correspond to other named concepts in this learning area (Clinical symptoms are unreliable, OSA is highly prevalent, Medical conditions, Clinical symptoms are unreliable), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 161, §8.9*

Q0813 §8.8 · Clinical-context application

Clinical context. A young child and adult both have AHI 6, prompting a discussion about age-dependent definitions. During this case discussion, how should the candidate interpret **Presentation**?

A. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

B. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

C. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

D. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

E. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

ANSWER: B · Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

Why this is best: In this context, the relevant notebook finding is **Presentation**: Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance. Adults: loud chronic snoring; stopped breathing episodes; morning headache or dry mouth; daytime drowsiness; decreased libido and mood swings; impaired memory or concentration The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0814 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **Paediatric AHI**?

A. normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

B. within pillars · <25% · 25–50% · 50–75% · >75%

C. AHI >1 in 69% · >5 in 50% · >10 in 34%; PSG by age 4; 53.8% asymptomatic

D. where craniofacial development suggests a narrow airway or malocclusion, consider early intervention

E. 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

ANSWER: A · normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

Why this is best: Paediatric AHI: normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal The distractors correspond to other named concepts in this learning area (Down syndrome OSA prevalence, Orthodontic intervention, PSQ threshold >0.33 (>8 of 22 items); FAIREST-6, Brodsky grades 0–4), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0815 §8.9 · Clinical-context application

Clinical context. A child with a genetic syndrome has possible OSA despite few reported symptoms. During this case discussion, how should the candidate interpret **Genetic syndromes**?

A. Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

B. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

C. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

D. only a weak correlation between parental perception and PSG findings, and 53.8% of affected children had no symptoms — the strongest argument for objective screening in a syndromic population

E. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

ANSWER: A · Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway

Why this is best: In this context, the relevant notebook finding is **Genetic syndromes**: Down, Prader–Willi, Crouzon and cleft lip and palate often involve craniofacial abnormalities — midfacial hypoplasia, macroglossia, retrognathia — narrowing the airway. In Prader–Willi, hypersomnolence and varying airway obstruction contribute. DOWN SYNDROME The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 161, §8.9*

Q0816 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **Circadian phase delay**?

A. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

B. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

C. chronic medical illness; anxiety and stress; prescribed psychotropics

D. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the…

E. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

ANSWER: A · Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

Why this is best: Circadian phase delay: Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later The distractors correspond to other named concepts in this learning area (Weekend catch-up, Electronic media, School start times, Other causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0817 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **Paediatric AHI**?

A. where craniofacial development suggests a narrow airway or malocclusion, consider early intervention

B. 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

C. normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

D. within pillars · <25% · 25–50% · 50–75% · >75%

E. AHI >1 in 69% · >5 in 50% · >10 in 34%; PSG by age 4; 53.8% asymptomatic

ANSWER: C · normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

Why this is best: In this context, the relevant notebook finding is **Paediatric AHI**: normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0818 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Palate and arches**?

A. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

B. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

C. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

D. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

E. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

ANSWER: B · high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

Why this is best: Palate and arches: high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing The distractors correspond to other named concepts in this learning area (Cognitive, behavioural and developmental assessment, Tongue posture and function, Jaw size and position, Medical, dental and family history), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0819 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Circadian phase delay**?

A. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

B. Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

C. Going to bed later and waking later at weekends reflects circadian biology and is also a response to insufficient weekday sleep — but later weekend sleep timing and catch-up sleep further delay the sig- nal for the…

D. chronic medical illness; anxiety and stress; prescribed psychotropics

E. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

ANSWER: B · Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later

Why this is best: In this context, the relevant notebook finding is **Circadian phase delay**: Early adolescent development and pubertal onset are associated with a more evening-type circadian phase preference — the preferred timing of sleep, wake and daytime activity shifts later The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 166, §8.5*

Q0820 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Contraindication**?

A. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

B. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

C. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

D. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

E. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

ANSWER: E · submucous cleft palate is relative — a partial adenotonsillectomy may be performed

Why this is best: Contraindication: submucous cleft palate is relative — a partial adenotonsillectomy may be performed The distractors correspond to other named concepts in this learning area (Medication, Other surgery, Intranasal corticosteroid, Positional and other), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0821 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Palate and arches**?

A. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

B. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

C. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

D. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

E. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

ANSWER: D · high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

Why this is best: In this context, the relevant notebook finding is **Palate and arches**: high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0822 §8.8 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on differences from adult sdb, which description is specifically associated with **Treatment**?

A. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

B. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

C. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

D. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

E. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

ANSWER: E · Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

Why this is best: Treatment: Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases. Adults: lifestyle change (weight loss, alcohol avoidance, position); CPAP; appliance therapy as a non-invasive alternative; surgery where necessary The distractors correspond to other named concepts in this learning area (Diagnosis, Presentation, Thresholds, Causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0823 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Contraindication**?

A. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

B. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

C. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

D. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

E. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

ANSWER: B · submucous cleft palate is relative — a partial adenotonsillectomy may be performed

Why this is best: In this context, the relevant notebook finding is **Contraindication**: submucous cleft palate is relative — a partial adenotonsillectomy may be performed The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0824 §8.9 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on underlying syndromes and conditions, which description is specifically associated with **Predisposing factors**?

A. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

B. macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

C. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

D. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

E. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

ANSWER: B · macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

Why this is best: Predisposing factors: macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages The distractors correspond to other named concepts in this learning area (Prevalence by AHI, Medical conditions, Clinical symptoms are unreliable, OSA is highly prevalent), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 161, §8.9*

Q0825 §8.8 · Clinical-context application

Clinical context. A young child and adult both have AHI 6, prompting a discussion about age-dependent definitions. During this case discussion, how should the candidate interpret **Treatment**?

A. Children: enlarged tonsils and adenoids lead; genetic predisposition; obesity in a minority but increasingly in adolescents; commonest at 3–6 y

B. Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

C. Children: snoring or noisy breathing; restless sleep or unusual positions; frequent bed-wetting; daytime fatigue and attention difficulties; behavioural problems that may mimic ADHD; poor academic performance

D. Children: thorough clinical evaluation of behaviour and sleep patterns; PSG; imaging (radiographs, endoscopy) may identify anatomical obstruction; end-tidal CO2 is a standard PSG measurement

E. Children: AHI ≥1 abnormal; mild 1–5, moderate 5–10, severe ≥10. Adults: AHI ≥5 with symptoms or ≥15 regardless; mild 5–15, moderate 15–30, severe ≥30. Age boundary 12 years

ANSWER: B · Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases

Why this is best: In this context, the relevant notebook finding is **Treatment**: Children: surgical removal of enlarged tonsils and adenoids first; weight management; PAP where surgery is ineffective; behaviour-focused therapy for sleep hygiene; orthodontics in selected cases. Adults: lifestyle change (weight loss, alcohol avoidance, position); CPAP; appliance therapy as a non-invasive alternative; surgery where necessary The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.8*

Q0826 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **PSQ threshold >0.33 (>8 of 22 items); FAIREST-6**?

A. AHI >1 in 69% · >5 in 50% · >10 in 34%; PSG by age 4; 53.8% asymptomatic

B. regular communication ensures coordinated treatment and monitoring, especially where SDB overlaps other conditions

C. where craniofacial development suggests a narrow airway or malocclusion, consider early intervention

D. 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

E. normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

ANSWER: D · 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

Why this is best: PSQ threshold >0.33 (>8 of 22 items); FAIREST-6: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation The distractors correspond to other named concepts in this learning area (Orthodontic intervention, Collaboration with paediatricians, Paediatric AHI, Down syndrome OSA prevalence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0827 §8.9 · Clinical-context application

Clinical context. A child with a genetic syndrome has possible OSA despite few reported symptoms. During this case discussion, how should the candidate interpret **Predisposing factors**?

A. obesity, asthma, allergic rhinitis, GORD and neuromuscular disorders all contribute to obstruction, inflammation or decreased airway muscle tone

B. Prevalence by AHI threshold: >1 in 69%; >1.5 in 76%; >2 in 75%; >5 (moderate–severe) in 50%; >10 in 34%. Compare 1–5% in the general paediatric population

C. macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

D. >1 in 69% · >1.5 in 76% · >2 in 75% · >5 (moderate–severe) in 50% · >10 in 34%. Compare 1–5% in the general paediatric population

E. only a weak correlation exists between parental perception of sleep problems and polysomnographic results, and 53.8% of children with OSA had no symptoms

ANSWER: C · macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages

Why this is best: In this context, the relevant notebook finding is **Predisposing factors**: macroglossia, midface hypoplasia, overweight, poor muscle tone. Moderate–severe disease is more prevalent at younger ages The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 161, §8.9*

Q0828 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **School start times**?

A. Sleep pressure builds more slowly with time awake in adolescents

B. chronic medical illness; anxiety and stress; prescribed psychotropics

C. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

D. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

E. anxiety, stress sleep, leading to residual daytime sleepiness

ANSWER: C · early starts conflict with the delayed rhythm, producing chronic insufficient sleep

Why this is best: School start times: early starts conflict with the delayed rhythm, producing chronic insufficient sleep. Later-starting middle schools report later rise times, more total sleep on school nights, less daytime sleepiness, less tardiness, fewer attention and concentration difficulties, better academic performance, and better safety with a reduced crash rate The distractors correspond to other named concepts in this learning area (Other causes, Sleep–wake homeostasis, Mental health problems, Electronic media), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0829 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **PSQ threshold >0.33 (>8 of 22 items); FAIREST-6**?

A. AHI >1 in 69% · >5 in 50% · >10 in 34%; PSG by age 4; 53.8% asymptomatic

B. normal <1 · mild 1–5 · moderate 5–10 · severe ≥10. Obstructive AHI ≥1 is abnormal

C. regular communication ensures coordinated treatment and monitoring, especially where SDB overlaps other conditions

D. 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

E. where craniofacial development suggests a narrow airway or malocclusion, consider early intervention

ANSWER: D · 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation

Why this is best: In this context, the relevant notebook finding is **PSQ threshold >0.33 (>8 of 22 items); FAIREST-6**: 2 mild / 4 moderate / 6 severe; paediatric ESS ≥10; tonsillar obstruction >50% warrants evaluation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0830 §8.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on paediatric screening, which description is specifically associated with **Tongue posture and function**?

A. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

B. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

C. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

D. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

E. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

ANSWER: E · tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

Why this is best: Tongue posture and function: tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development The distractors correspond to other named concepts in this learning area (Jaw size and position, Cognitive, behavioural and developmental assessment, Palate and arches, Medical, dental and family history), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0831 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **School start times**?

A. chronic medical illness; anxiety and stress; prescribed psychotropics

B. Sleep pressure builds more slowly with time awake in adolescents

C. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

D. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

E. anxiety, stress sleep, leading to residual daytime sleepiness

ANSWER: C · early starts conflict with the delayed rhythm, producing chronic insufficient sleep

Why this is best: In this context, the relevant notebook finding is **School start times**: early starts conflict with the delayed rhythm, producing chronic insufficient sleep. Later-starting middle schools report later rise times, more total sleep on school nights, less daytime sleepiness, less tardiness, fewer attention and concentration difficulties, better academic performance, and better safety with a reduced crash rate The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0832 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Partial tonsillectomy**?

A. leaves tissue covering the tonsillar fossa musculature

B. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

C. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

D. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

E. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

ANSWER: A · leaves tissue covering the tonsillar fossa musculature

Why this is best: Partial tonsillectomy: leaves tissue covering the tonsillar fossa musculature. Advantages: reduced morbidity, less immediate post-operative pain, earlier return to normal activity and diet. Disadvantages: regrowth risk 0.5–16%, increased intra-operative blood loss, recurrent infection in the remnant, and the need for long-term monitoring for OSA related to regrowth The distractors correspond to other named concepts in this learning area (Contraindication, Medication, Other surgery, Positional and other), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0833 §8.6 · Clinical-context application

Clinical context. A dentist screens a snoring child and documents oropharyngeal findings. During this case discussion, how should the candidate interpret **Tongue posture and function**?

A. chronic allergies, asthma, recurrent ear or throat infections, and a family history of sleep apnoea or SDB. Obesity, prematurity and syndromic conditions increase risk

B. high-arched palate or narrow arches restricting the airway; narrow arches may indicate habitual mouth breathing

C. retrognathia or micrognathia; subtle radiographic findings such as inferior hyoid position and reduced mandibular or maxillary length

D. tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

E. children with SDB often display hyperactivity, irritability and difficulty concentrating, closely resembling ADHD

ANSWER: D · tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development

Why this is best: In this context, the relevant notebook finding is **Tongue posture and function**: tongue tie, low posture, restricted movement; abnormal posture or tongue thrusting may affect patency and craniofacial development The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 158, §8.6*

Q0834 §8.10 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on referral and diagnostic pathways, which description is specifically associated with **Indications for PSG**?

A. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

B. overnight, attended, in-laboratory. It records EEG, EOG, EMG, ECG, respiratory airflow through oronasal sensors, respiratory effort via chest and abdominal belts, oxygen saturation, body position and (in children) end-tidal…

C. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

D. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

E. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

ANSWER: A · clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

Why this is best: Indications for PSG: clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery. Also for CPAP titration and evaluation of complex cases The distractors correspond to other named concepts in this learning area (Adenotonsillectomy failure risk, Sleep physician or ENT, Sleep duration, Polysomnography is the diagnostic gold standard), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0835 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Partial tonsillectomy**?

A. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

B. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

C. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

D. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

E. leaves tissue covering the tonsillar fossa musculature

ANSWER: E · leaves tissue covering the tonsillar fossa musculature

Why this is best: In this context, the relevant notebook finding is **Partial tonsillectomy**: leaves tissue covering the tonsillar fossa musculature. Advantages: reduced morbidity, less immediate post-operative pain, earlier return to normal activity and diet. Disadvantages: regrowth risk 0.5–16%, increased intra-operative blood loss, recurrent infection in the remnant, and the need for long-term monitoring for OSA related to regrowth The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 159, §8.7*

Q0836 §8.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on adolescent sleep and performance, which description is specifically associated with **Other causes**?

A. chronic medical illness; anxiety and stress; prescribed psychotropics

B. Sleep pressure builds more slowly with time awake in adolescents

C. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

D. anxiety, stress sleep, leading to residual daytime sleepiness

E. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

ANSWER: A · chronic medical illness; anxiety and stress; prescribed psychotropics

Why this is best: Other causes: chronic medical illness; anxiety and stress; prescribed psychotropics; chronic respiratory illness such as asthma; pain conditions such as migraine; stimulants (especially ADHD medication) used as a countermeasure to sleepiness or as performance enhancers; OTC or prescribed sleep medication causing residual sleepiness; alcohol; academic demands and extracurricular load The distractors correspond to other named concepts in this learning area (Sleep–wake homeostasis, Mental health problems, School start times, Electronic media), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0837 §8.10 · Clinical-context application

Clinical context. Parents ask the dentist which service should diagnose their child's suspected OSA. During this case discussion, how should the candidate interpret **Indications for PSG**?

A. 4–12 mo 12–16 h · 1–2 y 11–14 h · 3–5 y 10–13 h · 6–12 y 9–12 h · 13–18 y 8–10 h. <8 h in adolescence → ~3× suicide-attempt risk; caffeine users 2× more likely to have difficulty sleeping

B. overnight, attended, in-laboratory. It records EEG, EOG, EMG, ECG, respiratory airflow through oronasal sensors, respiratory effort via chest and abdominal belts, oxygen saturation, body position and (in children) end-tidal…

C. clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

D. for children with multiple risk factors or symptoms, for comprehensive evaluation which may include PSG or further imaging. For enlarged adenoids and tonsils, surgical removal is first-line

E. age <3 or ≥7, obesity, high pre-op AHI, nasal/craniofacial abnormality, family history, asthma. Overnight admission if SpO2 nadir <80%, AHI ≥24/h or peak PCO2 ≥60 mmHg. Partial tonsillectomy regrowth 0.5– 16%

ANSWER: C · clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery

Why this is best: In this context, the relevant notebook finding is **Indications for PSG**: clinical symptoms of SDB, especially before adenotonsillectomy in children under 2 years, in obesity, in craniofacial or neuromuscular disorders, in Down syndrome, or where symptoms persist after surgery. Also for CPAP titration and evaluation of complex cases The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 180, §8.10*

Q0838 §8.7 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 8**, in the revision material on treatment approaches, which description is specifically associated with **Medication**?

A. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

B. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

C. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

D. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

E. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

ANSWER: A · Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

Why this is best: Medication: Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. Effects: small AHI reduction; improvement in snoring and observed apnoea; demonstrated shrinkage of adenoidal tissue. Follow-up needed as long-term effects are unknown. Leukotriene receptor antagonists may help in mild cases or where allergic rhinitis contributes The distractors correspond to other named concepts in this learning area (Contraindication, Intranasal corticosteroid, Other surgery, Positional and other), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 160, §8.7*

Q0839 §8.5 · Clinical-context application

Clinical context. An adolescent sleeps late and struggles to function during early school hours. During this case discussion, how should the candidate interpret **Other causes**?

A. anxiety, stress sleep, leading to residual daytime sleepiness

B. Sleep pressure builds more slowly with time awake in adolescents

C. four mechanisms: media use directly displaces sleep; devices allow greater interaction with friends at night

D. early starts conflict with the delayed rhythm, producing chronic insufficient sleep

E. chronic medical illness; anxiety and stress; prescribed psychotropics

ANSWER: E · chronic medical illness; anxiety and stress; prescribed psychotropics

Why this is best: In this context, the relevant notebook finding is **Other causes**: chronic medical illness; anxiety and stress; prescribed psychotropics; chronic respiratory illness such as asthma; pain conditions such as migraine; stimulants (especially ADHD medication) used as a countermeasure to sleepiness or as performance enhancers; OTC or prescribed sleep medication causing residual sleepiness; alcohol; academic demands and extracurricular load The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 157, §8.5*

Q0840 §8.7 · Clinical-context application

Clinical context. A child with confirmed OSA is being discussed at an ENT-paediatric-orthodontic meeting. During this case discussion, how should the candidate interpret **Medication**?

A. Indicated only for children with mild OSA (AHI <5/h), and not for moderate or severe disease

B. submucous cleft palate is relative — a partial adenotonsillectomy may be performed

C. AHI is increased supine and lowest prone in children — but position is hard to maintain overnight

D. Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

E. turbinate reduction, septoplasty; expansion sphincter pharyngoplasty for refractory cases

ANSWER: D · Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe

Why this is best: In this context, the relevant notebook finding is **Medication**: Intranasal corticosteroid — only for mild OSA (AHI <5/h), not moderate or severe. Effects: small AHI reduction; improvement in snoring and observed apnoea; demonstrated shrinkage of adenoidal tissue. Follow-up needed as long-term effects are unknown. Leukotriene receptor antagonists may help in mild cases or where allergic rhinitis contributes The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 160, §8.7*

TOPIC 09
Best-practice, documentation and ethical responsibilities

60 questions | Full notebook pages 183–195 | Each question has a justification and linked learning code.

Q0841 §9.1 · Integrated clinical vignette

A patient asks whether a dentist can declare 'no OSA' after a home oximetry test. What is the best scope-of-practice response?

A. The device manufacturer establishes severity

B. The patient may diagnose based on snoring

C. A dentist may diagnose from oximetry alone

D. The diagnosis or exclusion of OSA belongs to an appropriately qualified physician

E. A normal ESS rules out OSA

ANSWER: D · The diagnosis or exclusion of OSA belongs to an appropriately qualified physician

Why this is best: The notebook repeatedly distinguishes dental screening and titration from physician-led diagnosis and formal objective efficacy verification.

Notebook anchor: *Integrated notebook, Topic 9, §9.1*

Q0842 §9.1 · All-of-the-above integrated judgement

At an OAT documentation audit, which is the MOST COMPLETE defensible record set?

A. Baseline medical sleep report

B. Dental records and care plan

C. Signed informed consent and education

D. Adherence, adjustment and physician communication

E. All of the above

ANSWER: E · All of the above

Why this is best: Appropriate records support diagnosis, selection, treatment, informed consent, ongoing monitoring and shared care.

Notebook anchor: *Integrated notebook, Topic 9, §9.1*

Q0843 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **Supervision**?

A. where in-laboratory PSG is impossible because of immobility, safety or critical illness

B. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

C. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

ANSWER: C · performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

Why this is best: In this context, the relevant notebook finding is **Supervision**: performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 184, §9.1*

Q0844 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **Supervision**?

A. where in-laboratory PSG is impossible because of immobility, safety or critical illness

B. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

C. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

ANSWER: C · performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

Why this is best: Supervision: performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination The distractors correspond to other named concepts in this learning area (In-laboratory PSG, Indication, Special indications, Portable monitoring), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 184, §9.1*

Q0845 §9.1 · None-of-the-above discrimination

Which activity may independently establish OSA diagnosis within the dentist's ordinary OAT role?

A. Scoring a snoring survey

B. Using a chairside dental photograph

C. Reading a pulse-oximetry trace without the physician

D. Measuring Mallampati grade

E. None of the above

ANSWER: E · None of the above

Why this is best: The document reserves diagnosis and confirmation of efficacy to a qualified medical practitioner; these dental activities are screening or adjuncts.

Notebook anchor: *Integrated notebook, Topic 9, §9.1*

Q0846 §9.2 · Integrated clinical vignette

An oral appliance was fitted without recording pretreatment overjet, overbite or models. What is the key risk?

A. The clinician cannot reliably quantify later appliance-related dental change

B. Occlusal records are irrelevant once AHI improves

C. A signed bill substitutes for baseline measurements

D. Later changes become impossible

E. The physician alone is responsible for dental records

ANSWER: A · The clinician cannot reliably quantify later appliance-related dental change

Why this is best: Baseline records are essential for monitoring predictable occlusal effects and resolving medicolegal disputes.

Notebook anchor: *Integrated notebook, Topic 9, §9.2*

Q0847 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Treatment planning**?

A. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

B. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

C. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

D. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

E. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

ANSWER: D · comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

Why this is best: In this context, the relevant notebook finding is **Treatment planning**: comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0848 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Treatment planning**?

A. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

B. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

C. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

D. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

E. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

ANSWER: E · comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

Why this is best: Treatment planning: comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded The distractors correspond to other named concepts in this learning area (Interprofessional communication, Baseline sleep study data, Ongoing clinical notes, Delivery documentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0849 §9.2 · Integrated clinical vignette

A dentist changes advancement settings but does not inform the sleep physician. Which best-practice principle is breached?

A. Only billing codes need updating

B. A dental assistant may provide the sleep diagnosis

C. Patient preferences are not relevant

D. Treatment never requires physician input after baseline

E. Documented interdisciplinary communication and shared care

ANSWER: E · Documented interdisciplinary communication and shared care

Why this is best: OAT care requires communication of fitting, adjustment, response and referrals, with physician participation in outcome verification.

Notebook anchor: *Integrated notebook, Topic 9, §9.2*

Q0850 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Consult authoritative sources**?

A. Keep clear, separate consultation notes differentiating between different insurers and different conditions

B. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

C. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

D. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

E. use DVA-specific codes, which can differ from those for the general public insurer or private insurance

ANSWER: C · the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

Why this is best: In this context, the relevant notebook finding is **Consult authoritative sources**: the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0851 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Consult authoritative sources**?

A. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

B. Keep clear, separate consultation notes differentiating between different insurers and different conditions

C. use DVA-specific codes, which can differ from those for the general public insurer or private insurance

D. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

E. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

ANSWER: E · the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

Why this is best: Consult authoritative sources: the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues The distractors correspond to other named concepts in this learning area (Private billing, Use DVA-specific codes, Maintain detailed notes, Department of Veterans' Affairs), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0852 §9.3 · Integrated clinical vignette

A practice sees patients funded through a veterans' scheme and proposes using private insurance codes interchangeably. What is appropriate?

A. Check the current payer-specific coding rules and document the service accurately

B. Use any code that increases reimbursement

C. Bill for sleep studies the dentist did not perform

D. Use unsigned clinical notes

E. Assume all jurisdictions use the same codes

ANSWER: A · Check the current payer-specific coding rules and document the service accurately

Why this is best: Billing codes, including DVA arrangements, vary by jurisdiction and payer; ethical coding requires current authoritative guidance.

Notebook anchor: *Integrated notebook, Topic 9, §9.3*

Q0853 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **PM minimum**?

A. airflow + respiratory effort + blood oxygenation

B. promote well-being and provide high-quality care improving oral and overall health

C. calibrated or uncalibrated inductance plethysmography

D. signal averaging time ≤3 s at heart rate ≥80 bpm; finger probe

E. work with other professionals, especially sleep physicians, for comprehensive care

ANSWER: A · airflow + respiratory effort + blood oxygenation

Why this is best: In this context, the relevant notebook finding is **PM minimum**: airflow + respiratory effort + blood oxygenation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 187, §9.4*

Q0854 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **PM minimum**?

A. promote well-being and provide high-quality care improving oral and overall health

B. calibrated or uncalibrated inductance plethysmography

C. airflow + respiratory effort + blood oxygenation

D. work with other professionals, especially sleep physicians, for comprehensive care

E. signal averaging time ≤3 s at heart rate ≥80 bpm; finger probe

ANSWER: C · airflow + respiratory effort + blood oxygenation

Why this is best: PM minimum: airflow + respiratory effort + blood oxygenation The distractors correspond to other named concepts in this learning area (Effort sensor, Oximetry, Collaboration and referral, Beneficence), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 187, §9.4*

Q0855 §9.4 · Integrated clinical vignette

A dentist with a commercial relationship to an appliance supplier recommends it without comparison. What ethical principle is at stake?

A. Patients need not know alternatives

B. The choice is ethical if most profitable

C. Commercial sponsorship proves superior outcomes

D. Conflict-of-interest management and evidence-based patient choice

E. Competence is irrelevant when devices are custom

ANSWER: D · Conflict-of-interest management and evidence-based patient choice

Why this is best: Device selection must not be dictated by financial arrangements; integrity and patient-centred informed choice are required.

Notebook anchor: *Integrated notebook, Topic 9, §9.4*

Q0856 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **Special indications**?

A. less expensive and quicker, and can be as accurate as PSG in selected populations

B. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

C. but expensive and technically complex

D. where in-laboratory PSG is impossible because of immobility, safety or critical illness

E. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

ANSWER: D · where in-laboratory PSG is impossible because of immobility, safety or critical illness

Why this is best: In this context, the relevant notebook finding is **Special indications**: where in-laboratory PSG is impossible because of immobility, safety or critical illness; and to monitor response to non-CPAP treatments (oral appliances, upper airway surgery, weight loss) where the diagnosis has already been made The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.1*

Q0857 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **Special indications**?

A. where in-laboratory PSG is impossible because of immobility, safety or critical illness

B. less expensive and quicker, and can be as accurate as PSG in selected populations

C. but expensive and technically complex

D. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

E. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

ANSWER: A · where in-laboratory PSG is impossible because of immobility, safety or critical illness

Why this is best: Special indications: where in-laboratory PSG is impossible because of immobility, safety or critical illness; and to monitor response to non-CPAP treatments (oral appliances, upper airway surgery, weight loss) where the diagnosis has already been made The distractors correspond to other named concepts in this learning area (Portable monitoring, Accurate, with a low failure rate, and attended by technical staff, In-laboratory PSG, Supervision), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.1*

Q0858 §9.4 · Integrated clinical vignette

A patient declines OAT after learning about permanent bite changes. What should the clinician do?

A. Dismiss bite changes as impossible

B. Apply the device without consent

C. Treat refusal as a dental emergency

D. Conceal available options

E. Respect the informed refusal, document it and discuss alternatives

ANSWER: E · Respect the informed refusal, document it and discuss alternatives

Why this is best: Autonomy includes the right to refuse treatment after understanding benefits, burdens and alternatives.

Notebook anchor: *Integrated notebook, Topic 9, §9.4*

Q0859 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Informed consent**?

A. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

B. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

C. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

D. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

E. complete signed forms explaining purpose, risks, benefits, alternatives and patient rights

ANSWER: E · complete signed forms explaining purpose, risks, benefits, alternatives and patient rights

Why this is best: In this context, the relevant notebook finding is **Informed consent**: complete signed forms explaining purpose, risks, benefits, alternatives and patient rights; clear, free of coercion, verifiable in the record; signed by the patient in the presence of the qualified dentist or team member, then countersigned and dated by the dentist The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0860 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Informed consent**?

A. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

B. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

C. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

D. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

E. complete signed forms explaining purpose, risks, benefits, alternatives and patient rights

ANSWER: E · complete signed forms explaining purpose, risks, benefits, alternatives and patient rights

Why this is best: Informed consent: complete signed forms explaining purpose, risks, benefits, alternatives and patient rights; clear, free of coercion, verifiable in the record; signed by the patient in the presence of the qualified dentist or team member, then countersigned and dated by the dentist The distractors correspond to other named concepts in this learning area (Study models, Delivery documentation, Ongoing clinical notes, Baseline sleep study data), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0861 §9.1 · Integrated clinical vignette

A clinician uses an HSAT in a patient with suspected narcolepsy and significant heart failure. Why is this problematic?

A. A dentist may replace PSG with a dental scan

B. HSAT always records sleep stages

C. A positive snoring questionnaire confirms narcolepsy

D. Cardiac disease improves device accuracy

E. Portable monitoring may miss other disorders and is inappropriate in important comorbidity

ANSWER: E · Portable monitoring may miss other disorders and is inappropriate in important comorbidity

Why this is best: The notebook reserves portable testing for appropriately selected high-risk uncomplicated patients; suspected complex disorders favour PSG.

Notebook anchor: *Integrated notebook, Topic 9, §9.1*

Q0862 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Separate conditions**?

A. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

B. Clear, accurate documentation supports all billing claims and ensures a smooth billing process — and screening test scores must be recorded in the clinical record because they may be subject to audit

C. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

D. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

E. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

ANSWER: C · For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

Why this is best: In this context, the relevant notebook finding is **Separate conditions**: For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0863 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Separate conditions**?

A. Clear, accurate documentation supports all billing claims and ensures a smooth billing process — and screening test scores must be recorded in the clinical record because they may be subject to audit

B. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

C. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

D. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

E. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

ANSWER: B · For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

Why this is best: Separate conditions: For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both The distractors correspond to other named concepts in this learning area (Ethical dimension, Use DVA-specific codes, Documentation is key, Private billing), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0864 §9.2 · Integrated clinical vignette

The patient disputes whether appliance-related tooth movement was discussed. Which record most directly supports a proper process?

A. Only an SMS appointment reminder

B. A laboratory invoice alone

C. The appliance's colour

D. Dated signed consent plus documented discussion, baseline findings and follow-up

E. A verbal claim without notes

ANSWER: D · Dated signed consent plus documented discussion, baseline findings and follow-up

Why this is best: A signed form alone is not the entire consent process; contemporaneous records of discussion and baseline dental status are essential.

Notebook anchor: *Integrated notebook, Topic 9, §9.2*

Q0865 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **Beneficence**?

A. promote well-being and provide high-quality care improving oral and overall health

B. oronasal thermal sensor (apnoea) + nasal pressure transducer (hypopnoea) — ideally both

C. work with other professionals, especially sleep physicians, for comprehensive care

D. basic sleep physiology including SDB pathophysiology; the roles of dentist and team members

E. oronasal thermal sensor for apnoea, nasal pressure transducer for hypopnoea — ideally both

ANSWER: A · promote well-being and provide high-quality care improving oral and overall health

Why this is best: In this context, the relevant notebook finding is **Beneficence**: promote well-being and provide high-quality care improving oral and overall health. Justice — fairness and equity for all patients. Veracity — be truthful, including about the limits of the evidence and the likelihood of side effects The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0866 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **Beneficence**?

A. oronasal thermal sensor for apnoea, nasal pressure transducer for hypopnoea — ideally both

B. basic sleep physiology including SDB pathophysiology; the roles of dentist and team members

C. promote well-being and provide high-quality care improving oral and overall health

D. work with other professionals, especially sleep physicians, for comprehensive care

E. oronasal thermal sensor (apnoea) + nasal pressure transducer (hypopnoea) — ideally both

ANSWER: C · promote well-being and provide high-quality care improving oral and overall health

Why this is best: Beneficence: promote well-being and provide high-quality care improving oral and overall health. Justice — fairness and equity for all patients. Veracity — be truthful, including about the limits of the evidence and the likelihood of side effects The distractors correspond to other named concepts in this learning area (Collaboration and referral, Airflow, Airflow sensors, Clinical knowledge), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0867 §9.4 · Integrated clinical vignette

A patient with financial barriers cannot access a custom appliance. Which ethical principle requires attention to equitable access?

A. Non-maleficence alone

B. Veracity only

C. Technician independence

D. Autonomy alone

E. Justice

ANSWER: E · Justice

Why this is best: Justice addresses fairness and equity in access to appropriate care, balanced with clinical safety and scope rules.

Notebook anchor: *Integrated notebook, Topic 9, §9.4*

Q0868 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **Accurate, with a low failure rate, and attended by technical staff**?

A. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

B. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

C. where in-laboratory PSG is impossible because of immobility, safety or critical illness

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. but expensive and technically complex

ANSWER: E · but expensive and technically complex

Why this is best: In this context, the relevant notebook finding is **Accurate, with a low failure rate, and attended by technical staff**: but expensive and technically complex The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 188, §9.1*

Q0869 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **Accurate, with a low failure rate, and attended by technical staff**?

A. but expensive and technically complex

B. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

C. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. where in-laboratory PSG is impossible because of immobility, safety or critical illness

ANSWER: A · but expensive and technically complex

Why this is best: Accurate, with a low failure rate, and attended by technical staff: but expensive and technically complex The distractors correspond to other named concepts in this learning area (Portable monitoring, Special indications, Indication, In-laboratory PSG), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 188, §9.1*

Q0870 §9.4 · Integrated clinical vignette

A dentist accurately discloses uncertainty about the appliance's expected success and limitations of research. Which principle is best illustrated?

A. Marketing loyalty

B. Unjustified paternalism

C. Avoidance of consent

D. Excessive treatment

E. Veracity

ANSWER: E · Veracity

Why this is best: Veracity is truthful communication about evidence, realistic response rates, side effects and uncertainty.

Notebook anchor: *Integrated notebook, Topic 9, §9.4*

Q0871 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Ongoing clinical notes**?

A. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

B. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

C. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

D. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

E. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

ANSWER: B · patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

Why this is best: In this context, the relevant notebook finding is **Ongoing clinical notes**: patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists. AADSM specifies SOAP format — subjective, objective, assessment, plan The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0872 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Ongoing clinical notes**?

A. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

B. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

C. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

D. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

E. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

ANSWER: B · patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

Why this is best: Ongoing clinical notes: patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists. AADSM specifies SOAP format — subjective, objective, assessment, plan The distractors correspond to other named concepts in this learning area (Delivery documentation, Baseline sleep study data, Study models, Interprofessional communication), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0873 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Identify the correct codes**?

A. Keep clear, separate consultation notes differentiating between different insurers and different conditions

B. use DVA-specific codes, which can differ from those for the general public insurer or private insurance

C. Consult the payer's specific fee schedule or guidelines to ensure correct item numbers

D. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

E. Use item codes and descriptions specific to the service provided and to the patient's in- surer

ANSWER: E · Use item codes and descriptions specific to the service provided and to the patient's in- surer

Why this is best: In this context, the relevant notebook finding is **Identify the correct codes**: Use item codes and descriptions specific to the service provided and to the patient's in- surer The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 191, §9.3*

Q0874 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Identify the correct codes**?

A. use DVA-specific codes, which can differ from those for the general public insurer or private insurance

B. Keep clear, separate consultation notes differentiating between different insurers and different conditions

C. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

D. Use item codes and descriptions specific to the service provided and to the patient's in- surer

E. Consult the payer's specific fee schedule or guidelines to ensure correct item numbers

ANSWER: D · Use item codes and descriptions specific to the service provided and to the patient's in- surer

Why this is best: Identify the correct codes: Use item codes and descriptions specific to the service provided and to the patient's in-surer The distractors correspond to other named concepts in this learning area (Refer to payer guidelines, Department of Veterans' Affairs, Maintain detailed notes, Consult authoritative sources), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 191, §9.3*

Q0875 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **Scope of competence**?

A. ethical principles including billing and coding; accountability and team training with adherence to applicable laws

B. use evidence-based practices, stay updated, and provide treatment only within established competence

C. effective patient communication; barrier and bias mitigation; interprofessional and team communication

D. AHI ≥5 with symptoms or ≥15 regardless. Paediatric: obstructive AHI ≥1 abnormal; end-tidal CO2 standard in children

E. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

ANSWER: E · treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

Why this is best: In this context, the relevant notebook finding is **Scope of competence**: treat only within your qualifications and expertise; deliver care within the qualified dentist's competence The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0876 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **Scope of competence**?

A. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

B. AHI ≥5 with symptoms or ≥15 regardless. Paediatric: obstructive AHI ≥1 abnormal; end-tidal CO2 standard in children

C. use evidence-based practices, stay updated, and provide treatment only within established competence

D. effective patient communication; barrier and bias mitigation; interprofessional and team communication

E. ethical principles including billing and coding; accountability and team training with adherence to applicable laws

ANSWER: A · treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

Why this is best: Scope of competence: treat only within your qualifications and expertise; deliver care within the qualified dentist's competence The distractors correspond to other named concepts in this learning area (Interpersonal and communication skills, Non-maleficence (do no harm), Adult diagnosis, Professionalism), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0877 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **Portable monitoring**?

A. less expensive and quicker, and can be as accurate as PSG in selected populations

B. but expensive and technically complex

C. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

D. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

E. where in-laboratory PSG is impossible because of immobility, safety or critical illness

ANSWER: A · less expensive and quicker, and can be as accurate as PSG in selected populations

Why this is best: In this context, the relevant notebook finding is **Portable monitoring**: less expensive and quicker, and can be as accurate as PSG in selected populations. Patients must be at high risk for OSA and must not have comorbid medical or comorbid sleep disorders. Failure to meet these criteria in a patient at high risk of moderate–severe OSA should lead to in-laboratory PSG The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 183, §9.1*

Q0878 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **Portable monitoring**?

A. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

B. where in-laboratory PSG is impossible because of immobility, safety or critical illness

C. but expensive and technically complex

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

ANSWER: D · less expensive and quicker, and can be as accurate as PSG in selected populations

Why this is best: Portable monitoring: less expensive and quicker, and can be as accurate as PSG in selected populations. Patients must be at high risk for OSA and must not have comorbid medical or comorbid sleep disorders. Failure to meet these criteria in a patient at high risk of moderate–severe OSA should lead to in-laboratory PSG The distractors correspond to other named concepts in this learning area (Special indications, Indication, Accurate, with a low failure rate, and attended by technical staff, In-laboratory PSG), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 183, §9.1*

Q0879 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Baseline sleep study data**?

A. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

B. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

C. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

D. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

E. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

ANSWER: E · the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

Why this is best: In this context, the relevant notebook finding is **Baseline sleep study data**: the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison. Without a baseline you cannot demonstrate treatment effect The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0880 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Baseline sleep study data**?

A. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

B. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

C. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

D. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

E. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

ANSWER: B · the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

Why this is best: Baseline sleep study data: the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison. Without a baseline you cannot demonstrate treatment effect The distractors correspond to other named concepts in this learning area (Delivery documentation, Ongoing clinical notes, Interprofessional communication, Treatment planning), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0881 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Use DVA-specific codes**?

A. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

B. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

C. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

D. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

E. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

ANSWER: E · Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

Why this is best: In this context, the relevant notebook finding is **Use DVA-specific codes**: Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0882 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Use DVA-specific codes**?

A. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

B. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

C. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

D. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

E. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

ANSWER: A · Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

Why this is best: Use DVA-specific codes: Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance The distractors correspond to other named concepts in this learning area (Private billing, Separate conditions, Consult authoritative sources, Ethical dimension), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 192, §9.3*

Q0883 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **Autonomy**?

A. disclose all material information: the nature of the disorder; the purpose of and alternatives to treatment; and the potential risks and consequences, including the risks of no treatment

B. dentists are not officially authorised to diagnose sleep-related breathing disorders but can screen for symptoms, including tooth grinding and jaw pain, and recommend further testing

C. evidence-based decision-making; continuing education; reflective practice and personal growth; identifying opportunities to standardise, simplify and improve quality

D. screening and risk assessment; comprehensive DSM-focused medical and dental examination and history, including relevant imaging

E. patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives

ANSWER: E · patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives

Why this is best: In this context, the relevant notebook finding is **Autonomy**: patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0884 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **Autonomy**?

A. patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives

B. evidence-based decision-making; continuing education; reflective practice and personal growth; identifying opportunities to standardise, simplify and improve quality

C. dentists are not officially authorised to diagnose sleep-related breathing disorders but can screen for symptoms, including tooth grinding and jaw pain, and recommend further testing

D. disclose all material information: the nature of the disorder; the purpose of and alternatives to treatment; and the potential risks and consequences, including the risks of no treatment

E. screening and risk assessment; comprehensive DSM-focused medical and dental examination and history, including relevant imaging

ANSWER: A · patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives

Why this is best: Autonomy: patients have the right to make informed decisions; in practice, genuine informed consent requiring clear understanding of purpose, risks, benefits and alternatives The distractors correspond to other named concepts in this learning area (Learning and improvement, Screening versus diagnosis, Informed consent, Patient care), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0885 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **In-laboratory PSG**?

A. but expensive and technically complex

B. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

C. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

ANSWER: C · accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

Why this is best: In this context, the relevant notebook finding is **In-laboratory PSG**: accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 183, §9.1*

Q0886 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **In-laboratory PSG**?

A. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

B. but expensive and technically complex

C. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

D. performed only in conjunction with a comprehensive sleep evaluation, by a board-certified sleep specialist or an individual meeting the eligibility criteria for the sleep medicine certification examination

E. less expensive and quicker, and can be as accurate as PSG in selected populations

ANSWER: A · accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

Why this is best: In-laboratory PSG: accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea The distractors correspond to other named concepts in this learning area (Indication, Supervision, Portable monitoring, Accurate, with a low failure rate, and attended by technical staff), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 183, §9.1*

Q0887 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Interprofessional communication**?

A. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

B. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

C. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

D. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

E. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

ANSWER: A · records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

Why this is best: In this context, the relevant notebook finding is **Interprofessional communication**: records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0888 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Interprofessional communication**?

A. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

B. comprehensive plans setting out rationale, appliance choice, adjustments, titration and patient education, with any shared decision-making discussions about options and intolerances recorded

C. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

D. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

E. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

ANSWER: A · records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

Why this is best: Interprofessional communication: records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission The distractors correspond to other named concepts in this learning area (Treatment planning, Baseline sleep study data, Ongoing clinical notes, Delivery documentation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0889 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Ethical dimension**?

A. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

B. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

C. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

D. Clear, accurate documentation supports all billing claims and ensures a smooth billing process — and screening test scores must be recorded in the clinical record because they may be subject to audit

E. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

ANSWER: C · billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

Why this is best: In this context, the relevant notebook finding is **Ethical dimension**: billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.3*

Q0890 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Ethical dimension**?

A. Clear, accurate documentation supports all billing claims and ensures a smooth billing process — and screening test scores must be recorded in the clinical record because they may be subject to audit

B. billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

C. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

D. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

E. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

ANSWER: B · billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics

Why this is best: Ethical dimension: billing and coding fall explicitly within the professionalism competencies of a qualified dentist. Appliance selection must never be driven by what an insurer will reimburse. 9.4 Ethics The distractors correspond to other named concepts in this learning area (Separate conditions, Documentation is key, Use DVA-specific codes, Private billing), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.3*

Q0891 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **Professionalism**?

A. effective patient communication; barrier and bias mitigation; interprofessional and team communication

B. screening and risk assessment; comprehensive DSM-focused medical and dental examination and history, including relevant imaging

C. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

D. ethical principles including billing and coding; accountability and team training with adherence to applicable laws

E. AHI ≥5 with symptoms or ≥15 regardless. Paediatric: obstructive AHI ≥1 abnormal; end-tidal CO2 standard in children

ANSWER: D · ethical principles including billing and coding; accountability and team training with adherence to applicable laws

Why this is best: In this context, the relevant notebook finding is **Professionalism**: ethical principles including billing and coding; accountability and team training with adherence to applicable laws; SOAP documentation, record maintenance and protection of patient information; a collaborative interdisciplinary model; informed consent The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 187, §9.4*

Q0892 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **Professionalism**?

A. effective patient communication; barrier and bias mitigation; interprofessional and team communication

B. screening and risk assessment; comprehensive DSM-focused medical and dental examination and history, including relevant imaging

C. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

D. ethical principles including billing and coding; accountability and team training with adherence to applicable laws

E. AHI ≥5 with symptoms or ≥15 regardless. Paediatric: obstructive AHI ≥1 abnormal; end-tidal CO2 standard in children

ANSWER: D · ethical principles including billing and coding; accountability and team training with adherence to applicable laws

Why this is best: Professionalism: ethical principles including billing and coding; accountability and team training with adherence to applicable laws; SOAP documentation, record maintenance and protection of patient information; a collaborative interdisciplinary model; informed consent The distractors correspond to other named concepts in this learning area (Adult diagnosis, Scope of competence, Patient care, Interpersonal and communication skills), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 187, §9.4*

Q0893 §9.1 · Clinical-context application

Clinical context. A dentist is asked whether a questionnaire or portable test is sufficient to diagnose a sleep disorder. During this case discussion, how should the candidate interpret **Indication**?

A. but expensive and technically complex

B. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

C. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

D. less expensive and quicker, and can be as accurate as PSG in selected populations

E. where in-laboratory PSG is impossible because of immobility, safety or critical illness

ANSWER: B · may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

Why this is best: In this context, the relevant notebook finding is **Indication**: may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. Limited to high-risk populations by clinical judgement informed by demographics, predisposing and precipitating factors, clinical The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 184, §9.1*

Q0894 §9.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on physician diagnosis and portable testing, which description is specifically associated with **Indication**?

A. but expensive and technically complex

B. may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

C. accurate with a low failure rate, attended by technical staff; expensive and technically complex. Also the standard for narcolepsy and central sleep apnoea

D. where in-laboratory PSG is impossible because of immobility, safety or critical illness

E. less expensive and quicker, and can be as accurate as PSG in selected populations

ANSWER: B · may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA

Why this is best: Indication: may be used as an alternative to PSG in patients with a high pre-test probability of moderate-to-severe OSA. Limited to high-risk populations by clinical judgement informed by demographics, predisposing and precipitating factors, clinical The distractors correspond to other named concepts in this learning area (Special indications, Portable monitoring, In-laboratory PSG, Accurate, with a low failure rate, and attended by technical staff), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 184, §9.1*

Q0895 §9.2 · Clinical-context application

Clinical context. During clinical audit, the practice reviews documentation for an oral appliance patient. During this case discussion, how should the candidate interpret **Delivery documentation**?

A. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

B. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

C. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

D. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

E. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

ANSWER: A · written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

Why this is best: In this context, the relevant notebook finding is **Delivery documentation**: written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained. Side effects — document presence, management and manner of resolution, and communicate to the medical provider and general dentist The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0896 §9.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on clinical records and communication, which description is specifically associated with **Delivery documentation**?

A. impressions and bite registrations used for fabrication and adjustment, retained as long as local regulation requires

B. patient assessments, symptoms, progress, adherence and response, including evaluations and observations from both physicians and qualified dentists

C. written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

D. records of communication between sleep physicians, dentists and others, ensuring coordinated care with shared assessment, adjustments and follow-up, documented with patient permission

E. the PSG or home test results with detailed sleep parameters, AHI, architecture and arousals, properly stored for treatment reference and follow-up comparison

ANSWER: C · written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained

Why this is best: Delivery documentation: written home care instructions, delivery form and warranty, signed and dated by patient and team member, one copy to the patient and one retained. Side effects — document presence, management and manner of resolution, and communicate to the medical provider and general dentist The distractors correspond to other named concepts in this learning area (Ongoing clinical notes, Baseline sleep study data, Study models, Interprofessional communication), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 185, §9.2*

Q0897 §9.3 · Clinical-context application

Clinical context. An administrator asks how to code a dental sleep medicine service under different payment systems. During this case discussion, how should the candidate interpret **Private billing**?

A. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

B. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

C. Keep clear, separate consultation notes differentiating between different insurers and different conditions

D. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

E. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

ANSWER: D · identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

Why this is best: In this context, the relevant notebook finding is **Private billing**: identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions; consult the payer's fee schedule or guidelines for correct item numbers The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.3*

Q0898 §9.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on billing and payer codes, which description is specifically associated with **Private billing**?

A. the relevant government health department or insurer website — for current guidance rather than relying on memory or on colleagues

B. Keep clear, separate consultation notes differentiating between different insurers and different conditions

C. Veterans' affairs schemes maintain their own set of codes, which can differ from those used for the general public health insurer or for private insurance

D. identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

E. For veterans with both DVA and other insurance, clearly separate discussion and documentation of unrelated conditions when billing multiple insurers. Best practice for both

ANSWER: D · identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions

Why this is best: Private billing: identify codes specific to the service and the insurer; maintain clear, separate consultation notes differentiating insurers and conditions; consult the payer's fee schedule or guidelines for correct item numbers The distractors correspond to other named concepts in this learning area (Consult authoritative sources, Use DVA-specific codes, Maintain detailed notes, Separate conditions), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.3*

Q0899 §9.4 · Clinical-context application

Clinical context. An OAT clinic faces a question about informed choice, professional competence and financial interests. During this case discussion, how should the candidate interpret **Non-maleficence (do no harm)**?

A. use evidence-based practices, stay updated, and provide treatment only within established competence

B. effective patient communication; barrier and bias mitigation; interprofessional and team communication

C. oronasal thermal sensor (apnoea) + nasal pressure transducer (hypopnoea) — ideally both

D. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

E. basic sleep physiology including SDB pathophysiology; the roles of dentist and team members

ANSWER: A · use evidence-based practices, stay updated, and provide treatment only within established competence

Why this is best: In this context, the relevant notebook finding is **Non-maleficence (do no harm)**: use evidence-based practices, stay updated, and provide treatment only within established competence The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 186, §9.4*

Q0900 §9.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 9**, in the revision material on ethics and professional competence, which description is specifically associated with **Non-maleficence (do no harm)**?

A. oronasal thermal sensor (apnoea) + nasal pressure transducer (hypopnoea) — ideally both

B. treat only within your qualifications and expertise; deliver care within the qualified dentist's competence

C. basic sleep physiology including SDB pathophysiology; the roles of dentist and team members

D. effective patient communication; barrier and bias mitigation; interprofessional and team communication

E. use evidence-based practices, stay updated, and provide treatment only within established competence

ANSWER: E · use evidence-based practices, stay updated, and provide treatment only within established competence

Why this is best: Non-maleficence (do no harm): use evidence-based practices, stay updated, and provide treatment only within established competence The distractors correspond to other named concepts in this learning area (Interpersonal and communication skills, Clinical knowledge, Scope of competence, Airflow), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 186, §9.4*

TOPIC 10
Informed consent and management of adverse effects

100 questions | Full notebook pages 196–215 | Each question has a justification and linked learning code.

Q0901 §10.1 · Integrated clinical vignette

Before a MAD is fitted, a patient signs a form but cannot explain any long-term side effects. What is the best response?

A. Proceed without further discussion

B. The signature proves valid comprehension

C. Repeat the explanation and verify understanding rather than treating a signature as sufficient

D. Avoid mentioning alternatives

E. Ask the technician to consent for the patient

ANSWER: C · Repeat the explanation and verify understanding rather than treating a signature as sufficient

Why this is best: Informed consent requires capacity, voluntariness and comprehension; teach-back helps verify meaningful understanding.

Notebook anchor: *Integrated notebook, Topic 10, §10.1*

Q0902 §10.1 · All-of-the-above integrated judgement

Before long-term OAT, which is the MOST COMPLETE informed-consent discussion?

A. Diagnosis, purpose and expected benefit

B. Alternatives including no treatment

C. Short- and long-term dental/TMJ risks

D. Follow-up needs and ability to change course

E. All of the above

ANSWER: E · All of the above

Why this is best: Valid consent requires meaningful understanding of treatment, alternatives, foreseeable harm and continued management obligations.

Notebook anchor: *Integrated notebook, Topic 10, §10.1*

Q0903 §10.1 · Clinical-context application

Clinical context. A patient is consenting to long-term mandibular advancement and wants to understand alternative options. During this case discussion, how should the candidate interpret **Possible dental side effects, before starting therapy**?

A. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant…

B. at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered…

C. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

D. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

E. capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

ANSWER: B · at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered…

Why this is best: In this context, the relevant notebook finding is **Possible dental side effects, before starting therapy**: at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered aesthetics The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 205, §10.1*

Q0904 §10.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on valid informed consent, which description is specifically associated with **Possible dental side effects, before starting therapy**?

A. at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered…

B. capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

C. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

D. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant…

E. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

ANSWER: A · at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered…

Why this is best: Possible dental side effects, before starting therapy: at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and altered aesthetics The distractors correspond to other named concepts in this learning area (Three tests of valid consent, CPAP is not innocent, What patients actually experience, Frequency), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 205, §10.1*

Q0905 §10.1 · None-of-the-above discrimination

A patient has significant appliance-related jaw pain. Which action is appropriate without any evaluation?

A. Advance to maximum protrusion

B. Ignore the new pain

C. Permanently reshape the bite immediately

D. Assume the pain proves OSA cure

E. None of the above

ANSWER: E · None of the above

Why this is best: All proposed actions are inappropriate; the clinician must examine, adjust or suspend as needed and discuss appropriate alternatives.

Notebook anchor: *Integrated notebook, Topic 10, §10.1*

Q0906 §10.2 · Integrated clinical vignette

A person develops reduced overjet after five years of nightly MAD use. How should this be framed?

A. An impossible adverse effect

B. An isolated event unrelated to mandibular protrusion

C. A definitive marker that OSA is cured

D. A recognised dose- and time-dependent occlusal effect that should have been disclosed and monitored

E. Proof the patient fabricated the complaint

ANSWER: D · A recognised dose- and time-dependent occlusal effect that should have been disclosed and monitored

Why this is best: Posterior forces on upper teeth and anterior forces on lower teeth predict changes in overjet, overbite and tooth position over time.

Notebook anchor: *Integrated notebook, Topic 10, §10.2*

Q0907 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Mechanism**?

A. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

B. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

C. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

D. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

E. occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight

ANSWER: E · occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight

Why this is best: In this context, the relevant notebook finding is **Mechanism**: occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight. Side effects are a product of protruding the mandible to achieve a therapeutic effect, rather than of specific appliance design The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0908 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Mechanism**?

A. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

B. occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight

C. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

D. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

E. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

ANSWER: B · occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight

Why this is best: Mechanism: occlusal changes occur because of a failure to reposition the mandible during the day following use of the device, rather than as a result of immutable changes caused by the device overnight. Side effects are a product of protruding the mandible to achieve a therapeutic effect, rather than of specific appliance design The distractors correspond to other named concepts in this learning area (Direction of force, Other occlusal changes, Frequency, What patients actually experience), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0909 §10.4 · Integrated clinical vignette

A MAD user wakes with a temporary posterior open bite. Which preventive strategy appears in the notes?

A. No baseline models

B. Prolonged overnight mouth opening

C. Maximal extra protrusion on waking

D. A custom morning occlusal guide used for about 15–20 minutes

E. Immediate irreversible grinding of the bite

ANSWER: D · A custom morning occlusal guide used for about 15–20 minutes

Why this is best: A morning guide and jaw exercises may help return the mandible to habitual occlusion and document progression.

Notebook anchor: *Integrated notebook, Topic 10, §10.4*

Q0910 §10.3 · Clinical-context application

Clinical context. A patient with limited pretreatment overjet develops increasing dental and TMJ symptoms. During this case discussion, how should the candidate interpret **Soft tissues**?

A. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

B. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

C. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

ANSWER: E · hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

Why this is best: In this context, the relevant notebook finding is **Soft tissues**: hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue; and soft tissue adaptation — muscular activity adapts to the device, leading to superior and anterior repositioning of the hyoid The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0911 §10.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on craniofacial and tmj effects, which description is specifically associated with **Soft tissues**?

A. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

B. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

C. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

D. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

E. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

ANSWER: C · hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

Why this is best: Soft tissues: hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue; and soft tissue adaptation — muscular activity adapts to the device, leading to superior and anterior repositioning of the hyoid The distractors correspond to other named concepts in this learning area (Factors influencing severity, Corollary, Direction of force, Realigns the bite), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0912 §10.5 · Integrated clinical vignette

A patient develops mild transient salivation soon after appliance insertion. What is the usual first response?

A. Automatic permanent cessation

B. Maximal advancement

C. Diagnose central apnoea

D. Surgical removal of salivary glands

E. Reassure, assess device fit and monitor while giving practical advice

ANSWER: E · Reassure, assess device fit and monitor while giving practical advice

Why this is best: Early excess saliva is a common transient side effect; persistent or severe symptoms need assessment and modification.

Notebook anchor: *Integrated notebook, Topic 10, §10.5*

Q0913 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Exercises increase contact between the teeth**?

A. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

B. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

C. good in the short term, with limited long-term evidence

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. some protocols specify around 10 minutes

ANSWER: C · good in the short term, with limited long-term evidence

Why this is best: In this context, the relevant notebook finding is **Exercises increase contact between the teeth**: good in the short term, with limited long-term evidence The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 210, §10.4*

Q0914 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Exercises increase contact between the teeth**?

A. good in the short term, with limited long-term evidence

B. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

C. some protocols specify around 10 minutes

D. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

E. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

ANSWER: A · good in the short term, with limited long-term evidence

Why this is best: Exercises increase contact between the teeth: good in the short term, with limited long-term evidence The distractors correspond to other named concepts in this learning area (Wear for 15–20 minutes in the morning after waking, Exercises the jaw muscles, Positions the condyles, Realigns the bite), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 210, §10.4*

Q0915 §10.5 · Integrated clinical vignette

Persistent severe unilateral jaw pain develops during titration. What action is most appropriate?

A. Increase protrusion to maximum

B. Ignore it because TMD never matters

C. Classify the patient as cured

D. Suspend advancement, evaluate the TMJ and consider appliance adjustment or temporary cessation

E. Avoid any clinical examination

ANSWER: D · Suspend advancement, evaluate the TMJ and consider appliance adjustment or temporary cessation

Why this is best: Ongoing severe pain is a reason to modify or pause therapy, potentially with specialist referral, rather than continuing automatic advancement.

Notebook anchor: *Integrated notebook, Topic 10, §10.5*

Q0916 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Minor bite change**?

A. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

B. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

C. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

D. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

E. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

ANSWER: B · mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

Why this is best: In this context, the relevant notebook finding is **Minor bite change**: mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0917 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Minor bite change**?

A. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

B. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

C. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

D. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

E. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

ANSWER: A · mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

Why this is best: Minor bite change: mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings The distractors correspond to other named concepts in this learning area (Monitoring and adjustment, Soft tissue irritation, Transient morning jaw pain, Dry mouth), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0918 §10.6 · Integrated clinical vignette

A patient cannot tolerate further protrusion but retains substantial residual OSA. What should be discussed?

A. Device wear without adequate benefit indefinitely

B. Alternative or combination therapy through the sleep physician while prioritising safety

C. Unilateral tooth movement as the sole treatment

D. No further care needed

E. Immediate orthodontics without consent

ANSWER: B · Alternative or combination therapy through the sleep physician while prioritising safety

Why this is best: If therapeutic benefit is inadequate at a tolerable setting, multidisciplinary alternatives and informed decisions about stopping or changing treatment are required.

Notebook anchor: *Integrated notebook, Topic 10, §10.6*

Q0919 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Patient-specific factors**?

A. excessive salivation ~27% · dry mouth ~38% · tooth pain ~36% · allergy <1%

B. CPAP is the primary alternative, particularly for severe OSA

C. worn 15–20 minutes on waking, custom-made in centric occlusion

D. overall health, tolerance for side effects and commitment to therapy

E. at least 6-monthly for the first year, then annually

ANSWER: D · overall health, tolerance for side effects and commitment to therapy

Why this is best: In this context, the relevant notebook finding is **Patient-specific factors**: overall health, tolerance for side effects and commitment to therapy The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0920 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Patient-specific factors**?

A. at least 6-monthly for the first year, then annually

B. overall health, tolerance for side effects and commitment to therapy

C. excessive salivation ~27% · dry mouth ~38% · tooth pain ~36% · allergy <1%

D. CPAP is the primary alternative, particularly for severe OSA

E. worn 15–20 minutes on waking, custom-made in centric occlusion

ANSWER: B · overall health, tolerance for side effects and commitment to therapy

Why this is best: Patient-specific factors: overall health, tolerance for side effects and commitment to therapy The distractors correspond to other named concepts in this learning area (Short term, Morning aligner, Alternatives, Regular reviews), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0921 §10.3 · Integrated clinical vignette

A patient with minimal baseline overjet develops an anterior crossbite during prolonged therapy. Why were they vulnerable?

A. Only the lower molars move under MAD therapy

B. Small starting overjet leaves little margin for expected incisor movement

C. Overjet generally increases with MAD use

D. Crossbite proves no night-time use

E. Baseline occlusion has no predictive value

ANSWER: B · Small starting overjet leaves little margin for expected incisor movement

Why this is best: Initial edge-to-edge relations create vulnerability because the appliance tends to reduce overjet and overbite over time.

Notebook anchor: *Integrated notebook, Topic 10, §10.3*

Q0922 §10.1 · Clinical-context application

Clinical context. A patient is consenting to long-term mandibular advancement and wants to understand alternative options. During this case discussion, how should the candidate interpret **Three tests of valid consent**?

A. capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

B. at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and…

C. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

D. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

E. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

ANSWER: A · capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

Why this is best: In this context, the relevant notebook finding is **Three tests of valid consent**: capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 196, §10.1*

Q0923 §10.1 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on valid informed consent, which description is specifically associated with **Three tests of valid consent**?

A. at minimum the chance of reduction in overjet and over- bite leading to a suboptimal occlusion, with fewer occlusal contact points, possible unilateral or bilateral crossbite in the (pre)molar region, and…

B. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

C. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

D. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

E. capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

ANSWER: E · capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard

Why this is best: Three tests of valid consent: capacity, voluntariness (free of undue influence) and comprehension — verified, not assumed, for example by asking patients to repeat back what they heard The distractors correspond to other named concepts in this learning area (Possible dental side effects, before starting therapy, Other changes, Where noticed, symptoms of bite change are, Causes), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 196, §10.1*

Q0924 §10.6 · Integrated clinical vignette

After an appliance is discontinued, minor occlusal change is detected. What is the preferred initial approach in the notebook?

A. Immediate full-mouth reconstruction for every small change

B. Restart extreme protrusion

C. Ignore patient preference

D. No record of the issue

E. Review, allow time and consider conservative measures before irreversible correction

ANSWER: E · Review, allow time and consider conservative measures before irreversible correction

Why this is best: The source advises that some changes may settle after discontinuation and exercises; irreversible intervention should not be rushed.

Notebook anchor: *Integrated notebook, Topic 10, §10.6*

Q0925 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **CPAP is not innocent**?

A. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

B. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

C. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…

D. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

E. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

ANSWER: C · CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…

Why this is best: In this context, the relevant notebook finding is **CPAP is not innocent**: CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental change The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0926 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **CPAP is not innocent**?

A. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

B. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…

C. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

D. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

E. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

ANSWER: B · CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental…

Why this is best: CPAP is not innocent: CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant dental change The distractors correspond to other named concepts in this learning area (What patients actually experience, Direction of force, Other occlusal changes, Frequency), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0927 §10.1 · Integrated clinical vignette

A patient has capacity and freely consents but does not know that CPAP and no treatment are alternatives. What is missing?

A. An ESS score

B. A physical examination

C. Voluntariness

D. Capacity

E. Adequate disclosure of relevant alternatives and their consequences

ANSWER: E · Adequate disclosure of relevant alternatives and their consequences

Why this is best: An informed decision requires disclosure and understanding of the condition, proposed care, alternatives including no treatment, and benefits/risks.

Notebook anchor: *Integrated notebook, Topic 10, §10.1*

Q0928 §10.3 · Clinical-context application

Clinical context. A patient with limited pretreatment overjet develops increasing dental and TMJ symptoms. During this case discussion, how should the candidate interpret **Corollary**?

A. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

B. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

C. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

D. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: B · an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

Why this is best: In this context, the relevant notebook finding is **Corollary**: an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects. Minimising titration is a side-effect prevention strategy, not just a comfort measure The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0929 §10.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on craniofacial and tmj effects, which description is specifically associated with **Corollary**?

A. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

B. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

C. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

D. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: B · an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

Why this is best: Corollary: an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects. Minimising titration is a side-effect prevention strategy, not just a comfort measure The distractors correspond to other named concepts in this learning area (Factors influencing severity, Soft tissues, Where noticed, symptoms of bite change are, How it works), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0930 §10.5 · Integrated clinical vignette

A device repeatedly traumatizes the gingiva while the patient's AHI is improved. What is appropriate?

A. Remove all retention permanently

B. Assume dry mouth is the only cause

C. Relieve the source of trauma and reassess fit rather than accepting tissue injury as a necessary cost

D. Ignore trauma whenever AHI falls

E. Increase wear without adjustment

ANSWER: C · Relieve the source of trauma and reassess fit rather than accepting tissue injury as a necessary cost

Why this is best: Soft tissue irritation is addressed by correcting appliance contours or fit, hygiene and follow-up while preserving therapeutic function.

Notebook anchor: *Integrated notebook, Topic 10, §10.5*

Q0931 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Perform slowly and gently**?

A. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

B. those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

C. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: C · opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

Why this is best: In this context, the relevant notebook finding is **Perform slowly and gently**: opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position. These stretch the muscles of mastication and reduce discomfort The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 210, §10.4*

Q0932 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Perform slowly and gently**?

A. those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

B. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: D · opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

Why this is best: Perform slowly and gently: opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position. These stretch the muscles of mastication and reduce discomfort The distractors correspond to other named concepts in this learning area (Two fabrication types, How it works, Monitors progress, Realigns the bite), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 210, §10.4*

Q0933 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Regular follow-up**?

A. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

B. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

C. asymmetric or excessive advancement; interocclusal distance

D. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

E. every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

ANSWER: E · every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

Why this is best: In this context, the relevant notebook finding is **Regular follow-up**: every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0934 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Regular follow-up**?

A. every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

B. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

C. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

D. asymmetric or excessive advancement; interocclusal distance

E. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

ANSWER: A · every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

Why this is best: Regular follow-up: every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider The distractors correspond to other named concepts in this learning area (Persistent TMJ pain, Soft tissue irritation, Minor bite change, Monitoring and adjustment), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0935 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Device and settings**?

A. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

B. OJ ~1.9 mm and OB ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 y median −1.1 mm OJ and −1.6 mm OB

C. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

D. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

E. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

ANSWER: A · efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

Why this is best: In this context, the relevant notebook finding is **Device and settings**: efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0936 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Device and settings**?

A. OJ ~1.9 mm and OB ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 y median −1.1 mm OJ and −1.6 mm OB

B. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

C. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

D. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

E. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

ANSWER: D · efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

Why this is best: Device and settings: efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment The distractors correspond to other named concepts in this learning area (Where treatment stops, Regular follow-up, Treating established occlusal change, Occlusal change), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0937 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Direction of force**?

A. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

B. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

C. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

D. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

E. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

ANSWER: D · appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

Why this is best: In this context, the relevant notebook finding is **Direction of force**: appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth. Over time there is a dose-dependent shift in occlusion, with lower teeth moving anteriorly, upper teeth moving distally, and a loss of chewing contact area posteriorly The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0938 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Direction of force**?

A. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

B. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

C. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

D. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

E. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

ANSWER: A · appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

Why this is best: Direction of force: appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth. Over time there is a dose-dependent shift in occlusion, with lower teeth moving anteriorly, upper teeth moving distally, and a loss of chewing contact area posteriorly The distractors correspond to other named concepts in this learning area (Long term (moderate–severe, ongoing), Frequency, Mechanism, Position of professional bodies), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 207, §10.2*

Q0939 §10.3 · Clinical-context application

Clinical context. A patient with limited pretreatment overjet develops increasing dental and TMJ symptoms. During this case discussion, how should the candidate interpret **Factors influencing severity**?

A. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

B. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

C. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

D. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

E. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

ANSWER: B · risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

Why this is best: In this context, the relevant notebook finding is **Factors influencing severity**: risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk; underlying masticatory musculoskeletal disorders increase risk; appropriate planning and monitoring are crucial The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0940 §10.3 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on craniofacial and tmj effects, which description is specifically associated with **Factors influencing severity**?

A. hypersalivation or dry mouth; the intended airway enlargement by advancing the mandible and repositioning the tongue

B. an appliance is already effective at a less protrusive position in mild and moderate OSA, and therefore causes less severe dental side effects

C. risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

D. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

E. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

ANSWER: C · risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk

Why this is best: Factors influencing severity: risk increases with greater advancement, with levels exceeding 50% of maximum often correlating with significantly higher risk; underlying masticatory musculoskeletal disorders increase risk; appropriate planning and monitoring are crucial The distractors correspond to other named concepts in this learning area (Soft tissues, Corollary, Monitors progress, Mechanism), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 199, §10.3*

Q0941 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Two fabrication types**?

A. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

B. those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

C. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

D. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

E. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

ANSWER: B · those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

Why this is best: In this context, the relevant notebook finding is **Two fabrication types**: those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0942 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Two fabrication types**?

A. those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

B. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

C. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

D. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

E. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

ANSWER: A · those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion

Why this is best: Two fabrication types: those using thermoacrylic beads, and those using a prefabricated sheet (AM aligner) placed in hot water, moulded around the teeth, with the patient asked to bite in centric occlusion The distractors correspond to other named concepts in this learning area (Perform slowly and gently, How it works, Monitors progress, Exercises the jaw muscles), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0943 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Transient morning jaw pain**?

A. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

B. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

C. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

D. supportive care to manage discomfort, especially during the initial adaptation period

E. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

ANSWER: B · muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

Why this is best: In this context, the relevant notebook finding is **Transient morning jaw pain**: muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0944 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Transient morning jaw pain**?

A. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

B. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

C. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

D. supportive care to manage discomfort, especially during the initial adaptation period

E. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

ANSWER: A · muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

Why this is best: Transient morning jaw pain: muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide The distractors correspond to other named concepts in this learning area (Dry mouth, Bilateral jaw pain, Palliative care, Unilateral jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0945 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Where treatment stops**?

A. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

B. most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

C. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

D. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

E. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

ANSWER: A · notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

Why this is best: In this context, the relevant notebook finding is **Where treatment stops**: notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.6*

Q0946 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Where treatment stops**?

A. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

B. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

C. most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

D. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

E. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

ANSWER: B · notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

Why this is best: Where treatment stops: notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk The distractors correspond to other named concepts in this learning area (Regular follow-up, Device and settings, Treating established occlusal change, Leave it alone within the first 12–18 months), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.6*

Q0947 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Where noticed, symptoms of bite change are**?

A. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

B. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

C. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

D. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

E. overjet ~1.9 mm and overbite ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 years median −1.1 mm overjet and −1.6 mm overbite

ANSWER: C · gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

Why this is best: In this context, the relevant notebook finding is **Where noticed, symptoms of bite change are**: gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 208, §10.2*

Q0948 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Where noticed, symptoms of bite change are**?

A. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

B. overjet ~1.9 mm and overbite ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 years median −1.1 mm overjet and −1.6 mm overbite

C. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

D. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

E. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

ANSWER: A · gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

Why this is best: Where noticed, symptoms of bite change are: gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth The distractors correspond to other named concepts in this learning area (Magnitude, Causes, Other changes, Mechanism), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 208, §10.2*

Q0949 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Exercises the jaw muscles**?

A. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

B. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

E. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

ANSWER: B · applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

Why this is best: In this context, the relevant notebook finding is **Exercises the jaw muscles**: applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0950 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Exercises the jaw muscles**?

A. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

B. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

E. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

ANSWER: A · applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

Why this is best: Exercises the jaw muscles: applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear The distractors correspond to other named concepts in this learning area (Positions the condyles, Realigns the bite, Monitors progress, How it works), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0951 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Palliative care**?

A. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

B. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

C. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

D. supportive care to manage discomfort, especially during the initial adaptation period

E. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

ANSWER: D · supportive care to manage discomfort, especially during the initial adaptation period

Why this is best: In this context, the relevant notebook finding is **Palliative care**: supportive care to manage discomfort, especially during the initial adaptation period The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0952 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Palliative care**?

A. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

B. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

C. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

D. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

E. supportive care to manage discomfort, especially during the initial adaptation period

ANSWER: E · supportive care to manage discomfort, especially during the initial adaptation period

Why this is best: Palliative care: supportive care to manage discomfort, especially during the initial adaptation period The distractors correspond to other named concepts in this learning area (Unilateral jaw pain, Dry mouth, Other jaw pain, Bilateral jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0953 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Discuss with the patient and record the discussion**?

A. at least 6-monthly for the first year, then annually

B. CPAP is the primary alternative, particularly for severe OSA

C. jaw pain ~34% · bite change 14–26%

D. worn 15–20 minutes on waking, custom-made in centric occlusion

E. the conversation is only protective if documented

ANSWER: E · the conversation is only protective if documented

Why this is best: In this context, the relevant notebook finding is **Discuss with the patient and record the discussion**: the conversation is only protective if documented The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0954 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Discuss with the patient and record the discussion**?

A. CPAP is the primary alternative, particularly for severe OSA

B. worn 15–20 minutes on waking, custom-made in centric occlusion

C. jaw pain ~34% · bite change 14–26%

D. the conversation is only protective if documented

E. at least 6-monthly for the first year, then annually

ANSWER: D · the conversation is only protective if documented

Why this is best: Discuss with the patient and record the discussion: the conversation is only protective if documented The distractors correspond to other named concepts in this learning area (Regular reviews, Morning aligner, Alternatives, Long term), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0955 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Realigns the bite**?

A. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

B. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

C. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

D. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

E. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

ANSWER: B · custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

Why this is best: In this context, the relevant notebook finding is **Realigns the bite**: custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0956 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Realigns the bite**?

A. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

B. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

E. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

ANSWER: E · custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

Why this is best: Realigns the bite: custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation The distractors correspond to other named concepts in this learning area (Exercises the jaw muscles, Positions the condyles, Monitors progress, Perform slowly and gently), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0957 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Excessive salivation**?

A. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

B. supportive care to manage discomfort, especially during the initial adaptation period

C. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

D. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

E. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

ANSWER: C · the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

Why this is best: In this context, the relevant notebook finding is **Excessive salivation**: the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0958 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Excessive salivation**?

A. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

B. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

C. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

D. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

E. supportive care to manage discomfort, especially during the initial adaptation period

ANSWER: B · the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

Why this is best: Excessive salivation: the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks The distractors correspond to other named concepts in this learning area (Other jaw pain, Unilateral jaw pain, Palliative care, Bilateral jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0959 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Regular follow-up**?

A. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

B. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

C. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

D. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

E. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

ANSWER: C · monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

Why this is best: In this context, the relevant notebook finding is **Regular follow-up**: monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0960 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Regular follow-up**?

A. notify the medical provider, and refer non-compliant patients back to consider alternatives, since untreated or subtherapeutic OSA carries long-term risk

B. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

C. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

D. monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

E. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

ANSWER: D · monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness

Why this is best: Regular follow-up: monitoring by qualified dentists is necessary to survey for dental side effects and occlusal change, and is essential for maximising adherence and effectiveness The distractors correspond to other named concepts in this learning area (Treating established occlusal change, Where treatment stops, Device and settings, Remember the rule from 10.5), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0961 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Position of professional bodies**?

A. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

B. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

C. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

D. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

E. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

ANSWER: A · for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

Why this is best: In this context, the relevant notebook finding is **Position of professional bodies**: for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 208, §10.2*

Q0962 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Position of professional bodies**?

A. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

B. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

C. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

D. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

E. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

ANSWER: B · for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

Why this is best: Position of professional bodies: for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment The distractors correspond to other named concepts in this learning area (Mechanism, Frequency, Direction of force, Long term (moderate–severe, ongoing)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 208, §10.2*

Q0963 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Monitors progress**?

A. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

B. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

C. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: A · the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

Why this is best: In this context, the relevant notebook finding is **Monitors progress**: the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0964 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Monitors progress**?

A. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

B. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

ANSWER: C · the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

Why this is best: Monitors progress: the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points The distractors correspond to other named concepts in this learning area (How it works, Realigns the bite, Exercises the jaw muscles, Positions the condyles), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0965 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Patient education**?

A. reassure that symptoms are likely temporary and manageable

B. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

C. 4 nights with the patient to remove the appliance on waking

D. monitoring and adjustment; palliative care during adaptation

E. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

ANSWER: A · reassure that symptoms are likely temporary and manageable

Why this is best: In this context, the relevant notebook finding is **Patient education**: reassure that symptoms are likely temporary and manageable The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0966 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Patient education**?

A. 4 nights with the patient to remove the appliance on waking

B. reassure that symptoms are likely temporary and manageable

C. monitoring and adjustment; palliative care during adaptation

D. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

E. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

ANSWER: B · reassure that symptoms are likely temporary and manageable

Why this is best: Patient education: reassure that symptoms are likely temporary and manageable The distractors correspond to other named concepts in this learning area (Reduce wearing time where appropriate; ask the consider alternating regimens, The dentist's role, Other jaw pain, Excessive salivation), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0967 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Regular reviews**?

A. worn 15–20 minutes on waking, custom-made in centric occlusion

B. overall health, tolerance for side effects and commitment to therapy

C. the conversation is only protective if documented

D. at least 6-monthly for the first year, then annually

E. CPAP is the primary alternative, particularly for severe OSA

ANSWER: D · at least 6-monthly for the first year, then annually

Why this is best: In this context, the relevant notebook finding is **Regular reviews**: at least 6-monthly for the first year, then annually The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0968 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Regular reviews**?

A. the conversation is only protective if documented

B. CPAP is the primary alternative, particularly for severe OSA

C. at least 6-monthly for the first year, then annually

D. worn 15–20 minutes on waking, custom-made in centric occlusion

E. overall health, tolerance for side effects and commitment to therapy

ANSWER: C · at least 6-monthly for the first year, then annually

Why this is best: Regular reviews: at least 6-monthly for the first year, then annually The distractors correspond to other named concepts in this learning area (Alternatives, Discuss with the patient and record the discussion, Morning aligner, Patient-specific factors), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0969 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Positions the condyles**?

A. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

B. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

C. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

D. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: D · assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

Why this is best: In this context, the relevant notebook finding is **Positions the condyles**: assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0970 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Positions the condyles**?

A. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

B. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

C. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

ANSWER: E · assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

Why this is best: Positions the condyles: assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort The distractors correspond to other named concepts in this learning area (Exercises the jaw muscles, Realigns the bite, Monitors progress, How it works), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0971 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Soft tissue irritation**?

A. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

B. every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

C. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

D. asymmetric or excessive advancement; interocclusal distance

E. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

ANSWER: E · screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

Why this is best: In this context, the relevant notebook finding is **Soft tissue irritation**: screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0972 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Soft tissue irritation**?

A. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

B. every 6 months to a year, to evaluate for and manage side effects appropriately, with documentation of presence, management and resolution, and communication to the general dentist and medical pro- vider

C. screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

D. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

E. asymmetric or excessive advancement; interocclusal distance

ANSWER: C · screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax

Why this is best: Soft tissue irritation: screw mechanism or rough surface rubbing tongue or cheek; cheek or lip biting from bite change. → polish; discontinue until healed; change appliance; orthodontic wax The distractors correspond to other named concepts in this learning area (Minor bite change, Regular follow-up, Monitoring and adjustment, Persistent TMJ pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0973 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Occlusal change the patient finds unacceptable**?

A. orthodontics; orthognathic surgery (very rare); optimising the occlusion by occlusal equilibration and restoration — refer to a prosthodontist if the patient simply wants to chew well; prosthodontic overlays

B. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

C. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

D. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

E. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

ANSWER: D · treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

Why this is best: In this context, the relevant notebook finding is **Occlusal change the patient finds unacceptable**: treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed. You cannot simply stop treating a patient with a potentially serious condition The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.6*

Q0974 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Occlusal change the patient finds unacceptable**?

A. orthodontics; orthognathic surgery (very rare); optimising the occlusion by occlusal equilibration and restoration — refer to a prosthodontist if the patient simply wants to chew well; prosthodontic overlays

B. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

C. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

D. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

E. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

ANSWER: C · treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

Why this is best: Occlusal change the patient finds unacceptable: treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed. You cannot simply stop treating a patient with a potentially serious condition The distractors correspond to other named concepts in this learning area (Remember the rule from 10.5, Where treatment is stopped, Treating established occlusal change, If treatment is required), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.6*

Q0975 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **Wear for 15–20 minutes in the morning after waking**?

A. some protocols specify around 10 minutes

B. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

C. good in the short term, with limited long-term evidence

D. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

E. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

ANSWER: A · some protocols specify around 10 minutes

Why this is best: In this context, the relevant notebook finding is **Wear for 15–20 minutes in the morning after waking**: some protocols specify around 10 minutes The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0976 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **Wear for 15–20 minutes in the morning after waking**?

A. assists in repositioning the condyles to their pre-treatment alignment, reducing strain and discomfort

B. good in the short term, with limited long-term evidence

C. some protocols specify around 10 minutes

D. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

E. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

ANSWER: C · some protocols specify around 10 minutes

Why this is best: Wear for 15–20 minutes in the morning after waking: some protocols specify around 10 minutes The distractors correspond to other named concepts in this learning area (Exercises increase contact between the teeth, Positions the condyles, Exercises the jaw muscles, Realigns the bite), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 209, §10.4*

Q0977 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Monitoring and adjustment**?

A. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

B. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

C. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

D. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

E. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

ANSWER: B · a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

Why this is best: In this context, the relevant notebook finding is **Monitoring and adjustment**: a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0978 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Monitoring and adjustment**?

A. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

B. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

C. a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

D. mechanical forces causing gradual shifts. → jaw exercises, passive exercises, morning occlusal guide; adjust the appliance or settings

E. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

ANSWER: C · a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan

Why this is best: Monitoring and adjustment: a qualified dentist is essential for monitoring side effects and making necessary adjustments to the appliance or plan The distractors correspond to other named concepts in this learning area (Minor bite change, Transient morning jaw pain, Dry mouth, Bilateral jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 211, §10.5*

Q0979 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Leave it alone within the first 12–18 months**?

A. most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

B. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

C. the conversation is only protective if it is documented. N U M B E R S TO M E M O R I S E — TO P I C 1 0

D. salivation ~27% · dry mouth ~38% · tooth pain ~36% · allergy <1%. Long term: jaw pain ~34% · bite change 14– 26%

E. OJ ~1.9 mm and OB ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 y median −1.1 mm OJ and −1.6 mm OB

ANSWER: A · most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

Why this is best: In this context, the relevant notebook finding is **Leave it alone within the first 12–18 months**: most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0980 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Leave it alone within the first 12–18 months**?

A. efficacy and side effects depend on appliance type, degree of protrusion and other design settings, so modification may be preferable to abandonment

B. the conversation is only protective if it is documented. N U M B E R S TO M E M O R I S E — TO P I C 1 0

C. OJ ~1.9 mm and OB ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 y median −1.1 mm OJ and −1.6 mm OB

D. salivation ~27% · dry mouth ~38% · tooth pain ~36% · allergy <1%. Long term: jaw pain ~34% · bite change 14– 26%

E. most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

ANSWER: E · most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available

Why this is best: Leave it alone within the first 12–18 months: most will self-correct. This is why it is important to review vulnerable patients early, so that this option remains available The distractors correspond to other named concepts in this learning area (Occlusal change, Short term, Discuss with the patient and record the discussion, Device and settings), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0981 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **What patients actually experience**?

A. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

B. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

C. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

D. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

E. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant…

ANSWER: D · Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

Why this is best: In this context, the relevant notebook finding is **What patients actually experience**: Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0982 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **What patients actually experience**?

A. distally directed on the upper dentition, forwardly directed on the lower teeth → dose-dependent shift with lower teeth moving anteriorly, upper teeth distally, and loss of posterior chewing contact area

B. lower arch expansion, with increased lower intercanine and intermolar distance and decreased lower crowding — intercanine width increases continuously while intermolar width increases rapidly then tapers off

C. CPAP also causes retroclination of the upper incisors, from posteriorly directed forces on the anterior maxilla by a tightly fitted nasal mask, and long-term use of both CPAP and appliances produces significant…

D. approximately 80% of patients who use an appliance every night over 7 years will experience occlusal change. 85% had bite change detectable on study models and cephalograms — though some bites do not change

E. Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

ANSWER: E · Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on

Why this is best: What patients actually experience: Mostly changes are minor and unnoticed; only a minority notice. Most are unaware, untroubled, and do not want to stop. Where noticed: gaps and food impaction; occasional lisping; rarely difficulty chewing on The distractors correspond to other named concepts in this learning area (Other occlusal changes, Frequency, Direction of force, CPAP is not innocent), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0983 §10.4 · Clinical-context application

Clinical context. A patient has difficulty achieving habitual occlusion immediately after removing the splint. During this case discussion, how should the candidate interpret **How it works**?

A. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

B. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

C. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

D. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: E · realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

Why this is best: In this context, the relevant notebook finding is **How it works**: realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear; assists repositioning of the condyles to pre-treatment alignment, reducing strain; and monitors progress — if it does not fit, contact the practice The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 199, §10.4*

Q0984 §10.4 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on morning guide and exercises, which description is specifically associated with **How it works**?

A. applying pressure exercises and stretches the muscles of mastication, which are tight after a night of wear

B. opening the mouth wide, moving the jaw left and right, resting the chin on a fist to let the jaw settle, and curling the tongue back to find the retruded position

C. custom-made to fit the patient's upper and lower teeth, guiding them back to their original maximum intercuspation

D. the patient uses it each morning to check jaw alignment; if it does not fit, they should contact the practice. Practical points

E. realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

ANSWER: E · realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear

Why this is best: How it works: realigns the bite toward original maximum intercuspation; exercises and stretches the muscles of mastication, tight after a night of wear; assists repositioning of the condyles to pre-treatment alignment, reducing strain; and monitors progress — if it does not fit, contact the practice The distractors correspond to other named concepts in this learning area (Monitors progress, Realigns the bite, Perform slowly and gently, Exercises the jaw muscles), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 199, §10.4*

Q0985 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Unilateral jaw pain**?

A. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

B. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

C. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

D. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

E. supportive care to manage discomfort, especially during the initial adaptation period

ANSWER: A · prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

Why this is best: In this context, the relevant notebook finding is **Unilateral jaw pain**: prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines; uneven occlusal contact; asymmetrical protrusion. → self-care programme; retrude the screw mechanism on the affected side by 0.5–1 mm; adjust occlusion for bilateral contact; confirm seating; reset to a new bite registration The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0986 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Unilateral jaw pain**?

A. the appliance itself. → towel on the pillow; reassurance, resolving in 2–3 weeks

B. supportive care to manage discomfort, especially during the initial adaptation period

C. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

D. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

E. bruxism while side-sleeping with pressure on the jaw; biting into food; trauma

ANSWER: C · prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

Why this is best: Unilateral jaw pain: prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines; uneven occlusal contact; asymmetrical protrusion. → self-care programme; retrude the screw mechanism on the affected side by 0.5–1 mm; adjust occlusion for bilateral contact; confirm seating; reset to a new bite registration The distractors correspond to other named concepts in this learning area (Palliative care, Other jaw pain, Excessive salivation, Transient morning jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0987 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Occlusal change**?

A. orthodontics; orthognathic surgery (very rare); optimising the occlusion by occlusal equilibration and restoration — refer to a prosthodontist if the patient simply wants to chew well; prosthodontic overlays

B. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

C. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

D. OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

E. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

ANSWER: D · OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

Why this is best: In this context, the relevant notebook finding is **Occlusal change**: OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0988 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Occlusal change**?

A. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

B. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

C. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

D. OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

E. orthodontics; orthognathic surgery (very rare); optimising the occlusion by occlusal equilibration and restoration — refer to a prosthodontist if the patient simply wants to chew well; prosthodontic overlays

ANSWER: D · OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

Why this is best: Occlusal change: OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant The distractors correspond to other named concepts in this learning area (Where treatment is stopped, Occlusal change the patient finds unacceptable, Remember the rule from 10.5, If treatment is required), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 201, §10.6*

Q0989 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Causes**?

A. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

B. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

C. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

D. overjet ~1.9 mm and overbite ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 years median −1.1 mm overjet and −1.6 mm overbite

E. reduced overjet ~1.9 mm and overbite ~2.3 mm; decreased overjet and overbite during the first 5 years; around one third of patients experience more than 1 mm of change

ANSWER: B · what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

Why this is best: In this context, the relevant notebook finding is **Causes**: what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change. Functional repositioning and neuromuscular adaptation: unlikely. Tilting of teeth is the main cause — retroclination of upper incisors, proclination of lower incisors, mesial tendency in the molar relationship, overall overjet reduction. WHO IS VULNERABLE? The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0990 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Causes**?

A. lower arch expansion — increased intercanine (continuously) and intermolar (rapidly then tapering) width, reduced lower crowding, opening of interdental spaces

B. gaps and food impaction; occasional lisping when talking; and — rarely — difficulty chewing on the back teeth or tearing food with the front teeth

C. what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

D. overjet ~1.9 mm and overbite ~2.3 mm reduction (average ~2.5 mm described); ~⅓ exceed 1 mm; at 17 years median −1.1 mm overjet and −1.6 mm overbite

E. reduced overjet ~1.9 mm and overbite ~2.3 mm; decreased overjet and overbite during the first 5 years; around one third of patients experience more than 1 mm of change

ANSWER: C · what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change

Why this is best: Causes: what it is not. Change in mandibular length: unlikely. Change in skeletal pattern: unlikely — appliances cause tooth movement, not craniofacial skeletal change. Functional repositioning and neuromuscular adaptation: unlikely. Tilting of teeth is the main cause — retroclination of upper incisors, proclination of lower incisors, mesial tendency in the molar relationship, overall overjet reduction. WHO IS VULNERABLE? The distractors correspond to other named concepts in this learning area (Other changes, Where noticed, symptoms of bite change are, Magnitude, Magnitude), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0991 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Dry mouth**?

A. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

B. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

C. supportive care to manage discomfort, especially during the initial adaptation period

D. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

E. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

ANSWER: D · poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

Why this is best: In this context, the relevant notebook finding is **Dry mouth**: poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult. → improve nasal breathing (treat rhinitis, refer to ENT); reduce vertical dimension; elastic or chin strap; dry mouth gels; increase fluids, humidifier; chewing gum or xylitol lozenges The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0992 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Dry mouth**?

A. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

B. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

C. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

D. supportive care to manage discomfort, especially during the initial adaptation period

E. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

ANSWER: A · poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

Why this is best: Dry mouth: poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult. → improve nasal breathing (treat rhinitis, refer to ENT); reduce vertical dimension; elastic or chin strap; dry mouth gels; increase fluids, humidifier; chewing gum or xylitol lozenges The distractors correspond to other named concepts in this learning area (Bilateral jaw pain, Transient morning jaw pain, Unilateral jaw pain, Palliative care), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0993 §10.6 · Clinical-context application

Clinical context. Severe side effects or inadequate OSA control lead the clinician to reconsider continued OAT. During this case discussion, how should the candidate interpret **Remember the rule from 10.5**?

A. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

B. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

C. OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

D. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

E. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

ANSWER: B · definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

Why this is best: In this context, the relevant notebook finding is **Remember the rule from 10.5**: definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0994 §10.6 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on suspending or abandoning treatment, which description is specifically associated with **Remember the rule from 10.5**?

A. OJ ~1.9 mm, OB ~2.3 mm (average ~2.5 mm); ~⅓ exceed 1 mm; at 17 y median −1.1 / −1.6 mm; ~80% of 7-year nightly users; 85% detectable on models and cephalograms; 13.3% by 2 years then constant

B. definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

C. notify the medical provider, and refer non-compliant patients back to the medical provider to consider alternative therapies, since untreated or subtherapeutic OSA carries long-term health risk

D. only a small group need anything. Leave it alone within the first 12–18 months — most self-correct, which is why reviewing vulnerable patients early keeps this option open

E. treatment should be discontinued only if the patient accepts another form of therapy from their attending physician, or understands the risks of discontinuation and their physician is informed

ANSWER: B · definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn

Why this is best: Remember the rule from 10.5: definitive correction should be undertaken only once appliance therapy has been discontinued and the patient is on another OSA treatment, because change continues while the appliance is worn The distractors correspond to other named concepts in this learning area (Occlusal change, Where treatment is stopped, Occlusal change the patient finds unacceptable, Treating established occlusal change), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 212, §10.6*

Q0995 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Frequency**?

A. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

B. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

C. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

D. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

E. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

ANSWER: E · ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

Why this is best: In this context, the relevant notebook finding is **Frequency**: ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change); the proportion rose over the first 2 years to 13.3% of cases then remained relatively constant The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0996 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Frequency**?

A. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

B. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

C. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

D. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

E. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

ANSWER: B · ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

Why this is best: Frequency: ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change); the proportion rose over the first 2 years to 13.3% of cases then remained relatively constant The distractors correspond to other named concepts in this learning area (Position of professional bodies, Mechanism, Direction of force, Long term (moderate–severe, ongoing)), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q0997 §10.5 · Clinical-context application

Clinical context. A patient attends urgently because of a new oral appliance-related adverse effect. During this case discussion, how should the candidate interpret **Bilateral jaw pain**?

A. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

B. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

C. supportive care to manage discomfort, especially during the initial adaptation period

D. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

E. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

ANSWER: E · advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

Why this is best: In this context, the relevant notebook finding is **Bilateral jaw pain**: advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); appliance too tight or loose. → retrude the mandible for a few weeks then gradually advance; modify vertical dimension; adjust retention; rest, ice, soft diet, anti-inflammatories; supportive exercise; decrease the titration rate The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0998 §10.5 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on managing side effects, which description is specifically associated with **Bilateral jaw pain**?

A. muscles adjusting to the forward mandible. → jaw exercises, massage, heat; morning occlusal guide

B. prone (stomach) sleeping; appliance not seating; imbalance in appliance or midlines

C. supportive care to manage discomfort, especially during the initial adaptation period

D. poor nasal breathing → mouth breathing; vertical dimension too large making lip seal difficult

E. advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

ANSWER: E · advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed)

Why this is best: Bilateral jaw pain: advancement beyond TMJ tolerance; vertical dimension outside tolerance (too open or closed); appliance too tight or loose. → retrude the mandible for a few weeks then gradually advance; modify vertical dimension; adjust retention; rest, ice, soft diet, anti-inflammatories; supportive exercise; decrease the titration rate The distractors correspond to other named concepts in this learning area (Dry mouth, Transient morning jaw pain, Palliative care, Unilateral jaw pain), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 200, §10.5*

Q0999 §10.2 · Clinical-context application

Clinical context. After several years of nightly OAT, a patient reports changes in bite and jaw comfort. During this case discussion, how should the candidate interpret **Long term (moderate–severe, ongoing)**?

A. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

B. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

C. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

D. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

E. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

ANSWER: E · jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

Why this is best: In this context, the relevant notebook finding is **Long term (moderate–severe, ongoing)**: jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues. Significant and persistent TMJ problems are rare. Change in facial height and jaw relation noted as early as 6 months. OCCLUSAL CHANGE — THE QUANTITATIVE DETAIL The remaining choices describe different mechanisms, indices or actions, so should not be substituted for this specific concept.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Q1000 §10.2 · Targeted knowledge / discriminative SBA

In IFDSM **Topic 10**, in the revision material on short/long-term side effects, which description is specifically associated with **Long term (moderate–severe, ongoing)**?

A. appliances produce distally directed forces on the upper dentition and forwardly directed forces on the lower teeth

B. jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

C. change occurs because of a failure to reposition the mandible during the day after use, rather than immutable overnight change

D. ~80% of nightly users over 7 years experience change; 85% detectable on models and cephalograms (some bites do not change)

E. for most people the change in bite is not annoying, is not a risk to oral health, and does not require further dental treatment

ANSWER: B · jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues

Why this is best: Long term (moderate–severe, ongoing): jaw pain ~34%; TMJ restriction or noise; bite change 14–26%; damage to teeth or dental work; appliance issues. Significant and persistent TMJ problems are rare. Change in facial height and jaw relation noted as early as 6 months. OCCLUSAL CHANGE — THE QUANTITATIVE DETAIL The distractors correspond to other named concepts in this learning area (Direction of force, Frequency, Mechanism, Position of professional bodies), rather than the term asked about.

Notebook anchor: *Integrated notebook, page 198, §10.2*

Master answer key

This is for marking only. The full explanation and notebook cross-reference remain beneath each question in the preceding sections.

Topic 1 — Normal sleep, sleep disorders and pathophysiology of SDB

1:**C** 2:**E** 3:**C** 4:**D** 5:**E** 6:**D** 7:**A** 8:**A**
9:**B** 10:**B** 11:**B** 12:**E** 13:**D** 14:**E** 15:**C** 16:**B**
17:**B** 18:**D** 19:**A** 20:**B** 21:**D** 22:**E** 23:**B** 24:**C**
25:**A** 26:**E** 27:**B** 28:**A** 29:**B** 30:**E** 31:**A** 32:**A**
33:**C** 34:**B** 35:**C** 36:**B** 37:**E** 38:**C** 39:**C** 40:**E**
41:**E** 42:**C** 43:**B** 44:**B** 45:**E** 46:**B** 47:**B** 48:**E**
49:**A** 50:**E** 51:**E** 52:**D** 53:**E** 54:**B** 55:**C** 56:**A**
57:**A** 58:**C** 59:**E** 60:**D** 61:**B** 62:**B** 63:**B** 64:**E**
65:**D** 66:**C** 67:**B** 68:**C** 69:**D** 70:**A** 71:**A** 72:**E**
73:**C** 74:**A** 75:**E** 76:**D** 77:**B** 78:**D** 79:**B** 80:**C**
81:**D** 82:**E** 83:**A** 84:**E** 85:**A** 86:**E** 87:**B** 88:**C**
89:**E** 90:**E** 91:**E** 92:**B** 93:**A** 94:**D** 95:**B** 96:**C**
97:**C** 98:**B** 99:**E** 100:**C** 101:**D** 102:**B** 103:**C** 104:**B**
105:**D** 106:**D** 107:**B** 108:**A** 109:**C** 110:**B** 111:**B** 112:**E**
113:**D** 114:**A** 115:**A** 116:**D** 117:**D** 118:**D** 119:**C** 120:**D**
121:**B** 122:**B** 123:**A** 124:**B** 125:**B** 126:**B** 127:**D** 128:**C**
129:**E** 130:**A**

Topic 2 — Diagnostic sleep testing and interpretation

131:**A** 132:**E** 133:**E** 134:**C** 135:**E** 136:**E** 137:**D** 138:**C**
139:**A** 140:**A** 141:**E** 142:**A** 143:**C** 144:**E** 145:**A** 146:**D**
147:**C** 148:**C** 149:**B** 150:**B** 151:**B** 152:**C** 153:**E** 154:**B**
155:**A** 156:**C** 157:**B** 158:**D** 159:**C** 160:**A** 161:**E** 162:**E**
163:**A** 164:**E** 165:**C** 166:**C** 167:**E** 168:**D** 169:**B** 170:**B**
171:**C** 172:**A** 173:**A** 174:**B** 175:**B** 176:**B** 177:**E** 178:**D**
179:**E** 180:**C** 181:**B** 182:**E** 183:**C** 184:**C** 185:**D** 186:**C**
187:**C** 188:**E** 189:**B** 190:**A** 191:**B** 192:**C** 193:**E** 194:**D**
195:**B** 196:**C** 197:**D** 198:**E** 199:**D** 200:**C** 201:**E** 202:**C**
203:**D** 204:**D** 205:**D** 206:**E** 207:**E** 208:**C** 209:**D** 210:**D**
211:**C** 212:**A** 213:**B** 214:**A** 215:**D** 216:**E** 217:**C** 218:**E**
219:**C** 220:**A** 221:**A** 222:**B** 223:**E** 224:**B** 225:**A** 226:**D**
227:**E** 228:**B** 229:**C** 230:**A** 231:**C** 232:**C** 233:**C** 234:**D**
235:**D** 236:**C** 237:**E** 238:**D** 239:**E** 240:**B** 241:**D** 242:**A**
243:**B** 244:**A** 245:**B**

Topic 3 — Oral appliance therapy and other treatments

246:**B** 247:**E** 248:**C** 249:**C** 250:**E** 251:**C** 252:**B** 253:**D**
254:**A** 255:**B** 256:**A** 257:**E** 258:**D** 259:**D** 260:**B** 261:**D**
262:**E** 263:**A** 264:**B** 265:**D** 266:**C** 267:**B** 268:**B** 269:**C**
270:**C** 271:**B** 272:**E** 273:**C** 274:**B** 275:**E** 276:**A** 277:**A**
278:**C** 279:**C** 280:**A** 281:**A** 282:**B** 283:**E** 284:**B** 285:**E**
286:**B** 287:**D** 288:**E** 289:**B** 290:**C** 291:**D** 292:**A** 293:**A**
294:**A** 295:**B** 296:**B** 297:**C** 298:**D** 299:**A** 300:**A** 301:**E**
302:**B** 303:**A** 304:**A** 305:**B** 306:**E** 307:**B** 308:**B** 309:**A**

310:**C** 311:**E** 312:**A** 313:**E** 314:**B** 315:**B** 316:**A** 317:**E**
318:**C** 319:**B** 320:**B** 321:**C** 322:**C** 323:**D** 324:**A** 325:**B**
326:**A** 327:**C** 328:**B** 329:**D** 330:**E** 331:**A** 332:**C** 333:**B**
334:**C** 335:**E** 336:**E** 337:**E** 338:**A** 339:**D** 340:**D** 341:**E**
342:**E** 343:**E** 344:**D** 345:**D** 346:**B** 347:**A** 348:**A** 349:**D**
350:**E** 351:**A** 352:**C** 353:**C** 354:**D** 355:**D** 356:**B** 357:**B**
358:**D** 359:**E** 360:**A** 361:**C** 362:**D** 363:**C** 364:**E** 365:**D**
366:**D** 367:**C** 368:**D** 369:**B** 370:**E**

Topic 4 — History, dental examination, imaging and patient selection

371:**C** 372:**E** 373:**E** 374:**D** 375:**E** 376:**D** 377:**E** 378:**E**
379:**B** 380:**B** 381:**D** 382:**E** 383:**B** 384:**C** 385:**A** 386:**D**
387:**C** 388:**B** 389:**B** 390:**E** 391:**B** 392:**C** 393:**C** 394:**D**
395:**B** 396:**A** 397:**D** 398:**E** 399:**D** 400:**C** 401:**D** 402:**A**
403:**E** 404:**E** 405:**C** 406:**A** 407:**C** 408:**E** 409:**B** 410:**A**
411:**B** 412:**D** 413:**A** 414:**E** 415:**C** 416:**B** 417:**E** 418:**E**
419:**B** 420:**E** 421:**C** 422:**B** 423:**B** 424:**D** 425:**B** 426:**C**
427:**E** 428:**E** 429:**E** 430:**D** 431:**D** 432:**C** 433:**C** 434:**C**
435:**E** 436:**E** 437:**D** 438:**C** 439:**E** 440:**A** 441:**B** 442:**C**
443:**A** 444:**E** 445:**E** 446:**E** 447:**B** 448:**D** 449:**A** 450:**E**
451:**C** 452:**B** 453:**E** 454:**D** 455:**D** 456:**E** 457:**B** 458:**C**
459:**A** 460:**E** 461:**E** 462:**A** 463:**D** 464:**C** 465:**D** 466:**D**
467:**B** 468:**B** 469:**C** 470:**C**

Topic 5 — Appliance design, registration, manufacture and fitting

471:**D** 472:**E** 473:**C** 474:**A** 475:**E** 476:**B** 477:**D** 478:**D**
479:**B** 480:**E** 481:**B** 482:**B** 483:**C** 484:**A** 485:**E** 486:**C**
487:**C** 488:**B** 489:**A** 490:**E** 491:**B** 492:**D** 493:**A** 494:**B**
495:**A** 496:**D** 497:**D** 498:**A** 499:**C** 500:**E** 501:**C** 502:**D**
503:**C** 504:**C** 505:**A** 506:**D** 507:**E** 508:**B** 509:**C** 510:**C**
511:**A** 512:**C** 513:**A** 514:**C** 515:**A** 516:**D** 517:**E** 518:**A**
519:**E** 520:**C** 521:**D** 522:**C** 523:**C** 524:**A** 525:**C** 526:**E**
527:**E** 528:**C** 529:**C** 530:**D** 531:**B** 532:**D** 533:**D** 534:**C**
535:**E** 536:**C** 537:**C** 538:**C** 539:**C** 540:**D** 541:**A** 542:**A**
543:**E** 544:**E** 545:**D** 546:**D** 547:**B** 548:**D** 549:**C** 550:**C**
551:**D** 552:**B** 553:**E** 554:**A** 555:**A** 556:**E** 557:**C** 558:**B**
559:**B** 560:**A** 561:**C** 562:**E** 563:**C** 564:**B** 565:**E** 566:**D**
567:**E** 568:**A** 569:**E** 570:**A** 571:**D** 572:**D** 573:**A** 574:**C**
575:**D** 576:**D** 577:**D** 578:**D** 579:**C** 580:**E** 581:**B** 582:**B**
583:**D** 584:**A** 585:**E** 586:**B** 587:**B** 588:**B** 589:**D** 590:**D**
591:**E** 592:**A** 593:**D** 594:**A** 595:**C** 596:**C** 597:**E** 598:**C**
599:**C** 600:**C**

Topic 6 — Titration, treatment effectiveness and adherence

601:**C** 602:**E** 603:**A** 604:**B** 605:**E** 606:**A** 607:**E** 608:**B**
609:**A** 610:**E** 611:**A** 612:**C** 613:**B** 614:**A** 615:**E** 616:**D**
617:**C** 618:**E** 619:**E** 620:**B** 621:**B** 622:**A** 623:**C** 624:**E**
625:**C** 626:**C** 627:**B** 628:**D** 629:**A** 630:**B** 631:**B** 632:**A**
633:**E** 634:**E** 635:**C** 636:**E** 637:**C** 638:**B** 639:**D** 640:**E**
641:**C** 642:**A** 643:**E** 644:**A** 645:**A** 646:**D** 647:**E** 648:**C**
649:**E** 650:**D** 651:**B** 652:**D** 653:**E** 654:**D** 655:**D** 656:**D**

657:**C** 658:**B** 659:**D** 660:**A** 661:**D** 662:**B** 663:**E** 664:**A**
665:**D** 666:**B** 667:**E** 668:**B** 669:**A** 670:**A** 671:**D** 672:**E**
673:**E** 674:**A** 675:**C** 676:**A** 677:**C** 678:**B** 679:**E** 680:**E**

Topic 7 — Long-term oral appliance follow-up

681:**A** 682:**E** 683:**E** 684:**D** 685:**E** 686:**E** 687:**E** 688:**C**
689:**E** 690:**B** 691:**A** 692:**B** 693:**E** 694:**B** 695:**B** 696:**B**
697:**D** 698:**A** 699:**E** 700:**B** 701:**C** 702:**C** 703:**C** 704:**C**
705:**A** 706:**B** 707:**A** 708:**A** 709:**B** 710:**D** 711:**B** 712:**B**
713:**E** 714:**A** 715:**B** 716:**A** 717:**D** 718:**D** 719:**E** 720:**E**
721:**B** 722:**E** 723:**D** 724:**B** 725:**B** 726:**D** 727:**D** 728:**E**
729:**A** 730:**A** 731:**A** 732:**D** 733:**A** 734:**C** 735:**D** 736:**D**
737:**D** 738:**D** 739:**B** 740:**A**

Topic 8 — Paediatric sleep-disordered breathing

741:**E** 742:**E** 743:**E** 744:**B** 745:**E** 746:**E** 747:**E** 748:**B**
749:**C** 750:**A** 751:**B** 752:**D** 753:**B** 754:**D** 755:**A** 756:**C**
757:**E** 758:**E** 759:**D** 760:**A** 761:**C** 762:**C** 763:**D** 764:**C**
765:**D** 766:**A** 767:**D** 768:**A** 769:**B** 770:**A** 771:**B** 772:**A**
773:**D** 774:**D** 775:**D** 776:**D** 777:**A** 778:**E** 779:**C** 780:**D**
781:**A** 782:**E** 783:**D** 784:**E** 785:**B** 786:**E** 787:**C** 788:**A**
789:**D** 790:**E** 791:**D** 792:**E** 793:**B** 794:**E** 795:**E** 796:**A**
797:**D** 798:**D** 799:**A** 800:**E** 801:**B** 802:**D** 803:**C** 804:**A**
805:**E** 806:**E** 807:**C** 808:**A** 809:**B** 810:**E** 811:**B** 812:**D**
813:**B** 814:**A** 815:**A** 816:**A** 817:**C** 818:**B** 819:**B** 820:**E**
821:**D** 822:**E** 823:**B** 824:**B** 825:**B** 826:**D** 827:**C** 828:**C**
829:**D** 830:**E** 831:**C** 832:**A** 833:**D** 834:**A** 835:**E** 836:**A**
837:**C** 838:**A** 839:**E** 840:**D**

Topic 9 — Best-practice, documentation and ethical responsibilities

841:**D** 842:**E** 843:**C** 844:**C** 845:**E** 846:**A** 847:**D** 848:**E**
849:**E** 850:**C** 851:**E** 852:**A** 853:**A** 854:**C** 855:**D** 856:**D**
857:**A** 858:**E** 859:**E** 860:**E** 861:**E** 862:**C** 863:**B** 864:**D**
865:**A** 866:**C** 867:**E** 868:**E** 869:**A** 870:**E** 871:**B** 872:**B**
873:**E** 874:**D** 875:**E** 876:**A** 877:**A** 878:**D** 879:**E** 880:**B**
881:**E** 882:**A** 883:**E** 884:**A** 885:**C** 886:**A** 887:**A** 888:**A**
889:**C** 890:**B** 891:**D** 892:**D** 893:**B** 894:**B** 895:**A** 896:**C**
897:**D** 898:**D** 899:**A** 900:**E**

Topic 10 — Informed consent and management of adverse effects

901:**C** 902:**E** 903:**B** 904:**A** 905:**E** 906:**D** 907:**E** 908:**B**
909:**D** 910:**E** 911:**C** 912:**E** 913:**C** 914:**A** 915:**D** 916:**B**
917:**A** 918:**B** 919:**D** 920:**B** 921:**B** 922:**A** 923:**E** 924:**E**
925:**C** 926:**B** 927:**E** 928:**B** 929:**B** 930:**C** 931:**C** 932:**D**
933:**E** 934:**A** 935:**A** 936:**D** 937:**D** 938:**A** 939:**B** 940:**C**
941:**B** 942:**A** 943:**B** 944:**A** 945:**A** 946:**B** 947:**C** 948:**A**
949:**B** 950:**A** 951:**D** 952:**E** 953:**E** 954:**D** 955:**B** 956:**E**
957:**C** 958:**B** 959:**C** 960:**D** 961:**A** 962:**B** 963:**A** 964:**C**
965:**A** 966:**B** 967:**D** 968:**C** 969:**D** 970:**E** 971:**E** 972:**C**
973:**D** 974:**C** 975:**A** 976:**C** 977:**B** 978:**C** 979:**A** 980:**E**

981:**D** 982:**E** 983:**E** 984:**E** 985:**A** 986:**C** 987:**D** 988:**D**
989:**B** 990:**C** 991:**D** 992:**A** 993:**B** 994:**B** 995:**E** 996:**B**
997:**E** 998:**E** 999:**E** 1000:**B**

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